

Global Health (MEDANTA)

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The General Manager
Dept. of Corporate Services
BSE Limited,
P J Towers, Dalal Street,
Mumbai - 400 001

Scrip Code: 543654 The Manager
Listing Department
National Stock Exchange of India Limited
Exchange Plaza, C -1, Block G,
Bandra Kurla Complex,
Bandra (E), Mumbai - 400 051

Symbol: MEDANTA
Sub: Disclosure under Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 – Earnings Conference Call Transcript

Dear Sir /Madam ,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find the below link of transcript of Earnings Conference Call held on Tuesday, February 14, 2023 hosted by JM Financials, for the Quarter III & Nine Months ended December 31, 2022 Results of the Company.
https://www.medanta.org/investor_relations/quarterly-financial-results/conference-call-transcript

The Transcript is also attached herewith for your reference.

This is for your information and record.

For Global Health Limited

Rahul Ranjan
Company Secretary & Compliance Officer
M. No. A17035

Encl: a/a

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Q3 FY 23 Earnings Conference Call
February 14, 2023

MANAGEMENT : DR. DR. NARESH TREHAN – CHAIRMAN AND MANAGING
DIRECTOR – GLOBAL HEALTH LIMITED – MEDANTA

MR. PANKAJ SAHNI – GROUP CHIEF EXECUTIVE OFFICER
AND DIRECTOR – GLOBAL HEALTH LIMITED – MEDANTA

MR. SANJEEV KUMAR – GROUP CHIEF FINANCIAL OFFICER
– GLOBAL HEALTH LIMITED – MEDANTA

MR. RAVI GOTHWAL – HEAD INVESTOR RELATIONS –
GLOBAL HEALTH LIMITED – MEDANTA

MODERATOR : MS. CYNDRELLA CARVALHO – JM FINANCIAL

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Moderator: Ladies and gentlemen, good day, and welcome to the Global Health Limited "Medanta " Q3 FY2022 -23 Earnings Conference Call hosted by JM Financial. As a reminder, all participant lines will be in the listen -only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star than zero, on your touchtone phone. Please note that this conference is being recorded. Some of the statements made in today's call will be forward - looking in nature and may involve risks and uncertainties. Please refer page number two, of the investor presentation for full disclaimer .

I now hand the conference over to Ms. Cyndrella Carvalho from JM Financial. Thank you, and over to you, ma'am.

Cyndrella Carvalho: Thanks Faizan. Good evening, everyone. I welcome you all today, on behalf of JM Financials, I Cyndrella Carvalho, we are hosting the first call of Global Health Limited Medanta. We will be discussing quarter 3 for FY '23 earnings, insight with the management today. At the outset, I thank the management of Global Health for giving us this opportunity to host this call. From the management team today, we have with us Dr. Dr. Naresh Trehan , Chairman and Managing Director; Mr. Pankaj Sahni, Group CEO and Director; Mr. Sanjeev Kumar, Group CFO; Mr. Ravi Gothwal, Head Investor Relations.

I will now hand over the call to Dr. Trehan for his opening remarks. Thank you, and over to you, Dr.

Dr. Naresh Trehan : Thank you Cyndrella. And good evening to all of you, and welcome to Medanta ' s first ever earnings call as the public listed entity. As you all know, the company got listed on 16 of November '22, and I'd like to take this opportunity to thank all our patients, our doctors , our employees, our partners and now our shareholders, who have joined us in this journey. For the benefit of all participants, I would like to quickly share the reasons for creating Medanta, that is why Medanta.

If you step back in history, I graduated from King George Medical College Lucknow , and on completion of my internship, I left for the United States. I joined New York University in New York, where Dr. Frank Spencer, who was considered the greatest teacher of heart surgery, was heading the whole program. I was lucky enough to get the job, for which I interviewed for and after finishing my training at NYU for general surgery and, then cardiac surgery ,the University asks me to stay on, as faculty. So I joined the faculty of NYU and was teaching and practicing cardiac surgery as well as doing research in cardiac surgery.

Soon after I went into practice, a large number of Indian patients would come to New York for me to do their coronary bypass surgery because we were the first generation of coronary bypass surgeons. It became quite clear to me that for everyone, who could come to the US for their surgery, there were hundreds or even thousands, who were in India who needed bypass surgery, but could not afford to come to the United States. So that is how my journey back to India started. First with the establishment of Escorts Heart Institute , which was a collaboration between Mr. H P Nanda , Chairman of Escorts, the tractor company , and myself. On completion of the Escort

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Heart Institute, I returned to India , and I ran it for the next 20 years. While I was at Escort Heart Institute, it was very clear that although we had made it into the largest Heart Institute in the world, that there was still big deficiencies in other specialties . When a patient needed, say, neurosurgery or gastroenterology or urology the standard of medicine was not the same as it was at the heart level . This difference between the stand

ards at Escort s Heart Institute and the other non cardiac specialties was quite stark.

So the patients were not getting the best of treatment across all specialties. Hence, the thought came to me that we needed to build an institution, which would be like Mayo Clinic or Cleveland Clinic, where all super specialists of world class could practice on the same platform and provide patients the finest medicine that is available anywhere in the world, at a fraction of the cost. Thus began the planning and execution of Medanta, the Medicity, which is built on a 43-acre campus in Gurgaon.

We have built the hospitals, which is 1,400 beds with all super specialties. We opened at the end of 2009, actually to early 2010 and since then, the journey has been quite exciting, quite fulfilling and quite successful. The whole idea being that we were able to execute the best treatment for any patients that came to us, in the most difficult of circumstances, our outcomes would be equal to or better than those of anybody in the world. So having started with this objective we rapidly achieved success and in the first 15 months, we broke even.

Our patients and their families embraced the idea of Medanta. It was by nature designed to be the, Cleveland Clinic or Mayo Clinic of the East. So then we felt that since this standard had been set and the acceptance was so huge, that other people also needed this kind of care in different locations. After we knew that we had robust protocols and that we have system, which was strongly rooted, we did our second greenfield facility in Lucknow, which is a approximately a 1,000 beds and this became operational a little over two years ago. Soon after we began it has become very successful.

With the success of Lucknow, it became clear to us that the idea of transporting the Medanta standard to other locations was possible. We wanted to maintain the same standards we have in Gurgaon in Lucknow and now in Patna. On the side, we picked up smaller facilities in Ranchi and Indore. Now we have our sixth facility coming out of the ground in Noida. We are carrying on the same ethics, the same philosophy, the same quality of care that we had established in Gurgaon, to all our patients whether they may be at Lucknow, Patna, Ranchi or Indore. So the movement is going on and has been very satisfying and successful.

All the facilities are doing well but our essential belief is that if you deliver the highest quality of care, with compassion and at an affordable cost so that more and more people can benefit of this kind of system then you will always have success. I have often said that good medicine will make good business, but good business does not make good medicine.

With this focus, we have traveled to other parts of the country. We serve over 400 million Indians in the heartland of India and this story will go on and on. I will now hand over to our Group

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CEO, Mr. Pankaj Sahni to walk you through the numbers and results and the future plans. Thank you.

Pankaj Sahni: Thank you, Dr. Trehan.

I will share with you some of the highlights that Dr. Trehan had mentioned have got us still today. As Dr. Trehan mentioned, we started our hospital at the end of 2009 in Gurgaon, that was a 43-acre campus and from there, over the course of the last 12 years, we have grown to five operating hospitals. We have hospitals in Lucknow, Patna, Indore and Ranchi, in addition to Gurgaon and we are building our sixth hospital in Noida. Our Lucknow Hospital started in 2019 and currently has about 475 beds, with the capacity to scale up to 950 beds. We are already adding beds in that facility as we speak.

Our Patna hospital, which started in January 2022, has a capacity of 650 beds and is currently operating with 330 beds. We are adding capacity and adding operating rooms

in both Lucknow

and Patna. The two of smaller facilities are in Indore and Ranchi. Indore is at 175 beds and Ranchi at 200 beds. Those hospitals were launched in 2014 and 2015. Our latest hospital, which is our Noida hospital, will be 550 beds, and is expected to come on board in financial year 2025. As you can see, and as Dr. Trehan mentioned, we currently have about 400 million people living in the states where Medanta hospitals are present, and we really cover a large part of the population heartland of the country. In addition to this, if you look at our presence, we have diversified out of Medanta Gurgaon across almost five or six different cities, thereby ensuring that we have a good diverse mix of hospitals across different areas.

The most important thing is the way in which Medanta delivers its healthcare. We have a unique model of delivery, which focuses on ensuring that we operate at the highest end of care. We have been rewarded for delivering exceptional clinical care. This comes with having exceptional clinical talent, and cutting-edge infrastructure and technology. In fact, our infrastructure is comparable to some of the leading institutions in the world. We have built a very strong value system around delivering high-end and high integrity care and also having very robust processes. If you look at the way in which we deliver care, we really operate at a very complex level. Our

facilities across the 2,500 beds that are currently operational, have almost 70 operating rooms. About 30% of our bed capacity is critical care or ICU beds. This shows that we are really focused on providing care at a very high end and of a very complex nature. We employ our doctors on a full-time basis. We have over 1,400 doctors. We have been fortunate to get some of the best talent in the country to work for us across our units and that is why a lot of the care we deliver is really 'destination care'.

Our patients do not only come to us from the community or the neighborhood where our hospitals are located, but they also come to us from around the country and indeed from around the world. That is why we have come to be known as the last stop in Indian health care. Thus, the way we are set up and the way we think is to ensure the delivery of complex care at the highest end.

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I will now share with you some of the performance parameters over the course of the last nine months, that is, the nine-month ending 31st December 2022.

If you look at our performance, we have been able to deliver robust performance across all of the key performance indicators. Our consolidated total income was about INR 20,273 million, and that grew by 21% year-on-year. This growth has been driven by a variety of factors, including growth in patient volumes, growth in our bed occupied days as well as changes in specialty and pay mix. Our EBITDA was INR 4,856 million, and that has a growth of 22%, with very strong EBITDA margins at 24%. Our profit after tax showing even greater growth at 26% has come in at INR 2,250 million and our PAT margins have improved to 11.1%.

During this period, we have been able to add about 167 new beds. This includes 65 beds in Lucknow and 102 beds in our Patna hospital. As I mentioned earlier, this bed addition continues as we speak. We continue to add capacity, not only in our existing hospitals, but we also will be adding capacity in our upcoming hospitals. We will talk a little bit more about that shortly. One important point to mention is that, as we continue to add bed capacity, the appropriate metric to look at for us is bed occupied days, which have actually increased year-on-year in nine-month period by about 11%.

Also our ARPOB has grown by 8.5% to INR 58,494 in the 9M FY23. This growth is driven by increased contribution from higher complex specialties

as well as the lowering of the contribution from internal medicine. As COVID recedes, we see the revenue from internal medicine as a percentage of the total revenue reducing and a greater share shifting towards the complex specialties that we are known for.

When I look at our inpatient count, it increased by 32%. Our outpatient count has increased by 12% during this period. International revenue also sees an increase of 83% to INR 1,197 million, and that is driven by increased volume in international patient admissions. You would be aware that there was a dip in international admissions because of the COVID pandemic. That seems to be coming back as we speak and we see improvements in the international volumes quarter on quarter.

And lastly, our in-house pharmacy business has been doing very well. That continues to register strong growth. Our outpatient pharmacy revenue has increased by 58% from INR 396 million in the 9M FY22 to INR 625 million in the 9M FY23. This concludes the summary of the high-level financial metrics for the 9-month period ended December 31, 2022.

Important to point out that we continue to strengthen our core business, continue to build out a sustainable business model. A few important facts for you to be aware of, during the 9 months, we have added over 130 super specialist doctors across the various units. We continue to maintain ourselves as one of the most preferred destinations for senior doctors.

We have added some exceptional clinical talent across the units. Some of the recent senior doctor additions include Dr. Randeep Guleria, the former Director of AIIMS Delhi, who joined us as Chairman in our Internal Medicine and Respiratory Department, Dr. Praveen Khilnani, joined

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us as our Chairman of Pediatrics in our Gurgaon facility. Dr. Gagan Gautam, joined us as our Vice Chairman in Uro Oncology and Robotic surgery. Dr. Preeti Rastogi joined us as our Head of Obstetrics in Gurgaon, and Dr. Neelam Vinay has joined us as our Director of Obstetrics and Gynecology in our Lucknow facility. These are just a few of the illustrious faculty that we have

across the Medanta system.

When you look at the new specialties that we have added, in Gurgaon we started our Lung Transplant program. We have started our Mother and Child services in both Lucknow as well as in Gurgaon. In Patna, we have also started gastroenterology, surgical oncology services and we are also building out our radiation oncology services in Lucknow as well as in Patna.

In line with our plans to deliver the Medanta model of care, we continue to focus on being able to deliver the highest end of care with the highest clinical specialties and the best clinicians. We also look at delivering care across the continuum and extending this care outside the hospital. We launched our Medanta Labs business in January of this year, and we have already started out home care services and our outpatient pharmacy, as I mentioned earlier.

When I look at all of the services that we have, we are now able to deliver care closer to the patient's home, deliver care at the high end and ensure that we are always focused on delivering the highest standard to our patients.

Looking forward, we are confident of maintaining our growth trajectory. We believe some key levers for this growth will be, of course, first, our ramp-up of the developing hospitals in Lucknow and Patna. The second will be our strengthening of the core by adding new specialties, new doctors and we also have new hospitals coming on board. The first of those is the addition of our Noida facility, which will be about 550 beds that should commence operations by FY 2025. We have also added a new hospital in our ecosystem, which was announced yesterday. That is a new asset-light O&M partnership model for a 300-plus-bed hospital in Indore that

will

have about 100 critical care beds over 15 operating theaters and this will help us to fortify our presence in Central India.

Finally, we will be adding about 700 beds at the existing facilities, and that will allow us to deliver the beds at a slightly lower capex per bed because a lot of the primary capex by putting up the building and putting up the facility has actually already happened and we already have these hospitals operating. This concludes our overview over the course of the 9 months and the brief summary of our future plans. I will now hand over to our CFO, Sanjeev Kumar to walk us through some of the quarterly performance numbers.

Sanjeev Kumar: Thank you, Pankaj. I will now share the key highlights of Quarter 3 FY 2023 results that we have announced yesterday. We have delivered the highest ever quarterly total income of INR 7,062 million, seeing a growth of 19%. In terms of EBITDA, it was INR 1,719 million, a growth of 17.9% with healthy EBITDA margin of 24.3%. Our profit after tax was also INR 806 million, that is a growth of 15.3%. In terms of some of the operational parameters, that you look at, our occupied beds during this quarter increased by 13.5%, and our inpatient volumes grew by 23.1% to 35,000. Outpatient volumes grew 16% to over 5.5 lakhs. ARPOB grew 4.3% year-on-year.

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This is about our quarter 3 performance and now we are happy to take questions from the participants.

Moderator: The first question is from the line of Amit Kadam from Canara Robeco Mutual Fund.

Amit Kadam: Question is, mature hospital growth was flat for the nine months in terms of EBITDA and there was actually a decline in occupied beds. Can you just throw some light on it?

Pankaj Sahni: Sure. You want us to address that or you want us to take both the questions together.

Amit Kadam: So second question is on other expenses. Other expenses in this quarter has gone up by 38%. So, how do we look at this particular line item going forward?

Pankaj Sahni: Let me take the first question, and Sanjeev will walk us through the question on the other expenses. When you look at the performance in the mature hospitals, keep in mind that this is our Gurgaon, Indore as well as our Ranchi facility; all the three are grouped together in our mature hospitals.

If you look at our occupied bed days in the nine-month period of December 2021 versus the nine-month period December 2022 it is important to take you back to what was happening last year. Typically, as you would be aware, the quarter 3 is a slightly lower quarter in terms of the volumes because of the festival season that happens around Diwali, Dussehra, etc. However, last year, because of the COVID impact in the first quarter of the year, we did see a very strong bounce back in some of the numbers post covid that saw exceptionally high numbers in the nine-month period ending December 2021 versus the nine-month period ending December 2022. When you look at the occupied bed days in the mature facilities, the dip is really related to the fact that last year was an exceptionally high volume growth because there was a lot of pent-up demand, which got addressed as we move forward into the subsequent quarters.

When you look at our EBITDA performance over the group over that period, we continue to see very strong EBITDA margins across the group. You see that we are already delivering about

24% EBITDA margins. Some amount of growth in other expenses, which Sanjeev will walk us through some of that, linked to our repairs and maintenance costs or some of our facilitation charges. So Sanjeev, over to you to walk us through some of the other costs.

Sanjeev Kumar: Yes, sure, thanks. When we look at the other expenses, you are right that in these nine months, it has gone up. Some of the expenses which have gone up are like repair and mai

ntenance

expenses which have gone up specifically because some of the assets had to undertake some kind of repair activity. The second one is some of the facilitation charges increased because of the increase in international business. The third one is that some of the professional charges increase because of an assignment that we had undertaken for optimization of our material cost and as you can see that the impact of that has been that material costs have come down as well. The fourth reason was the increase in sales and marketing cost, which if you actually recall that during the last two and half years, during the COVID as well as the very next year after the COVID, we were very limited in terms of our sales and marketing. This year now, again, we

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have incurred sales and marketing costs. Other expenses also include conference expenses, which actually increased because of the activity of conferences that we started in these nine months once the normal activity has resumed. These are some of the reasons why other expenses went up.

Moderator: The next question is from the line of Dheeresh Pathak from White Oak Capital.

Dheeresh Pathak: Congratulations, Dr. Trehan and team for the successful listing. Just continuing with the same question on Slide 31. So as I understand that revenue for the mature hospitals, revenues increased by 8%, but EBITDA is flat because of higher sales and marketing and higher facilitation fees for international patients. Is that the main reason, sir?

Sanjeev Kumar: Yes, that is one of the main reasons. I have also mentioned that there were some other expenses like repairs and maintenance as well

Dheeresh Pathak: The margin that you are seeing for nine months in FY 2023 is 22.3%, is this understated, or do you think this is more of a normalized margin. Is FY 2023 nine months more effective of a normalized period or is there scope for the margin to improve?

Pankaj Sahni: Let me answer that question in two parts. First, let me answer the question on what are the scopes for improving efficiency. As you may be aware in the presentation you would have seen that we undertook our JCI audit in our Gurgaon facility this year. Typically, as that audit happens every three years, you do have some amount of repairs and maintenance costs that happens over the period. That may not happen every single year, but it happens maybe every two or three years. So I wouldn't say it's a one-off cost, but at the same time, it may or may not come in at every single year. That is one aspect.

The second aspect, as Sanjeev had alluded to is that we have already taken some kinds of activities towards trying to bring greater efficiencies into the cost structure, specifically with material costs, you can see some of the improvements play out in terms of the overall numbers. We continue to look at opportunities to add efficiencies in material costs as well as in manpower costs and across all the line items of other expenses. So to answer your question, are there opportunities for efficiencies? Absolutely, there are opportunities.

The other thing that you will see is that over the course of the last couple of years, we have not really taken any tariff increase partially because of COVID. That activity has started now step-by-step so you may see some amount of tariff increases and that could add to potential margin uplifts as we move forward.

The third aspect, of course, is that we have new clinicians coming on board. We have some new services that I had mentioned earlier, being added, including complex work. So as you look at the sales mix improving towards more complex work, you do expect to see an ARPOB growth and the corresponding revenue growth as you move forward. These are some of the opportunities for improvement. While we do feel that there are margin growth oppor

tunities to be captured we

don't really give any firm guidance for the next quarter.

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It is important for you to understand that in our industry and in our business and based on our

belief in the way healthcare is supposed to be delivered we do not really think quarter -to-quarter, but actually, focus more on the longer term by looking at the right aspects of delivery of care. As you think about how margins will stabilize over this period, we have always maintained that any performance which is anywhere in the range of 20% to 25% EBITDA margins is really very good performance. We don't see any reason why Medanta would not be able to deliver that kind of strong performance.

Dheeresh Pathak: So this slide is very interesting because your developing hospitals are showing KPIs, margins and occupancy just similar to the mature hospital, in fact, better. So on the developing hospital side, I am assuming these KPIs are because in the one of this later slides, you mentioned Patna turned EBITDA positive in the last quarter itself. So a lot of the EBITDA margin, EBITDA here on this slide, INR 153 crores for nine months is attributed, I would assume to Lucknow asset, right?

Pankaj Sahni : Just to give you the very quick history, our Lucknow hospitals, which we started towards the end of 2019, really January 2020 was able to deliver EBITDA breakeven in its first full year of operations. In fact, it delivered about a 15% EBITDA margin in that year. If I recall this was also disclosed in our RHP and some of our IPO documents. As we look at the Lucknow margin, that margin has continued to stay strong, in fact, continue to grow not only in percentage terms, but also in absolute amounts. You are correct a large part of the margin that you are describing does definitely come from our Lucknow facility, which is now about 500 beds. While we still consider it as a developing hospital it is important to keep in mind that the 500 -bed facility in Lucknow is one of the larger facilities in Indian health care. We continue to see growth coming out of Lucknow.

Our Patna facility, although it is very new, only started in January 2022 has also done exceptionally well, in a very short period of time. We continue to add beds. We continue to add doctors and clinical teams there. We are building 10 more operating rooms.

We do not yet say that Patna has reached a stable state, however, we did deliver a positive EBITDA both in Quarter 2 and Quarter 3. We believe that Patna also is on its way to a strong performance. One word of caution or also just clarification, is that today, our Lucknow facility is 100% cash plus TPA. We have zero schemes there and we don't have much corporate or PSU business either. So a lot of the realizations and the ARPOB which you are seeing, as you say, comparable to some of the mature facilities are driven by a very high percentage of cash business. That is in a little bit different from what you may be familiar to seeing in the industry in general.

Dheeresh Pathak: So as the newer beds at capacity gets added, it would be fair to say that you might take some scheme patients. So maybe the absolute amount will increase, but the EBITDA margin and ARPOB might go lower than what we are currently seeing, right?

Pankaj Sahni : When you look at the bed growth, we are about 470 beds, so you can say about 500 beds right now. If you look at our last quarter occupancies and availability of beds in Lucknow, it has

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actually been tight, which is why we have been trying to add beds. So at present, we have not really taken on any schemes in our Lucknow facility. As we move towards 1,000 beds, there is a possibility that we may look at augmenting the patient mix, not only in terms of schemes, but there are also

corporates, there are also public sector undertakings and we would like to be able to service those profile of patients as well. So yes, you may see that there may be some change in the payer mix. However, just to reflect on our overall payer mix across the group, including all the mature hospitals. If you look at our payer mix, which has been consistent year -on-year for last several years, upwards of 80% of our payer mix is cash plus insurance. So while we may have some scheme, it would possibly be on the similar lines as what you are seeing across the group.

Absolute margins will increase depending on the cost to serve the incremental patients and even your margin percentages may or may not change, it really depends on how you think about it. Another important point, which I did mention in the opening section, was that our doctors are all full timers. So unlike many of the other hospitals, where the doctors are more on a visiting or fee for service model, our doctors are all full timers so we benefit, from a lower marginal cost for every incremental patient.

Moderator: The next question is from the line of Nikhil Mathur from HDFC Mutual Funds

Nikhil Mathur: I will be harping on the same slide number 31. If I look at the EBITDA in nine months FY '23, I mean, it is flattish. This includes Gurgaon, right? And I also remember that most of the overhead for the new facilities sit in the Gurgaon facility. Is that the right to understand that you are showing here some of the overhead associated with your newer facilities as well?

Pankaj Sahni: That is absolutely right, Nikhil. So as you may recall, and as I think we may have also mentioned in our prospectus all our group costs, including the costs which are there for some of the newer business units, are all currently part of the mature hospital costs. So all of that cost, including the corporate overheads, the shared services in terms of IT costs, etcetera all of these costs sit in the mature facilities and are recorded as part of our Gurgaon cost. We don't actually split out the corporate overheads from the Gurgaon unit. Of course, if we did that, you would see a slightly different kind of a margin profile in the mature hospitals.

Nikhil Mathur: So if I strip out the overhead associated other facilities, would the base Gurgaon, Indore, Ranchi combined would have grown EBITDA on a Y-o-Y basis?

Pankaj Sahni: Like I said, we haven't split out the costs specifically on each of those aspects. I can, however, tell you that those costs have increased over the nine-month period with additions of variety of manpower, including some of the newer services, whether it is pharmacy or labs. So, my assumption would be definitely, yes, there would be margin growth, which would be better than what you're seeing today if you did split out those costs. However, we have not reported that so I cannot give you a definitive number.

Nikhil Mathur: My second question is on, if I understand correctly, also your Gurgaon facilities caters a lot of upcountry footfalls from Eastern UP and Bihar region and all. Now that you have a full service facility in Lucknow and in Patna, wouldn't that result in some bit of cannibalization in your

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Gurgaon unit? And why I asked this question is because in the vicinity, you have many upcoming bed additions from your competitors. So just trying to understand if there is going to be any cannibalization, any competitive pressures in the Gurgaon because of all this?

Pankaj Sahni: Let me address that in two parts. The first part around our patient mix in our Gurgaon facility over the years. You are right, upwards of 50% of our patients in our Gurgaon facility over the years have come from outside of Gurgaon. That includes Haryana, UP, Bihar, Rajasthan, Punjab, as well as, of course, the international business. I

interestingly, as we opened our Lucknow facility, we did actually see that with the awareness of our brand in UP, in the first year of opening itself, some of our patients from Lucknow actually increased in our Gurgaon facility. So, we didn't really see too much cannibalization because of that the new hospital opening. Our thought is that for every patient that travels all the way from up country to Gurgaon or from UP or Bihar to Gurgaon, there are thousands of other patients that are already in UP and Bihar, who are unable to make the trip. We believe that with increasing presence in these territories, you also have increasing brand awareness for Medanta as a whole. Some are not overly concerned with the cannibalization. That being said, we did experience during the COVID pandemic, some increased demand from our local community here in Gurgaon. We had people who were reaching out to us saying, please do look at serving the community closer to you in a greater way. So, we have put in effort to expand our connect into the local Gurgaon community. We have opened about 30-plus clinics in various residential colonies. In the last nine months, we have opened a new large clinic facility in Gurgaon, in Subhash Chowk. This now gives us two clinics running in Gurgaon itself with a third one in South Delhi. There is some movement to increase the servicing and connect to the community in Gurgaon and in the greater NCR region as well.

As far as your question around capacity, see, if you look at Delhi NCR as a whole, and that includes Noida, Delhi, Gurgaon, there is still a fair amount of demand in the city. When you look at the absolute population as well as the population growth that is likely to come versus the beds that are available, especially the high-quality beds. We do not have a huge amount of concern around too much saturation of supply. Also, keep in mind; it is somewhat easier to put up a bed, you still need a doctor to treat the patient. Therefore, that supply is not increasing just because the beds are increasing.

Moderator: The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services.

Tushar Manudhane: Sir, just on this asset-light hospital to be set up in Indore, while the partner is going to give the warm shell. What kind of investment is going to be there from our side?

Pankaj Sahni: If you look at the Indore facility, the model, which is there, which is I think is now reasonably well known in the industry is that the real estate partner would actually own the land and would put up the complete warm shell. Our investment will be only to the extent of the medical equipment, the partner will give the rest of the entire facility equipment investment, capex, etcetera, interiors.

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Our investment will range anywhere from INR 130 crores to INR 160 crores over the course of the first cycle of all the equipment that we need to put in. We have very long and deep relationships with our equipment partners so the payments to be made for this equipment will actually be staggered. Therefore, we do not see this as something where there would be a huge cash outflow in any one particular year.

Tushar Manudhane: And in the matured category, would current Indore hospital be at a similar EBITDA margin as is the matured segment or is it lower or more? If you could just explain that?

Pankaj Sahni: Indore is a little bit of an interesting scenario. I think Sanjeev mentioned we do not report out Indore separately but our current facility is significantly smaller than some of our larger established facilities. It is only about 170 beds and we have got the majority of the work in Indore coming in from just maybe two or three specialties. We have cardiac and neuro, which are the lead specialties in that unit. We do also have gastroenterology,

digestive services as well as

kidney and urology institute. We do kidney transplants there as well. However, when you look at the entire range of oncology services, these services are not available there because we do not have the space to be able to deliver these services. We also do not have the space to be able to put the kind of critical care capacity, that is demanded of us and that we are well known for in our other units.

So definitely, the EBITDA margins in Indore are lower than the ones, which you see across the mature groups. It is also significantly smaller in terms of absolute contribution, both on the top line and the bottom line. However, it is not really fully equipped to deliver the comprehensive range of clinical specialties, that we can offer and that there is a demand for.

Tushar Manudhane: Secondly, on labs as a business. While we have seen over last 12 months to 15 months, there has been significant amount of competition entering, be it online or other corporates and given that the existing guys are facing challenges in terms of volume growth, and which gets reflected in pricing. So, what from a capital allocation and from our effort allocation for this part of the business will the next three years to five years look like and what is your thought process for getting into this business? What kind of return ratios do you expect from this business?

Pankaj Sahni: Let me take the three different aspects. First, in terms of the capital allocation, you would be aware that the lab business has really very limited capital requirements because the model for most of the equipment is on what they call a reagent rental basis. It is a 'pay per use' or a 'pay-as-you-go' type of model. Therefore, you do not actually spend a huge amount of money in capex for the lab. In addition, we already have existing capacity in our hospital labs.

When you look at the cities where we started out, it is really the cities, where we already have hospitals. We have large laboratory services in each of those hospitals. Therefore, the incremental sample load that comes in, can actually be well serviced by the labs that we have right now. We may do some selective lab additions, but that is really more to manage the logistics than to manage any kind of capacity constraints that we have in our existing labs. So that is one point, we do not see a huge amount of capex, in this businesses.

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You are right, there has been, I would say, a COVID induced enthusiasm for some of these services like labs. Our approach is very different. One, unlike some of the stand-alone labs, we do not really have the same kind of customer acquisition costs that would be there in newer age stand-alone lab businesses because we already have the patient population from our hospitals. Most importantly, what we found is that many of our patients are actually demanding of us the ability to help service them a little bit closer to home. Our thought is that this allows us to service the patients across the life cycle. It allows us to deliver care to the patient closer to home and is really something that COVID showed us is something that our patients are demanding. We look at these new businesses as a comprehensive continuity of care model rather than a purely as a stand-alone separate business.

Therefore, our move into this segment, whether it is outpatient pharmacy, or our labs business is actually very deliberate and is linked to the communities which we already serve and the extended communities where our patients are demanding the services.

Tushar Manudhane: That is simple.

Moderator: The next question is from the line of Cyndrella Carvalho from JM Financial.

Cyndrella Carvalho: I just wanted to understand, we were contemplating some price increases. Have we taken those or are we still thinking

g about it? If we have taken, have we taken them across the facilities? Any thoughts will be helpful.

Pankaj Sahni: Sure. Our price increases, as I may have mentioned in the opening section, are oriented towards making sure that we remain as affordable as possible for the communities that we serve. We actively try not to be the most expensive in the city where we operate, but try to be a little bit more reasonable, maybe benchmark as the third or fourth most expensive in that particular city or community. This has been the broad philosophy that we started with and the philosophy that we currently maintain.

Our tariff increases are all done in line with our clinical leadership, each department actually decides what is the appropriate tariff for the kind of complexity of work, which they are doing as well as, the nature of the costs associated with servicing that kind of complex work. That has been our approach. Because of COVID for almost three years, as you mentioned rightly, we had not taken any tariff increase across the network. We have now started to take that tariff increase. We take it systematically over the various specialties.

I would say the majority of that tariff increase would be in Gurgaon because Lucknow and Patna are still new, although we are seeing some selective tariff increases in Lucknow as well. That process is underway. Some of that tariff increases have already been done. Some is currently in process and some will be done later. We will do this over the course of the next several months. We will maintain our positioning and will not be the most expensive, but as the industry tariff grows, you will also see that our tariffs move in line with that growth.

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Cyndrella Carvalho: That is very helpful. I am just going to try this. Any luck you will be able to help us with Gurgaon, Lucknow, and Patna, ARPOB's, ALOS and the occupancy levels. Any color will be helpful any of it.

Pankaj Sahni: I am sure that many people are trying to fill out their excel models, and I do empathize. However, I also must defer to the guidance that I get from our CFO and our compliance team to make sure that whatever we are disclosing are numbers which are appropriately audited and appropriately displayed. The only thing that I can say is, as you can probably guess, our mature facilities are still fairly heavily weighted towards Gurgaon. You can just get a sense that if you take the total number of beds that are there in those facilities. If I say it probably it is about 1,700 beds would be in the mature facilities, almost 1,300 of those are in Gurgaon itself.

In addition, obviously, Gurgaon would have a slightly higher tariffs and ARPOB profile than Indore and Ranchi. This may give you an indication how the numbers are weighted in favor of Gurgaon.

Moderator: The next question is from the line of Anubhav Sahu from McPro.

Anubhav Sahu: Sir, to my understanding some of the capacity additions will happen in phases. For example, for Noida facilities, though we aspire to be reaching 550 bed facility it would take 300 beds to reach this stage. And so for Patna say around, it will reach 500 beds facility by fiscal year 25. So if we take it as fiscal year 25 as the time line, would it be fair to assume that around 1,000 beds you would be adding? And if so, what would be capex for that?

Pankaj Sahni: Let me just quickly touch on the bed additions and then Sanjeev can shed some light on the capex. You are right, we add our beds in phases. Important point as I think I mentioned earlier is that although the bed additions happened in phases, the bulk of the capex, because we build the superstructure, the shell, the complete facade of our buildings in one shot, has already gone into the building. On day one, about 70% of the capex has already gone into the building, thus the i

ncremental beds will be at a significantly lower capex.

If you look at our Gurgaon facility that is mature, we may add some beds there. We are actually revisiting a lot of movement towards daycare, we see a lot of trends of patients shifting to daycare and you will see that reflected in a significant reduction in our average length of stay. We expect that this trend will continue. Therefore, we are adding some amount of capacity in our cancer services in our Gurgaon facility as daycare. Of course, the bulk of the bed addition will come in Lucknow, Patna and Noida.

Lucknow will scale up over the course of the next couple of years from the 500 beds that it is today, probably at least scaling up another 200beds -300 beds depending on as the demand rolls out. That hospital is constructed as two towers. One tower is now completely full and the second tower, which can house another 300 beds - 400 beds will be built out. We have, post December, already activated some beds there in our critical care units. We have already added ICU beds between December and March 31 and you will see some bed additions in that period as well.

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Patna, as I said, is 330 beds now. Those beds will also scale up. We will probably scale from 331 beds to 400 beds and then to 500 beds before we move to the 650 beds. Noida, while it has a capacity of 550 beds will likely open with the bed number of 300 beds. That will be the way in which the beds build out. Sanjeev, over to you on the capex.

Sanjeev Kumar: As Pankaj has just explained most of the beds would be actually coming in Lucknow, Patna and Noida. In these three facilities, we would be adding almost 1,100 beds. In case of Lucknow we will go from 475 beds at present to actually 950 beds. In case of Patna from almost 330 beds to 650 beds and in case of Noida, we will be starting with almost 300 beds, which will eventually go up to 550 beds. So we will be adding approximately 1,100 beds and we will be incurring almost INR 1,100 crores for putting up the beds.

Most of this, more than 60%, would be towards the Noida hospital because that is the active project that we have. We will be implementing the entire warm-shell, the entire structure. So our total capex would be approximately INR 1,100 crores, out of which more than 60% would be for Noida. This is for the next three to four years in terms of the specific projects identified till date.

Moderator: We will take the next question from the line of Gagan Thareja from ASK Investment Managers.

Gagan Thareja: Sir, the first question is on -- in your P&L, is there any one-off expense related to the IPO in this quarter? And what's the gross debt position on the balance sheet post the IPO?

Sanjeev Kumar: In terms of one-off expenses, there are very minor expenses, let us say, almost INR 2 crores in terms of the advertisement expenses to be precise INR 1.5 crores incurred during the IPO process. There are also some one-off expense in terms of the travelling, etcetera. Nothing more than that.

Sanjeev Kumar: What was your second question?

Gagan Thareja: On the gross debt on the balance sheet, what is the position?

Sanjeev Kumar: In terms of the gross debt, we actually have three loans. One is the loan in case of MHPL, which is approximately INR 420 crores, then we have a loan in case of Patna, which is another INR 250 crores and then we have some debt in GHL which is our holding company from Asian Development Bank, which is at INR 66 crores. These are the three loans that we have today.

Gagan Thareja: And related to the capex one, how will you be funding it? I mean what would be the debt equity mix for that? And two, while you'll get efficiencies in Patna going as utilization or occupancy ramps up there, will that be offset by the new bids that come in at Lucknow and

Patna and also

the Greenfield at Noida. So I mean, if you could address these two aspects. The funding of the debt and the margin impact of new capacities?

Sanjeev Kumar: I think let me first talk about the funding aspect. As you are aware that we have a good liquidity position in terms of our balance sheet. So, the funding required for our capex would be done out of the internal accruals as well as the debt that we will take against each project, which would

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be there in case of Noida. You have seen that our net debt to EBITDA is in a very good position. We will maintain a very comfortable debt-to-equity ratio. That is what we expect to have for funding our future capacities.

Pankaj Sahni: Just quickly, you asked about the ramp-up as we get Noida mobilized. Definitely, there are some challenges in getting a new hospital up in running -- and if we look at our trajectory over Lucknow and Patna, we have been able to deliver positive EBITDA in a reasonably short period. We do not keep that as our standard operating model but we have now shown short break even in all three of our green fields Gurgaon, Lucknow as well as in Patna. The only advantage, I would say, Noida has versus the others is that because it's in the NCR region, we already have talent across clinical and non-clinical areas in our Gurgaon facility, that are being groomed or are ready to move across to our Noida facility. We may have some opportunities for ensuring that we can optimize talent across the NCR facilities as we operationalize Noida.

Moderator: The next question is from the line of Dhara Patwa from SMIFS Ltd

Dhara Patwa: Since you are expanding in non-metro cities, do you think attracting talent would be a challenge in those cities?

Pankaj Sahni: If you look at our performance and if you look at the profile of clinical names that we have been able to attract across the cities where we operate, whether that is Lucknow, Patna or Indore, we have not experienced this as a challenge yet. Now obviously, we do keep in mind the availability of talent when we look at the cities where we move to -- we are fortunate that the Medanta brand and our operating model, which is really clinician-friendly and doctor-led and doctor first type

of model has allowed us to become a preferred destination for really high -quality clinical talent. We have also been fortunate that we have been able to cross -pollinate some of these units. If you look at some of the clinical leaders in our , Indore facility, both the head of cardiac surgery as well as neurosurgery were doctors that were in our Gurgaon unit before we move d them to Indore . Our Head of Cardiac Surgery in Patna worked here in Gurgaon with Dr . Trehan, before he moved to Patna . We have had doctors move into Lucknow facili ty as well from here. In Patna, we even had clinicians come from the UK and move back to India to come and work in our Patna facility.

Now one important point to note is that the nature of the facilities that Medanta is able to put up in these cities has n ever been seen before . When you look at the kind of facility that we have in Lucknow or in Patna, there is really no facility of this caliber , either in terms of size or in terms of just the quality of infrastructure and technology in pretty much the entire state, at least not in the private space.

When we are looking at clinicians, we are able to give them a really exceptional platform for where they can practice their craft. We are able to do things like give them huge amounts of operating rooms, additio nal space, really give them all the tools and technologies that allow them to deliver the highest standard of medicine. That is one of the reasons why they feel Medanta is the right platform for them to join.

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We have not had too many challenges with respect to hiring clinicians in any of these areas. We are, however, very selective, and we are very determined to maintain our full -time model. We do not allow the visiting model where a doctor would work in one hospital on one day of the week and in another hospital on another day of the week . Any restrictions on bringing in talent are really more from our side than from a supply side.

Moderator: The next question is from the line of Anubhav Sahu from McPro.

Anubhav Sahu: So, my question is from the asset lig ht model, which we are trying to execute in Indore . I wanted to understand, is it a change in business model in some way or is it a one -off thing? Is something which we may like to follow up for other cases?

Pankaj Sahni: The way I would answer that is to say that neither is it a change in business model, nor is it a one-off or a first time. If you look at our facilities, we have different types of models in each.

Our Gurgaon and our Lucknow facilities, we own and Noida as well. We have owned the land and we built up those facilities. Patna is a PPP project with the government of Bihar. So that's a slightly different model. Our Ranchi facility is also in O&M. So there as well, we did not invest in the infrastructure . , There was an existing hospital that we took over and turned around . The Indore model will be the one where our partner will be building up the warm shell and then we will be putting in the medical equipment and running it.

We are open to all types of models, whether it is O&M, whether it is greenfield, whether it is brownfield or even whether it is M&A or taking over any existing facilities. The key aspect though, in our model is that we must be able to ensure that these faci lities deliver the highest quality and standard of care that we have come to be known for. What that means is that in each city, there are certain requirements for what it takes to deliver this kind of high standard of care that may be different in Indore and in Lucknow or Gurgaon.

Each city has its own unique characteristics and our approach into those cities is in line with that, but always making sure that we are able to deliver very high end, high quality of care. We do not want to go into cities or enter into partnerships just for the sake of showing bed additions; they have to be able to deliver the quality and standard of care with the value s that we stand for.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I now ha nd the conference over to the management for closing comments.

Pankaj Sahni: Thank you very much to all of you who joined the call as well as for everybody who asked questions to try to understand our approach to healthcare delivery a little bit better. We remain aligned towards delivering and focusing on high end of care. We remain aligned to ensuring that we are managing our institution with the values that we believe are the right values for healthcare delivery. We remain committed to delivering growth for all of our stakeholders - the kind of growth that you can be proud of. Thank you very much for taking the time and for participating in this journey along with us, and we look forward to speaking with you all soon.

Moderator: Thank you. Ladies and gentl emen, on behalf of JM Financial, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

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Notes:

1. This transcript has been edited for readability and does not purport to be a verbatim record of the proceedings
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The General Manager

Dept. of Corporate Services

BSE Limited,P J Towers, Dalal Street, Mumbai - 400 001

Scrip Code: 543654The Manager

Listing Department

National Stock Exchange of India Limited Exchange Plaza, C-1, Block G,Bandra Kurla Complex,Bandra (E), Mumbai - 400 051

Symbol: MEDANTA

Sub: Disclosure under Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 – Earnings Conference Call Transcript

Dear Sir/Madam,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements)

Regulations, 2015, the transcript of the Earnings Conference Call held on Monday,

May 29, 2023, hosted by Motilal Oswal, for the Quarter & Financial Year ended

March 31, 2023 is enclosed herewith.

The Transcript is also available at our website: www.medanta.org .

This is for your information and record.

For Global Health Limited

Rahul Ranjan

Company Secretary & Compliance Officer

M. No. A17035

Encl: a/ARAHUL

RANJANDigitally signed by RAHUL RANJAN Date: 2023.06.03 18:01:41 +05'30'

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"Medanta – Global Health Limited
Q4 FY23 Earnings Conference Call "
May 29, 2023

MANAGEMENT : DR. NARESH TREHAN – CHAIRMAN AND MANAGING
DIRECTOR – MEDANTA – GLOBAL HEALTH LIMITED

MR. PANKAJ SAHNI – GROUP CHIEF EXECUTIVE
OFFICER AND DIRECTOR – MEDANTA – GLOBAL
HEALTH LIMITED

MR. SANJEEV KUMAR – GROUP CHIEF FINANCIAL
OFFICER – MEDANTA – GLOBAL HEALTH LIMITED

MR. RAVI GOTHWAL – HEAD, INVESTOR RELATIONS –
MEDANTA – GLOBAL HEALTH LIMITED

MODERATOR: MR. TUSHAR MANUDHANE – MOTILAL OSWAL

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Moderator: Ladies and gentlemen, good day and welcome to Global Health Limited, also known as Medanta Q4 FY2023 Conference Call , hosted by Motilal Oswal. As a reminder, all participant lines will be in the listen -only mode . And there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star, then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Tushar from Motilal Oswal. Thank you and over to you, sir.

Tushar Manudhane: Thank you, Aman. Good evening and a very warm welcome to all the participants on Global Health Limited Q4 FY23 Earnings Call. Joining with us today from the management side, we have Dr. Naresh Trehan, Chairman and Managing Director, Mr. Pankaj Sahni, Group CEO and Director, Mr. Sanjeev Kumar, Group CFO, Mr. Ravi Gokhwal, Head of Investor Relations. Over to Dr. Trehan for the opening remarks.

Dr. Naresh Trehan: Good morning to all of you. Welcome to the Q4 FY23 call. I would like to give you a little background. Most of you may be already familiar with it, but as you know, we created Medanta as a very specific platform to bring all super specialists from different clinical specialties on the same platform to be able to deliver, collectively, the highest end of care. We started our journey in Gurugram in late 2009/early 2010. By 2023 we have grown to 2,700 beds across Gurgaon, Lucknow, Patna, Indore and Ranchi. Our Noida facility is under development and should come online in the next 18 months or so. That will take us to the over 3,300 beds, that's including 550 beds in Noida.

Our focus has always been to deliver the highest quality care across all specialties. We have been able to achieve that in all our specialties across all our facilities. Our expansion is focused basically on areas which are underserved. And we have built a sustainable platform delivering the highest quality of care, making sure that the infrastructure, the technology and the medical facilities like the doctors and paramedical and nurses are all of the same high quality. We are very proud of the teams that have worked across all our facilities and are happy to report that we have been declared the best private hospital by Newsweek for four years in a row. Not only that, we are also recognized by Newsweek as one of the top 150 hospitals of the world. So this is a good achievement and our journey has been quite successful.

FY2022-23 has been a landmark year, in the journey of Medanta.

- o We transitioned from a private company to a public company in November '22, with our successful IPO that was oversubscribed almost 10x.

- o Our Patna facility is now fully functional for a year and the remarkable part is that it has also achieved EBITDA break even in the first year of operation.

- o Over 150 specialist doctors have joined us across the units during FY23, further strengthening Medanta's healthcare delivery capabilities.

- o Medanta has also added new specialties during the year such as lung transplant, the woman and child specialists. We have done triple way liver transplants, heart transplants. So all these new accomplishments have happened. Medanta continues on our journey of providing

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the most complex procedures like bone marrow transplants, endocrine surgeries and very complicated cancer surgeries across the whole body.

Also the recognition as the best private hospital has given us the boost and we continue on our journey with a renewed focus and taking care of our doctors and teams to work together in the service of humanity because we do want to maintain the highest standard of ethics.

For further financial details, which

if I'm sure you are all waiting for, I'll hand over the call to

Mr. Pankaj Sahni, Group CEO, who shall walk you through the financials for the year. Thank you and over to you, Pankaj, please.

Pankaj Sahni: Thank you, Dr. Trehan. Good evening and welcome everyone, to our first full year as well as the quarter 4 financial year 2023 earnings call. We've concluded our first full year on a very strong note and we have reported our highest quarterly revenue in this quarter of INR 7,318 million, which is a 37.3% year-on-year growth. Our CFO Sanjeev will talk shortly about our quarterly performance, but let me spend few minutes before that giving you some of the highlights for the year.

As Dr. Trehan mentioned, this has been a landmark year for Medanta. We are proud of what we have accomplished and would like to thank our doctors, nurses, our staff, as well as our shareholders, and of course, finally, our patients for the trust that they have placed and continue to place in us.

Some highlights starting with our financial performance, on the revenue side, our consolidated total income of INR 27,592 million has shown a strong growth at about 25.1% year-on-year. This growth was driven by a combination of factors. Some of it was higher inpatient volumes, increased bed occupied days, as well as changes in specialty mix towards higher revenue specialties.

Our EBITDA as well has grown 38.3% and has come in at INR 6,771 million. EBITDA margins as well have improved by 234 basis points from 22.2% in FY22 to 24.5% in FY23. Profit after

tax was at INR 3,261 million, which is a growth of 66.2% year -on-year. And our PAT margins have improved as well by 292 basis points to 11.8%.

During the course of this year, our expansion continues. We have added 293 new beds. That's a growth of about 12.2% year -on-year. Out of these 293 beds, we have added 191 in Lucknow and 102 beds in our Patna hospital. Our average occupied bed days has increased by 14.1%, representing an occupancy of about 59% at the group level. And that is largely because of the increased bed capacity as well. ARPOB have grown as well to about INR 59,098 in FY23. This growth, as I mentioned earlier, in line with the revenue increase, is driven by contributions from higher complex specialties such as cardiac, GI, cancer, and neuro. Inpatient volumes have increased by 32% and outpatient counts have increased by 15.4% in FY 23 as well. When it comes to the breakdown of our developing hospitals, which is largely Lucknow and Patna, our revenue share has increased from 18% to the group in FY22 to 27% in FY23, amounting to INR 7,426 million. The developing hospital EBITDA share has increased from

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16% in FY22 to 32% in FY23, amounting to INR 2,159 million. So we continue to de-risk and diversify our portfolio from a single unit in Gurgaon to multiple units across the country. Medanta Patna, Dr. Trehan mentioned as well, has shown phenomenal growth during the course of the last year. We started our in-patient operations in January 2022 and we have delivered a revenue of INR 1,707 million in FY23. Once again, a Medanta Hospital has been able to achieve EBITDA break-even in its first full year of operations. We are very pleased with the performance in Patna. You will recall Lucknow also had been able to achieve EBITDA break-even in its first full year of operations.

When it comes to our international patients, our revenue has increased by 68% to INR 1,564 million and that of course is driven by higher realizations as well as increased volumes post-COVID from countries such as Iraq, Nepal, Bangladesh, as well as the CIS countries. Our in-house OPD pharmacy business, which we had started a few years ago, continues to register strong growth. Revenues have increased by 58% from INR 536 million in FY22 to INR 850 million in FY 23.

From

from a balance sheet perspective, a lot of good news. We became a net debt-free company. In fact, we had a net cash surplus of INR 4,692 million at the end of March 2023. Cash proceeds of INR 5,000 million or INR 500 crores which came in from the IPO, out of which INR 3,750 million utilized towards debt repayment which were completed in April of 2023. During the year, we have also incurred net capital expenditure of about INR 2,846 million, out of which INR 659 million has been incurred towards our ongoing Noida Hospital construction project. One very important highlight, Medanta continues to generate very strong operating cash flows, a very robust EBITDA to cash flow conversion of about 95%. So overall, from a balance sheet point of view, our balance sheet remains well capitalized, we are well positioned for funding the future expansion as well as the growth that we see coming.

A few developments during the year in addition to the financial numbers that I would like to share, we have been very focused on ensuring that we execute the plans and strategies that we have outlined for each of our units and I would like to share a couple of updates.

The first one is around bed additions. As I mentioned earlier, we have added a total of 293 new beds, which is a growth of about 12.2% in bed capacity. At Lucknow, strong demand resulted in us bringing forward our bed expansion plans a little bit. We added 191 beds, out of which 126 beds were added in the last quarter of FY23. Patna as well has seen bed additions, we have added 102 beds, including 29 new critical care beds during the course of the year. Both these facilities will see bed additions in FY24 as well. In fact, some of those bed additions and operating room additions are already underway.

We have also launched some new specialties and new services over the course of the year. At Gurgaon, we started our lung transplant program, already completed six successful lung transplants. We have, in fact, one of the leading chest specialty departments that comprises leading clinicians in lung transplant, chest surgery, respiratory and pulmonary medicine, collaborating together to deliver some of the highest quality in chest care services.

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We have also launched our mother and child specialty, both in Gurgaon as well as in Lucknow.

And we have further enhanced some of our cancer care with advanced treatments in bone marrow transplant, endocrine surgery, etc., in Lucknow. Our Patna facility also continues to add specialties as we speak and continues to expand its operations.

New technology added in Gurgaon includes the Da Vinci robot. We have added one of the fastest CT scanners in the world in terms of next generation sequencing and molecular work. We have also added a new edge Varian EDGE LINAC machine, which I think is only the third or fourth installation in the country, in our Lucknow facility. We will operationalize this by the first quarter of FY 24.

In terms of our ongoing digital transformation, we continue to enhance our existing clinical applications, which include the e-Clinic application, which enables our patients to book online appointments as well as access their medical records from anywhere in the world. Our doctor EMR app, allows our doctors to review their patient's records from anywhere as well as receive real-time clinical alerts, engage in real-time manner with their clinical teams. This year, we have developed a third application, which is a 'smart nursing application', which is used to identify critical and sick patients and support intelligent nurse staffing based on predefined parameters. Finally and perhaps most importantly, as Dr. Trehan mentioned, we have added over 150 full-time super specialist doctors during the year, including some well-known names like Dr. Randeep Guleria, Chairman of our respirator

and internal medicine department in Gurgaon, Dr.

Gagan Gautam and his team of robotic uro-oncology specialists, various doctors including Dr. Praveen Khilnani, Dr. Rajiv Uttam, Dr. TJ Anthony, Dr. Preeti Rastogi in terms of our paediatrics, obstetrics, gynaecology and neonatology specialists in Gurgaon, as well as Dr. Neelam Vinay and her team of experts in mother and child care in Lucknow.

We continue to strengthen our large pool of medical, surgical and radiation oncologists with the addition of senior faculty across Gurgaon, Lucknow and Patna. We continue to scale up in newer facilities with the addition of over 50 doctors in Lucknow and over 35 senior specialists in Patna. Over the past three years, that's FY2020-23, Medanta has delivered consistent financial growth. Our total income has grown at a compounded annual growth rate of 21% over this time period, while our EBITDA and profit after tax have grown at a compounded annual growth rate of 43% and 108% respectively.

As we look forward, we are confident that we shall continue our deliberate and sustainable growth by focusing on the ramping up of our Patna and Lucknow facilities to their full capacity. Continuing to attract and retain the highest quality of clinical talent across the country. And by concluding the construction and launching our 550 bed facility in Noida and finalizing our already announced plans for our 300-plus bed hospital project in Indore.

With that, I will hand over to Sanjeev to discuss our fourth quarter financial performance.

Sanjeev Kumar: Thank you, Pankaj and good evening to you all. I would like to share some of the consolidated financial performance of Medanta for the quarter ending FY 23, that is March 31, 23.

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Medanta has delivered total income of between INR 7,318 million, registering a growth of 37.3% on year-to-year basis. EBITDA was also INR 1,915 million, which is a growth of almost 108% year-on-year with EBITDA margins improving from 17.3% to 26.2% in quarter 4 FY23. Our profit after tax was INR 1,011 million, which is a growth of almost 478%. During the quarter, 126 new beds were added at Lucknow.

In terms of our operational performance, our occupied bed days has also increased by 25.6% and occupancy was at 58% on increased bed capacity. Our ARPOB also stood at around INR 61,000. It has grown by 7.5% year-on-year, primarily driven by change in specialty mix. During the year, we have also initiated our tariff revision. In-patient volume also grew by 32% to almost 33,000-plus and outpatient volume also grew by 26% to 5.7 lakhs. In terms of our developing hospitals, that is Lucknow and Patna, our total income has increased from INR 1,102 million to INR 2,025 million, a growth of almost 84%. And EBITDA increased from INR 133 million to INR 625 million in quarter 4 FY23.

Now we are happy to take questions from the participants.

Moderator: Thank you very much. We will now begin the question-and-answer session. First question is from the line of Dheeresh from Whiteoak. Please go ahead.

Dheeresh: Thank you for the opportunity and congratulations on good operational performance. First question is, in April first week there was an article about Gurgaon administration wanting more beds. Is there any update on that?

Pankaj Sahni: I am sorry, let me just clarify the question. You are saying that the Gurgaon administration, do you mean the Medanta or the government administration?

Dheeresh: No, so in April first week there was an article which said that instead of 5%, 20% beds in

Gurgaon, and then I had checked the thing with you and you had said that we are going to, as per our obligation, we are only obligated to provide 5% . So have you exchanged anything in communication with the administration and what is an update on this?

Pankaj Sahni: Yes, so I understand the question

now. Thank you. In terms of the requirements and any notification or information from the government, there has been no change in the course of the last several months. Like you mentioned on the call and like we have constantly reiterated, we continue to abide by the terms of our lease as well as the terms of the allotment ad that was there which mentioned that 5% of the beds should be reserved. We do service whatever requests come to us in this nature from the government authorities. We have not seen any significant uptake or any significant change in the volume of patients coming in from the underprivileged category. So no real significant impact on anything thus far.

Dheeresh: So in the Gurgaon facility, right now you are providing only 5% of the census beds free of cost.

Pankaj Sahni: So the terms of our allotment require us to service up to that much at present and for the starting of the hospital, we have never received that high of a number. We continue to provide services

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to the EWS patients that come to us. There are certain requirements that the authorities follow when they refer patients to us and as and when they come in, we service them, yes.

Dheeresh: And just to understand, when you report the KPIs like occupied bed days and ARPOB, for example, for Gurgaon facility, the occupied bed days would include whatever bed days you have provided to this section of the population, right?

Pankaj Sahni: That is correct. And actually, if I understand correctly, you would be referring to the occupied bed days of the mature facilities. So it is not Gurgaon, it does include Indore and Ranchi as well, if I'm not mistaken.

Dheeresh: Understood. No, what I'm asking is whatever it is, it is not shown net. You're showing occupied bed days, ARPOB, everything is gross of the beds that you are obligated to provide for the EWS section, right?

Pankaj Sahni: Absolutely, correct.

Dheeresh: Understood. Okay. Thank you for that. And then on slide 44, which the right hand side, there is this exhibit which talks about capacity from 2,697 beds to 4,000 beds. So can you just like discuss in terms of, you know, each project, you have mentioned the beds, but in terms of timeline and in terms of capex spent, can you just give us a, you know, refresher on this slide, please?

Pankaj Sahni: Sure. So let me do the following. Let me just very quickly walk you through the bed numbers that you see on this graph, and then I'll hand over to Sanjeev to give us a little bit of an overview on the capex. Let me start with our Gurgaon facility. Actually, somewhere in the presentation, it is also mentioned we are currently undergoing two major projects within our Gurgaon facility. One is the development of our mother and child floor. We will be adding about 30 beds or so into that area that will include operating rooms, a neonatal ICU, etc. We are also in the process of renovating our complete oncology services in the unit. So we will be adding in approximately 50 beds of medical oncology daycare services. That is a renovation, so it is not a new build as such. That is the first major part of the capex that is happening in our Gurgaon unit. The second part is in our Lucknow facility. Our Lucknow facility as at March 31st is at 601 beds. That includes about 200+ beds of critical care, as well as about 12 operating rooms. We added in about seven operating rooms in Lucknow during the course of this year. As we move forward, Lucknow will move from 601 beds to 950 beds. Our estimation is that approximately, 150 of those beds will come during the course of this financial year.

As far as Patna goes, Patna Hospital is 330 beds today and will also add in probably around 70 beds or so, which will take us to maybe slightly over the 400 bed limit during the course of this financial year. The more important part

of Patna is that we will add in almost 10 operating rooms.

In fact, since the end of March, we have already activated about four operating rooms and six more operating rooms will come on board in Patna. So Patna will finally then have another six operating rooms will come on board. So Patna will finally have then the 14 operating rooms that it was planned for. That is the plan for Patna.

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Noida, which is built as a 550 bed facility, we anticipate opening somewhere towards the latter part of financial year '25. That gives you some sense of the build timeline. Let me hand over quickly to Sanjeev for the high level overview of capex on these projects.

Sanjeev Kumar: Yes, thanks, Pankaj, and thanks, Dheeresh. As Pankaj has explained we will actually move from almost 2,700 bed capacity to ~4,000 bed capacity in the next three years or so. We expect to incur approximately INR 1,000 crores capex for almost 1,200 to 1,300 beds. We expect that this will come over the period of the next three years or so.

Dheeresh: So how much, Sanjeev, your voice, audio quality is not very good, but if I understood what you're saying is INR 1,000 crores capex for this, between FY24 and FY25 for this 1,300 bed. How much will it spread out in '24 and '25, the capex?

Sanjeev Kumar: In case of FY24, which is actually the current year, since our Noida project is actually in full swing, and we actually might see almost INR 250 crores capex getting incurred in case of Noida. Overall, this year we will incur approximately around INR 450 crores or so. The balance of INR 550 crores in the next two years, that is FY25 and FY26.

Dheeresh: Okay, and just one last clarification, Noida, as Pankaj mentioned, towards the end of FY 25, 550 beds, so it's not in phases. The full 550 will become operational at the end of FY25, and that capacity will be available for commercialization in FY 26, right?

Pankaj Sahni: No, let me clarify. The facility in Noida is a single tower facility, unlike our Gurgaon, Lucknow and Patna facilities. So it will be built in one shot, but we plan to operationalize only about 300 beds on opening day, and then we will scale as and when the requirement comes.

Dheeresh: Understood. Thank you so much, Pankaj. Thank you.

Pankaj Sahni: You are welcome.

Moderator: Thank you. The next question is from the line of Rishabh Tiwari from Allegro Capital. Please go ahead.

Rishabh Tiwari: Hi, thanks for the opportunity, and congratulations for the stellar numbers. My question is regarding the decision for Indore O&M facility that we are opening. So if the management could please add some colour to the current Indore unit, what are the occupancy and ARPOB levels that we are seeing, and the basis on which the management has decided to set up another facility? So some colour on the current business of the Indore unit.

Pankaj Sahni: Okay, let me give you a little bit of quick context. You know, our Indore facility is actually just a rental building. It was an existing hospital that wasn't functioning that we had taken over. So it's a very small infrastructure facility, just about 150 beds there. We have four operating rooms there, and about 50 critical care beds. What we are finding in Indore is that our ability to add in services is hindered by size, for example, we do not have much medical oncology services there. There is, of course, no space to add things like a radiation oncology service, et cetera.

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We are fairly constrained with that facility, and therefore, we have actually decided that the appropriate and logical thing for us to do is to build out a reasonably big facility there, which would be somewhere in the range of 300 beds to 350 beds, to be able to cater to some of the deficiencies that we have. For the most part,

the services that are currently running in Indore are mainly cardiac and neuro.

As we look at how we should build this out, we decided that it would make sense for us to go more towards the asset-light approach in conjunction with a partner. So we have been able to tie up with a partner who will build out the infrastructure and deliver to us a warm shell, which is about 5 lakhs sq.ft, which should accommodate for us about 300 beds to 350 beds. Medanta will put in the medical equipment and, of course, then run the hospital. So this is our plan. At present, our Indore facility is functioning at a kind of suboptimal level and that is why we have decided to move forward with this approach.

Rishabh Tiwari: Just if I could have a ballpark number on the occupancy or ARPOB levels?

Pankaj Sahni: So we don't report out the financial data in terms of the Indore facility separately. What I can tell you is that if you look at the ARPOB levels that we are able to achieve in Indore today just with a couple of specialties that are there, it is probably about 40% to 45% lower than our Gurgaon facility. We see occupancies there more or less on a similar line, fluctuates a little bit depending on the season, because other than cardiac, neuro, a lot of the care is secondary care.

Rishabh Tiwari: Okay. That helps. Thank you.

Pankaj Sahni: Thank you.

Moderator: Thank you. The next question is from Nikhil Mathur from HDFC Mutual Fund. Please go ahead.

Nikhil Mathur: Hi. Good evening. My first question is on the ARPOB. If I remember correctly, at the time of IPO Roadshow, the sharp run-up that happened in Lucknow and perhaps was next in Patna as

well, I may be wrong here, was largely driven by non-scheme patients, mostly cash-paying patients. Has that started to normalize in Lucknow and Patna both wherein the scheme-based patients and insurance-based patients are kind of coming into the mix? Because the A RPOB continues to look good. Over the last couple of quarters, you have expanded your A RPOB. So I'm not sure if there is any ARP OB dilution, that's happening from Lucknow and Patna?

Pankaj Sahni: So, if I draw your attention to the slide number 31, where we have detailed out the payor mix for the year and indeed, even for the quarter. You will see that, we continue and have for several years now, continued to maintain a fairly healthy payor mix. Upwards of 85% of our payor mix is in the cash and insurance segment. And that hasn't really changed over the course of the last quarter or any change with the Lucknow and Patna facilities scaling up. In fact, both Lucknow and Patna continue to be towards the end of FY23, almost exclusively cash and TPA. So, no real change in the year mix from say, when we last spoke.

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Nikhil Mathur: Yes, so that's the question. In Lucknow and Patna, if we return mix will skew to a certain extent towards scheme-based patients as today, it's entirely cash and TPA, wouldn't that lead to some bit of A RPOB dilution?

Pankaj Sahni: Is the question that going forward, will that mix change?

Nikhil Mathur: Yes.

Pankaj Sahni: Yes. So, let's go unit by unit. Let's first start with our Patna facility. As you're probably aware, Patna is a PPP project, with the Government of Bihar. So, 25% of the beds are reserved at State Government rates. We have been working with the Government to get those activated but that hasn't been activated yet. We do believe that it will get activated in the first quarter of this year. You may see some A RPOB dilution in Patna specifically because of the PPP project scheme-based patients coming into play.

As far as Lucknow goes, we have not seen nor have we any immediate plans to change the payer mix there. Obviously, as we continue to scale up, we will keep re-evaluating this at the appropriate time. At present we d

on't have any major plans to change the payor mix in Lucknow.

Nikhil Mathur: Got it. That's quite helpful. Also, my second question is slightly higher level. In the entire north to east belt, do you see any major bed additions happening by non-incumbents? I will include Medanta amongst the incumbents now given its presence in three or four cities now. But do you see any major bed additions by non-incumbents? Perhaps, any player from south or west that, you might be seeing?

Pankaj Sahni: You are asking me to do a little bit of crystal ball gazing. So, I don't know. What I can tell you is that, there are many people who are not in the healthcare industry, who continuously approach us to say, will you please partner with us, to help us build a hospital or we will build a hospital and you run it. So, maybe that is just an indication of the success of Medanta or the brand of Medanta or the industry in general.

I do not know, who will decide to come into which territory of the country. However, what I can tell you is that, there is still a huge need for capacity build out across many, many parts of the country. So, the more people that are invested in the success of the healthcare industry, the better it is.

Nikhil Mathur: Would it be safe to say that, at least for the next two years to three-year horizon, even if someone tries to enter and it takes a few years to kind of set up a hospital, at least from a two years to three year perspective, the incumbents are kind of sorted on supply demand balance?

Pankaj Sahni: Yes, you have to see two, three factors. One is obviously, you cannot produce a hospital out of thin air, especially any hospital of meaningful size and quality. It takes at least three years or so, to build if you are starting from scratch. I also do not see too much visibility on hospitals, which are ready made and are just waiting to be taken over for somebody to run. So, you are absolutely right. It takes a certain amount of time to put up this kind of infrastructure, especially in terms of quality infrastructure. But let us not forget that, building and construction is the easy part.

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After that you have to actually hire the doctors and nurses and get the clinical talent to be able to operationalize and run the facility. We have done very well to be able to break even both Lucknow and Patna, within one year of starting operations, but that is not a normal trend. Normally, hospitals take at least three years to four years to deliver, EBITDA break even. So, I

would say that, yes, some of your conjecture is correct.

The other thing is that, I do not see too much construction actually underway beyond, what we have been doing. So, I cannot pinpoint that there is any specific asset that, is likely to come on board, in the immediate future. So, yes, that is the correct assumption, I would say from your part. As far as the supply-demand mismatch goes, the northern part of the country is hugely underpenetrated, whether it is UP or Bihar, Haryana, even NCR still has a lot of scope. So, a lot more to be done.

Nikhil Mathur: Right. That is pretty helpful. One more question I had is on ALOS. If I look at versus pre-COVID levels, I have some data possibly from your DRHP. The ALOS used to be 3.9 days, somewhere around that in FY19, and currently, it is standing around 3.3 days, 3.2 days. Can you give some colour of, what is happening and what has changed?

Pankaj Sahni: Sure. You are absolutely right with one exception. If you look at our ALOS over the years, and I will just quickly flag out some of the numbers, in FY19, it was 3.9 days, we were able to reduce that very actively in FY20 to 3.5 days, and I will talk about that in a minute. It bounced back up in FY21 to 3.9 days, then 3.8 days in FY22, and we have been able to bring that back down in FY23 to

3.3 days.

Now, we had actually started some of these initiatives in FY20, just before COVID to actively work on bringing the ALOS down, and that benefits patients and it benefits the hospital. We worked with some of our surgical specialties to get them to admit patients the day of surgery instead of admitting the patient the night before. We also worked with our doctors to get the patient discharged the day of the surgery itself. So we did work on ALOS reduction and efficiency.

These are standard operational improvement metrics, and it's good for everybody because we get the patient home, we are able to possibly even close down some of the wards at night to save some operating costs, and most importantly we felt that it was the right thing to do. We have reactivated some of that work in FY '23, after COVID has gone away, and we do see some amount of ALOS reduction which is very much intentional.

The other thing that's very important to keep in mind, especially in context of the Medanta group, is that, we have a very high percentage of cash and TPA patients, and we have seen that cash and TPA patients do have a shorter length of stay as compared to the scheme patients, who tend to have a little bit of a longer length of stay. So these are some of the factors that, could be leading to the lower ALOS that, you see.

But active strategy was implemented in FY 20, active strategy has been implemented in FY23, and we will continue to do our best to get our patients home as soon as possible.

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Nikhil Mathur: Right. So, it's a right deduction, right, that while your occupancy might look lower versus some of the peers that report Delhi NCR-centric numbers, but that gets offset by ALOS, right? The focus is more on the right efficiency in terms of getting the ALOS right, which might be at the expense of what occupancy numbers, we are able to see.

Pankaj Sahni: That's right. If you look at the basic mathematics, obviously the ARP OB as well as the occupancy levels are a function of the volume of admissions and the volume and the length of stay. Like I said, some of the scheme patients tend to stay a little bit longer because frankly, they're not too concerned about, who is paying for their stay. We have seen this even in our Gurgaon facility, scheme patients for the same specialty, do tend to have a little bit of a longer length of stay. So you're right.

The deduction that you did is correct. You do see differences in occupancy levels in ARPOBs, but the real thing that, we should be looking at frankly is, how the volumes have grown in terms of the absolute amount of inpatient admissions or discharges and more than even ARPOB, the average revenue per patient, which gives a slightly better perspective on what each patient is actually paying.

Nikhil Mathur: Got it. And just one final question, just kind of a bookkeeping. When I look at your operating beds to capacity beds ratio, it is somewhere around 75 %-76%. Once all these bed additions are laid out in the slide in the presentation that happens, should we assume the same percentage of operating beds with the denominator being the capacity beds?

Pankaj Sahni: So, just to give you a quick context, right, it's in line a little bit with the length of stay point that you mentioned earlier. We do have, (and we have realized this, in fact, after building Gurgaon), a very strong focus on daycare work because we have a lot of people, who come into the system during the course of the day, who may not need to stay at night.

If you look just at our Gurgaon facility, you would have maybe almost 200 beds or 300 beds on the daycare side, which is, the size of a pretty large hospital itself. So we cycle through these

patients efficiently , whether it is endoscopy daycare, emergency, chemo daycare, dialysis, etcetera.

We have seen that , we are able to deliver high quality care in this model. So we feel good about this kind of daycare model. We feel that , as we build out the other units, we will continue to maintain a fairly high daycare to census bed ratio. But , it does differ depending on the city or the area which you're in. So there are some nuances depending on , where patients come in from.

Some of our patients in our Gurgaon or our Lucknow facility , may be traveling from further away, so they may struggle with the ability to stay outside of the hospital. When you look at our Patna facility, it's located in the heart of town, so there's plenty of options for people to stay. So these things do make a difference. But in general, yes, we will try to maintain a fairly high daycare to overall bed ratio.

Nikhil Mathur: Understood. Thank you so much and all the best.

Pankaj Sahni : Thank you.

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Moderator: Thank you. The next question is from the line of Amit Kadam from Canara Robeco Mutual Fund. Please go ahead.

Amit Kadam: Yes, good evening, sir. So I have a couple of questions. So starting with this, one of the observations on the slide , where they have mentioned how the OPD and IPD have grown. In this, I guess it's slide 39. So your IPD volume growth was lower sequentially quarter -on-quarter, which my understanding about the subject is that , usually quarter 3 is a little lean quarter and then quarter 4 is a kind of normalized quarter. Still our IPD volumes were sequentially lower. Any particular reason to it? That's point number one.

And so another extension to is that , for the full year, what we have seen that OPD volume growth has been 15%, whereas IPD volumes has been for the full year 32%. Any reason that by such a stark difference between how OPD and IPD volume growth has been? Yes, this is a first question I would say. And there are a couple more.

Pankaj Sahni : So let me start with your second question first , in terms of OPD and IPD. You are referencing slide 39 metrics that reflect the whole group. An important point to note as , we look at the year that has gone by , is that we still have hospitals that are growing and in fact , growing in a very big way. So , when you look at our Patna facility, that really came on board in January 2022 we really have had only one full year of operations.

Now in new facilities, their OP D-IPD trends tend to differ a little bit as the specialties come in. So we started Patna just to give you a sense, largely with cardiac and neuro. Now , we have added in many other specialties. Some of them have a faster growth towards OPD. Some of them have a faster growth towards IPD. So , you will see some amount of variability in the newer facilities until they stabilize.

Also, don't forget we are adding in doctors fairly aggressively. So when you look at some of the doctor additions in both Lucknow as well as in Patna, and that goes to your first question as well, you do see sometimes , when a doctor comes on board, there's an immediate uptick because there's a pent up demand of that doctor and they bring in their practice to the facility, whether it is a Lucknow or whether it is a Patna. So, up until the time that you find some amount of stability in the bed addition and the doctor addition, we do find variability both in the OPD and IPD basis. Frankly, since we don't look at this on a quarter to quarter basis, but look much more at it on an overall basis, we don't stress too much on how things move on a quarter -on-quarter basis. But , we do keep in mind that we see sequential growth as the specialties are maturing and as the unit is maturing. I don't know if that answers your question, but that's how we think about it.

Amit Kadam: Okay, it does answer my question. Maybe let the things stabilize and then maybe this will be a correct metric to track

thereafter. I got it. So second question is that , our international patient contribution for this quarter was 5%, vis -a-vis like quarter 3, saw 7% contribution. And on top line, we are broadly saying , somewhere around INR 160 crores odd . Just wanted to know because the peers have actually saw some reasonable improvement in both sequential and Y-o-Y. Any particular reason? This is I'm trying to refer from slide number 40.

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Pankaj Sahni : Yes. So, on an absolute basis, we have also seen the same improvement , both in terms of volumes as well as in terms of realization. The absolute amount of international patients has grown. In fact, we have about a 68% growth year -on-year. And , we are also seeing a breakdown of that by quarter. You do see an absolute growth i n terms of quarter 3 versus quarter 4 of financial year '23.

The difference , when you're looking at this graph as a percentage of overall revenue is that , this now includes the revenue additions in Lucknow and Patna. And as you'll understand, Lucknow and Patna is almost negligible international patients. So as the base of Lucknow and Patna adds in, international as a percentage may look slightly different. But on an absolute basis, we have actually seen a growth from financial year '22 to '23.

Amit Kadam: Okay. It answers. My final question is that , this like the scheme patient , so that there were some price revisions for most of the like for certain consumables to some kind of service room rates, etcetera. So , what is that particular impact? Because we also cater to lots of CG HS or those scheme patients. Any impact , what we witness in this particular quarter? How do we see it going forward?

Pankaj Sahni : Yes. So just to clarify, we have actually under 10% of our total revenue is coming from scheme or CG HS patients. The fourth quarter it is about 8%. And if I look at the full year also, more or less the same in terms of the CG HS, ECHS. So it's not that high, especially compared too many of the others. The tariff increase that , were announced by the governm ent have been adopted in different ways. This includes the tariff increase on the CG HS patients as well as the Ayushman and other schemes.

So of course, some of the public sector undertaking, some of the corporate contracts , the partnerships which we have, those tariffs got affected immediately . When the government made the announcement. In certain other cases, the tariff increases are dependent on , how the States adopt those tariff increases. So , we may see this playing out fully, over the course of financ ial year '24, more than in financial year '23.

In addition to that, we did actually take some tariff increases towards the last quarter of financial year '23. I think Sanjeev mentioned it in his opening comments. But obviously, we will see the full impact of that play out only in this year.

Amit kadam: Okay. Thanks. Thank you. That's it. I'm done.

Moderator: Thank you. The next question is from line of Cyndrella Thomas Carvalho from JM Financial Limited. Please go ahead.

Cyndrella Carvalho: Thanks for the opportunity. And congratulations on great set of numbers. Pankaj, if we are looking at our Gurgaon facility or the standalone numbers per se, how should we see the growth from here onwards as well as , at the margin level? How should we be seeing it? Becaus e somewhere , we see that , these margin levels are coming to around 20 % odd or more excluding the other income. So how should we see and what could be the leve rs of margin expansion,

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including the standalone business of Indore and Ranchi with it? And if you can help us understand the growth path for Gurgaon as a focus.

Pankaj Sahni: Yes. So let me give you a couple of thoughts. And I'll ask Sanjeev also to clarify. There's some conf

usion amongst the community on the true definition of other income and what's included and what's not included. But let me just first give you a couple of thoughts , in terms of conceptually. See, when it comes to the growth opportunities in terms of volumes and in terms of the levers that are there for Gurgaon or the NCR market as a whole. First of all, we have added almost about 80 doctors at the senior level across our mature facilities and many of those doctors coming in towards the second half of this year.

So there has been a very definitive understanding that , given the demand that was there, that we experienced during the course of COVID, given the responsiveness of various clinicians , who want to work for us. So we have added in a significant clinical talent pool in our Gurgaon facility. And you will see that , continuing as w e move forward in financial year '24. So the first aspect is that , there's new services, new clinicians, mother and child coming on board , as an example, lung transplant, chest surgery coming on board is another example. So we are adding in the clinical fi repower in Gurgaon and we do expect that to start taking effect in the coming months and years.

The second aspect is that , we are doing some amount of realignment, as I mentioned, we're adding in about 30 beds of mother and child, 50 plus beds of premium cancer services. So we are looking at , how we reorient some of the care that , we deliver, including looking at how we can think about more daycare, better utilization etcetera . So, those will be all positive levers that , should add to the top line and volu me growth in the facility.

As far as margin goes, there are, of course, three or four levers there. The first one, as I mentioned in the question before, was on tariff. We hadn't taken very significant tariff increases for several years, some of that was because of COVID but in general, we are reasonably conservative on the pricing that we have. We have triggered that exercise in Gurgaon, more significantly in January, February, March of this year, concluding in actually towards the end of March, middle of April.

So, we will see some impact of tariff, which should be positive towards the margin side in financial year '24. We also continue to look at, how we can increase our operating efficiencies, whether it is our material cost efficiencies or the employee costs. Last year, because of COVID we didn't do much sales and marketing, and we were also trying to conserve the cash and the costs there. So, some of that cost has come back.

Also very important to note that, when you look at our mature facilities or our GHL facility, this includes all the corporate costs. So we don't actually report out, the corporate costs separately, whether that is employee costs or other shared services costs. So that's fully loaded in on Gurgaon. And to that extent, the margin is a little suppressed in Gurgaon vis-a-vis that apportionment, which you would see across all the units in a normal case.

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So, these are some of the factors that may be impacting what you're looking at in terms of the numbers as you try to dissect them. And then also on the other income, you know, it's not all interest income which should be taken out. And Sanjeev will give you a very quick overview as to what is really other income and what is actually a normal hospital income. Sanjeev?

Sanjeev Kumar: So, I think, you know, as far as other income is concerned, we have actually seen that other than the interest income, it also includes some of the income like rental income, provisions which actually get written back. And those also have been considered appropriately as far as the expenses earlier. Some of the income like the sponsorship income, this year if you actually look at, you will find that, sponsorship expenses are high. So that g

ets captured in your miscellaneous

income. So, the expenses have been captured in other expenses, but the income is captured in other income. Some of the revenue shares from, food centres, etc., etc. I think somewhere we actually feel that these are all the income which actually wouldn't have been here had our hospital not been operational. So, that's the reason why we do include other income as part of the overall EBITDA margin. And that we have been doing consistently.

I think when we actually look at the consistent basis, the margins excluding interest income it would be more or less in the same region. And I think some of the reasons which that have been explained by Pankaj as well, largely in terms of the higher expense on account of marketing expenses, including the facilitation charges, which are on account of the international operations, that's one. The second one is, also some of the expenses like conference expenses, which I just said, has gone up by almost, that also impacted almost 2.2% increase in your traveling costs, etc. Some of those expenses have also increased. But Yes, the margins remain in the range of approximately 20%-22% excluding the other income.

Cyndrella Carvalho: Thank you for an elaborate answer. Just to clarify, Pankaj, you said 80 doctors, we have senior doctors we have added at Gurgaon, right? Along with all the bed addition that you mentioned. So, all this cost is already in this number that we are seeing today?

Pankaj Sahni: Yes. Just to clarify this does, include some of the senior additions at Indore and Ranchi, as well.

As you're probably aware, most of our doctors are all on a full-time fixed kind of salary model.

So, yes, their costs will be fully captured in the P&L.

Sanjeev Kumar: For the period that they have been here.

Cyndrella Carvalho: Yes. Yes, thanks for that. And if we look at the overall new hospital margins, they are looking upwards of 30%. And if I try and understand, I mean, our Lucknow was already doing healthy margins. But can we conclude that even Patna is doing very healthy margins at least in Q4 aspect? Or how should we look at it, if you can help us understand the Patna level EBITDA margin?

Pankaj Sahni: So, Cyndrella, you may be familiar with this from our last conversations as well. We definitely do feel that Patna has delivered fairly healthy margins in not only in Q4 of this last year, but also for the full year. We have been able to deliver a strong break even in Patna. Obviously, the margins in Patna are not as strong as they are in Lucknow or even in Gurgaon, because it's still new and still upcoming. But, yes, we feel fairly confident about our

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financial performance there. And we feel that that will only continue to improve. However, to point out that Patna is still in a very strong kind of growth phase. So unlike Lucknow, which I think maybe had a slightly shorter time to achieve a reasonable scale, Patna continues to expand every quarter. It's a slightly, more quarter-on-quarter scale up in Patna. And some of that is, of course, post-COVID, we didn't have the same scenario which we had in Lucknow. So, I think that it may be a little premature to look at Patna margins from any kind of a stable state.

With Lucknow at 600 beds now firing quite well for almost two years. We do feel that is a little bit more stable. Patna, I think you have to still give us another six to nine months or maybe the whole of FY '24 before we feel really confident about what's the stable situation there.

We do intend to add in more doctors, so you will see more cost. We do also hope that the doctors that have been added in will continue to kick in. And we are also adding in very high-end expensive equipment. So, we may have mentioned this earlier, we have ordered the radiation oncology LINAC, and we will be s

tarting our radiation oncology services, hopefully early this

year. So, all these things will come in. So, Patna, you'll still see some bouncing around of margins. I think give us six to six months to 12 months to see some stability there.

Cyndrella Carvalho: Thanks, Pankaj. We understand definitely that it's just one year of operation that we are already at the gap. We can even make it up. Of course, we do understand.

Pankaj Sahni: Don't get carried away with the strong performance in Patna. And assume that is normal. We also feel it is better than normal.

Cyndrella Carvalho: Yes, Yes, that we totally agree with you on. Some of the bookkeeping questions largely on the side of, I mean, if you would like to share our, at least Gurgaon, our ARPOB and occupancy, would you be able to share that for the full year?

Pankaj Sahni: So, Cyndrella, I can see on the queue, a couple more folks who asked questions and we are already running a little late. I request, if we can set up some specific calls for any detailed bookkeeping questions that you may have. If there's overall kind of questions which may benefit everybody, you can quickly ask them there. Otherwise, if there's specific points, we set up some time, Sanjeev and team will be able to walk you through whatever you need to kind of help you build out, I guess, the models that you're working on.

Cyndrella Carvalho: No worries. Thank you so much.

Moderator: Thank you. The next question is in the line of Akash Dobhada from Motilal Oswal Financial Services. Please go ahead.

Akash Dobhada: Yes, thank you for the opportunity. I have just one question. What kind of locations would you look for in organic opportunity apart from Lucknow, Patna, Noida? And your comment on valuation on such in organic front?

Pankaj Sahni: Our approach to looking at which locations to go into is driven by four or five very fundamental values. The first, of course, is that there is a need for providing this kind of high-end, high-

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quality services. As you're aware, we operate very high-end tertiary care hospitals, so we're not really looking in at just going in in the secondary care or very lower-level complexity of care. So, the market should be able to sustain high-end tertiary, quaternary care. And that means two, three things.

On one hand, of course, there has to be a certain amount of demand as well as ability to afford those quality of services and that complexity of services. But there also has to be the appropriate clinical talent and employee pools that you can bring in on board to be able to offer and deliver those services. It's not really just about putting up a building. That's the first part.

The second part is that we do like to go into markets where there has to be some element of really being underserved in terms of at least the quality. And by underserved, I don't mean only the places with lower per capita income. As I mentioned many times, there are parts of Delhi and Bombay that you would also consider as underserved. At the same time, there may be tier three cities which may be completely fully served. So, what we look at is whether there is an underserved need or an unmet need in terms of clinical care and specifically whether Medanta can fulfil that need. Of course, lastly, we have to make sure that this can all be done at the quality standards and values that we operate in. So, that's how we look at locations.

Obviously, given our historical experience, I would say, the central and northern part of the country to be more closer to what we have historically operated in. And therefore, our bias would be towards looking into those areas first. And that includes, by the way, Greenfield as well as inorganic opportunities. Although, never say never. As far as the approach towards the inorganic opportunities, again, two, t

three fundamental factors . One, as I mentioned, it has to make sense for us in terms of location and the kind of quality of assets that we're getting into. But also, we don't want to buy an asset just for the sake of buying out EBITDA . It has to be able to fit into our ecosystem of delivering high quality care. And very importantly, it has to be in sync with the value system and the ethics which we practice. So, we have walked away from many a deal where we felt that it was not in line with the values, ethics, and governance standards that we would like to practice. And, lastly, I can say that for the most part, we are more comfortable where we are the sole provider of care and our partners, whoever they may be, are more silent partners.

It is something which is completely clean and kosher in terms of the due diligence, etc, that we look at. Of course, after all this , can you get the right kind of value for the asset that you're buying ? And frankly, I think that in many a deal, you find one or more of these things are challenging.

Akash Dobhada: Okay. Thank you.

Moderator: Thank you. Ladies and gentlemen, that would be our last question for today. I now hand the conference back to the management for the closing remarks. Thank you and over to you.

Pankaj Sahni: Thank you very much. I just wanted to thank everybody for their ongoing support. It has been our first full annual conference call after our listing . We thank you for your interest as well as your support and look forward to the same going forward. If there are additional questions or

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clarifications that you may require, we'd be happy to provide them on a secondary follow-up discussion. Thank you.

Moderator: Thank you very much. Ladies and gentlemen, on behalf of Motilal Oswal, that concludes this conference. Thank you all for joining us . And you may now disconnect your lines.

Notes:

1. This transcript has been edited for readability and does not purport to be a verbatim record of the proceedings
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Dear Sir (s),

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, the transcript of the Earnings Conference Call held on Friday, August 11, 2023 , hosted by JM Financial , for the Quarter ended June 30, 2023 results , is enclosed herewith.

The Transcript is also available at our website: <https://www.medanta.org/investor-relation>

This is for your information and record.

For Global Health Limited

Rahul Ranjan
Company Secretary & Compliance Officer
M. No. A17035

Encl: a/a

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Q1 FY24 Earnings Conference Call
August 11, 2023

MANAGEMENT : DR. NARESH TREHAN – CHAIRMAN AND MANAGING
DIRECTOR – GLOBAL HEALTH LIMITED

MR. PANKAJ SAHNI – GROUP CHIEF EXECUTIVE
OFFICER AND DIRECTOR – GLOBAL HEALTH LIMITED

MR. SANJEEV KUMAR – GROUP CHIEF FINANCIAL
OFFICER – GLOBAL HEALTH LIMITED

MR. RAVI GOTHWAL – HEAD, INVESTOR RELATIONS –
GLOBAL HEALTH LIMITED

MODERATOR: MS. CYNDRELLA CARVALHO – JM FINANCIAL
LIMITED

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Moderator: Ladies and gentlemen, good day and welcome to Global Health Limited Q1 FY24 Earnings Conference Call hosted by JM Financial. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Ms. Cyndrella Carvalho from JM Financial. Thank you and over to you, ma'am.

Cyndrella Carvalho: Good afternoon, everyone. I, Cyndrella Carvalho, on behalf of JM Financial, welcome you all on the quarter 1 FY24 Earnings Conference Call of Global Health Limited, Medanta. At the outset, I thank the management of Global Health Limited for giving us this opportunity to host the call and looking forward to have an insightful interaction on a strong earnings performance. Today from the management team we have with us Dr. Naresh Trehan, Chairman and Managing Director, Mr. Pankaj Sahni, Group CEO and Director, Mr. Sanjeev Kumar, Group Chief Financial Officer, Mr. Ravi Gothwal, Head Investor Relations. I now hand over the call to the management for their opening remarks.

Dr. Naresh Trehan: Good afternoon to all of you and welcome to Medanta's Q1 FY24 earning call. As you are aware, the results were published yesterday and the company has reported strong operational and financial performance during the quarter. The details of which of course Mr. Pankaj Sahni, our group CEO will give you later. But in the meantime I will walk you through some of the developments.

Our performance in the Q1 FY24 is on the back of delivering the highest quality of medical care with highest ethics and Medanta continues to replicate this model as we go across other parts of the geography. In the network hospitals, we have five hospitals and the sixth one, that is Noida, is on its way and hopefully in the next 18 months or so, we will be functional there too. Furthermore, most of you already know the fact that we have announced the partnership between Medanta and DLF in developing a hospital in GK1 in Delhi. For a long period, there has been a huge demand because being aware of Medanta's ethical and quality practices, people in Delhi have been actually requesting, demanding that we develop an institution over there also. We welcome this partnership, that is Medanta and DLF, to start work on this hospital which will be a 400 plus bed hospital situated over 2 acres in GK1. It will have about 20 or more super specialties and also a great emphasis on oncology, that is cancer. It will of course benefit a large number of patients not only from Delhi, but across Uttar Pradesh, Haryana, Rajasthan and other neighbouring states. In addition, we have a very strong demand for our services from international traffic and we shall also be able to fulfil that between our existing facilities which is in Gurgaon and the new one that is coming up in Noida and also addition of Delhi will make it a seamless operation across the NCR region. This is actually very much in line with our strategy to build a long-term sustainable platform and deliver, or I would say continue to deliver, the highest quality of healthcare in this region, not only in this region, but across the heartland of India.

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I would like to hand over the call to our group CEO, Mr. Pankaj Sahni, to provide an update on the quarter results, performance and new initiatives. Thank you and over to you Pankaj.

Pankaj Sahni: Thank you Dr. Trehan. Good afternoon and welcome everyone to our Q1 FY24 earnings call. We have concluded our first quarter of FY24 on a strong note with the highest ever quarterly

performance

Total Income of INR 7,945 million, registering a growth of 26.8% year on year. The company continues to deliver strong year-on-year and sequential growth driven by higher patient volumes and improvements in realization.

We are grateful for the continued trust of our patients placing us across our network. Needless to say, none of this would have been possible without the tireless commitment of our doctors,

nurses and other caregivers.

Now coming to some of the highlights for the quarter :

Both the matured and developing hospitals continue to deliver year-on-year and sequential growth. We have delivered consolidated total income of INR7,945 million, which is a growth of 26.8% year-on-year and 8.6% on a quarter-on-quarter basis, driven by higher patient footfalls and improved realizations in this quarter. EBITDA of the quarter was at INR1,993 million, registering a growth of 40.7% year-on-year, with EBITDA margins improving from 22.6% in the first quarter of FY '23 to 25.1% in the first quarter of FY '24. Our profit after tax was at INR1,020 million, a growth of 73.7% year-on-year.

During the quarter, we have added 28 new beds in Patna, taking our total bed capacity at the group level to 2,725. These additional 28 beds are dedicated ICU, which we have built at our Patna hospital. In terms of our operational performance, average occupied bed days for the quarter increased by 16.2% year-on-year, representing an occupancy of 58.1% on increased bed capacity.

ARPOB grew by 7.1% to INR63,148 in Q1 FY 24, driven in part by increased tariffs at our Gurgaon unit, as well as the increased complexity of work in Lucknow and the scale up of our facility at Patna. We continue to witness strong patient volumes across all facilities. Inpatient count increased by 19.4% year-on-year to 37,000+. Our outpatient count increased by 17.5% year-on-year to ~6.6 lakhs in the first quarter of FY '24.

In terms of developing hospitals, that is Lucknow and Patna, the total income increased from INR1,468 million to INR2,297 million, a growth of 56.5% year-on-year. EBITDA of the developing hospitals increased from INR331 million in Q1 FY23 to INR720 million in Q1 FY24, representing a growth of 117.6% year-on-year. Our developing hospital revenue share also increased from 23.4% in Q1 FY23 to 28.9% in Q1 FY24. While our developing hospital, EBITDA share increased from 22.6% in Q1 FY 23 to 31.4% in Q1 FY24.

During the quarter, international patients' revenue also increased by 45% to INR477 million driven by increased volumes and realizations. Our in-house outpatient pharmacy business revenue increased by 43% from INR186 million in Q1 FY23 to INR266 million in Q1 FY24.

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Additionally, a few key developments during the quarter :

First of all, Medanta Lucknow launched its tele-ICU services with tie-ups across ICUs and nursing homes and hospitals in Jodhpur, Maharajganj and Prayagraj cities. We have also launched our comprehensive oncology program in Lucknow with the installation and launch of our new Siemens Varian Edge radiosurgery machine for radiation oncology. It was inaugurated by the Honorable Chief Minister of UP.

We also launched our first multi-service out-of-hospital retail pharmacy in DLF Cyber Park, Gurgaon, which offers doctor consultations with Medanta experts, medicine deliveries, as well as lab services to corporate employees.

In addition to the financial and operating numbers, as Dr. Trishan outlined earlier, we have entered into an understanding with DLF to form a company to launch a 400-bed multi-specialty hospital to provide highest quality of health care in the heart of Delhi in Greater Kailash 1. Both Medanta and DLF will own 50-50 equity in the new company. DLF will be the strategic investor and Medanta will run the hospital and have operational control.

T

This new facility will strengthen Medanta's presence in the national capital region of Delhi. Medanta already operates Gurgaon facility there as well as our upcoming Noida facility and we believe that the Gurgaon, Noida and Delhi facilities collectively will integrate in a seamless manner to be able to link our 1,391 bed flagship hospital in Gurgaon, our 550 bed hospital in Noida and the new 400-bed hospital in Delhi to deliver the same high-quality seamless care across the entire NCR region. With that, we are happy to take questions from the participants. Thank you.

Moderator: Thank you very much. We will now begin the question and answer session. First question is from the line of Bino from Elara Capital. Please go ahead.

Bino Pathiparampil: Hi, good afternoon. Just a couple of quick questions regarding the DLF joint venture, what's the timeline for the beds to come up?

Pankaj Sahni : Like with any other hospital, it will probably take us somewhere in the three to four years' time frame. Obviously we need to get all the regulatory approvals initially and then typically it takes about three years to construct. So three to four year is what we anticipate.

Bino Pathiparampil: Is there a land already in the JV? Land and building?

Pankaj Sahni : I'm sorry, could you repeat that question? It wasn't very clear. Is there land where?

Bino Pathiparampil: Is the land already identified and acquired by the JV?

Pankaj Sahni : Okay, so the land has already been identified. The JV is not necessarily going to be acquiring the land. It will most probably be a leasing or a rental construct , but yes, the land parcel has been identified and as Dr. Trehan had mentioned earlier, it's about two acres of land.

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Bino Pathiparampil: Okay, got it. Second question, in your P&L, there is an item called loss on impairment of financial assets. What does that refer to?

Pankaj Sahni : Let me just hand over to Sanjeev to take that.

Sanjeev Kumar: Impairment is actually a provision for doubtful debts, which has increased in this quarter, in line with the increase in total debtors which have increased by almost INR50 crores. And out of that, almost INR30 crores is on account of the state government dues, and around INR12 crores is on account of TPA. Some of the collections , which are from the government side have been slower and therefore we have created a provision for doubtful debts .

Bino Pathiparampil: Understood. I'm sorry, just one follow-up question on the DLF deal. So the investment into building the structure and the hospital, would that be borne half-half by DLF and Medanta?

Pankaj Sahni : Yes, that is right. That's right.

Bino Pathiparampil: Okay, thank you very much.

Moderator: Thank you. Next question is from the line of Banshi Desai from JP Morgan. Please go ahead.

Banshi Desai: Yes, hi. Thanks for the opportunity. So just on this expansion , which is announced, sir, if you look at the Delhi NCR landscape, optically Delhi seems to have slightly lower bed density but when we hear about people and large private players, your competition, all of them seem to have aggressive plans of expansion into this market. So looking at that, how do you, what prompted you to basically look at this market and how confident you are of this capacity getting absorbed as you ramp up this facility?

Pankaj Sahni : Okay, let me take that in a couple of ways. First of all, to mention, or to reiterate rather what Dr. Trehan had mentioned, given that we already operate a large 1,300 plus bed facility in Gurgaon , which is close to the South Delhi area , we already had a very strong brand presence, brand equity in the region. And in fact, we run a clinic that is not too far from this location in South Delhi and there has been for many years a demand for us to open out facilities there. This demand comes

from a variety of reasons.

First of all, people want to be able to access the kind of high quality and high integrity care that we have come to be known for, whether it is for emergency care, whether it is for more day-care types of procedures, for example, chemotherapy, radiation oncology, where people feel that , it may be more convenient to get that care delivered a little closer to where they are in that community. So the demand has actually been there for some time. We also have a number of clinicians , who have practiced in that area for many years before they joined us. So we do have a good sense of confidence in the demand that , we will receive from this region.

The second point is that , when you look at our plans in the NCR region, we already plan to open up a Noida facility , which will be 550 beds in sector 50 in Noida. So if you literally look at the line drawn from the Gurgaon facility to the Noida facility, in fact if you actually just drive that

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road, you will find that this new site comes almost bang in the middle of that road, really connecting the two sites. So it allows us to create a strong network effect across the three sites. And given our full-time and our kind of model of care that we deliver, we do believe that , we will be able to develop significant synergies, both in terms of clinical as well as operational, across these three facilities across the NCR region. So that is how we thought that this was a good opportunity.

We have of course been engaging with DLF for some time on this. We have strong relationships with them as well. And so this is not something which is new and just suddenly coming out of the blue. The other thing is that Medanta has always tried to focus on making sure that , we put our head down and deliver the care that we're known for and deliver the kind of operational excellence that we stand for and not overly worry too much about what is happening on the left and the right. And that if we do that, we have seen across our units that the patient community responds in a positive manner and our intent is to continue to adopt this approach.

Banshi Desai: Okay, this is helpful. And my second question is on, if you can throw some color on, how our Patna unit has done, we know it has broken even last year, but how has been the performance in

this quarter?

Pankaj Sahni : So let me give you a little bit of context just before I jump into that question . As highlighted in our last couple of quarterly earnings calls, that Patna is still very much in a ramp up phase. While we have been able to deliver EBITDA break even in the last financial year, and continue to show very strong financial performance in this quarter, I just also want to highlight that we continue to add in clinicians and we are continuing to build out that hospital . In fact, medical equipment is in the process of getting commissioned, new beds capacities in the process of getting added . So currently it is still not really in a stable state, so to speak. Whenever you look at the financial performance of Patna, you keep this in the back of your mind.

That being said, our performance in Patna has been fairly strong. We have increased our revenue performance approximately about 20% when you look at it on a quarter -on-quarter basis, which is Q4 FY23 to Q1 FY24. We also see significant improvement in EBITDA levels. But as you may be aware, we don't report out the financial numbers for each unit on a quarterly basis, although you will see the full year financials for Patna once the annual report is published.

Bansi Desai: Okay, noted. Thanks.

Moderator: Thank you. Next question is from the line of Cyndrella . Please go ahead.

Cyndrella Carvalho: Yes, thanks for the opportunity. So just want to understand , what is working for our Lucknow facility in specific. If you can highlight

what , what are the key, like, therapy -wise also, what are we seeing the key driving forces here in Lucknow and what has worked in our favor typically if you can highlight these two aspects will be very helpful ?

Pankaj Sahni: Let me make an attempt to give you three or four levers. I don't know , which one specifically contributes how much but they all can work collectively to be able to deliver the performance

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of our Lucknow unit. So first of all, as you are aware, in some of our earlier calls we had also mentioned that , this unit got off to a start right in the middle of COVID. And at that point in time, we were able to demonstrate to the community in Lucknow and to the community in UP that we were standing firm to be able to deliver this kind of high quality care in a very challenging environment right in the middle of the COVID pandemic. We started this facility literally a few months before the lockdown was announced. And right through COVID, we were able to stand firm with the community, not only in terms of delivering care to COVID patients, but also to deliver the much -needed care to patients in other diseases because you know COVID doesn't stop cancer from increasing, COVID doesn't stop heart disease from happening. So we were able to bring that balance in and that care to the community.

The second aspect is , we were fortunate to be able to get very highly accomplished, highly skilled clinicians and care providers in Lucknow from across the Lucknow and UP region, but also clinicians coming in from other areas. We had doctors moving there from Delhi, we had doctors moving there from other parts of the country. And so we were able to assemble a highly capable accomplished team and that is evident , if you study over the last several months , the kind of quality or complexity of work that they have been doing whether it is neurosurgery, whether it is cardiac surgery, whether it is complex minimally invasive cardiac intervention. We have also got an exceptionally talented cancer team there , that is not only in the medical oncology or the radiation oncology field, but also surgical oncology, who are super specialize in a particular disease or a particular organ for treatment.

So we have been able to build out that team. In fact, in the last quarter, as I have mentioned earlier, inaugurated our radiation oncology services with the new Siemens Varian Edge machine, which is only the second installation in the entire country and the first installation of this new model for this highly sensitive and highly precision radiation oncology machine. So with that, we now have the complete suite of cancer services. We expect that the cancer work should also grow as we build that out over that area.

Overall, it has been a combination of these three, four factors. And then lastly, what I would say , we have been able to do this because we maintain the same standards in our Lucknow or even in our other units as we have in our Gurgaon unit, the same standards, the same ethical practices. Structurally, we have the same models in terms of full -time doctor employment, in terms of the way in which we think about quality, think about standards.

And so that has translated into a very positive response from the patient community. And let's not also forget that, and this is of course one of the reasons for choosing Lucknow and UP to start as we are dealing with a state with a population of over 22 crores. So there is lack of critical care infrastructure , at least in the private space , of this calibre and this scale in the entire state. And so therefore , we have become the destination of choice for complex healthcare.

Cyndrella Carvalho: Thanks for that, Pankaj. And any color on the, you see the scope for enhancing further capacity

in Lucknow from what we have already announced? Do you see the

need for that?

Pankaj Sahni: As you may have noticed in our investor presentation, we highlight that , we are currently about 600 beds in our Lucknow facility. That facility itself is expected to scale up to about 950 beds

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on the current infrastructure. In fact, this build out of that is already underway with respect to further fitting out some of the additional wards and beds for in the existing facility.

Our first target will be to complete out the 950 beds before we actually consider a second facility in Lucknow. Although given UP is such a big state, I do absolutely feel that there is a need for a lot more healthcare in that area, and especially a lot more healthcare of a higher standard and a higher quality.

Cyndrella Carvalho: This is my last one, and I'll fall back in the queue. Can you help us understand the Gurgaon outperformance this quarter, what worked for us in Gurgaon specifically?

Pankaj Sahni: In Gurgaon , as we had mentioned in our full year earnings call , we have been focusing on what we call strengthening of the core and really building out additional capabilities in terms of bringing in some additional clinicians. We have added things like a new surgical robot for robotic surgery, brought in a new super specialist in terms of oncology, cardiology, respiratory and internal medicine, some leading clinicians, well -respected, well -known clinicians from the community have been brought in. So we kind of continue to double down our efforts on bringing in the talent and building out. We have also added in a new service which is our Women and Child Service , which we didn't have in Gurgaon. That has come in and that is starting to perform well now. But overall , we have also been looking at how we can further focus in on optimization across the facility.

We are also currently looking at building out a new chemo day-care unit in the building. So renovation for that is underway. In fact, about 50 or more of the census beds are actually under renovation and are closed at the moment. So really just doing, what we have always said that we stand for, which is trying to continue to bring in high -quality talent and allow them to practice the highest end of medicine. In addition, we have done some things with respect to trying to get access and outreach to the community in general. That is through our clinics, through some of our home care and our lab businesses to try to increase the continuity of care to the extent possible. Those activities have been appreciated by our patients and our communities, especially near to the Gurgaon facility. And we do intend to continue to do that.

Moderator: Thank you. Next question is from the line of Nikhil Mathur from HDFC Mutual Fund. Please go ahead.

Nikhil Mathur: Hi. Good afternoon . I wanted to probe a bit more on the Delhi 400 bed hospital. If I look at this way, from this hospital in GK1 and your existing Gurgaon facility, the airport will not be very far of f. More or less it would be equal distance. Even for the Delhi market, if you're setting up a hospital in Noida, Eastern Delhi could well be served by that facility.

Your Southwest and West Delhi can be serviced by your Gurgaon facility. And you have land bank available in Gurgaon and you're just setting up Noida. So I'm not sure, how to look at this. That too in a setup where you are sharing economics with a JV partner. So can you just throw some more light on what's the need of this hospital in such an affluent neighborhood in Delhi, where some of your existing networks in Noida and Gurgaon could also have well taken care of, whether international or upcountry, or even for that matter, local Delhi footfalls as well ?

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Pankaj Sahni : So, three -four points that we have found as the advantageous reasons for getting into Delhi. One,

I will not go deep into the population and the current underserved area of Delhi, especially given the huge growth that is happening there. So when you look at all the density ratios, etc etc , despite what people talk about in Delhi, they're still pretty low for covering that population. So there's still a huge demand supply gap in Delhi.

The second thing is that , if you look at this facility, it's about over 30 kilometres from our Gurgaon facility, approximately 25 to 30 kilometres from our Noida facility. So it's almost bang equidistant between the two. And as you rightly said, when you look at the servicing of the catchment area, although we do get in Gurgaon many patients from outside NCR, when you look

at the servicing of the catchment area, and just in terms of a travel and a commute point of view, you will find that , Gurgaon does cater a little bit more to the South West Delhi market and Noida will cater a lot more to the Eastern Delhi and then of course , parts of UP market as well.

Hence, South Delhi, Central Delhi area does at times find it challenging especially given the traffic conditions in the city to get access to the hospital on a very regular basis. Now this becomes important given what we see as the changing trend in healthcare. So if you look over the years, of course, barring COVID, you will see that the average length of stay continues to reduce in our Medanta facilities. And part of that reason is because more and more work is moving into a day care nature. So, whether I said before, whether it is chemotherapy day care, whether it is radiation oncology, patients from Delhi do find it at times inconvenient to travel an hour and a half in the car to get to their care. So, they have actually been demanding, and we see that in our Delhi clinic, they have been demanding access to this kind of standard of care closer to where they live. The other important point, of course, is emergency care. So, a gain, there has been a lot of demand for us to open up especially cardiac emergencies, stroke etc etc in and around Delhi. I think the feeling is obviously that there is a pull for the brand, there is a pull for the quality of medicine and the ethics of medicine that we practice.

These are some of the reasons that we feel that this makes sense to us. In fact , one may argue that Delhi just being so big and continuing to grow, there are opportunities even beyond South Delhi and other micro geographies of Delhi. So, I think it's important to understand Delhi and this region operates very much in a micro market kind of way and we believe that this will be a very big boom.

The other thing is that when you look at the complexity and the nature of care that we deliver many of our patients come in with highly complex issues, many times multi -organ issues. So , there is an opportunity to service these patients from a continuity of care point of view across units in a seamless manner. Just to give an example, you may have a patient who comes in with a GI surgical issue, but lives in Delhi and they come in for an elective procedure, but they actually already have a cardiac issue or they already have some other comorbidity which requires more continuous monitoring. They now feel that they can get that kind of seamless care of the standard that Medanta is famous for, closer to their home and across the network.

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So, this is what we found, and this is the demand really from our patient population. So, we believe it will be highly profitable, highly successful, and most importantly, much needed for the patient community.

Nikhil Mathur : Sir, if I may just hop a bit more on this, what you're saying makes a very strong business case and I think that's what it is looking like today. But if I fast forward three years , four years, in just not just five,

six kilometre away, you have such massive expansion happening in Saket by one of your large competitors. With you also having 400 beds, do you think the things would look the same three years , four years out? I mean, today it might be sounding a very big, good business case, but with so much of beds coming on stream in a radius of 5 kilometres , 7 kilometres , that will still hold true three years , four years down the line as well?

Pankaj Sahni : You know, 2-3 points ; first, we've tried to focus our efforts always on making sure that we do the right thing and we operate at the right standard, rather than worry too much about what the others may or may not be doing and that has served us well so far. But I am also reminded of our journey, which I think now is about two years ago, when we started our entire IPO road show process and at that point in time, there was a lot of questions from people around all the capacity which is supposed to come up in Gurgaon, in our backyard. I mean, I haven't yet seen any of this capacity. And even if you look at Medanta history from the time we started in 2009 till today, I mean, hospitals have opened and closed, and we're still standing tall. So , this doesn't worry us too much and like I said, there is a dire shortage of high -quality healthcare facilities. But finally, at the end of the day, we believe that if we continue to focus on doing the right things, which is practicing the highest quality of medicine, everything else in terms of the financial success will follow from that.

So, the model has worked for us, it has worked for us in Gurgaon, it has worked for us in Lucknow. I was briefly mentioning this when I was in Lucknow at the launch , we also had many people in Lucknow wondering, what are you doing in Tier 2, Tier 3 cities , but we have proven that to be successful and the same story in Patna. So, I think that we feel fairly confident on our capabilities and we feel that this also will be a strong and successful operation.

Nikhil Mathur : Got it. And just final bookkeeping question. So , this will be consolidated line by line item and you will adjust it in a JV line item or there will be proportionate consolidation or you'll be paying a yield to DLF on this? How will the accounting work?

Pankaj Sahni : I'll let Sanjeev take that. My understanding from the accounting rules is that since we are completely in operational control, we will be consolidating this operation in our holding company, Global Health Limited.

Sanjeev Kumar: Yeah, so at present, all the definitive agreements have to really take place. But as Pankaj said, our understanding is that in terms of the IndAS ; the entity having operational control shall be able to consolidate it line by line and in our case Medanta will have operational control

Nikhil Mathur : Great, sir. Thank you so much and all the best.

Moderator: Thank you. Next question is from the line of Sumit Gupta from Motilal Oswal. Please go ahead.

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Sumit Gupta: Hi, thank you for the opportunity. Sir, I just want to know regarding the margin of the Gurgaon facility. Could you provide some like overall trend, what has been the like over margin for this particular facility?

Sanjeev Kumar: So, as you are aware that we actually don't report our margin for each unit. And as far as Gurgaon is concerned, this is part of the standalone GH L, which is actually Global Health Limited. And this actually has the other two units as well, which is Indore and Ranchi. At present , if you actually look for this particular quarter, the EBITDA margins are at 24.7 % for the standalone entity .

Sumit Gupta: So, but Gurgaon facility would be having more margins?

Sanjeev Kumar: Actually, in case of GH L, a large portion is contributed by Gurgaon.

Sumit Gupta: Okay, sir. And sir can you provide us an outlook on the

margins of the developing facility over
the next one years or two years?

Sanjeev Kumar: I would say that both the units are doing pretty well and they are having very healthy margins and there is no reason that at this point in time where we actually think that the margins will get impacted except to the fact that in case of Lucknow at present the entire margins that we are having is on cash and TPA basis . Once we increase the capacity from 600 beds to over 950 beds over a period of time, we do expect that we will be moving away from only cash and TPA to other payer categories as well. Once that happens, it may have some impact on the ARPOB's and margins . Secondly, we also need to keep in mind that in case of Patna, at present , the business is largely from cash and TPA . In addition, this is a PPP project, 25% of the capacity is actually reserved at the state government rates, which are more or less on the lines of CGHS rates. We have not seen any significant patient inflow from that site and however we expect that probably in the coming two or three quarters, this will really pick up in a more meaningful manner. So, once that happens, that's also likely to have an impact on our overall ARPOB profile. Secondly in terms of the expenses , as our Lucknow and Patna facility now starts getting mature , some of the expenses in terms of your AMC, CMC etcetera will start coming in because at present, most of the equipment 's are new. Now, if you look at some of these expenses may actually go up, but the fixed costs which are there in terms of manpower, in terms of the other expenses etcetera , we will be able to apportion such costs it over a large capacity of patients and that should help us in margin improvement. Overall, I think based on all these parameters, we expect that our developing hospitals will continue have a healthy margin profile .

Sumit Gupta: Okay, sir. And so regarding CGHS patient , so was there any price revision undertaken by the government or was there any contract change?

Pankaj Sahni : Can you just repeat the question? I'm sorry, I missed the first part.

Sumit Gupta: So, regarding the CGHS patient contracts, so was there any price revision which was undertaken?

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Pankaj Sahni : Yeah, so CGHS announced a price increase a couple of months ago. So, that price increase has of course translated into whatever it is, the standardized rates for CGHS, those are applicable typically across the country as well as some of the public sector undertakings which use those rates. So, that of course has led to beneficial revenue or upliftment for the hospitals and for the industry in general.

But I just want to point out, if you see in our investor presentation as well, we have a significantly lower share of CGHS in the business. About 85% of our business continues to be consistently quarter over quarter from cash and TPA. So, while it has benefited us, keep in mind that it's only for about 10%, 12% of the overall.

Sumit Gupta: Okay so one more regarding the industry point of view just wanted to understand about it the majority of the hospital chains are focusing on Delhi NCR market. So, like why not they're focusing majorly on the like West area like Western India like Rajasthan or Gujarat, so just wanted to understand on that why is Delhi NCR the core focus area for majority of the hospitals from industry point of view like if you can...

Pankaj Sahni : If you look at the industry I'm not exactly sure whether your hypothesis is correct . I would assume actually that South India has a far higher percentage of private hospitals than North India. It is possible that some of the groups which you may be referencing may be the ones which are listed and therefore are more in the public domain in terms of communication.

But if I look at the industry as a whole, I think that South

India has a far higher density of hospital

beds and also private hospital beds. Of course, these vary from established unlisted chains to listed chains to maybe smaller hospitals which maybe have just a few hospitals which are operating under their bed. In many cases, those would be unlisted. So I don't necessarily think that it's only the north, the city of Delhi, which is having demand for hospitals. Hyderabad, South, Bangalore, Chennai, a lot of these metro cities, but also some of the Tier2 cities are having good build-outs. As far as the West goes, you do see movement there as well. Obviously, one of the big constraints in hospitals, especially bigger hospitals in terms of size, is land.

So, land is, of course, less easily available in a city like Bombay, maybe than a city like Delhi. And obviously, the more you move out to Tier 2 and Tier 3 cities, I'm assuming that the land availability would be easier. So maybe that's why you're seeing the growth there. But yes, Delhi has always been an attractive market. It has also always been a market which is talked a lot about.

I think the combination of being a high population, slightly maybe better structure in terms of its layout for scale out and build out because Delhi has expanded in almost concentric circle beyond what it was which is maybe a little challenging for Bombay to do. So you've seen Delhi grow as the city, population grow and therefore the demand for health care grow. So I think these are some of the factors, but I do also see growth happening in many, many other cities in the country.

Moderator: Thank you Sumit. Next question is from Dheeresh Pathak from White Oak Capital Management Consultants LLP . Please go ahead.

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Dheeresh: Yeah, hi, this is Dheeresh . So on this 400 bed in Delhi, just to understand, so DLF is bringing the land, sorry I missed the opening commentary so you might have covered it there. So in that partnership with DLF of 400 beds in South Delhi, so DLF is bringing the land to the SPV or what else? They are bringing capital and then you will buy the land?

Pankaj Sahni : I think I mentioned in a question which came earlier, so we are forming a company with DLF, so it will be a new company that is established. DLF will provide equal equity participation as will Global Health Limited. And Global Health will run this company and run the operations and have control over this company. And I think somebody had asked whether that will be consolidated and our belief is yes, it will be with GHL. So, we will have complete control to run the business operations . This entity will lease land for the hospital operations , I don't think that we will be buying this land. So the new entity will be in entering into some kind of an operating lease or rental kind of an arrangement to run the hospital. Right now the definitive agreements have to be finalized, the company will be incorporated, etcetera, and only once the company is incorporated , the lease will be executed by this company.

Dheeresh: Okay, so land will be on lease and building will be constructed, and what is the capex outlay for this?

Pankaj Sahni : So, we haven't finalized the business plan obviously because that will be approved by this company's board as well but if I just go by what we're seeing in in our Noida facility , which is currently under construction I think today given the escalation in construction costs, you're talking about fully loaded somewhere around INR1.4 crores to INR1.5 crores per bed . If you do that math on the 400 -bed hospital, you're probably talking somewhere in the INR600 crores to INR700 crores range in terms of this facility.

Obviously, in Delhi, you may have slightly higher construction costs and as we move from today to a couple of years down the line when the construction will really be in full swing, there may be some escalat

ion in construction costs, etcetera . Although , a lot of it depends because we did see this in Noida, the steel , cement prices really badly escalated during the immediate post - COVID times and then they rationalized a little bit. But that's what we're assuming it to be

somewhere around that number.

Dheeresh: This was earlier asked also, I think, why are we sharing 50% with them? They're just bringing the capital. So we are not constrained by capital, right? So INR300 crores, we could have done it ourselves right, their portion of the capital because you are going to be running it, you are going to value creation. I just feel that we should have got 100% unless they are helping with the land or something.

Pankaj Sahni : As you are obviously aware, in any partnership, there are a number of factors that go into how things get concluded the way that they do. I would only say that our belief, given DLF, given its reputation, and given its strength that we believe that this is an exceptionally strong partner to have not only for us in this particular operation but well into the future. Both the parties, both the companies feel that we have similar thought process in terms of how we want to take this relationship forward.

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And we believe that there are a huge number of opportunities in the healthcare space. So for us, we see a strong ethical and values-based partner in DLF, and I hope that they see the same. And that is why we believe that this partnership is not something that we are looking at as a single transaction, but as a longer-term successful partnership for many, many years to come.

Dheeresh: Understood, thank you. Thank you for taking my question.

Moderator: Thank you. Next question is from the line of Alankar from Kodak Securities. Please go ahead.

Alankar: Hi, thank you for the opportunity. Sir, first question, how many beds are we planning to add in Lucknow in this fiscal?

Pankaj Sahni : If you look at our investor presentation and I think we've mentioned it in the year-end one also, it will be about 150 beds. We have actually started the construction for almost four floors or five floors because of the way in which the building is. So just the interior fit out is done. And then depending on as the demand scales up, currently we are planning out three floors, which is, I believe, just short of 150 beds. And then it depends on how quickly the demand scales and what is the minimum optimal number of beds to construct one at a time.

Alankar: So in that case Pankaj, you had earlier mentioned that once you cross 700 beds in Lucknow, broadly that's when we will start having some scheme patients. So would it be fair to assume that by the end of the fiscal, somewhere around maybe first quarter of FY '25, when our scheme makes the institutional patients will start coming in Lucknow?

Pankaj Sahni : Little early to conclude on that I think that as a responsible healthcare provider, we do feel that we have an obligation to service the broader community. I think right now, what we see is a very positive response from the community. We are treating patients from across the socioeconomic strata. We are very fortunate to be able to get a good amount of support in terms of the numerous schemes that are available, not only at the state government level, whether you talk about things like the C M Relief Fund, but also other state patients who come in. So there are opportunities for us to serve across the spectrum.

I think that we will take this on almost like on a contract-by-contract basis. So there are a certain number of PSUs that may be operating a certain way. There may be a certain number of patients that we bring in at slightly different tariffs, which may not necessarily be scheme tariffs, but maybe a little bit somewhere in the middle, or there's some combinations of those

. So there are many opportunities for us to look at this. I don't think it's definitive that I cross a certain bed number and then I turn on switch on and off. I think this is an evolution as things change as we are able to serve.

We also are cognizant of the fact that we should be affordable. One of the things that we have not yet done is taken a tariff increase in Lucknow at all really, since we started the hospital. And at present, at least we haven't really made any plans to do that. So I think when you look at the overall pay or mix and you look at the overall tariff and revenue structure, it's a far more nuanced approach and one that we evaluate almost on a quarterly or monthly basis but I don't think that there is a goal in mind to say, once this happens and you'll do that. What I did articulate earlier

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was that when we reach this number of 700, 750, you kind of are able to then fully utilize the baseline infrastructure and thereafter, the build out typically is dependent on what services are more in demand.

I had mentioned it in the last call that sometimes you may over-index in cardiac, or sometimes you may over-index in cancer. In the case of Lucknow, certain specialties which we didn't anticipate having a very high demand have started demanding more beds. So these are all the ways in which we kind of decide how to build out. So that is really where we go. We have a certain set of specialties and infrastructure in mind, till about 750 beds and thereafter I think it is linked a little bit to how the demand plays out.

Alankar: Fair enough, that's clear. My second question is at present how many beds are being reserved for the EWS patients in Gurugram?

Pankaj Sahni : Our lease requires us to keep about 5% of the beds available for EWS patients. That being said, we have never stopped any of the patients from coming in. The various government authorities that send across patients to us are all accommodated. And we have also never yet received that many number of patients either. So as and when they are sent across, we are treating them fully. In fact, if you walk around the hospital, you would not be able to tell which patient in which ward is EWS, because we don't have any dedicated ward. They all get treated in exactly the same way as any other patient.

Alankar: Thank you. And maybe one final question if I may. Just wanted to understand the reasons behind this delay in the PPP inflow in Patna. I assume that it's already started but you said that it would kind of ramp-up over the next few quarters. So how quickly should we expect the intake to be what's causing the delay any inputs there would be really helpful. Thank you.

Pankaj Sahni : Some of the reasons for this delay, I think are just you know kind of operational and tactical challenges that the authorities may have had. One of the things that I am aware of that they had requested us to do was assist them with building out the digital website and portal which will be used for referring patients. So we have actually completed that last quarter and handed that over to the government. They had some operational challenges and sought our help to build that out on their behalf, which we have done. We have handed over access passwords, etcetera, for them to refer patients because they want to keep a track and a log and have a database for all of this. The second is obviously these are hospitals and authorities across the state that need to be lined up and sent in. We have been engaging very closely with the government and supporting them in whatever information they want, a awareness, information, etcetera. I can report that the first patient has been admitted through the PPP portal which we had handed over to them. As to how this flows and how that information disseminates across all the government officials in the state of

Bihar, I'm not sure. What I can tell you is that at the senior most levels in the government, there is a strong enthusiasm to push this. We are all hopeful that this will scale rapidly so that we can serve this population.

Alankar: Sure, thank you and all the best.

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Pankaj Sahni : Thank you.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I now hand the conference over to the management for closing comments.

Pankaj Sahni : Thank you. Let me take a minute to say that while we are very happy and pleased with the financial performance of the first quarter of FY24, we do continue to focus in on the things that we believe matter with looking at slightly longer-term perspective on ensuring that we build out the right capacity in the right manner with the right standard. As well as operate that capacity with the right ethics and the right focus on delivering high-quality care. While we are pleased with our quarterly performance and continue to focus on driving that on a daily basis, we do look at this slightly more holistically and in the long run. And we hope that all the participants appreciate that this business is slightly different from many other industries. We continue to ensure that we focus on doing the right thing, practicing the right kind and right quality of medicine. And believe that the results of that will speak for themselves both in terms of the reputation as well as the financial performance.

Thank you very much for listening and for your support.

Moderator: Thank you very much. On behalf of JM Financial that concludes this conference. Thank you for joining us. You may now disconnect your lines. Thank you.

Notes:

1. This transcript has been edited for readability and does not purport to be a verbatim record of the proceedings
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Sub: Disclosure under Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 – Earnings Conference Call Transcript

Dear Sir (s),

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, the transcript of the Earnings Conference Call held on Friday, November 10 , 2023 , hosted by JM Financial , for the Quarter and Half year ended September 30, 2023 Results , is enclosed herewith.
The Transcript is also available at our website: <https://www.medanta.org/investor-relation>

This is for your information and record.

For Global Health Limited

Rahul Ranjan
Company Secretary & Compliance Officer
M. No. A17035

Encl: a/a

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Q2 FY24 Conference Call "
November 10, 2023

MANAGEMENT : DR. NARESH TREHAN – CHAIRMAN AND MANAGING
DIRECTOR – MEDANTA – GLOBAL HEALTH LIMITED

MR. PANKAJ SAHNI – GROUP CHIEF EXECUTIVE
OFFICER AND DIRECTOR – MEDANTA – GLOBAL
HEALTH LIMITED

MR. SANJEEV KUMAR – GROUP CHIEF FINANCIAL
OFFICER – MEDANTA – GLOBAL HEALTH LIMITED

MR. RAVI GOTHWAL – HEAD, INVESTOR RELATIONS –
MEDANTA – GLOBAL HEALTH LIMITED

MODERATOR: MR. JAINIL SHAH – JM FINANCIAL

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Moderator: Ladies and gentlemen, good day and welcome to the Global Health Limited, also known as Medanta, Q2 FY 2024 Conference Call hosted by JM Financial. As a reminder, all participant lines will be in the listen-only mode. There will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star and then zero on your touch-tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Jainil Shah from JM Financial. Thank you and over to you, sir.

Jainil Shah: Thank you. Good afternoon and very warm welcome to all the participants on the Global Health Limited Q2 FY '24 Earnings Call hosted by JM Financial. Joining us today from the management team, we have Dr. Naresh Tr ehan, Chairman and Managing Director, Mr. Pankaj Sahni, Group CEO and Director, Mr. Sanje ev Kumar, Group CFO, Mr. Ravi Go thwal, Head, Investor Relations. I will now hand over the call to Dr. Tr ehan for his opening remarks. Thank you and over to you, doctor.

Dr. Naresh Tr ehan: Good afternoon and Happy Dhanteras to all of you. From my knowledge, I've always heard Dhanteras, which comes before Diwali, is a big trading event. So I guess you guys have been busy.

As far as Medanta or Global Health is concerned, this is the completion of one year of us having launched the IPO and we became listed. We have experienced that there has been a very strong response from the shareholders and stakeholders and they have become more and more integral part of our journey. The real DNA of Medanta has been that we deliver the highest end of care with the best quality and at the most affordable cost that can be made possible.

Integrity and ethics is our central point and everything revolves around these principles so that we can serve as many people who need help, who have the most complex of problems because the real platform of Medanta is having the best of doctors in every specialty functioning on the same platform as a team. And this gives the patient confidence that they have attention of the best of doctors to be able to solve the most complex of problems. As you may be already aware of the fact that we are at a very high end, you know we do every kind of transplant, lung, heart, kidneys, liver. So we are actually going to places where we feel that Medanta kind of quality is missing and can be accessed by the maximum number of people.

Now as you may have seen that after building Gurugram, we have now gone to Lucknow which has completed four years of operations and still growing very strongly. And Patna is completing two years and also ramping up very favorably. More and more patients are accessing quality care there and their confidence grows. We have also received NABH in less than two years which is not the norm for hospitals to be accredited by National Accreditation Board.

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Furthermore, we have launched Medanta TB-free Uttar Pradesh project in partnership with the UP government. We have already been doing it in Haryana. Now we are extending it to Bihar also. This is in line with CSR activity and give back to society, TB which is a very sort of debilitating disease can be detected early and treatment started early so that we can get rid of TB from this country.

During the period, we have continued our commitment of delivering the most complex care and our doctors offer the latest robotic treatments. We have done over 200 robotic procedures during this quarter. Our transplant teams are offering critical care. Patients flying from thousands of kilometers to get treated by us and so

it has been a good journey and I want to thank you all for supporting us in this journey. Our patients have been very supportive and growing more and more.

Now I will hand over the call to Mr. Pankaj Sahni, Group CEO, who will walk you through the

numbers and also be able to give you the growth story. So a very happy and safe Diwali to all of you. Pankaj, can I request you to take over? Thank you.

Pankaj Sahni: Thank you, Dr. Trehan. Good afternoon and welcome everyone to our second quarter FY '24 earnings call. We are extremely pleased to report that the company has delivered another quarter of strong financial and operational performance. This growth is driven not only by our new hospitals but by our mature hospitals as well.

Just to summarize for you a few quick points:

Medanta has delivered a total consolidated income of INR8,647 million, a strong growth of about 24.5% year-on-year and this growth was primarily driven by higher patient volumes across all of our hospital units. EBITDA was INR2,336 million which is a growth of 35.7% year-on-year. EBITDA margins have also improved by 224 basis points from 24.8% in Q2 FY '23 to 27% in Q2 FY '24. The improvement in EBITDA margins is led by higher occupancies which is of course benefited with a higher operating leverage.

Profits after tax was INR1,252 million which is a growth of 46.1% year-on-year and PAT margins have improved by 214 basis points year-on-year to 14.5%.

Our overall growth is driven by higher patient footfalls. Our inpatient volumes in Q2 FY '24 increased by 19.4% year-on-year to 41,506 and the outpatient volumes increased by 23.2% year-on-year to 7,27,728. Volume growth was driven by the impact of new clinical talent additions, bed additions, some of which were done towards the end of the last financial year and some portion of seasonal disease impact as well for this quarter.

Average occupied bed days for the quarter have increased by 17.8% year-on-year representing an occupancy of approximately 65% in Q2 FY '24. It's important to note that our census bed capacity has also increased by about 7% on a year-on-year basis and therefore our occupancy on an increased bed capacity has been around 65%.

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Our ARPOB grew by about 5% year-on-year to INR61,003 in Q2 FY '24 led by an increase in tariffs at our Gurgaon facility as well as the ramping up of our Patna facility.

Developing hospitals which comprise Lucknow and Patna, their revenue share has increased from 27% in Q2 FY '23 to 31% in Q2 FY '24 amounting to INR2,643 million and developing hospital EBITDA share has increased from 33% in Q2 FY '23 to 38% in Q2 FY '24 amounting to INR897 million. This is along the lines we had articulated earlier around the further geographic diversification of the Group. Occupied bed days at developing units has increased by 38.7% year-on-year with largely consistent ARPOB reflecting that this growth has been led largely by volume increases.

Matured hospitals have also registered strong revenue growth of 19.5% year-on-year and an EBITDA growth of 32.8% year-on-year amounting to INR6,064 and INR1,545 million respectively. During the quarter, international patients' revenue has increased by 20% year-on-year to INR507 million driven by increased volumes and realizations.

Our outpatient pharmacy business continues to register strong growth, revenue increased by 39% year-on-year to INR298 million in Q2 FY '24.

In addition to the financial numbers I just articulated, I would like to share some major developments during the quarter.

During the quarter, we have added additional operating theatres in our Patna facility taking total operating room headcount from four to eight and this has allowed us to add in the ability to perform more-and-more complex surgeries. We continue our expansions in Patna as we speak.

We have added 40 doctors in this quarter across all our facilities and have also strengthened our research capabilities with the appointment of Dr. Shoibal Mukherjee as our Director of Medical Research.

As far as our updates on our bed expansion and capacity expansion, our expansion at Lucknow and Patna are progressing well. We are planning to add another 125 to 150 beds at Lucknow and approximately 50 beds at Patna by the end of the current fiscal year. Construction in both these facilities is currently underway.

Our Noida Hospital project is on track and is expected to commence operations by FY '25 end as we had mentioned earlier. I am also pleased to report that our project has received pre-certification of the green building certification under the IGBC Green Healthcare Facilities Rating System. Finally, our recently announced Medanta Delhi Hospital in partnership with DLF is on track. The definitive agreement between the two parties has now been executed.

With this, I would like to wish you all a happy Diwali and hand over to our Group CFO, Sanjeev, to discuss the half-year FY '24 performance.

Sanjeev Kumar: Thank you, Pankaj. Now, I would like to share consolidated financial performance of our first half that is H1 FY '24. Medanta has delivered consolidated total income of INR16,592 million

in H1 FY '24 , reaching a strong growth of 25.6%. This strong growth in revenue is in both

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matured and developing hospitals. EBITDA for the H1 FY '24 is at INR4,329 million, reaching a growth of 38% with EBITDA margins improving from 23.7% in H1 FY '23 to 26.1% in H1 FY '24.

Our profit after tax was at INR2,272 million, that is reflecting a growth of 57.3%. This strong performance is driven by higher patient footfalls . Our inpatient volumes in H1 FY '24 increased by 19.4% Y-o-Y to 78,934 and the outpatient volumes increased by 20.5% Y-o-Y to 13,84,907. Addition of critical clinical talent and increase in bed capacity also contributed to these higher volumes.

Average occupied bed days for H1 FY '24 increased by 17.1% Y-o-Y, representing an occupancy of 61.5% in H1 FY '24 . I would also like to inform you that our census bed capacity has increased by 12.7% on Y-o-Y basis and therefore on increased bed capacity we have been able to deliver 61.5% occupancy. Our ARPOB grew by 5.9% Y-o-Y to 62,011 in H1 FY '24 , led by increase in tariff at Gurgaon facility as well as ramping up of Lucknow and Patna facilities.

We have increased our capacity beds from 2,569 in H1 FY '23 to 2,725 in H1 FY '24 . It is important to note that during this period number of ICU beds has increased from 563 to 664 in H1 FY '24 . So out of 156 bed additions, 101 is in ICU beds which shows Medanta focus on expanding its critical care infrastructure.

During this period we have also added five new operating theatres out of which four were added at Medanta Patna and while one operating theatre was added at Medanta Lucknow. This will allow us to perform complex surgeries and further enhance our critical care capabilities. We have also added 250 -plus doctors across our facilities over H1 FY '23 during the period.

From a balance sheet perspective, we have incurred a capex of INR1,276 million during H1 FY '24, out of which INR308 million was incurred towards Noida Hospital. We have a strong and net cash positive balance sheet and are very well positioned to fund our future expansion plan and growth.

Now we are happy to take questions from the participants.

Moderator: Thank you very much. We will now begin the question -and-answer session. The first question is from the line of Tushar Manudhane from Motilal Oswal Financial Services. Please go ahead.

Tushar Manudhane: Thanks for the opportunity. Congratulations on good set of numbers and happy Diwali to the entire team. Sir, my question refers to more on

the Patna. If you could share some ballpark profitability for Patna Hospital now ?

Pankaj Sahni : Sorry, could you just repeat that question , Tushar, if you could share what about Patna?

Tushar Manudhane: The profitability of the margin side Patna for first half FY '24 or 2Q FY '24?

Pankaj Sahni : So, Tushar, as you are aware, we reported the numbers on the quarterly basis for the mature and developing hospitals separately. You will get a chance to see the GHPPL, which is our Patna

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facility at the end of the year. What I can share with you is that overall we've seen very strong and robust growth in Patna. If you recall, our financial year FY '23 , we had ended with approximately 10 % EBITDA margin, and that has grown significantly during this quarter. You know, getting to a very healthy profitability margin and almost doubling the kind of profitability . If you look at our growth across Patna, it has come in from good amount of addition of clinical talent, ability of the teams that were already existing to expand their scope of services because of the additional operating rooms that we've been able to add. Of course, a normal growth happening because of the increased awareness of our facilities, and we see now patients coming in from various parts of the region as well. Overall the Patna growth story continues very strongly. We believe that just in the second year to deliver these kind of EBITDA and profitability metrics is not a small feat. However, I would like to add that we have still quite a long way to go in Patna. We've got a lot of growth still to come. We've got clinical talent still being onboarded.

One point , I think which may be important to note with respect to Patna, as you may be aware, it is a public -private partnership with the government of Bihar. And this quarter, we have seen the government patients start to now flow in. While this activity has taken some amount of time

to get started because of various logistical challenges, I'm pleased to report that we are now able to service the patient community, which is being referred to us through this partnership with the government of Bihar. So, on an overall basis, I think strong performance in Patna, and we're very happy with the way that is growing up.

Tushar Manudhane: Understood, sir. That's helpful. At the same time, ARPO B for developing hospitals is pretty stable now for almost three quarters, four quarters, while we have been adding clinical talent. So, any thoughts over there?

Pankaj Sahni : So, when you look at the ARPOB across the developing hospitals, one, of course, we don't really look at this on a quarter -to-quarter basis. There is some amount of fluctuation and seasonality depending on what is the disease profile. But if you look at this across the year, what you will find is that Lucknow Hospital, which has been now in operations for just about four years, we haven't really taken any tariff increases in that at all. So, a lot of the growth that you see coming in, not only in terms of ARPOB , but also in terms of the average revenue per patient, is largely driven by the volume growth and as that expands. Some amount of this is a little bit dilutive in this quarter, because the patients who come in for dengue and some of the other medical management patients typically have a little bit of a lower realization. But more than a lower realization, you will notice that the average length of stay also has increased a little bit. So, of course, that has a dilutive effect on ARPOB .

Patna, of course is a newer facility there we haven't taken any tariff increases as well. So I think that as we look at the years coming forward or the months coming forward, we will evaluate what is the right time to look at tariff increases in both these facilities. But we do also expect that as the sales mix stabilize

s beyond the seasonal fluctuations, you will see a better picture of how this ARPOB moves on an annualized basis.

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Tushar Manudhane: Okay. And sir just lastly, for this 100 beds addition at Gurgaon, so how much is the approximate investment and when would they be operational?

Pankaj Sahni: Sorry, for Gurgaon, how much is the what? Sorry?

Tushar Manudhane: 100 beds, the presentation indicates 100 beds will be added at Gurgaon. So just would like to understand how much is the investment and when will that be operational?

Pankaj Sahni: As we have mentioned before, the bed additions in Gurgaon are largely linked to some amount of interior work which is happening. So the capex on that is fairly limited. We'll be adding one complete floor of our mother and child services. We added in this clinical team towards the end of last financial year. And their working space or their area is being outfitted. We expect this to be on -board by the start of the next calendar year. So that work is underway.

As far as the capex goes, since it is really just an interior fit out, it is almost negligible amount of capex that is involved because there is no incremental build, which is happening in terms of civil infrastructure in Gurgaon.

The second area is our medical oncology floor, which we are renovating, as we may have mentioned as well earlier, converting additional chemotherapy beds, scaling that up. And that also is more of a renovation and an increase in capacity in our existing infrastructure. So also fairly negligible capex.

As far as the capex goes for Gurgaon bed additions, I would say, it would be negligible.

However, as we look forward, we do anticipate some amount of capex, maybe somewhere in the range of maybe INR70 crores to INR100 crores over the course of the next year -plus, which would be largely medical equipment refresh and upgradation of technology in our Gurgaon facility.

To answer your question, the bulk of the capex, which we anticipate coming in in Gurgaon, will be more around technology than around infrastructure.

Tushar Manudhane: Thank you, sir. Thanks for the elaborate answer and all the best.

Pankaj Sahni: Thank you.

Moderator: Thank you. The next question is from the line of Jainil Shah from JM Financial. Please go ahead.

Jainil Shah: Yes. Hi. Thank you for the opportunity. My question is on the capex. So like which other regions are we targeting now ? We have announced Indore, we have announced DLF JV. So, which other regions are we planning to expand?

Pankaj Sahni: I think with respect to capex, as I said, I let Sanjeev handle that. But a little bit respect to what the capex that we have already visible. I think really your question is not just capex, but around, where do we see additional facilities coming up?

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When we look at Medanta's approach to care, we have always maintained that our approach is to make sure that we are able to deliver a very high standard of care at the right complexity and care level. We are more in the business of bigger, definitive steps, high-end, high-complexity kind of institutions.

We have also articulated that we believe that in the immediate future, our strategy will remain focused around central India and north as a region. We are currently evaluating opportunities in several cities in these regions, both in territories where we already have existing hospitals as well as in newer cities.

A lot of our thought process around this is to make sure that we are able to get the right asset or the right location at the right scale. Therefore, as we think about this growth, which you will, of course, come to hear about as and when we conclude on that, it has to be the growth which links up to our strategic footprint.

Last quarter, we articulated that the

strategy around our facility in Delhi was not only to look at

Delhi as a stand-alone facility, but actually to look at the entire NCR region now linking up to start with our Gurgaon facility, our Noida facility and then our Delhi facility, which very nicely links up along a major arterial road connecting these three facilities.

So we do have thoughts on how we can serve the people of the NCR region, but also other areas in Uttar Pradesh. We already have a big facility doing very well in Lucknow. And obviously, with the success of that, there is a demand from the communities to see if we can provide additional services there. But also, as I mentioned, other cities beyond our existing regions continue to be in focus. As and when these become slightly more definitive, we will definitely share them with you and the larger community.

Jainil Shah: So, sir, can you specifically call out which cities are these and what would be the asset size that we would be looking at? And what kind of business model or rather how would we be financing it? So will it be a pure green field? Will it be in a partnership? What is the thought process there?

Pankaj Sahni: Jainil, you would obviously appreciate that there is only so much that we can call out, both with respect to where we are on what is definitive as well as in context of, too much crystal ball gazing into the future, which may or may not be allowed. So I'm not at this point in a position to give you specifics on which city we're looking at beyond what I've said.

As far as the partnership model, you're aware we just signed the deal with DLF. So obviously that is a partnership that we feel is a very strong and well-respected partner. You are also aware that the O&M model where a partner would build out the warm shell and hand over to us is something that we already are looking at doing.

And then, when you look at our balance sheet, I think Sanjeev mentioned we are already sitting on approximately INR600 crores of net cash. So we do have a fairly healthy balance sheet to be able to look at opportunities directly, both in terms of greenfield or brownfield or acquisitions as they come up. So I think we are not constrained on any one particular model. However, the

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model has to make sense for that location and has to make sense from a quality and our brand point of view, as well as from a financial point of view.

The one thing that I can say for you is that we will unlikely make very small bets. So you are unlikely to see, 70, 100 bed big footprint facilities from Medanta, especially if they're in the greenfield arena. They will typically be larger in size.

Now, again, a city like Lucknow, a 1,000 beds may also be too small. But we believe that certain other cities, like, for example, Indore, 300, 400 beds is an optimal size. So the size of the facility will be linked to the territory, but it will be a substantial facility for the most part. So I think the sweet spot we typically believe is 300 beds and upwards is a good size for the kind of care that we deliver.

Jainil Shah: So that's helpful. I have a few questions, but I'll get back to you. Thank you.

Moderator: Thank you. The next question is from the line of Bansil Desai from JP Morgan. Please go ahead.

Bansil Desai: Yes. Hi. Congratulations on a good set of numbers. My question is on occupancy. So we've seen very strong outcomes in this particular quarter, both in mature and in developing clusters. And for mature specifically, the increase is not just quarterly, but even on a year-over-year basis, we are seeing a 500 basis points of expansion. So I understand international patients has grown significantly for us. But should we consider this as a new base and barring quarterly fluctuations? You know, can we expect your overall mature cluster to trend at 65% and then, above?

Pankaj Sahni

i: Banshi, let me answer that on two, three dimensions. First of all, as far as the mature hospitals goes, we have seen strong growth coming in from all specialties, not just the specialties, which are a little bit more seasonal in nature for this particular quarter. And as you very rightly pointed out, this growth is not just a quarterly growth. This is actually growth, which we are seeing on an annualized basis.

I would also like to just refer you back to the strategy that we had articulated, several quarters ago where we really talked about investing in and focusing on our core capabilities. So if you recall, I think it was in our last quarterly presentation or maybe even both the last earnings calls, we have mentioned that we have made very significant clinical additions in our mature facilities. Almost 80 doctors added in during the course of 12 months in our mature facilities, which is for the most part, in case of Gurgaon, over 10 -year old facility. So what we are really doing is delivering and executing well on what we had said. And some of that is now playing out over the months, which you are seeing.

So what you are actually seeing is the playing out of the new talent coming in, the new equipment getting utilized, and all of the strategies around hardcore execution and focus on our operations and our quality. And that gets reflected, whether it comes in this quarter or the previous quarter. Yes, there is some amount of seasonality with respect to internal medicine, dengue, etc. But frankly speaking, we've seen a little bit more of that in our Lucknow facility than our Gurgaon facility.

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This is how we see ourselves playing out. Like I said earlier, we don't necessarily just look at this on a quarterly basis. We are committed to putting in place all the tools that allow us to grow consistently and in a stable manner over the period. And by the way, we are still adding in clinical talent into our mature facilities and, of course, also into our developing facilities. So from our point of view, the growth in clinical talent, technology and patient servicing in terms of volumes continues. And we do hope that that will play out as the quarters go forward. There will be some seasonality that changes based on festivals, typically the Diwali time is a little bit more conservative in terms of elective procedures. But these things, even out over the course of the year, which is really how we look at it. So that's our approach.

Just to touch on quickly on the developing facilities, Lucknow, pretty big now in terms of 600 beds. But like I said, adding in about 100, 150 beds there. So we do anticipate growth coming in Lucknow in terms of bed addition as well. We are also adding in clinical talent into Lucknow. In fact, both going deeper in the existing specialties as well as filling out other specialties that may not be as mature as some of the others. So Lucknow growth will continue both on beds as well as in talent.

Finally, Patna. Patna is, as you are aware, is a little bit further behind. So that talent will continue with addition and that bed growth will continue. And frankly, in Patna, we've already seen very high demand, especially for services like critical care, emergency care, etc. So we are trying to ramp that up as fast as possible. And our immediate focus actually in Patna is to add in approximately 40, 50 ICU beds because there's a big demand for it.

And then lastly, as we move, it's not a quarterly play, but as you think about the next 12 months to 18 months, we do already have our sights set on the talent that will be required to get Noida off the ground. I may have mentioned earlier as well that we have already started to look at some of this talent coming in in advance so that we can get everybody geared up to the Medanta way of functioning and also get as advanced a team as

possible ready so that we can hit the ground running in Noida.

So that's how we look at it across all the three pools of developed assets, mature assets, developing assets, and then the upcoming Noida facility.

Banshi Desai: Understood. Thanks for that. And therefore, just thinking from a profitability standpoint, if we continue to see improvement in our occupancies at both these clusters and given the bed addition, barring Noida is not going to be significant at Lucknow and probably is also measured in case of Patna as well. We can continue to see improvement over the current levels as well, right? Because the drag from the newer units are not going to be as much. Is that correct understanding?

Pankaj Sahni: So just to clarify, we do have, on a base of 600 beds in Lucknow, we will be adding approximately 130 to 150 beds. So from a percentage capacity increase, that is not a trivial amount. So there will be some addition. However, as you rightly pointed out, a lot of that addition is going to be in, because of the way in which we build out within the existing structure.

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As far as the profitability and the drag goes, look, I mean, you all are better experts to kind of project some of these things out. The way I think about it is that when you look at the company as a whole and you look at the portfolio of assets that we have, we feel fairly confident about how well balanced portfolio of assets we have. So you've got a strong and so to speak, well-sweated asset in terms of our mature facilities, of course, led by Gurgaon and very big in scale. That's delivering well. And as that volume scales up, obviously, you get the benefits of profitability coming in there.

Lucknow is now hitting a little bit of a more mature state, even though we're still growing there. And there we see as we add in volumes, as we add in capability there, you will obviously see some amount of operating leverage benefit. And we still don't have any subsidized patients as such in Lucknow. So that's all at a fairly healthy realization. And then, of course, Patna, Patna as it scales up, you will see the EBITDA profile increase there as well. So the drag that, may come in maybe next 12, 18 months as we start to put some expenses into the pre-operationalization of Noida, kind of gets well balanced out as the operating leverage kicks in in some of the developing hospitals and as we continue to grow in our Gurgaon facilities.

Overall, we're not very concerned about, how this moves. But like I said, we don't view this on a quarterly basis, but a little bit more longer term. But I think if you look at this across the year, we feel fairly good that we have got this portfolio well balanced to be able to manage both the profitability ups and downs, as well as the investment that will be required to build out into the future.

Bansi Desai: Got it. This is very helpful. Thank you. And I wish you a happy Diwali.

Pankaj Sahni: Thank you. And to you.

Moderator: Thank you. The next question is from the line of Rishabh Tiwari from Allegro Capital Advisors. Please go ahead.

Rishabh Tiwari: Hi, thanks for the opportunity and congratulations for the amazing set of numbers. Just wanted to understand the capex cycle regarding the INR100 crores mentioned for the medical equipment's and technology upgradation of Gurugram facility. So how what is the capex cycle of such kind of cost increase that we incurred, say, four years or five years? Because it was also mentioned that Lucknow facilities are on four years old now. So maybe in a year or two, that would be incurring such kind of expenses at other facilities?

Pankaj Sahni: Let me hand over to Sanjeev to give you a slightly more holistic perspective on capex beyond just the medical equipment. But looking at how we're thinking about this over the next couple of years. So,

Sanjeev, you want to give a quick overview on the capex?

Sanjeev Kumar: If we actually look at the capex, we need to incur capex on three things. One is the increase in actual capacity in case of Lucknow, which is expected to go from 601 beds to almost 950 beds. So I think that's that one area of capex that we really need to incur. The second area of capex is that when we actually have to increase the capacity in case of Patna, which also has to go up to almost 650 beds.

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The third place where we will be incurring capex would be in Noida. And in case of Noida, this is going to be a 550 bed facility, which we have already said that we will have 300 beds, you know, in the initial phase. So these are three places as far as the capex is to be incurred in terms of the existing projects. And apart from that, there would be a maintenance capex. So as to say for the purpose of technology upgradation or, you know, maybe some of the assets which are getting worn out and need to be replaced.

So primarily when we look at these four segments, we actually find that we are likely to spend almost in the region of INR1000 crores to INR1100 crores over the next three years or so. Out of which the maintenance capex, which you look at in case of Gurgaon, we have been seeing that it would be somewhere in the region of, almost INR60 to INR75 crores on an average in some years it could be lower and actually some years could be higher.

Pankaj Sahni: Also, just to clarify on the question around the Lucknow four years, much of the medical equipment is in the nature of bigger assets, CT-MRI, cat h labs, etc. And their life cycle is not four years. It's actually closer to 10 years. So, some of our assets in Gurgaon are now running for 13 years and running very well. So just to give a clarity that the medical equipment, especially the bigger, more expensive medical equipment does have a longer life than the four years you articulated. It's probably closer to, you know, eight to 10 and in some cases maybe even longer.

Rishabh Tiwari: Thanks for the clarification. And what would be the maintenance capex at a consol level?

Pankaj Sahni : What is a maintenance capex at a console level?

Sanjeev Kumar : At a console level, as I explained that probably in case of Gurgaon, it would be a capex. But in case of MHPL, which is Lucknow and at Patna facility, at present, they may not be major maintenance capex. So it would be largely only in case of their matured units, which could be in the region of approximately INR60 to INR75 crores per annum. In a particular year, it may be lower or higher, but it would be within this range.

Rishabh Tiwari: Thank you.

Moderator: Thank you. The next question is from the line of Sourabh Kapadia from Sundaram Mutual Fund. Please go ahead.

Sourabh Kapadia: Yes, thank you for the opportunity. So first question on the, how we should look at the ramp up of the new beds, both at Patna and Lucknow? And will that change the pay or mix in the overall company?

Pankaj Sahni : Okay, so let me go aspect by aspect, since you asked about Patna and Lucknow. First, Patna, we are currently at about 358 beds in Patna. And as I mentioned, we are building out about 50 beds more. Those 50 beds actually for the most part are ICU beds, although we are also adding chemotherapy, dialysis, etc. beds in those facilities. So some of the demand for the beds in Patna are actually linked to, potentially higher realization patients because of the fact that it may be critical care.

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We are already operating with about 84 ICU beds in Patna, but we will be building out. So that asset will go from somewhere around 350 beds today to maybe, a little bit over 400 beds, maybe 410, 420 beds by this end of this financial year. As I mentioned

already, that is part of the existing build out. So the bulk of that capex is pertaining to interior fit out, etc. Largely, again, for the ICUs that we're building. One important point also is that our radiation oncology services in Patna, as we speak, not in quarter two, but post end September have actually begun. We have started our radiation oncology services in Patna as well and in the coming weeks, we hope to have an inauguration there of that facility. And so that should help ramp up our cancer services in Patna as well.

So that's how we see the bed build out happening in Patna. As far as the supporting clinical teams, etc. there, I would say for the most part in place, we would have some additional doctors being hired in to manage our ICU, etc. And of course, the supporting staff for that as well. When it comes to Lucknow, we have already got 600 beds operational in Lucknow. And out of those 600 beds, about 200 beds are ICU beds.

So there is a strong demand for adding in some of the ward beds in Patna. So not only will we be adding ward beds across specialties, but there's also some talk about increasing our mother and child capacity there, because we have seen a very, very strong response of our mother and child services in Lucknow. As I mentioned, somewhere around 100, 150 beds getting added in to Lucknow, mostly ward beds, and that will scale out.

Now, in Patna, to answer your question on a payor mix, Patna may see some slight shift in payor mix because of the fact that the PPP work has really only started towards the latter part of the second quarter of this year. As that scales up, you will see that not only the regular cash and insurance business, but you may see some of the PPP mix come in as well. As far as Lucknow goes, currently no real change or no real plans on any changes in the payor mix. But we do believe that that should be able to sustain us towards the end of this financial year as well. So no changes in Lucknow payor mix anticipated.

Sourabh Kapadia: Okay. And my second question on the long-term margins for these developing hospitals. Right now, it's much higher than the mature hospital, but if you look at two, three years down the line, how would you look at the margin profile for these developing hospitals?

Pankaj Sahni: Again, you're asking for a little crystal ball gazing. I would definitely say that the margin profile on the developing hospitals has been very strong. Obviously, a lot of that is currently our Lucknow facility, which, as I mentioned just earlier, is currently 100% cash and TPA business. So, you know, as Patna becomes a little bit more part of this, you will see Patna contributing a certain percentage to that profitability or a greater percentage to that profitability. And so Patna does have a lower margin profile than Lucknow.

As well as when you look at this out by the end of this year, we will probably be somewhere around 700, 750 beds in Lucknow, around 400 beds in Patna. And we don't see very significant shifts that will happen there. But as you move forward, there are possibilities that you may have different payor mixes kicking in. But then also, as I mentioned earlier, you we have not taken tariff increases in these two facilities. So you will have the positive impacts of tariff coming in.

Hence, I think the way to think about the margin profile is on levers. Right. So there are some levers which may be slightly dilutive, lower tariff patients in terms of PPP, lower pricing maybe as compared to both the developing hospitals, which is Patna versus Lucknow. Patna tariffs are slightly lower, but then there are positive levers as well. One of those positive levers was on the margin profile is, of course, tariff increases that may get taken.

And then very importantly, you have operating leverage. So just don't forget that we have

a fairly

robust teams in place in both the institutions. And many of those teams are in place to cater to the full facility, not just the 600 or the 300 beds that are there right now. So as the facility scales, the costs should not scale linearly. And therefore, you should get the benefit of operating leverage kicking in as we move out on this.

So I think overall, we feel fairly confident about the margin profile in the in the developing units.

But there will be some up downs as we move along the quarters over the next few years.

Sourabh Kapadia: Okay, thank you. And all the best.

Pankaj Sahni: Thank you.

Moderator: Thank you. The next question is from the line of Alankar Garude from Kotak Institutional Equities. Please go ahead.

Alankar Garude: Hi, thank you for the opportunity. Sir, my first question is more of a continuation of the previous discussion. If you look at our margin profile between mature and developing, there is a big difference, as you said, almost 10 percentage points. Without giving any numbers, is it possible to broadly rank all the five facilities in the order of their profitability?

Pankaj Sahni: Alankar, today, when you look at our Gurgaon facility, all the so-called corporate overhead costs are fully accounted for in our GHL, which is our mature facility. So logically, if we were to look at a costing or a profitability by assets, it would make sense for some of that cost to be apportioned across all of the units, on some basis, maybe revenue or whatever. So you will definitely see some dilution of the EBITDA profile in the developing hospitals and some increase in the mature hospitals to the extent that that margin profile gets spread out across.

I think it's no secret, even if you look at the full year financial results, which declare out the three entities, which we have, which is MHPL, GH PPL and GHL, that currently on a percentage basis, the Lucknow facility has got the highest margin profile. And then, I would say, that the Gurgaon facility would be, next in line. And, of course, Patna is little bit unfair to compare financially on FY '23 margin profile or even now, because it's still very new compared to the other two. And it's a question of about a year and a half of full operations in Patna versus about four years in Lucknow and maybe, about 10 years, 12 years in Gurgaon.

I would say that, since you asked for a ranking, that's how I would rank it. But I don't think that it is necessarily a fair ranking, because in some ways, they're all running in slightly different races or at slightly different periods of time. And that, as I said, from a portfolio level is actually quite beneficial.

Alankar Garude: Fair enough, Pankaj. That's helpful. The second question is, if you look at, I mean, Patna and compare it to Lucknow, if I exclude the difference, which will potentially come up in the future because of the PPP partnership, is there any other fundamental reason which can lead to our margins in Patna being different than Lucknow?

Pankaj Sahni: I think we should be very clear. The market in Patna is not at a tariff level or a patient realization level as high as Lucknow. There is definitely a gap of around 10% to 15% from a realization point of view between Lucknow and Patna or UP and Bihar, you may say. So there is absolutely a difference in that vis-a-vis the two units, not just linked to PPP or payor profile.

The second thing is that, because of the historical nature of the clinical talent in Lucknow and because, frankly speaking, of the exceptionally fast ramp-up that we saw when Lucknow opened. You may recall Lucknow opened right around COVID time and while everybody else was struggling, Lucknow just took off like a rocket. So, we did get a very, very fast ramp-up and therefore a fast recovery of costs, although we have been able to break

even in almost all our

facilities within one operating year. But Lucknow had an exceptionally fast ramp-up, even better than what we had anticipated. So I don't think that payor mix is the only reason. I do think that

Lucknow benefited from that fast ramp -up.

Patna is moving along very well, but a little bit more slow and steady. And maybe everybody has got a little bit spoiled with the speed with which Lucknow achieved whatever it achieved.

But I think if you compare Patna on an overall basis at the size it is, I think and you compare that across industry, I think also doing exceptionally well in terms of how it ramps -up. But I do believe that the EBITDA profile between these two facilities will be slightly different in the long run as well, just because of the nature of the market and the payor ability in these markets.

Alankar Garude: Sure, that's helpful. And one final question. How has been the offtake of this PPP partnership over the past few months? I mean, broadly in terms of patients, how many patients are being admitted on a daily basis? Are there more efforts by the government to kind of spread awareness about this partnership? Any color on this?

Pankaj Sahni: Just give you the quick context. You may be aware that there had been some logistical challenges with respect to how we can actually operationalize this partnership in terms of things like building out the website and how the various government authorities can send in and refer the patients to our facility in Patna.

That, for the most part, has been fixed. The websites are up. Access has been given to all the government medical colleges and other appropriate officers by the government authorities. And they have been very, very supportive in this. I must say that across the board, the partnership is fairly strong. The support from the government authorities has been very, very good. And we have also been helping them to build out this.

We have right now a majority of the patient population still coming in from the Patna area. And we are actively working on our side as well as the government is working to actually spread this information to all the districts where the patients can be referred from. So today you see, I would say almost 80%, 90% of the patient population still coming in from Patna. As far as PPP is

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concerned, we do believe that in the coming months that will spread out to all the government medical institutions around all the districts of Bihar. And we will see patient flows coming in from those regions as well.

Alankar Garude: I'm sorry, broadly out of the 25%, what would be the occupancy right now negligible?

Pankaj Sahni: It's right now negligible. If you look at the Q2 numbers, because this really started in maybe last couple of weeks of September. So as far as the occupancy percentages come in, it's almost negligible. And we haven't really captured exactly the occupancy for PPP versus cash separately. As we scale that up and as we see the volumes grow, we'll assess whether that's something that we can capture and report.

Sanjeev Kumar: Just to add to that, Alankar, I think the good thing which has happened is that the process has started. You know, all the processes of escrow mechanism, etc, that is now settled. So we have received the actual payment of the very first set of such PPP patients. So I think those systems have been now established and then in the subsequent quarters, we will see much better inflows on account of PPP.

Alankar Garude: Understood, sir. Thank you and wish all of you a very happy Diwali.

Sanjeev Kumar: Happy Diwali to you as well.

Moderator: Thank you. We have no further questions. I would now like to hand the conference over to the management for closing comments. Over to you, sir.

Pankaj Sahni: Thank you very much. And on behalf of Dr. Trehan, our entire

clinical leadership, as well as our

entire management team, I would like to thank you all for participating. Just to close by saying that, over the course of the last couple of interactions we've had with you, we have remained committed to executing on the strategies we've talked out.

We believe that focusing on the right quality, the right ethics and value system is the right way forward. We hope that is translating into the appropriate results as well. And we do intend to continue this focus on quality values and a slightly longer term approach towards how we move forward.

With that, I would like to take the opportunity to wish all of you a very Happy Diwali and Healthy Diwali. And thank you all for joining this call.

Moderator: Thank you. On behalf of JM Financial, that concludes this conference. Thank you all for joining us. You may now disconnect your lines.

Notes:

1. This transcript has been edited for readability and does not purport to be a verbatim record of the proceedings
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Call: Feb 2024

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Symbol: MEDANTA
Sub: Disclosure under Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 – Earnings Conference Call Transcript

Dear Sir (s),

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, the transcript of the Earnings Conference Call held on Friday, February 9, 2024 , hosted by Motilal Oswal , for the Third Quarter and Nine Months ended December 31, 2023 Results , is enclosed herewith.

The Transcript is also available at our website: <https://www.medanta.org/investor-relation>

This is for your information and record.

For Global Health Limited

Rahul Ranjan
Company Secretary & Compliance Officer
M. No. A17035

Encl: a/a

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"Medanta – Global Health Limited
Q3 FY24 Earnings Conference Call "
February 09 , 2024

MANAGEMENT : DR. NARESH TREHAN – CHAIRMAN AND MANAGING
DIRECTOR – MEDANTA – GLOBAL HEALTH LIMITED

MR. PANKAJ SAHNI – GROUP CHIEF EXECUTIVE
OFFICER AND DIRECTOR – MEDANTA – GLOBAL
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MR. YOGESH KUMAR GUPTA - GROUP CHIEF
FINANCIAL OFFICER – GLOBAL HEALTH LIMITED

MR. RAVI GOTHWAL – HEAD, INVESTOR RELATIONS –
MEDANTA – GLOBAL HEALTH LIMITED

MODERATOR: MR. TUSHAR MANUDHANE – MOTILAL OSWAL
FINANCIAL SERVICES LIMITED

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Moderator: Ladies and gentlemen, good day and welcome to Medanta Global Health 3Q -FY24 Earnings conference call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Tushar Manudhane. Thank you and over to you, sir.

Tushar Manudhane: Thanks, Sagar. Good afternoon and a very warm welcome to all the participants on the Global Health Limited 3Q FY24 Earnings call hosted by Motilal Oswal. Joining with us today from the management side, we have Dr. Naresh Trishan, Chairman and Managing Director, Mr. Pankaj Sahni, Group CEO and Director, and Mr. Ravi Goyal, Head, Investor Relations. Over to you, Naresh, sir.

Dr. Naresh Trishan: Good afternoon to all of you and welcome to Medanta's Q3, FY24 earnings call. I am also pleased to welcome Mr. Yogesh Kumar Gupta as our Group CFO and he is with us on this call. Now, coming to Q3 FY24 results, we have yet again delivered strong operational and financial performance.

More importantly, our clinical expertise, quality infrastructure, technology and compassionate approach are all aligned to ensure the best outcomes to our patients. The success of our strategy is visible in the growing patient footfalls across our facilities and also encourages us to serve more and more people in areas where these kind of facilities are required.

During the quarter, Medanta Group became the first hospital in India to use advanced AI-enabled Penumbra Lightning technology for treating patients who have suffered a very severe case of pulmonary embolism, which means that blood clots from the legs have gone into the lungs and are life-threatening. In the past, the patient in this condition will either have to be given very high doses of blood thinners, which led to many complications, or we would have to operate on them physically removing the clots. Now, this was all very high risk for the patient, but with the introduction of the AI-enabled Penumbra Lightning technology, we have been able to suck these clots out with the AI-monitored, assisted device which sucks out these clots, only the clots, not the blood. The patient suffers very minimum trauma to the body and is relieved immediately of the lack of oxygen that was happening because the pulmonary arteries were choked and lungs could not function. Overall, this has been a great advantage from the patient's point of view and a great technological movement forward.

Second, Medanta Lucknow has also launched advanced NICU ambulance and child development center amid growing demand for quality mother and child care health services. And this was much needed in the community because many newborns were losing their life because immediate care was not available to them. With the introduction of the NICU ambulance, we are now able to pick them up immediately and bring them to our NICU, that is the Neonatal Intensive Care Unit, which is a very large unit at our Lucknow facility and has been able to save many lives.

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Medanta Patna has just launched a comprehensive cancer institute. Now, this cancer care program, which includes the most advanced radiation oncology machine, known as the Varian Edge, is the first of its kind in that part of the country. And Patna is now one of the most advanced facilities for cancer treatment because the precision with which the Varian Edge machine can deliver radiation to add a focus to where the cancer is without damaging the surrounding tissue is much more precise. With the introduction of this, I think the patients of all of Bihar will benefit hugely.

Overall, as a group, we continue to explore territories where we can make a difference in the standard of healthcare delivery. At Medanta, patient wellbeing is at the heart of our strategy and these values will continue to guide our decision making as we move forward in other locations.

I would now like to hand over the call to our group CEO, Mr. Pankaj Sahni, who will provide an update on quarterly results and walk you through them. Thank you.

Pankaj Sahni: Thank you, Dr. Trehan. Good afternoon and welcome to everyone for our Q3 FY24 earnings call. First of all, I would like to introduce our new CFO, Mr. Yogesh Gupta. He has joined us yesterday. He was appointed in our board meeting, as you would have heard. He is a highly experienced and accomplished chartered accountant with 27 years of experience in finance, accounting and commercial management across diverse industries, including pharmaceutical, manufacturing, retail and healthcare. He joins us from Sheares Healthcare. While during his career journey, he was also a vital part of various other organisations such as Max Healthcare, Fortis, TPG, Reliance Retail and Ranbaxy.

Now, coming to our quarterly financial performance, we have concluded yet another quarter with strong all-round operational and financial performance. Medanta reported consolidated total income of INR 8,545 million, a strong growth of 21% year-on-year. The growth was driven by higher patient volumes across our hospitals and ARPO B growth at our Gurugram facility. EBITDA was INR 2,340 million, which is a growth of 36% year-on-year. EBITDA margins increased by 305 basis points from 24.3% in Q3 FY23 to 27.4% in Q3 FY24. Improvement in EBITDA margins is mainly because of higher operating leverage. Profit after tax was INR 1,235 million, a growth of 53% year-on-year. PAT margins improved by 304 basis points year-on-year to 14.5%.

As I have highlighted in earlier calls, our growth is led largely by patient volume growth, which is reflected in higher IPD volumes, which increased by 13% year-on-year. Average occupied bed days for the quarter increased by 15% year-on-year, representing an occupancy of 64% for the group.

As anticipated, there is seasonal dip in occupancy levels at matured hospitals on a sequential basis on account of various festivities in the quarter. However, this was offset by higher occupancy growth at Lucknow and Patna. ARPO B grew by 4% year-on-year to INR 60,571 in Q3 FY24, primarily driven by increases in tariffs and realisations at our Gurugram unit.

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Developing hospitals, which comprise of Lucknow and Patna, registered strong growth of 33% year-on-year. Revenue share increased from 29% in Q3 FY23 to 32% in Q3 FY24, amounting to INR 2,742 million. Developing hospitals' EBITDA share increased from 37% in Q3 FY23 to 41% in Q3 FY24, amounting to INR 968 million.

Mature hospitals also registered strong revenue growth of 17% year-on-year and EBITDA growth of 37% year-on-year, amounting to INR 5,875 million and INR 1484 million respectively. During the quarter, international patient revenue increased by 6% year-on-year to INR 473 million, driven by increased volumes and realisation.

Our OPD pharmacy business continues to register consistent growth. Revenue increased by 28% year-on-year to INR 287 million in Q3 FY24.

We continue to strengthen our core and have on-boarded 253 doctors in the nine-month ended December '23 against the same period last year. Out of 253 doctors, 159 doctors were added in our mature facilities, while 94 doctors were added in developing hospitals.

We have incurred capex of approximately INR 1,770 million during nine-month-ended December '23, out of which 66%, which is INR 1,170 million, was incurred towards our Noida hospital project.

Overall, our balance sheet remains well capitalised to fund future expansion and growth plans.

Coming to a few updates on our bed expansion. As you are aware, our Noida Hospital construction is underway, this is on-track, with about 82% of the superstructure completed. Mechanical, electrical and plumbing work is currently under progress, with the hospital expected to commence operations by the end of FY25. We expect to add another 140-150 beds in Lucknow and approximately 30-40 beds in Patna by the end of March 2024.

In Medanta Gurugram, we are in the process of converting one of the floors, which had approximately 60 beds to day care for our medical oncology chemotherapy procedures. In addition, we are adding approximately 50 beds more for our mother and child unit.

In Q3 FY24, Medanta and DLF Limited had incorporated a SPV company by the name of GH Hospitals Limited to construct and operate our super-specialty Medanta Hospital in South Delhi. With this, I am now happy to take any questions from the participants.

Moderator: Thank you very much. We will now begin the question and answer session. The first line of question would be from Tushar Manudhane. Please go ahead, Sir.

Tushar Manudhane: Yes, while the question queue assembles, just a couple of questions from my side. Sir, we have a good amount of cash available on the balance sheet, while we have grown organically, you

know, since inception, so how to think about the usage of surplus cash?

Pankaj Sahni: Thank you, Tushar. So, there are two or three stated strategies which we have articulated over various conference calls. For the most part, these strategies remain consistent. The first one, as

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I have mentioned many times, is of course to continue the various build-outs that have already been planned and disclosed.

Most immediate amongst them, of course is Noida, followed by the scale-up in our remaining hospitals, which are already constructed, as well as now the build-out of our facilities in Delhi as that project comes on board. We are, of course, also continuing to look at opportunities beyond these stated six or seven hospitals. And as those come on board and become definitive, we will be sharing them in due course with all of you.

The second aspect, when you think about the utilization of our cash, of course, we are looking at growth opportunities beyond our organic. And as we have mentioned in the past as well, that we are always open to looking at what potential opportunities may come in, provided they meet the criteria that we have very clearly laid out in many of our presentations, as well as our annual report, around ensuring that they are able to continue to create assets for us that deliver the highest quality of healthcare and provide healthcare in areas where we feel it makes a difference in the quality to serve in those communities.

We feel that the opportunities in healthcare delivery, as well as in its ancillary industries are quite strong and we continue to remain optimistic about them.

Tushar Manudhane: Sure. So, can you also update on Indore project?

Pankaj Sahni: Our Indore project, as we had articulated last time, is still undergoing various due-diligence processes. We need to make sure that everything that we do is 100% transparent, above board, and completely in line with our values and ethics. We do not want to rush into anything where the due-diligence and the legal processes of ensuring 100% compliance to the various real estate and construction approvals, etc are in place. And therefore, we continue to wait for our partners to obtain the necessary approvals before moving forward.

We are also continuing to evaluate opportunities in Indore, in our existing facilities scale-up, as well as opportunities that remain with our partner who we have engaged with

, as well as

opportunities that may be above and beyond that. But we will not move forward until we have complete clarity and transparency on the appropriate regulatory requirements.

Tushar Manudhane: Thank you, sir. I will join back in the queue. Thank you, sir.

Moderator: Thank you. The next question is from the line of Bino Pathiparampil from Elara Capital. Please go ahead.

Bino Pathiparampil: Hi. Good afternoon and congrats on a good set of numbers. Just one question around ARPOB.

We are seeing that most of your peers, competitors are reporting very high ARPOB growth this year, in the range of 15% and above. As I understand, there has been quite a few revisions in insurance rates, CGHS rates, etc. this year, which is helping people, apart from, of course, maybe some cases.

I was wondering why your ARPOB growth is much lower at around 4% levels.

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Management: Is that the complete question?

Bino Pathiparampil: Yes, yes.

Management: Okay.

Pankaj Sahni: So, a couple of points. First of all, when you look at our growth across units, both developing as well as the mature unit, as well as the developing units, you will find that, unlike many in the industry, our growth has been driven mostly by volume growth. For the quarter, we see about 13% growth in inpatient footfalls, 15% growth in occupied bed days.

And the same applies for the nine-month period, about 17% growth in inpatient footfalls, and about 16% growth in occupied bed days. And when you look at this for our mature hospitals as well, it is not only the new hospitals that are showing volume growth, our mature hospitals are also growing very well on the volumes. So, we are very happy with the kind of volume growth that is coming.

And much of our revenue growth is actually driven by volumes. What this means is that we have not been forced to resort to tariff increases to drive the revenue growth. We've always maintained

that the tariffs which we charge to the public should be as reasonable as possible, both in our newer facilities as well as our mature facility.

Just to give you a sense, we did take some tariff increases in our Gurgaon facility, starting in January 2023, and that was continuing out till about April 2023. We've also seen the tariff increases from the CGHS and some of the insurance renegotiations. But frankly speaking, these tariff increases are not really the largest driver of our revenues.

Also, to let everybody know, we've seen very strong growth in Lucknow and Patna. And Lucknow has not taken a tariff increase since it started four years ago. And Patna also has not taken a tariff increase since it started two years ago. Again, in these developing hospitals, we see very strong momentum. Therefore, we have felt that it makes sense to retain the tariffs at the rates they are, again, with the same philosophy that I articulated earlier. So, we continue to believe that our top line growth should be taken in a measured manner.

We are quite happy with the growth that we see in Gurgaon, the mature hospitals actually having grown a little bit higher than the 4% which you see at the consolidated level. So, that are the reasons. Some of these are intentional strategies, and some of them are because of the high growth in volume.

Bino Pathiparampil: Understood. So, it's a very good thing that your growth is primarily driven by volume. It shows the quality.

But at the same time, at an industry level, I was just asking for the purpose of understanding. When the insurance negotiations happen and all the other facilities around you take a 10% or 15% increase, wouldn't those kinds of increases apply to you as well from the same insurance companies or CGHS, etc.? I mean, how does the industry practice work?

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Pankaj Sahni: Coupl

e of points to clarify for your understanding. I think the first thing to understand is that there are actually different payer categories, and the tariffs apply differently to those payer categories across the industry. First of all, when it comes to payer categories like CGHS, ECHS, you will see from our presentation that our revenue mix for CGHS, ECHS, and these kind of scheme patients is quite low.

Only about 10%, 12% comes from CGHS patients, about 9%, in fact, actually, we'll see on the investor presentation. So, it is not a very big part of our overall sales mix. That being said, in the industry, when CGHS tariffs increase, they increase for the industry as a whole, although there are some small deviations in rates depending on which state you operate in. That is how the industry tariffs move for CGHS.

As far as the insurance companies go, both the nationalized insurance companies as well as the private insurance companies, these companies engage with commercial negotiations with each hospital or hospital group independently. In some cases, the hospital group may have the same negotiation rates across all of the units.

And in some cases, these may be different for different units depending on individual strategies of hospital groups. So, those are far more commercial negotiations. And very typically, they would take some form of a baseline with respect to the cash tariff, which is published by the hospital.

And then there is a discussion around how that moves with respect to the cash tariff. This is very often how the negotiations would move in the insurance. And then, of course, each hospital company or each hospital unit decides what its tariff strategy should be for the particular hospital in the market where they operate.

As we have said, again, many times, in our case, just because we can take tariff increases doesn't mean we should take tariff increases. And we continue to maintain this approach with all of our assets. So, we do take tariff increases from time to time to ensure that we are able to manage the inflation, manage the input cost as well. But we try to do it in as reasonable a manner not so as not to create excessive burden on the patient population that we serve. I don't know if that answers your question both on the industry as well as our philosophy.

Bino Pathiparampil: Yes, yes, that answers. Thank you. Thank you very much.

Moderator: The next question is from the line of Kunal Damesha from Macquarie. Please go ahead.

Kunal Damesha: Thank you for the opportunity. On this retrofitting of a floor for medical oncology, I'm sure rationale would be demand for that service. But is there any meaningful difference in terms of profitability, return on investment that we would be doing here versus some of the other specialties?

Pankaj Sahni: I think the important point to note first before we get into the return on investment or profitability is that the medical oncology, specifically the chemotherapy, typically functions in two types or maybe three types of care. The first, of course, is where a patient, because of their particular

disease or their particular medical condition, sometimes there may be multiple comorbidities, requires admission and overnight stay, whether for one day or multiple days, depending on the medical condition of the patient. So, that typically gets treated in the inpatient care and the patient stays overnight.

The second type of chemotherapy that is typically administered is what we call chemotherapy daycare, which happens across many hospitals around the world. And these are typical chemotherapy, which is medicine infusions into the patient, which has to be done in a hospital setting, but doesn't necessarily require the patient to stay overnight. Typically, we see these infusions are of two types.

Some are shorter in duration.

So, the medicine is delivered over maybe a one hour, two hour or three hour period. And some of these infusions take a little bit longer, maybe four to six hours. So, when it comes to how the medicine is administered, and of course, these are decisions which the doctors take individually based on each patient, you may find that a particular daycare bed may get utilized twice or thrice in a day, because if you're only required to administer the medicine over a course of, say, three hours, then that bed could, you know, in a nine hour period be utilized thrice or in an eight hour period be utilized twice.

So, it really depends on what kind of disease the patient has and how that bed gets utilized. What we have found, and again, in line with our stated strategy many times is that if you look at the way in which healthcare gets delivered, there is no reason to keep a patient in hospital if the same treatment can safely and effectively be delivered on daycare basis. So, when you look at our length of stay, which across the industry is quite low, this has been in part an active strategy to try to get patients back home as soon as possible. We realized that there was no real need to keep patients overnight in certain medical oncology procedures, and that is why we took the decision to actually convert a complete inpatient ward into a chemotherapy daycare ward. Does that address your question?

Kunal Damesha: Yes, on the business side, you know, how things operate here, but, some of you on the financial side, how kind of, you know, it helps us because I believe one of our peers has a very strong oncology contribution, you know, and that's been growing really fast as well as I think it also has a favorable profitability profile. So, is that the similar kind of trend that you are also witnessing?

Pankaj Sahni: Yes, so two points I think in your question, let me try to split them out. The first is with respect to the profitability or the financial economics on medical oncology. You know, obviously a large part of the treatment in medical oncology is the actual chemotherapy drugs that are administered, and of course the clinical inputs given by the medical oncologists and cancer experts towards that. Also important to understand that typically cancer doesn't appear as one disease. There may be multiple situations with the patient associated with various comorbidities, etc., associated. So, the large part of the patient's bill in medical oncology is driven by the cost of the pharmaceutical chemotherapy drugs that are administered, and therefore the room rent kind of component is much lower because the patient doesn't need to stay for a long period of time.

So, the economics is largely driven by the pharmaceutical consumables and the clinical input of the doctors. As far as your second question, which pertains to the contribution of cancer in general. See, it is important to understand each hospital looks at cancer slightly differently. If you look at our sales mix and also what was articulated in our various investor presentations over the quarters, we view cancer as a disease management group basis. We don't actually record all of our cancer care under the cancer tagline which you are seeing in our sales mix which is presented in the investor presentation. We actually show much of our surgical oncology in the various different aspects.

For example, our uro-cancer, which is a prostate cancer and kidney cancer, that will be reflected in our kidney and urology institute. Our neuro-cancer, brain tumors, etc., would be reflected in our neurosciences institute. And our stomach cancer, GI cancer, colon cancer, pancreatic cancer, that will be reflected in our digestive institute.

And our lung cancer, etc., will be reflected in our chest surgery institute. So, when you see cancer, and it's also incl

uded in the footnote, it really only includes medical oncology, radiation oncology, head and neck surgery, and breast surgery. All other surgical cancers are actually split out.

So, if you look at our cancer contribution vis-a-vis most other big hospital groups as a percentage of the total revenue, it's more or less exactly the same. And that obviously makes sense because the incidence of disease occurred for all the hospitals the same way as in the country. So, I would assume most groups would have the same situation. It just depends on how you classify.

Kunal Damesha: Sure, Great. Thank you on that detailed answer. And second one on the payer mix, now we have shown pretty strong growth. Our occupancy levels are also around 64%. At any point in time, are we planning to refine payer mix? Maybe one year down the line, it could come into play for us. Is there any plan there?

Pankaj Sahni: I think two parts again to this question. The first is it's important to understand what's happening in the industry as a whole. We do see, especially post-COVID, some momentum towards increased insurance penetration on the healthcare insurance side across the country.

So, definitely our expectation is that over the next several years, I don't know whether that will be next year or next three years or next five years, but we do expect insurance penetration to increase. And therefore, the natural assumption would be that the insurance share of business for healthcare would increase. And maybe that would be at the expense of some of the cash patients.

Now, overall, we believe that this is a good thing for the industry. It's a good thing for the country and of course, a good thing for patients. We actively support the increasing insurance penetration and insurance coverage, because obviously we want patients to be secured from what could potentially be devastating financial consequences of you know, sudden illnesses.

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So, I do believe that insurance sales mix will grow over time. As far as the other mix, whether it's our scheme business or some of the other mix that we have, we don't have any active strategy to move in one direction or the other. We believe that it's important to continue to serve as many sections of society as possible.

And that is why we do take patients at slightly lower scheme rates. And we believe that we will continue to support that segment of the society and that segment of our patients. But we do also feel that with the increasing focus of healthcare in India, with the government's focus on medical value added travel, we do believe that as an industry, international business will grow and medical value added travel will grow. And obviously, hospitals will benefit from that increase as well.

Kunal Damesha: Thank you.

Moderator: The next question is from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal: Hi, on the new hospitals in Lucknow and Patna, we've had a very, very strong occupancy this quarter, about 71%. You talked about adding another 100-150 beds in Lucknow. Now, at the base where it is and with the occupancy that we've really got to, how do we see these two hospitals really moving ahead over the next two to three years?

Pankaj Sahni: Let me take both the units slightly separately. Let's first start with Lucknow. Our Lucknow unit has been doing very well and we've had very strong occupancy, especially this quarter. Some of it is also, of course, the hangover of Dengue, which extended quite long this year in Lucknow. But what we do see is over the last several years, I don't know if you've been party to those conversations around how our occupancies have grown. Last year, we had very high demand of critical care beds in Lucknow. So, we focused in on adding beds in the ICU. I think towards the end of financial year '23, we had

added almost 50 or more beds in ICU because the ICUs were overflowing, and that demand continues to remain strong.

This year, what we realized is that while we were able to provide ICU capacity, the demand for the ward beds has increased as well. So, we are actually adding in about 150 beds of ward capacity. We also find that the demand for our mother and child services is growing, both on the critical care side in the NICU and PICU, as well as on the ward side.

So, you know we may be playing a little bit of catch up on demand. We had ICU demand, we built ICU beds, we have ward demand, we'll build ward beds. By the time we are done with this current build out, which we hope to conclude over the next couple of months, we will be almost at about 750 to 800 beds in Lucknow out of the total capacity of about 950 beds.

So, the bulk of the hospital will be complete. And then we will take some decisions based on how this plays out as to how we want to add out the additional beds. The important thing for us, you know, is that we are far more focused on the bed days and the IPD volumes, because occupancy by just very nature of the metric is a moving target. You add capacity, the percentage will fall. You know, you took the capacity, the percentage will increase. We really internally

manage this much more through absolute volumes than the percentage occupancy, which is frankly just a derived number. That's the story in Lucknow.

In Patna, what we've seen is that we continue to add the beds in Patna and we continue to roll that hospital out. We are at about 358 beds right now. There also we have a very strong need for additional ICU beds. We are actually currently in the middle of constructing two ICUs the re, one of which we hope to come on board in the next few weeks and the other one probably in the first quarter of next year. And that should alleviate some of the burden because we are really choked on the ICU in Patna.

We also have already fully utilized our dialysis capabilities there. So, we are adding in some dialysis beds, because again, while it wasn't really planned, the demand has been very high. And we're also building out our chemotherapy ward there to be able to provide the same kind of chemotherapy daycare services as I articulated in one of the earlier questions.

As Dr. Trehan mentioned, we have inaugurated and launched our radiation oncology services in Patna this quarter, which is Q4, a few weeks ago actually. So, the complete suite of cancer services is now available and we do expect cancer work to grow across the board in Patna. So, this is how we are kind of building it out.

I think by the time we are done with this current build out towards, you know, maybe the first quarter of next year, Patna will hit almost about 450 beds or so. And then we will be able to complete one tower of the hospital and we'll have the second tower, which we will then again move towards building out as we move forward.

Moderator: Sorry to interrupt, sir. We have lost the audio from your side. Ladies and gentlemen, the line for the management has been dropped. Please stay connected while we connect them back. Ladies and gentlemen, we have the management line connected back. So, you can go ahead.

Pankaj Sahni: Hi, I'm sorry. I didn't realize. Where did you lose me? Did I complete the Lucknow story?

Nitin Agarwal: Lucknow was completed. You were talking about, you know, after the round of expansion, Patna is done. You'll be probably reaching about 450 odd beds there, 400 odd beds.

Pankaj Sahni: Okay, good. So, not too much you missed. So, you know, this round of expansion, which is currently already underway, we'll probably hit somewhere around 450 beds or so. It is possible we may add some specialties in Patna that is under discussion and we hope to be able to conclude on that b

y year end, which are not yet there.

And that may require some additional beds to be put into the system. But as you're aware, the capacity of the facility is about 650 beds. So, once we get to about 450, maybe even, you know, closer to 500 beds, we'll have a fairly big sized hospital there.

And then we look at the build out of the second tower, which typically will take about six to nine months because we have to put all the services into the second tower, etc etc. And then that will help us with the growth there.

The other thing that I was mentioning, I think when the call dropped is that while it started late, our PPP business from the government of Bihar has now started really only kicking off towards the start of Q3. Now we do see some increase in patients coming in from the government of Bihar. So, we're very happy that we're able to serve those.

So, depending on how that scales up, there may be additional demand for beds in Patna. But once again, as I mentioned earlier, we don't really worry too much about the occupancy percentage, as long as we are seeing good growth in the volumes and the bed days.

Nitin Agarwal: If I can just probably squeeze a little bit on that. On the Patna, obviously, it's a relatively new hospital, you are a relatively long way to go. In Lucknow, I mean, when you take say, three, five years of the business, you know, while I understand you, the focus on metrics like volumes and occupied bed days, but is there enough there, I mean, can the Lucknow with the pace of the demand that you see, can the hospital take on the next three to five years of growth, which you can see in the region for yourself?

Pankaj Sahni: Is your question oriented around do I have enough capacity to meet the growth? Or is the question around is there enough demand?

Nitin Agarwal: I think by the looks of it, capacity is a challenge, would be a challenge rather than demand

Pankaj Sahni: I mean, I would broadly agree with you before. Just if you sit back and reflect on the state of

UP, it is a population of 250 million, and really no high quality healthcare, definitely nothing of the size and quality of Medanta. So, probably we need five, six Medantas to come up in UP and that also will not be sufficient to meet the demand there.

It's very interesting, obviously, because two, three years ago on investor calls and meetings, people used to ask us, why are you going to UP? I think those questions are well answered. And we had firm conviction in the need and our ability to deliver. And you're seeing that.

So, I would agree with you. I think the real issue in Lucknow and UP will be, can you continue to meet this supply? Because the demand for high quality healthcare there of the highest standard with the highest ethics is, of course, very high.

Nitin Agarwal: So, how are you thinking about leveraging this opportunity? Is it -- are you looking to get into more cities in Lucknow? And at what time period does it begin to happen?

Pankaj Sahni: Yes. So, we are definitely looking at opportunities in UP beyond Lucknow. Just to clarify, Noida also technically qualifies as UP, although it's part of NCR. We do believe that there is opportunities in Lucknow area, as well as in Eastern and Western UP both. And we are currently engaging in identifying and driving some opportunities in those regions. Obviously, as you're aware, we can't really disclose where we are until any of these things are definitive. But looking at UP beyond Lucknow and Noida is very much part of the agenda. And that process is actually already underway. We will, of course, relay that information to you as and when it becomes definitive. But I think there's no doubt about the opportunity in this area.

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Nitin Agarwal: Thanks so much. Best

of luck.

Pankaj Sahni: Thank you.

Moderator: Thank you. The next question is from the line of Ankit Shah from Canara Robeco Asset Management Company. Please go ahead.

Ankit Shah: Hi. Thanks for the opportunity. I had a few questions related to Patna. Most of them have been answered. But firstly, I just want to get a sense, with the addition of ICU beds and radiation oncology block, would we see any ARPOB growth from current levels? Because as of now, developing hospitals, ARPOB has been flattish. So any improvement can be expected there?

Pankaj Sahni: So I think two things, right. One, as you very rightly pointed out, the ICU bed will have a higher average revenue per patient and a higher ARPOB just by nature of its services. And the other thing, when we look at ARPOB, which again, important to point out is a derived metric, radiation oncology revenue will be driven largely through daycare.

So the denominator is not getting added, but the numerator in terms of revenue is getting added.

So yes, you could assume that ARPOB could come with radiation oncology and ICU beds increases. That being said, as I mentioned earlier, we also do hope to see an increase in the PPP patient volumes that we have committed to.

And given the tariffs of that are slightly lower than our listed tariffs, so that may have a slightly diluting effect. So it really depends on how this plays out in terms of the mix of all these specialties. And also to re-mention that we haven't taken any tariff increases in Patna at all. Same goes for Lucknow. So the reason your ARPOBs are flat in both those units, despite volume increases, is really because there's been no tariff increase in any of the developing hospitals.

Ankit Shah: And so as a mix of the mixed impact of both these factors, PPP patients plus ARPOB increase, should we expect the Patna margins to remain stable over here or can they increase further as occupancy goes up?

Pankaj Sahni: As I have mentioned in the past, we don't really give guidance on where the margins will go. But let me try to articulate it from this point of view. Obviously, as the hospital is still growing, and given the nature of our model, which I may have mentioned in the past, which is largely full-time doctors, fixed cost kind of a model, obviously, as the volumes grow, you get the benefits of operating leverage and those are margin accretive for the most part.

The second aspect, as you rightly pointed out, is that if you are able to generate revenues on businesses which have lower cost base, now obviously, radiation oncology at a P&L level doesn't have too much material costs and too much manpower cost to revenue, but obviously, there's a huge amount of capex that goes in. So if you look at it only at a P&L level, radiation oncology business is margin accretive. But again, there's obviously a huge amount of capex that has gone in towards the acquisition of the machine. So these are all things which are margin accretive.

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As I mentioned, reduction in the payer mix towards PPP, although I don't anticipate it to be that drastic, you may be aware it's capped at 25% of the beds as per our contract, would have somewhat of a dilution effect on ARPOBs. But since it's volume at lower marginal cost, it may still be beneficial on the margin and the EBITDA. So we feel fairly confident. I think that if I look at the financial performance of Patna Hospital over the last several quarters, and we do anticipate this full year financial results.

Last year, I believe the EBITDA margin in the first year was about 8%. This year, we expect a significant improvement on that for the year. So overall, I would say in a second year of operations to deliver these kind of margins, which we are seeing in Patna, I think we are very, very happy with the performance of Patna.

Ankit Shah: Got

it. My second question pertains to the Noida coming hospital, I understand that around 300 beds are expected to come by FY'25 end. So can you give some color on how this will be scaled up, or what period would EBITDA breakeven be achieved?

And also, considering that most of corporate costs are loaded in the Gurgaon Hospital, can we expect the margins over here to be much closer to the developing hospital versus the mature ones, as they are now at maturity?

Pankaj Sahni: So let me take that question also in two, three different buckets. Obviously, when you look at building out any hospital, especially of this size and scale and the quality that Medanta does, I think that the typical thought process is that hospital like 550 beds that's coming up in Noida, normally one would expect to breakeven in maybe three to four years. And that's typically how we internally also model it out.

That being said, we have proven that we are able to generate a breakeven in Lucknow in one year, in Patna also in the first year itself. Although, as I always tell people, please don't hold us to that just because we have achieved breakthrough performance in Lucknow and Patna doesn't mean that one should expect normally a hospital will breakeven in one year. It is not normal performance.

It is actually quite exceptional performance from that aspect. So we continue to model out our breakeven on a more rational basis. That being said, the Noida Hospital is a little bit different in its construction and layout as many of the other hospitals, which is most of our hospitals are built around two towers of inpatient beds. Noida is one tower. And therefore, our ability to scale up is a little bit faster because of the way in which the construction is done. So while we plan to start out with 300 beds, scaling up thereon is just a question of doing the interiors and fit-outs of one floor at a time.

So we hope that the scale up to 550 could be faster depending on what the demand is. As far as your question on corporate costs and margin profile goes, you're right. Currently, we load all the corporate costs on the margins of the stand-alone Gurgaon, Indore and Ranchi Hospital Company. Also, just to clarify from a financial point of view, the Noida facility will also be part of Global Health Limited, which is the same company as Gurgaon, Ranchi and Indore. So that will be part of that. So there's no real question of taking the corporate overheads, loading to a

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new company. As you're aware, Lucknow and Patna are 100 -percent subsidiaries. So that's one of the reasons for the difference.

But we feel very confident in the market in Noida. We feel very excited about the expectations of the community there. A lot of positive demand from the clinical community to get associated with Medanta Group and Medanta Noida specifically. So all the indications are positive for our opening and build-up. How the margins play out, I think time will tell. And once again, we don't really crystal ball gaze into that. But we feel confident on the broad success of Noida as an institution.

Ankit Shah: Thank you.

Moderator: Thank you. The next question is from the line of Jainil Shah from JM Financial. Please go ahead.

Jainil Shah: Hi. Thank you for the opportunity. Just trying to understand the developing hospital performance better. So we've reported 70 % occupancy and flatter sharp ups. Is it fair to assume that Patna's occupancy is somewhere around 60 % plus and its margins are, you know, have reached 20 % and above?

Pankaj Sahni: So to your question, is it fair to assume? Yes, I think it's fair to assume. Obviously, full results we will declare in the year-end when we publish the independent accounts for both entities. But yes, things are going quite well in Patna, both as far as occupancy goes as well as the margin profile.

But again, just one word of caution on both the units is that, as we add beds, occupancy percentage tends to be a little volatile because obviously you are changing the denominator on a constant basis. So I think slightly more holistically, because again, this business is not run quarterly when you look at it over the course of the full year.

I think we will see good occupancy numbers, good inflation volumes, good bed days, as well as the kind of margin profiles that, you know, you're talking about, which, frankly speaking, is much better than what we would have originally anticipated just in a short two -year period.

Jainil Shah: And we have reported, you know, margins of 25 % plus over the last two quarters. So just want to know your thoughts. Is this sustainable? Are we planning any more price hikes next year?

Because we have a few bed additions and we have Noida coming up. So just want to understand the trajectory going forward?

Pankaj Sahni: Look, like I mentioned earlier, I don't want to be giving out what the EBITDA guidance will be, and especially not quarter -on- quarter. So I think the margins are an outcome of the efforts we put into the work that we do. That being said, I think the smarter way to think about this is to lay out what are the potential accretive and dilutive impacts.

As I mentioned already, with respect to the developing hospitals, the incremental volumes on a largely fixed cost base will provide disproportionate profitability because the marginal cost is lower. Frankly speaking, if you look at our volume growth over the last nine months or so, in

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even our mature facilities, you see good amount of volume growth coming in. And that, of course, again, is getting delivered on an asset base and a cost base that is fully mature and also fixed to a large part.

So obviously that also flows towards the bottom line. So that all ends up being margin accretive. We are conservative as far as tariff increases go. So to the extent that we maintain that conservatism, there may be some input cost increases as we think about doctor costs, as you think about the tendency in the industry for the various elements of input cost to increase. Obviously, there's always this question which people ask us around, how do you think the regulation may impact the pricing? So regulation will obviously have a margin dilution effect. So I think that none of these impacts are very different from what we would have seen last quarter or even the few quarters before that.

So if you step back and look at this slightly more holistically on an annualized basis, I think the trends are very positive as far as the Medanta Group goes. Just because we see good volume growth, we see increased utilization of the hospitals that are built out in Patna and Lucknow. We are seeing very strong volume growth even in a hospital like Gurgaon, which has already been operational for 12 -13 years.

So I think these are all positive. Definitely there will be some initial margin reduction with the ongoing operational costs of Noida as that unit becomes operational. But of course, that will be determined whether it's capitalized as part of pre -operating expenses or whether it gets into the operational expenses.

So I think once the finance accounting of all that is sorted out, you'll be able to see where those numbers fall. But these are, I would say, the levers that will drive the profitability of our group.

Jainil Shah: Sure, that's helpful. Thank you so much.

Moderator: The next question is from the line of Lakshminarayanan KG from Tunga Investments. Please go ahead.

Lakshminarayanan KG: Thank you. I have a couple of questions. The first question is, conceptually, how to think about ARPOB in NCR? Because what puzzles me is that all the leading hospitals continue to add beds and the ARPOB is the highest

in India. And despite the city having the highest number of beds, including leading providers, AIMS, etc . I just want to understand that question ?

Pankaj Sahni: I think various different elements there. One, of course, let me first just touch upon my perspective on the NCR area as a whole. Obviously, you're aware that the demand and the population is still growing in NCR and is still hugely short of the supply and capacity required from a bed to population ratio point of view, including vis -a-vis some of the other major cities in the country.

So from that point of view, Delhi NCR is still significantly undersupplied from a hospital bed point of view. The city is continuing to grow and therefore, that problem is only likely to

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get worse. And also Delhi, just because of its nature, drains a lot of patients, not only from other parts of the country, but also internationally. So I think that when you look at the demand supply mismatch in Delhi, as far as infrastructure capacity goes, of course, not to mention doctors is still fairly large. So I don't see any concerns with additional capacity coming on board. I think that will continue to be needed.

Now, as far as the ARPOBs and tariffs go, look, Medanta as a group has always maintained that we do not want to be the most expensive in town. We want to maintain a reasonable level of tariffs to be as affordable as possible, given the context of the quality of healthcare, the cost of infrastructure and the cost of the inputs that we provide. So we don't aspire to charge more just because it's possible to charge more. We try to charge as reasonably as possible. That may or may not be the approach of others in the industry. That's our approach.

And therefore, we always view our tariffs in these lines. The other thing that I mentioned earlier is look, ARPOB a derived metric. If your average length of stay will fall, your ARPOBs will rise. If your average length of stay will increase, your ARPOBs will fall. So we really much more view this on average revenue per patient basis than ARPOB, because depending on your sales mix, depending on your patient mix and the complexity, the length of stay, etc, ARPOB's tends to move around. So we don't obsess on ARPOBs growth.

I think for us, the real focus is, are we able to serve the patients that come in and are we able to continue to see growth in that dimension?

Lakshminarayanan KG: Got it. And second, your inpatient revenues, does it include the inpatient pharmacy charges also?

Pankaj Sahni: Yes, it does. So all the charges that the patient incurs while admitted in the hospital are part of inpatient revenue. The outpatient pharmacy business that I mentioned in the earlier remarks is purely the OPD pharmacies, which are the retail pharmacies. For the most part, they are the ones which are in the various hospital lobbies, etc, and one or two outside hospitals as well.

Lakshminarayanan KG: No. What kind of profit margin for the gross profit you make on either inpatient or outpatient pharma?

Pankaj Sahni: We don't disclose gross profit by category of revenue stream. And frankly, neither do we disclose gross profit across the board. I think it's important for you to understand also that healthcare delivery is not about just giving medicines or just having a doctor or just doing a surgery.

It's far more complex than that and it consists of a very complicated, integrated level of care. In Medanta, we also believe that care is not delivered by one doctor alone. It is the team that delivers care. And that team comes together comprehensively to deliver care. So neither do we worry too much about individual line items in the bill, and neither do we report out the economics in that front.

Lakshminarayanan KG: And the last question from my side is, when you said profitability, do you actually have every

hospital is taken as a different PnL? And because you mentioned some of that earlier, that the corporate overheads get slowed, and I just want to understand that part.

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Pankaj Sahni: Go ahead.

Lakshminarayanan KG: Yes. So, and also I just want to understand, how do you drive procurement efficiency as you increase the footprint?

Pankaj Sahni: Okay. So two different questions, I think. The first one is that, as mentioned in our, right from our IPO days, the Medanta group is structured as the Global Health Limited, which is the hospital, which is the listed entity, as well as the company which houses Indore, Ranchi and Gurgaon. And our accounts are disclosed as such. And that includes all the corporate overheads that you were mentioning. Lucknow is 100% subsidiary in the company called MHPL.

And Patna is 100% subsidiary in a company called GHPPL. So, at the end of the year, you will see three sets of accounts being filed with the ROC. And that is how we disclose our financials on across the board.

So, we of course have many different ways that we manage the company internally. But this is what we disclose out across the board to the public as well as to the authorities and the exchanges. As far as procurement efficiencies, I think that's a far deeper question.

But obviously, there are all the basics, which is economies of scale, ensuring that you are actually able to optimize your procurement across the group rather than looking at it individually, which is how I think most hospital groups would do it. And also ensuring that you are able to use that to get the best possible rates from the suppliers. That being said, it is not the cheapest only. Medanta has a very, very high bar on quality. And so, there are certain inputs and certain manufacturers that would not qualify for procurement in us, even though they may be cheaper as far as the rates go. So, we take a very high focus on quality of inputs that we utilize.

Lakshminarayanan KG: Sorry, I just want to squeeze in one more question if it's possible. May I?

Pankaj Sahni: Yes.

Lakshminarayanan KG: Yes. So, what I understand is Q3 is slightly lower in terms of utilization for the industry, given the festival period coming in. So, I want to understand at an overall level, where does your hospital utilization gets?

It's very difficult. That is, you cannot inch beyond 75% or 80%. What's the band you actually look at it? if you just look at utilization over a period of time, and what kind of de-bottlenecking you normally do?

Pankaj Sahni: Look, we don't run this company or view the business optimization quarterly. It just so happens that we are required to report our quarterly results. So, absolutely nothing that we do in the company runs the business on a quarterly basis.

We are far more long term in our orientation, and far more focused on ensuring that we deliver a high standard of care. And we believe that if you do that, the numbers will kind of speak for

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themselves. So, this is not the kind of organization or in our opinion, the kind of industry, where you're optimizing things on a quarterly basis, trying to produce good results, quarter on quarter. So, the numbers are what they are based on how we run the business. And our personal belief is that at a minimum, you should be looking at this on an annualized basis, if not even on a longer term period. And that's how we think about the business.

Obviously, on a daily basis, we look at where the inefficiencies are that can be kicked out. And what are the improvement opportunities that we can bring in. But like I said, none of this is at the cost of the quality baseline and benchmarks that we have.

Lakshminarayanan KG: And actually, the quality goes down with what point in

utilization, it actually ...

Pankaj Sahni: It doesn't work like that, actually. Because when you look at utilization, again, at a very macro level, it's too difficult to say. Quality differs in daycare, quality differs in OPD, quality differs in the IPD. So, to really understand this, I don't think a one hour conference call will be sufficient, you really have to get into how a hospital functions.

So, quality utilization, I can't tell you that there's a particular number where quality will fall, totally differs depending on what's the nature of the specialty and the nature of the institution. And frankly speaking, what your baseline and benchmark is for you to define quality. So, I don't think there's an answer I'll be able to give you on that front.

Lakshminarayanan KG: Thank you so much. This is very helpful.

Pankaj Sahni: Thank you.

Moderator: Thank you. Thank you. As there are no further questions, I would now like to hand the conference over to the management for closing comments. Sir, you have any closing remarks?

Pankaj Sahni: Thank you everybody for participation in this call. And just to reiterate, we remain committed to our outline strategy. And I hope you will agree that we have been consistently reverting back to you that we are working on the outlined strategy, which we laid out several quarters ago. And we will continue to maintain that focus on delivering the highest standard of care with the highest standard of ethics and values. And we thank you for your support and participation.

Moderator: Thank you. On behalf of Medanta Global Health, that concludes this conference. Thank you for joining us. You may now disconnect your line.

Notes:

1. This transcript has been edited for readability and does not purport to be a verbatim record of the proceedings
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Call: May 2024

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Sub: Disclosure under Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 – Earnings Conference Call Transcript

Dear Sir (s),

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith, the transcript of the Earnings Conference Call held on Monday May 20, 2024, hosted by JM Financial, on the Financial Results for the Fourth Quarter and Financial Year ended March 31, 2024. The Transcript is also available at our website: <https://www.medanta.org/investor-relation>.

This is for your information and record.

For Global Health Limited

Rahul Ranjan
Company Secretary & Compliance Officer
M. No. A17035

Encl: a/a

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DIRECTOR –MEDANTA – GLOBAL HEALTH LIMITED

MR. PANKAJ SAHNI – GROUP CHIEF EXECUTIVE
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MODERATOR : MR. AMEY CHALKE – JM FINANCIAL

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Moderator: Ladies and gentlemen, good day and welcome to Medanta Global Health Q4 and FY24 Earnings Conference Call hosted by JM Financial. As a reminder, all participant lines will be in the listen - only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on a touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Amey Chalke from JM Financial. Thank you and over to you, sir.

Amey Chalke: Thank you, Zico. Good afternoon and very warm welcome to all the participants on the Global Health Limited 4Q FY24 Earnings Call hosted by JM Financial. Joining with us today from the management side, we have Dr. Naresh Trehan, Chairman and Managing Director, Mr. Pankaj Sahni, Group CEO and Director, Mr. Yogesh Kumar Gupta, CFO, and Mr. Ravi Gothwal, Head, Investor Relations. Over to you, Naresh, sir.

Dr. Naresh Trehan: Good afternoon to all of you who have joined us for the Medanta Q4 and full year FY24 Earnings Conference Call. I'm pleased to inform you all that for the fifth year in a row, Medanta has been recognized as "India's best private hospital" and also it is one of the only hospitals from India which is in the first 200 hospitals of the world. Now, this is fifth year in a row, so it's really a testimony to our consistent care, quality, and what we call Medanta model of care.

With that, let me just walk you through what is happening as far as our commitment to excellence and the growth of our medical services and also the standards that we are raising continuously. Medanta Gurugram became the first in India to use advanced AI-enabled Penumbra Lightening technology for treating pulmonary embolism. This is a technology which has replaced the acute need for major operation to extract clots from the lungs which used to carry a lot of risk.

But with this device, which is AI-enabled, we can actually suck out the clots and save people's lives and many lives have been saved which otherwise would have had to undergo this risky operation. Further, Medanta piloted the Bharat program helped by a specialized team which is dedicated to raising awareness about reducing the incidence of pulmonary embolism in the Indian scenario and also enhancing the knowledge around the world.

Our doctors also pioneered in using non-invasive mitral valve repair technology which actually come under the heading of structural heart and India now actually has caught up with the rest of the world. Medanta is at the leading edge of all structural heart procedures which include TAVI, and other complicated procedures. All that is happening here and we are the leaders in this part of the world. Also, we have made major strides in robotic surgery. All procedures now are routinely done, robotically and with the least amount of intervention for the patient and increasing the safety.

One of the major things that we have accomplished which is routine now is that the robotic donor surgery for liver transplant. This has actually reduced huge amount of pain and morbidity on the

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patients who are donating livers for their relatives. So, these are major advances that have taken place in the last year.

Digital innovation is key enabler in bridging the accessibility gap in healthcare services especially in underserved Tier 3 and Tier 4 towns. Last year, we had launched Medanta e-ICU program with setting up command centers at Lucknow to enhance the access to critical care in underserved areas. During the year, Medanta ICU saved lives of many patients including a 16-year-old boy, patient with critical conditions and non-recordable blood pressures admitted to the

hospital in Prayagraj. Utilizing Medanta e-ICU, our intensivist team managed to patient until stabilization then swiftly transferred him to Medanta hospital. The patient was successfully treated and discharged in good health. Overall, we continue to focus on delivering higher standard of clinical care and as a result Medanta consistently delivers strong financial performances year-on-year.

Now, I will hand over the call to Mr. Pankaj Sahni, our Group CEO, to walk you through the performance during the year and the quarter. Thank you for your patience and joining the call. Pankaj, over to you please.

Pankaj Sahni: Thank you, Dr. Trehan. Good afternoon and welcome everyone to our Q4 and full year FY24 earnings call. Let me take you through the key highlights and annual performance for the year. After that, I will hand you over to our CFO Mr. Yogesh who will discuss the quarterly performance.

Financial year FY24 was another strong year for Medanta with consistent execution and sustained growth across all revenue and profitability metrics. During the year, Medanta delivered a total consolidated income of INR 33,498 million, a strong growth of 21% year on year. EBITDA was INR 8,737 million compared to INR 6,771 million in FY23, registering a growth of 29% year-on-year.

EBITDA margins have also improved by 154 basis points from 24.5% in FY23 to 26.1% in FY24. Profit after tax for the period was INR 4,781 million, an impressive year on year growth of 47% y-o-y. Our average occupied bed days for the year increased by 14% year-on-year, representing an occupancy of 62% on increased bed capacity. ARPOB increased 5% year on year to INR 61,890 driven by a change in case mix. Inpatient count increased by 15% year on year to 1.6 lakhs.

During FY24, we incurred capex of INR 3,035 million, out of which INR 995 million was incurred towards Noida Hospital. In line with our growth strategy, we added 126 new beds. Of these, 98 beds were added in Lucknow in the last week of March 2024 and 28 beds were added in Patna hospital. Additionally, four new operating theatres were made operational in Patna hospital to meet the increasing demand for surgical procedures and critical care.

In FY24, we added over 150 senior clinicians across all our units, further enhancing our medical expertise and ensuring the delivery of superior healthcare services to our patients. We have also launched comprehensive oncology care services with the operationalization of the most advanced radiation oncology machine known as the Varian Edge at both Lucknow and Patna.

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Our patients will benefit from complete comprehensive cancer care under one roof in both of these units.

Medanta Labs created a network of 8 new labs and over 120 collection centres set up since its launch in January 2023. Also, from a balance sheet perspective, we closed the year with a net cash surplus of INR 7,720 million, providing us with a strong financial foundation to support our future growth initiatives and support our growth trajectory.

Our developing hospitals, which includes Lucknow and Patna, registered a strong revenue growth of INR 9,948, which is a 34% year-on-year growth. The overall revenue share from these hospitals increased from 27% of our consolidated revenue in FY2023 to 30% in FY2024.

The EBITDA contribution from these hospitals rose 32% in FY23 to 37% in FY24, reaching INR 3,208 million, a robust growth of 49% year-on-year. The occupied bed days at our developing units increased by 31% year-on-year, with an improved occupancy rate of 61% on an increased bed capacity, reflecting strong growth driven primarily by increased patient volumes. This growth clearly indicates the successful ramp up of operations and the increasing trust and preference of patients in our quality healthcare services in these regions.

Now coming to our mature

hospitals, our mature hospitals have also contributed significantly to the overall growth. Mature hospitals have registered a year-on-year growth of 18%, amounting to INR 23,829 million. EBITDA grew by 29% year-over-year reaching INR 6,002 million. This growth is driven by consistent efforts of strengthening the core through clinical talent additions in our existing units.

During FY24, revenue from international patients increased by 24% year-on-year to INR 1,935 million which contributes approximately 6% of our overall revenue. Our in-house outpatient pharmacy business continues to register strong growth with revenue increasing by 32% from INR 850 million in FY23 to INR 1,120 million in FY24.

Now let's take a closer look at the performance trends over the past several years from FY20 to FY24 which is important to illustrate the consistent strong profitable growth Medanta has been delivering and this growth is backed on strong fundamentals across the network. Our total income has had a compounded annual growth rate (CAGR) of 22% while EBITDA has grown by 40% CAGR from FY20 to FY24.

This robust growth is a clear indication of our successful strategy and our ability to scale operations effectively driven in large part by volume growth across all units indicating strong demand for our high-quality comprehensive healthcare services. During the period, our EBITDA

has grown at a CAGR of 40% with EBITDA margins improving from 15.1% to 26.1%. This underscores our portfolio strength and focus on operational efficiency and ability to drive profitability while scaling our operations.

A quick update on our upcoming projects : The construction of our Noida Hospital, a state-of-the-art facility designed for optimal patient care is progressing well. As at 31st of March 2024, 95% of the superstructure is complete and the MEP work is in progress. As of today, I am proud

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to tell you that our complete roof has been cast and the finishing work is underway across the hospital. Our hospital is expected to commence operations in the end of Q4 FY25/ Q1FY 26. Our asset light hospital partnership in Indore has moved forward with a favourable judgement from the Madhya Pradesh high court and we are awaiting final clearances to proceed further. Our super-specialty 400-bed hospital in South Delhi in partnership with DLF is progressing. As you would be aware already, a subsidiary company has been formed and regulatory approvals are in process.

Before handing over the call to our CFO Mr. Yogesh to discuss the financial performance for the quarter, I would like to highlight that the Q4 FY24 performance was impacted due to a little subdued performance in our Lucknow unit given a seasonally weak quarter. While other units, including our mature hospitals have continued to deliver very strong year-on-year growth and moderate sequential growth. We are very confident in our performance across all of the units and continue to invest heavily in each one of them. Let me now hand over the call to Mr. Yogesh to share with you the performance for the quarter.

Yogesh Gupta: Thank you, Pankaj and good afternoon to everyone. Now I would like to discuss consolidated financial and operational performance for the quarter. During the quarter, Medanta has delivered a total income of INR 8,361 million registering a growth of 14% compared to the same quarter last year. EBITDA for the quarter was INR 2,068 million an increase of 8% year-on-year basis with the EBITDA margin of 24.7%.

Our profit after tax for the period was INR 1,273 million registering a growth of 26% year-over-year. During the quarter, 98 beds were added at Lucknow in the last week of March 2024. Occupied bed days have increased by 9.5% year-over-year representing an occupancy of 59% on increased bed capacity. ARPO B grew by 4% to 63,063

in Q4 FY24. Inpatient counts

increased by 10% year-over-year and outpatient counts increased by 11% year-over-year. During the quarter, international patient revenue increased by 30% to INR 477 million driven by increased volume in international patient admission.

Now coming to the developing hospitals : In terms of developing hospitals that is Lucknow and Patna total income increased from INR 2,025 million to INR 2,265 million compared to the same period last year a growth of 12% year-over-year. EBITDA of the developing hospital was INR 622 million with EBITDA margin of 27.5%. Average occupied bed days increased by 10% year-over-year representing an occupancy of 53% on increased bed capacity. ARPO B grew by 3% year-on-year to INR 57,696.

Now coming to the matured hospitals, total income of INR 6,112 million growth of 15% year-over-year. EBITDA of INR 1,548 million growth of 18% year-over-year. EBITDA margins were 25.3% compared to 24.7% in the same period last year. Average occupied bed days increased by 9% representing an occupancy of 63%. ARPO B grew by 4% to INR 65,746.

With this, I request operator to open the line for questions. Thank you everyone.

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Moderator: Thank you very much. We will now begin the question and answer session. The first question is from the line of Amey Chalke from JM Financial. Please go ahead.

Amey Chalke: Till the time the queue assembles I will ask the first two questions. Basically the lower margin for this quarter, is it solely led by the lower occupancy in Lucknow or developing hospital or you want to call out any cost related items which could have led to the lower margin during this quarter?

Pankaj Sahni: So, let me quickly address that. There is no significant change at all in any of the cost structures across any of the units in terms of things like the cost-to-revenue ratios and some of the other traditional metrics that get evaluated. You are of course aware that we have a fixed cost model when it comes to much of our employee costs and so as you very rightly pointed out, if the

volumes or the total top line changes there is a potential impact on the bottom line.

But there has been no fundamental change in our cost structure. That being said, when we look at our individual units as well as the mature and developing units which we do report out we do believe that the EBITDA levels on a percentage basis are still very strong and in many cases, exceptionally high.

Amey Chalke: Sure. So, as we see the occupancy recovering in coming quarters we could see the margins coming up?

Pankaj Sahni: Obviously, as the occupancies grow the margins will rise. Once again as I mentioned because of our fixed cost model the occupancy growth has maybe even a disproportionate impact on margin growth. So, yes, you are right and we are very confident with our Lucknow asset as you may be alluding to. If you notice we have already invested in additional approximately 100 beds.

We are also building another ward of about 50 beds which will come on board in the first quarter of this year. So we remain fully invested. We have full clinical teams there. We do expect and we do see recovery coming up there and as I mentioned many times we do not look at this business month on month or quarter on quarter. We have a slightly longer horizon. I understand we have to report out statutorily quarter numbers, but that is not how we run the business. So, yes, we are very confident and remain so.

Amey Chalke: Sure. And second question I have related to the Insurance price revision, which is expected this year. Some of the hospitals have said that it could be in the range of 15% to 20% which comes maybe once in 3 - 4 years. So, how are we placed with respect to that, how much impact we expect to have on our business on account of that?

Pankaj Sahni:

Every insurance company that we interact with we have negotiations and interactions with them on the various rates and tariffs that are there. You are right that we have over the course of the last 12 months or so been undergoing negotiations with many of the providers of insurance and we have been successfully able to close many of those insurance negotiations for price revisions or bringing the prices up to 2023 and 2024 rates and that has been ongoing over the course of the last several months.

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That being said, it is important to point out and if you actually look at the growth not only last year, but frankly speaking for the last several years. Medanta has always tried to drive the growth more oriented around volumes than tariff. We have not been very aggressive with tariff hikes even this year we have taken a tariff hike only in Gurgaon, no other unit and that also has a fairly negligible impact on the absolute revenue growth.

So, a lot of the growth is driven by extremely strong fundamentals on volume. So, yes, we will negotiate with each of the providers. I am not able to tell you, of course, what each of the negotiations leads to, but there will be some positive movement on tariff, but important for you to understand that much of the growth fundamentally is volume linked not tariff. Tariff over FY23 and FY24 has had a very negligible impact on the absolute revenue growth.

Amey Chalke: Sure. Thank you so much. I will join back the queue.

Moderator: Thank you. The next question is from the line of Dhara Patwa from S MIFS Limited. Please go ahead.

Dhara Patwa: Thank you for the opportunity. Sir, my question was regarding Noida Hospital. So, it is expected to get operationalized somewhere in Q4 FY25 or Q1 FY26. So, what will be the operating cost we might incur in FY26?

Pankaj Sahni: We do not give future projections of operating costs or any financials for that matter, but if you look at the historical facts across hospitals that have been operational. Obviously in normal course it takes a couple of years for these facilities to get ramped up. I may refer you back to our history and show you that Medanta both in Patna as well as in Lucknow in recent times has been able to achieve breakeven in a very short period of time.

However, I do not think it would be fair for me to assume that that will happen on a consistent basis going forward. So, you should assume that, yes, there would be some ramp up which will come in the first year of operations. Obviously, there will be some amount of scale up, some amount of initial costs that get incurred, but that being said, as I mentioned before as well, we remain very confident on the Noida region.

There is a lot of excitement already both with the clinical community that would like to join us as well as we understand with the patient community from that area. So, to the extent that we operationalize this, we do plan in our internal activities for some amount of EBITDA losses in the first few years, but hopefully, we will be able to scale these also as soon as possible.

Dhara Patwa: Just from the modeling perspective will that be fine to assume that whatever cost we have in FY24 below the gross margin divided by your operational beds in Noida, your total operational

beds and whatever cost we get will that be fair to assume that Noida Hospital will have a similar level of cost as we have in Medanta currently?

Pankaj Sahni: I think it will be a little difficult for me to assist you. with your modelling over this call. However, what I may suggest, if you have detailed questions, maybe I can refer you to our Investor Relations team. And to the extent allowable and possible, we'll be able to guide you on the

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specifics. But I don't think we'll be able

to give you formulas of what to project in your model just on this call, given the queue of questions.

Dhara Patwa: So, next question was on Lucknow as a region. So, even other competitors are looking out to expand in Lucknow. And they're adding up a good, you know, 500 beds - 1000 beds kind going forward. So, how do you see this competition going forward? Or do you think that the market has enough potential for every player to grow further?

Pankaj Sahni: My belief and our belief in Medanta is that not only Lucknow, but pretty much every region in the country has a huge underserved demand. Therefore, I firmly believe that if you take all the capacity that people are claiming to put up over the next few years, and maybe even more, there is still plenty of opportunities for healthcare providers just because there's such an acute shortage, and even more so an acute shortage of the high quality healthcare that Medanta is known to provide. So, I'm not concerned about additional capacity enhancement in Lucknow or any other region.

In fact, Medanta has always been a trendsetter. You know, similar questions, maybe a couple of years ago, people used to ask, why are you going to Lucknow? Or why are you going to Patna? And very clearly, we have seen the success and very clearly that success has been followed by others. So, they always say that, imitation is the best form of flattery. So, to that extent, I guess we can feel flattered on our performance.

I also wanted to point out that as of yet, there is no additional capacity expansion. So, all that is happening in many of the places is that the ownership of one entity is changing hands from one party to another, that doesn't actually translate into additional capacity. In our particular case, we have not seen any impact of any changes on any of the ownerships or any additional impact of competition so far.

Moderator: Thank you. The next question is from the line of Rishabh Tiwari from Elagro Capital. Please go ahead.

Rishabh Tiwari: Hi, I have two questions. I'm not sure if these were answered earlier in the call. First one is why do we see a decline in the margin in this quarter? Is this related to seasonality purely or there were some other factors that this can be attributed to?

Pankaj Sahni: Yes. So, we had actually addressed this earlier in the call, as I mentioned, and I had detailed out that there has been a bit of a volume drop in our Lucknow unit in the fourth quarter. And that has resulted in what you see as a sequential margin decline from Q3 FY24 to Q4 FY24. That being said, we believe that the margins which are actually reflected on a full year basis as well as on a quarter basis for our Lucknow as well as all our units actually are still exceptionally high. And we continue to have very strong confidence in the performance across all these units.

Rishabh Tiwari: Okay. My second question is regarding our upcoming unit with the partnership at DLF. We recently came across a news article about a couple of litigations regarding the land for the same. So, do we see any problems or how does the management view the partnership and the plan?

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Pankaj Sahni: So, to the best of our knowledge and our partner's knowledge, there is actually no litigation that is currently being filed by anybody. Therefore, maybe we should be cautious on what we read in the media. It's not always accurate information. We are 100% committed to our partnership with the DLF group, as I believe they are with us. And we are 100% committed to the construction and building of this hospital after all due statutory approvals and processes are in place. So, there is absolutely no change in anything with respect to this particular hospital site or this partnership.

Moderator: Thank you. Our next question is from the line of Tushar Manudhane from Motilal Oswal

Financial Services. Please go ahead.

Tushar Manudhane: Sir, just on capex, if you could share for this year, FY25 and FY 26, which projects and what quantum of capex will be incurring? I guess because Noida is largely done, DLF might take some time. So, if you could share some thoughts on capex first ?

Pankaj Sahni: Yes. So, let me hand that over to Yogesh. He can share with you some of the thoughts on it and then I can give you a bit of the qualitative flavor.

Yogesh Gupta: Noida capex is still undergoing, we have just completed the superstructure. Rest of the capex on Noida is yet to be done. We have already given guidance that the overall capex over next two to three years will be in the range of INR 1,000 crores to INR 1,200 crores. If you need any unit wise breakup, maybe we can take this information to you later on.

Tushar Manudhane: Got it, sir. And secondly, sir, given that directionally now that almost 98 beds already added in Lucknow, another 47 beds to be added in Q1, FY25. So, directionally, we will have some increase in operational cost or the current four h quarter number already reflects the additional cost on because of these beds ?

Pankaj Sahni: So, Tushar, let me answer that in two parts. I think the question which you are getting to, which is fundamentally this capacity expansion does not require significant cost increases because of the fact that we already have the teams in place to be able to service the patients in these wards. So, there are three new floors which we have built out, which are all in the ward category, not the ICU category.

Therefore, no very significant increase in manpower or any fundamental operating structure is required for the purposes of this incremental bed capacity. So, that is, I think, the main part of your question. However, just to give you some more clarity, we do continue to invest in senior clinical talent in Lucknow. And that is not really linked to these particular beds, but that is in general. So, we are currently hiring as we speak. We intend to continue to hire clinical talent over the course of the full year. And we remain committed to driving not only senior clinical talent, but also new and innovative clinical talent. So, things like robotic expertise, things like minimally invasive surgery expertise. So, we will be adding in clinical talent in also niche areas based on our philosophy and strategy which we have adopted across the group. So, the cost which may come may not be linked to the beds per se, but linked to the already stated strategy.

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Tushar Manudhane: Understood. So, but despite that, directionally, the profitability should be improving because this number of beds will clearly then have an operating leverage. Is that the right way to understand?

Pankaj Sahni: Yes. So, you are right. As we add beds because of our model and our system, operating leverage kicks in. Obviously, you know, depending on which metric you're looking at, whether it's occupancy or other things, things may, you know, move around because we are still not in that situation like in, say, Gurgaon, where you are fully stable, it's still a growing unit, still adding in capacity.

But otherwise, like I said earlier, we are very confident in how we continue to move on investing in this unit. And, you know, over some period of time, like I said, we don't look at it over quarter-on-quarter, over some period of time, this will stabilize towards being a more mature unit and you will see probably then, this the typical mature unit kind of profile from the financials of this unit as well.

Tushar Manudhane: Got you, sir. And now that Indore project, if you could just share in terms of timeline for, let's say, the commercialization or the start of construction and all ?

Pankaj Sahni: So, in Indore, we have or rather our partners have received a positive judgment from the High

Court. We are just waiting for all the final I's to be dotted and T's to be crossed on all the legal aspects, including, if anything, what may happen in any of the potential litigations that may crop up. And once that is done, we will then enter into a definitive agreement, which, of course, we will obviously inform to everybody and then move rapidly on construction. Our partner remains committed and we remain committed. So, as soon as possible, we would like to move forward.

Tushar Manudhane: Got you, sir. And just lastly, on the matured hospital side, we haven't added bed in FY24, I guess. So, but then any scope of adding bed there for, let's say, in FY25 - FY26 or will we be having Lucknow, Noida and Indore to be the key bed driver for next two years and then subsequently DLF project?

Pankaj Sahni: So, in our Gurgaon unit, I may have mentioned this earlier as well. We had one floor, which was our chemo daycare ward, which had actually been under renovation. So, those beds were actually out of commission and out of service. So, while you may not see them on our bed addition, they are now almost, I would say, about 75 % in operation. Still, some more renovation is going on in the remaining 25 %. So, that bed count is back up to complete full strength.

And we actually have in the first quarter of FY25 added in one additional floor, which is our

mother and child obstetrics and gynaecology floor. And that got operationalized just a couple of weeks ago. So, it is not reflected in the FY24 numbers, but you will see that in Q1 FY25. So, a new floor for the service, which we added in last year.

Moderator: Thank you. The next question is from the line of Ankit Shah from Canara Robeco AMC . Please go ahead.

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Ankit Shah : Hi, thanks for the opportunity. So, my question pertains to the Lucknow hospital. If I heard correctly, you mentioned that there was a Y-o-Y decline in occupied bed days. So, can you confirm that and also explain if there's any slowdown in demand or any particular reason for this Y-o-Y decline?

Pankaj Sahni: So, yes, I can confirm that there has been a slight decline in the year-on-year occupied bed days. There are a couple of reasons for this. One, of course, is that we've seen some amount of volume decline because of various aspects of the increase and very severe winter that we had this year in that part of the region.

We've also seen some amount of challenges with respect to how this particular quarter performs vis-a-vis some of the other units which traditionally have a stronger and more robust Q4. So, if you look at across the years, in Lucknow, we do see actually the fourth quarter performing a little bit weaker than, say, even the third quarter. Of course, in the first nine months and even leading up to that, we had very robust performance in Lucknow.

And so, this may be a reflection of increased performance in the past, which you are seeing as a dip. That being said, like I've mentioned, we are very confident on the trajectory of this unit and we do see that we continue to invest, we continue to have the demand there, we continue to have the clinical teams there. So, no very big concerns on our part.

But also important for you to understand that this is not something that we look at just one quarter on one quarter. So, if I give you an example, in Lucknow last year, we saw that at Q2 actually had a very high growth, which is also unexpected to some extent because the nature of the disease profile or the nature of the patients, etc., that came in. So, I think in our particular case, we look at it over a slightly longer time frame.

Ankit Shah : Right. Got it. And the new beds which are being added in Lucknow, any timelines you're looking at for these beds to come to overall hospital level occupancy and margins? Any expectation on that?

Pankaj Sahni: So, that's not really how we actually look at the way in which we practically operate any of our units. So, I don't know if I may have mentioned this in the past, but we operate what we call a cohorting model, where we try to group all the patients of a particular specialty or a particular type of disease profile in a similar manner. And that allows for us to have highly trained nurses, better quality of care, etc. etc. So, we don't look at it to say that, okay, will one particular floor have occupancy of X or occupancy of Y. We actually look at it much more at the unit as a whole and see, are we able to provide the right clinical infrastructure in combination of wards and ICUs to be able to, and of course, the procedure rooms like OTs and cath labs, to be able to deliver that kind of care. And so, we typically group these by that categorization.

So, you will find, for example, in our more mature unit like Gurgaon, we will have dedicated floors for cardiac surgery, dedicated floors for orthopaedics, and so on and so forth. And we then provide those floors to our clinicians and to our clinical teams. And, you know, those get filled in over the time and this is how we actually run the show. So, it is a slightly different way of

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operating from what your question implies. And we believe that this is the right way to ensure high quality of care delivery.

Ankit Shah : Got it. And similarly, some of the brownfield bed addition, any costs that is yet to come, you know, P & L related to new specialties being added, which is still not there in the numbers yet. Anything you want to highlight on that?

Pankaj Sahni: So, I mentioned before, I think it's important for you to understand that the new specialty addition is not linked to beds in, frankly speaking, any of the units, with the exception that last year, we added our mother and child services in and we have created a dedicated ward for that, which came on board in FY25 in our Gurgaon unit. Other than that, specialty additions are not necessarily linked to bed additions. So, bed additions is a larger growth strategy and specialty additions is also a larger growth strategy and they don't have a one to one correlation.

So, we will be adding in specialties and there may be costs associated with those specialties. And we will also be adding in doctors in the existing specialties. This is largely in both Lucknow and Patna, where we are still very much in growth phase in those units.

Moderator: Thank you. The next question is from the line of Madhav from Fidelity . Please go ahead.

Madhav: Hi, good afternoon. Just one question from my side was on the timeline for the unit with the DLF . So, could you give us some sense of the timeline there? Thank you.

Pankaj Sahni: So, unfortunately, we can't give you too many specifics on that because, as you are aware, the first step is to ensure that we get all the appropriate approvals related to construction. Once we do that, I would say, it would take somewhere between three to four years to get all the construction up and running and get the hospital operationalized. So, I would assume at least three to four years in that time frame is what we can expect.

Madhav: And what are the nature of the approvals which are pending for this unit? And three to four years is the construction timeline or you're saying the timeline including the approvals which are expected?

Pankaj Sahni: Well, it depends. I mean, you know, any building and especially hospitals have a standard list of approvals which are required everything from pollution to fire to the sanctioned approvals for actually executing the construction activity. And these are fairly standard for, you know, all construction projects.

Obviously, with elections going on, that has its own set of challenges in getting, you know, approvals through various authorities and agencies. But that's a fairly standard process. Three to four years maybe with approvals or once we get the approvals, we'll, of course, be in a position to communicate to you that the construction activities have started and so on and so forth.

But all the activities are moving forward. So, during this financial year, we have incorporated the company to actually move forward on this. We have partially capitalized that company also. We are moving forward with all the things that can be done in terms of agreements which are

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not government related. So, those activities are on. We will, of course, try to expedite it as much as possible. That is in everybody's interest, especially the patients of that region.

Madhav: Wonderful. Thank you so much.

Moderator: Thank you. The next question is from the line of Alankar Garude from Kotak Institutional Equities . Please go ahead.

Alankar Garude: Hi, sir. Thank you for the opportunity. So, first question, would it be fair to say that the sharp 36% sequential drop in EBITDA for the developing hospitals is only because of Lucknow and Patna would have actually been sluggish or maybe would have grown a bit in terms of EBITDA in this quarter?

Pankaj Sahni: Yes, fair to say.

Alankar Garude: So, sir, then if we have to break down the reasons for this decline, you mentioned three reasons. One is severe winter. Then the second reason was Lucknow has always been a bit weak in the fourth quarter. And the third reason is we had talked about, I mean, in the third quarter, some benefit of dengue. So, that had led to some inflated numbers in the third quarter. Would that be a fair assumption, sir? I mean, any other reason for the sequential drop apart from these things?

Pankaj Sahni: Your reasons are broadly in line with our analysis as well. We have seen no talent attrition at all as on date. We have seen no significant increased activity in terms of the market or the competitive intensity or any kind of behaviour like that. In fact, nothing much has changed. So, I think that, you know, as with all units, things grow through different kinds of cycles.

We look at this a little bit more broadly. One of the things that, you know, we have found is that we do have some amount of constraint in our doctors' capacity. They're already very busy in OPDs and very busy with their operations, etc etc., which is why I was mentioning that we continue to look at adding talent in the existing specialties. But broadly, I think your assumptions are correct.

Alankar Garude: Understood, sir. And maybe one final on that particular line is any particular reason why seasonality is adverse in the fourth quarter in Lucknow compared to any of our other facilities?

Pankaj Sahni: I don't know if that's the right way to look at it. Maybe the better way to look at it is to say that, you know, some of these areas are a little bit more prone to what we call the seasonal or the communicable disease profile. And so, that's why you see, if I look back at, let's say, all of our units, you see a far greater amount of patients for diseases like dengue, etc etc., in Lucknow and Patna than maybe Gurgaon. I'm not a public health expert, so I don't know for sure, but it is possible that in some of these units you actually see a higher amount of patients coming in for some of the diseases which have a little bit of a more seasonal like dengue, etc., and that may have spiked during the Q2, Q3 time frame. We did definitely see a greater number of patients as

a percentage in Lucknow and even Patna for dengue as an example than we did in Gurgaon last year. Now, whether that is because of some particular reasons on how the demographics or how

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the ability to manage these diseases in those territories or whether this will change over time,

I'm not sure, but this could be one of the reasons.

I think also, to be fair, you still have to give it a little amount of time because with our hospital coming up in Lucknow, it was really the first hospital of its kind in that entire area, and you also have to give some amount of time for not only the hospital to stabilize, but also for those communities to understand the kind of care that gets delivered across all the range of super specialties. So, this is, in our opinion, a natural progression of the region with more and more hospitals coming up in the state of UP.

It will also bring an increased awareness of the kind of healthcare that can be delivered, and that will be good for everybody because it also provides greater access and greater availability of complicated services for the people of that region.

Alankar Garude: Fair enough, sir. That's helpful. And maybe one final question, with your permission. Any thoughts on the payer mix in Lucknow and Patna as of FY24 end, and how do you see the payer mix changing in both these facilities over the next couple of years?

Pankaj Sahni: So, let me answer that in two parts. Actually three. First, let's take our Patna facility, which, as you may be aware, is a PPP project with the government of Bihar. We have last year, which is the financial year 2024, started to see the inflow of patients from the PPP part of the business.

Still pretty negligible as far as the overall 25% occupancy, which is part of the contract.

So, still not the complete flow, which we would like to see, but that has started. I think approximately 500 patients have been discharged successfully under the PPP scheme in the Patna unit over the course of FY24, and we really only started to see that flow coming in around middle towards end of Q3 of last year.

We do hope that scales up and we are able to provide those patients with the kind of care that we are known for. So, you may see some pay or mix increase there as depending on how that scales. As far as Lucknow, FY24, no real change in payer mix for the full year. However, we have applied now as part of our stated strategy for availing the Ayushman scheme in cardiac sciences in our Lucknow unit.

And we believe that this is something which is important for us to do, to be able to serve a larger community as well. And this is in line with our doing the Ayushman work in cardiac in Gurgaon as well. And as we get our capacity up to speed in Patna, we will evaluate what payer mix and what programs we need to take in there beyond the PPP of the government.

Alankar Garude: Understood, sir. That's helpful. Thank you and all the best.

Moderator: The next question is from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal: Thanks for taking my question. Two questions. One is, from a seasonality perspective, is it fair to assume that you also go through a cycle of Q1 increments which bring some pressure and margins for the business and some of the other hospitals typically allude to that for Q1? What I

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would observe in other hospitals is typically from Q4 to Q1, there is a margin dip which comes in on that account. Is there a trend that we should expect in our seasonality profile also?

Pankaj Sahni: Let me answer that this way. Yes, like every other organization, we also have given employee increments with effect from the 1st of April and that will of course play out over not only Q1 but the full year. That being said, if you look at our historical performance, I don't know whether there is any very significant difference in, cost to revenue ratios or margin profiles historically and we have given increments every year. So, I would assume that whatever would be the natural flow through of this increment cost in the P&L will continue in the

same way it has in the past. I don't see anything fundamentally different.

Nitin Agarwal: Secondly, in a couple of times during the call, you mentioned that some of our hospitals do have exceptionally higher EBITDA margins. So, how should we, should one interpret, this sort of classification? Is it, does it suggest that maybe some of these margins are a little on the higher side and they have a chance to get, as hospitals mature to get even more rationalized or how

should one think about it?

Pankaj Sahni: So, I think the real answer to that is, it depends from whose perspective you are looking at things. We as an organization have always tried to be a little bit financially conservative. If you go back in history from even pre-listing to today, you see that or pre-COVID to today, you see that what was maybe a 15% to 18% EBITDA margin at some of the top performing units suddenly jumped to, 22%, 23% numbers and then, hospitals started reporting, 25% and even upwards. I think at some point, we have to understand that this particular industry is not an industry where you can be talking about 30% - 40% EBITDA margins on a sustainable basis. So, we firmly believe that a rational and reasonable margin profile where we deliver more sustainable growth over a longer period of time is the correct approach.

And when I look at some of the margins that we have right now, I feel that, they are doing really well, especially in the historical context. Now, whether they will go up or they will go down, I guess time will tell. Maybe you guys are more savvy at that part. But I do not think under any stretch of imagination that our industry currently is seeing anything less than really exceptional high-end margin profiles. And that is true for many players in the industry.

Nitin Agarwal: Thank you. Sir, lastly, when we look through the next 12 months, barring the Noida hospital comes up towards the end of the year, what are the other major capacities that will get commissioned as we go through the next 12 months?

Pankaj Sahni: So, next 12 months, I mentioned that we already have a one ward which we have activated in our Gurgaon facility. It is approximately 40 - 50 beds of mother and child that is currently already operational as of today. Patients already there, surgeries being performed, etc. The second thing that is underway is we have one more floor which should come on board in the next few weeks in our Lucknow facility that will again add another approximately 50 beds. In addition to that, over the course of this quarter itself, we hope to add in at least two more ICUs in our Patna facility that is almost 60 beds. As well as our dialysis extension, our dialysis is

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actually doing, almost chock-a-block already. So, we had to expand it very quickly in Patna.

And we have also started, along with the radiation oncology which we started last year, we are building out a chemotherapy daycare ward in Patna.

And all of that capacity, we hope, will come on board in this quarter or maybe towards the early part of Q2, but most of it should come on board this quarter itself. These are projects which are already underway and very near to conclusion, interior fit-outs, etc. Activity is already underway. In addition to that, we may see some additional bed fit-outs for some of the unfinished floors in our Patna unit which may happen over the course of these 12 months.

And then, as you mentioned, Noida. Beyond that, I don't think that we are able to definitively tell you that there will be any specific capacity enhancement in FY25. Obviously, the other projects that are on board and a few more sticks in the fire may have a slightly longer time frame than the next 12 months.

Moderator: The next question is from the line of Vishnu, who is an Investor. Please go ahead.

Vishnu: Sir, there is an

MCD department in the hospital panel. Will you include it in this? There is a larger employee in this?

Pankaj Sahni: No, Mr. Vishnu, I could not understand your question. Please ask again.

Vishnu: I would like to know that there is a good number of employees in the MCD department. For example, the Delhi Government Employee Health Scheme and your CDHS. Is there anyone who can include them in the panel of Medanta Group?

Pankaj Sahni: Look, the focus of Medanta is always to serve all our patients. Now, I will not be able to tell you that there is a contract of MCD or NDMC or BMC or any other agency. We have a lot of contracts of all kinds. But I will be able to say that all the patients who come to Medanta, we try to treat them with the highest quality.

Moderator: That was the last question for today. As there are no further questions from the participants, I now hand the conference over to the management for closing comments.

Pankaj Sahni: Thank you. Just, to quickly wrap up, I know we are already over time. Thank you to everybody for joining in and for all of your questions. In line with the very positive trends that we see, not only in Medanta, but also across the industry, we believe that we continue to maintain our strong trajectory of growth and continue to focus on the stated strategy with consistent and deliberate focus.

Beyond the numbers that have been the focus of the conversation today, we believe that our impact resonates in the lives we touch and the communities we serve. And we continue to hold that as the number one priority for our organization. Looking ahead, we are well positioned to continue this momentum into the next year.

We continue to leverage on our strengths and believe that we will deliver sustainable growth that you all can be proud of and continue to focus our journey on transforming Indian Health

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Care. Please feel free to reach out to our investor relations team in case you have any questions that remain unanswered. Thank you very much for joining and all the best.

Notes:

1. This transcript has been edited for readability and does not purport to be a verbatim record of the proceedings
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Global Health Limited

Call: Aug 2024

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Scrip Code: 543654 The Manager
Listing Department
National Stock Exchange of India Limited
Exchange Plaza, C -1, Block G,
Bandra Kurla Complex,
Bandra (E), Mumbai - 400 051

Symbol: MEDANTA
Sub: Disclosure under Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 - Earnings Conference Call Transcript

Dear Sir (s),

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the transcript of Earnings Conference Call held on Friday, August 9, 2024 , for the Quarter ended June 30, 2024 results .
The Transcript is also available at our website: <https://www.medanta.org/investor-relation>

This is for your information and record.

For Global Health Limited

Rahul Ranjan
Company Secretary & Compliance Officer
M. No. A17035

Encl: a/a

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"Global Health Limited – Medanta
Q1 FY 2025 Earnings Conference Call"

August 09, 2024

MANAGEMENT : DR. NARESH TREHAN – CHAIRMAN AND MANAGING
DIRECTOR , MEDANTA GLOBAL HEALTH LIMITED

MR. PANKAJ SAHNI – GROUP CHIEF EXECUTIVE
OFFICER AND DIRECTOR , MEDANTA GLOBAL HEALTH
LIMITED

MR. YOGESH KUMAR GUPTA – CHIEF FINANCIAL OFFICER , MEDANTA GLOBAL HEALTH LIMITED

MR. RAVI GOTHWAL – HEAD INVESTOR RELATIONS , MEDANTA GLOBAL HEALTH LIMITED

MODERATOR : MR. TUSHAR MANUDHANE - MOTILAL OSWAL FINANCIAL SERVICES LIMITED

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Moderator: Ladies and gentlemen, good day and welcome to Medanta , Global Health Limited Q1 FY25 Earnings Conference Call hosted by Motilal Oswal Financial Services Limited .

As a reminder, all participant lines will be in listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal a n operator by pressing “*” then “0” on a touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Tushar Manudhane from Motilal Oswal Financial Services Limited . Thank you and over to you, sir.

Tushar Manudhane: Thanks Sejal. Good afternoon and a very warm welcome to all the participants on Global Health Limited Q1 FY25 Earnings Call hosted by Motilal Oswal Financial Services .

Joining with us today from the management side, we have Dr. Naresh Trehan – Chairman and Managing Director, Mr. Pankaj Sahni – Group CEO and Director, Mr. Yogesh Kumar Gupta – CFO, and Mr. Ravi Gothwal – Head, Investor Relations.

Over to you, Dr. Naresh , sir for Opening Remarks .

Dr. Naresh Trehan: Good afternoon. Welcome to all of you. Thank you for joining us today for the Medanta Q1 FY25 Earnings Conference Call. Our Results were published yesterday, and we have already uploaded the Press Release and Earning Presentation on the website and the Stock Exchanges. As you know, Medanta has always been at the forefront of providing world class healthcare services . This quarter has been no different, and we have continued to deliver exceptional patient care while introducing the latest technology and best treatments.

During the quarter, Medanta Lucknow has achieved a significant milestone in the field of Oncology . The team has successfully treated leukemia patient using Chimeric Antigen Receptor Therapy , better known as CAR T -cells, and this is the first time in this part of the country that this therapy has been administered . This is an advanced cancer treatment that will stimulate the power of the patient's own immune system to fight cancer. This was the first CAR T -cell therapy in Medanta Group, and also, as I said earlier, in this whole area.

Medanta Lucknow has also performed 50th successful liver transplant and became first hospital in Central and Eastern UP to achieve this milestone . We have also launched a new dedicated floor for Mother and Child in Gurugram . The department has state of the art infrastructure for 50 beds dedicated to mother and child, including 25 neonatal beds, where treatment and recovery of little babies can be actually brought to the best level anywhere in the world. We continue to strengthen our clinical capabilities and have on -boarded 35 + senior clinicians in Q1 FY25. We continue to grow, not only in numbers, but in quality and keeping the standards of health care that we maintain at the highest standard available anywhere in the world.

Now , I would like to share an important update with all of you :

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We are extremely pleased to acquire 2.2 acres of prime land in Mumbai Oshiwara in an auction from Maharashtra Housing and Area Development Authority (MHADA). This is an important milestone in Medanta's growth journey, as we plan to establish a state of the art ~500 bed super specialty hospital in Oshiwara. We are delighted with the prospect to bring Medanta Model of Care to Mumbai and provide world class medical services to the residents of Mumbai and surrounding regions.

We are pleased with our progress, and our core desire to serve people better and better at the highest end of healthcare is now journey is continuing .

With this, I now hand over call to our Group CEO , Mr. Pankaj Sahni who will provide details , over view of our financial

performance for the quarter. Thank you all for joining us.

Pankaj Sahni: Thank you, Dr . Trehan. Good afternoon and welcome everyone to our Q1 FY25 Earnings Call. During the quarter, Medanta has delivered a total income of Rs. 8,830 million, registering a

growth of 11.1% compared to the same period last year. The growth continues to be driven by higher patient volumes.

EBITDA for the quarter was Rs. 2,082 million, an increase of 4.5% year-on-year basis, with an EBITDA margin of 23.6%. Our margins have declined primarily due to increase in employee cost on account of annual increments and some sluggish growth in our Lucknow unit. Our profit after tax for the period was Rs. 1,063 million, registering a growth of 4.2% year-on-year.

Moving on to our Operational Performance during the quarter:

Our inpatient volumes increased by 10.3% year-on-year to 41,293 and our outpatient volumes increased by 10.1% year-on-year to 7,23,692. We have added 49 census beds in Gurugram in our mother and child department, as Dr. Trehan already mentioned. Furthermore, we have added 18 new dialysis beds at our Patna unit, as the demand for our nephrology services in the region continues to remain strong. On average, our occupied bed days for the quarter increased by 7.3% year-on-year, representing an occupancy of approximately 59%. Please note that this occupancy is based on increased bed capacity across the group.

Average revenue per occupied bed, or ARPOB for the quarter grew by 1.4% at Rs. 64,035 compared to Rs. 63,148 in the same quarter last year. During the quarter, revenue from international patients was Rs. 475 million, compared to Rs. 477 million in the same quarter last year. This accounts for approximately 6% of the total revenue. Our outpatient pharmacy business revenue increased by 25.6% from Rs. 266 million in Q1 FY24 to Rs. 334 million in Q1 FY 25.

Now coming to our Matured Hospital Performance:

Our matured hospital performance, which consists of Gurugram, Indore and Ranchi, continues to deliver very strong volume led growth. Mature hospitals register a total income growth of 10.4% and an EBITDA growth of 11.6% amounting to Rs. 6,381 million and Rs. 1,590 million, Global Health Limited – Medanta

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respectively. Inpatient volumes grew by 10.2% in matured facilities, resulting in an average occupied bed days increase of 6.9%. Our occupancy was 63% on an increased bed capacity. ARPOB grew by 3.2% to Rs. 67,160 in Q1 FY 25 due to a change in case mix.

Developing Hospitals

Developing hospital's contribution in terms of those, which includes Lucknow and Patna, the revenue from operations during the quarter was Rs. 2,369 million, compared to Rs. 2,297 million in the same quarter of last year, registering a growth of 3.1%. EBITDA was Rs. 581 million, contributing 27.9% of the total consolidated EBITDA. Average occupied bed days during the quarter increased by 8.2% year-on-year, representing an occupancy of 52% on increased bed capacity. ARPOB declined by 1.7% year-on-year due to a change in payer mix to Rs. 58,205. Our Patna hospital continues its impressive growth trajectory with strong year-on-year growth, while the performance in the Lucknow unit was relatively subdued during the quarter. Lucknow performance was down compared to the same period last year. However, we have seen improvements in inpatient volumes and occupied bed days on a sequential basis. We have seen some improvements in our surgical procedures, interventional radiology and gastro procedures. However, some of the specialties continue to lag behind historical performance.

In Lucknow, the management has taken several initiatives, including on-boarding of additional clinical talent, and expansion of our existing specialties and services. During the quarter, we have on-boarded 14 senior clinicians, and we continue to strengthen our clinical capabilities

as

we move forward.

As you are aware, there has been an increased competitive intensity, not only in Lucknow, but across the country. And this is expected given the increased focus on the healthcare industry. We have taken steps to improve our connect with the communities we serve in Lucknow, as well as beyond in other districts of UP. As indicated in our earlier calls, with the increased bed capacity, we have already signed up and started treating patients in Ayushman Bharat Scheme. Furthermore, we have applied for empanelment with the CGHS and other government schemes. This is in line with our vision of building large format hospitals in underserved healthcare markets to be able to provide the highest standard of care to all sections of society.

Overall, we remain highly committed to serving the people of UP and Bihar and are continuing to make significant investments in equipment, infrastructure and human capital in both our units. We will be adding additional beds both in Lucknow and in Patna in the coming quarters, and will commission new operating rooms as well as add new equipment including a new Cath Lab and the Da Vinci Xi surgical robot in Lucknow.

As Dr. Trehan already highlighted, we are also delighted to announce the acquisition of a prime land parcel in Mumbai with a potential to construct an over 500 bed hospital subject to regulatory and customary approvals. We have paid the Rs. 125.11 crores to the authorities for this land.

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This investment reflects our stated strategy of expanding our Medanta Model of Care to new communities.

We also remain committed to ensuring continuity of care for our patients, and have secured Board approval to construct the service apartment guest house on the Medanta Medicity Gurugram campus. This project, with an investment of approximately Rs. 250 crores, will cater to the growing demand from international as well as out of town domestic patients for comprehensive healthcare and allied services. This facility will also help us to optimize our length of stay in our Gurugram hospital by providing continuity of care in a safe and secure out of hospital environment with seamless access to medical services of Medanta.

With this, I request the operator to open the line for questions. Thank you.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question is from the line of Amey Chalke from JM Financial. Please go ahead.

Amey Chalke: The first question I have is on the matured hospital, here our growth is around 10% and ARPOB growth is around 3%, is it possible for the management to give some outlook for next two years how the performance would be for this for next two to three years, and what levers we have to tie the growth in the mature hospital. Thank you.

Pankaj Sahni: As we have stated multiple times, we don't give any future projections or any expectations with respect to the specific numbers on either of our developing or matured hospitals. However, what I can share with you is some of the thoughts around how we think about our matured facilities growth. So, first of all, just to reiterate, some of the numbers I mentioned, we have seen now over the last several periods, in fact, over the last several years, very strong volume led growth in our matured hospitals. And just to give you a sense, these are hospitals like our Gurugram facility is approximately 14 - 15 years old. Normally, I don't see too many institutions showing a 10% or thereabouts, IPD volume led growth. Many of the institutions seem to get a lot of the revenue growth from the tariffs and case mix change. As we have stated multiple times in the past, Medanta has always tried to be reasonably conservative in its annual tariff increases, and therefore we are very happy with the growth that is coming

in the mature

facilities, which is largely driven through volume.

We have added in some clinical talent and capabilities in our mature facilities over the last several quarters, I would say, over maybe the last 18 to 24 months. This includes bringing in senior talent in new departments, like starting the Mother and Child services, which we did last year, and adding in infrastructure for the mother and child services, which we did this quarter, as Dr. Trehan also mentioned. But not only that, we have added depth in the existing facilities. You may recall several quarters ago we had talked about the addition of additional teams in our robotic surgery program. We will be making additional investments in equipment, which you will hear about in the coming quarters, both on the surgical side as well as on the cancer side. There is a lot of work being done, to continue to remain at the cutting edge of technology and medical care in our Medanta Gurugram and our matured facilities. We are also making additional

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equipment investments in our facilities in Indore and Ranchi, some of which have already undergone post the quarter end, and some of which you will hear about as we roll out in the coming months. So, there is a lot of work happening in growing these services across the matured facilities and we do expect this focus on continuing to make sure we are always at the cutting edge and delivering the highest technology, bringing in the best clinical talent into our mature facilities. And, I again take you back to our stated strategy in very early on, when we first went public, which very clearly talked about that the first aspect of our strategy was to continue to invest in the exceptional clinical talent and high end clinical capabilities we have in our mature facilities.

We believe that if we continue to do this, we should continue to see growth coming into these facilities, and we hope that we will be able to continue to serve the patients with the highest care. So, that is the answer for our mature facilities.

As, I also mentioned in my opening remarks, that we have initiated the activity of building out the service apartment and guest house, and this should give us some amount of opportunity to further streamline and improve the efficiencies with respect to the length of stay, with respect to being able to manage, especially patients who come from out of town, like international patients, and provide them with a seamless experience. We do believe that this is something that is been missing in the sense that people come from out of town, because we have a lot of destination care patients, and they have to stay in all different places in and around our hospital, it may not necessarily be the easiest thing. We believe that providing them with this kind of a seamless experience, both the international as well as out of town domestic patients, may give us some

benefits, and then there are also opportunities to optimize length of stay for patients who may need to stay in Gurugram for certain period of time before they travel back home, but are fit enough to be discharged from the hospital. So, these are kind of initiatives that we will continue to work on as we move forward.

Amey Chalke: So, just to summarize, so is it fair to say the improvement in the occupancy, some optimization on the ALOS and the improvement in the international patients going ahead would be the drivers for these segments?

Pankaj Sahni: Yes, these are definitely focus areas for us. And in addition to that, what I would also say is that, we are looking at beefing up our clinical talent pool, and obviously that clinical talent pool continues to go into deeper levels of expertise. So, we will be looking at doing more and more complex work, bringing in the latest technologies, whether it is for robotics or whether it is

for

cancer. So, what you articulated are the ways in which you see the outcomes on some of the financial metrics, but the inputs are also related to clinical talent acquisition, technology investments, etc. And then, as we roll out also our focus on research and academics in the coming years, we expect that also to add to the overall reputation, as well as situation of the unit.

Amey Chalke: Sure, thank you so much. And the second question I have is, in the developing hospitals, so here we have seen the occupancy as high as 70% in the last year in one of the quarters, which has gone down to 52% this quarter, I believe like you said, it must have been led by the Lucknow Global Health Limited – Medanta

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unit. So, how long do you think will it take for the occupancy to move up, if not 70%, but close to 60% in this segment of developing hospital?

Pankaj Sahni: Yes. So, let's first break this up into two categories. Let's first talk very quickly, just about Patna.

Patna, our hospital continues to grow well. As you may recall, we have added in our PPP business only rally towards the end of last financial year maybe Q3 FY2024. So, that business continues to scale, we continue to add beds there and actually we see quite a strong demand there. Like I said, we continue to put in beds there, in fact, we are trying to get the beds live as fast as possible. So, you will see some amount of bed additions coming in in the next several quarters in Patna. We have been triggering more and more beds given such strong demand, both on the cash side as well as on the PPP business side, so that will of course be one of the drivers which should move in a positive direction. Obviously, the realizations in Patna are little bit lower than the realizations in Gurugram and Lucknow. So, when you blend it all, it has somewhat of a dilutive effect on ARPOB but on a standalone basis, Patna this quarter actually delivering a first time PAT profit as well. So, we are very happy with the way Patna is proceeding.

Now, when it comes to your question on Lucknow, Lucknow has been a little bit disappointing for us for the last several months. Some of that has also of course been further exacerbated by what you exactly said, which was exceptionally strong performance in the first couple of quarters of last financial year, where you did see occupancy levels extremely high, you saw realizations extremely high. Some of that, of course was seasonal, with respect to dengue and some of the other occupancies in our ICU, as you said going as high up to 70% but also don't forget that we have added in a fair amount of capacity in our Lucknow facility. So, some of the occupancy percentages which you saw was when we were somewhere around 400 - 450 beds, today we are now at about 700 - 750 beds and so if you really look at the occupancy percentages on a standalone basis that may not necessarily be correct, because the denominator keeps changing. So, by the end of this quarter I believe, we will probably be somewhere around 700 - 750 beds. So, just one word of caution there, we should not look at absolute percentage.

That being said you are right, there has been a decline, especially with respect to the same quarter last year, on the absolute volumes as well as the absolute revenue. And some of that is as I mentioned, various aspects of competitive intensity, some of the challenges in that market, which we have been working through, as I mentioned earlier, to get back up to speed, get running, get refocused. The real important point here is to think about, when we talk about our Lucknow market, we are really committed in this market for the long run. So, we have been hiring in, just as I mentioned, ~13 - 14 senior doctors hired in the last quarter. You will see more hiring coming in in this quarter when we announce our Q2 Results as well. So, really driving

in a lot of the

longer term, sustainable business has been our focus. I have repeatedly mentioned on multiple calls that, we should not take the baseline business that we were seeing in Lucknow as a realistic number for the extremely long term. We do believe that we can do better than we are, and we are working towards that. So, we are by no means satisfied with this performance. But I would caution if we put a baseline of saying, you will be at 75% let's not forget that we have increased

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bed capacity and some of the performance, which you saw last year were really, really I would say above normal performance .

One last thing, just on the absolute financial numbers. As you are aware, a lot of the costs, especially with respect to the annual increments on manpower cost, they get booked in the new quarter when the new year starts vis-à-vis, let's say what was there earlier. So, some of the impact that you are seeing from an EBITDA point of view, rather than from a top line or a volume point of view, is the negative impact of the increments. And since Medanta operates on a largely fixed cost model , we are hopeful that as occupancies and as volumes grow like always, we will have the benefits of operating leverage as we scale up.

Amey Chalke: Sure. Just final question, is there any change of plans or delaying of plans because of the lower occupancy on the Lucknow side , or are we still on track with that timeline for the expansion on the Lucknow?

Pankaj Sahni: No, we are still on track , there's no delays. In fact, as I did mention we have added in, about a 100 beds towards the end of Q4 FY24 . We have added in about 50 beds in this quarter as well and we are actually also adding in investments. So, if you look at some of the CAPEX investments that we have done, we will be putting in a new Cath Lab , we already have two Cath Labs, we will be adding in our third Cath Lab , in fact it's a biplane Cath Lab , so it's a pretty high end machine. We will be adding in a Da Vinci Xi surgical robot that should come on board in next several weeks. So, absolutely no looking back as far as our investments and our plans in Lucknow go . And like I said, when we look at our facilities we have not gone into Lucknow with a 200 or 300 bed facility. We have gone with a very long term commitment to build an institution that will sustain that area for the longest period of time. In fact, it is not only Lucknow, we continue to remain extremely excited about the UP region in general, and this will continue to remain a focus region for us.

Moderator: Thank you. The next question is from the line of Damayanti Kerai from HSBC. Please go ahead.

Damayanti Kerai: As you mentioned you have started empaneling Ayushman patient in Lucknow and then you will proceed for CGHS scheme, etc. So, just want to understand how should we look at the payer mix change for this hospital, like how do you balance between your mix and then occupancy?

Pankaj Sahni: As you mentioned on the occupancy , if you look at our mature facility, and if you look at our Gurugram Gurugram facility specifically, we Gurugram are at 1,400 beds. And if you put all the beds together, probably somewhere around 1,700 beds across all the mature facilities. We have been taking scheme patients in Ayushman , we have been taking scheme patients in CGHS , railways, etc, in these facilities for many years. And if you look at our payer mix across the board, we have had one of the healthiest payer mix in town. We continue to have in the ranges of around 80% of Cash and TPA, if you add in some of the private corporates maybe even little more. So we believe that it's our right, it's our duty to serve every segment of society. That is one of the reasons why we put up these big facilities , it goes very well with our fixed cost model, because the marginal cost to serve

every incremental patient is of course significantly lower. So ,

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in our particular philosophy, we are not opposed to serving the kinds of patients that may otherwise be considered to have a slightly lower realization. We believe that we should serve all of these.

We have been able to maintain, a fairly healthy payer mix in the mature facilities, and, of course across the group and we see no reason why we cannot continue this in Lucknow.

We had been asked many times that will you take Ayushman business, and will you take scheme business in Lucknow. And we had consistently maintained that as soon as we have the capacity to be able to sustain this volumes, we would like to sign up for these schemes. So, we have never said that we would not do it. We obviously, when you had 300 - 400 beds and we had a slight catch up which we were doing, vis-à-vis the bed availability, it was not possible for us to open up this business as well. But as soon as we have crossed into the financial year of FY 25 which was always our plans, we did say that we would be applying for these schemes and treating these patients. So, we believe that as we have always said, 10 % - 15% of patients said this , at these realizations is absolutely a good thing for us to do. And very interestingly, as you may be aware our Patna project, which is a PPP project with the government of Bihar, has an obligation of 25% of the patients at state CGHS rates. And we actually do find that realizations of those patients is in many cases even higher than some of the cash patients. So , I think that we are not averse to this business , it is maybe a lot more nuanced to go into understanding how the various payer mixes work. I don't think that any particular payer mix is a good or a bad business for us, that's at least how we think about it.

Damayanti Kerai: Sure. And just in Lucknow, do you have any obligation to reserve certain beds or it's at pure discretion?

Pankaj Sahni: No, it is a pure discretion. We have no obligation in Lucknow.

Damayanti Kerai: My second question is on Mumbai facility, so when you are likely going to start this unit?

Pankaj Sahni: So, Mumbai unit, as with any unit will take, I would say at least three to four years because right now we are just in the process of acquiring all the necessary approvals to close out on this complete transaction. And then, it will take a certain amount of time to construct the facility, because this will be a pure green field facility. So, I would say that if everything went really fast, I would say at least three years. But normally we say three to four years to build out a hospital, especially the size which we are talking about.

Damayanti Kerai: Sure. And last clarity, you mentioned you will continue to invest in critical talent and some equipment, etc, to maintain the growth momentum. So, should we assume current quarter level cost to continue on a broader basis?

Pankaj Sahni: Let's look at it into two categories. First of all, on the equipment side obviously this would be mostly, I would assume capital expenditure so that will have maybe more of a balance sheet impact than a P & L impact from the numbers point of view. But yes, of course we do expect to

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continue to invest in medical equipment. As far as the clinical talent goes yes, I do believe that we will be adding in clinical talent, and therefore there would be some impact on the manpower cost. And like I said, we do have fixed cost doctors who are full timers, so there would be some impact of that. Obviously, that gets offset depending on how quickly these teams ramp up with respect to revenue. So, in some cases we have seen very, very fast ramp ups for certain specialties. In some cases, it takes a little bit longer depending on either the region or

the

particular nature of the specialty, or the demand for that specialty, and also sometimes it is seasonal. So, I would say that we don't necessarily look at the manpower cost purely quarterly, but if you look at it across the year, I did mention this last quarter as well, we do anticipate and have budgeted a certain increase in the manpower cost on an overall basis, as well as some maybe on a percentage of revenue basis, just because we feel that there will be increasing intensity for high quality talent given all the various hospitals and all the excitement and investment going into hospitals and healthcare. So, we do believe that we are a very strong doctor employer, and we do believe that we have the ability to attract very high quality and high capability talent, and we will continue to invest in that. We are not overly worried about whether that pays off in one quarter or two, in the long run this is the right approach for us.

Moderator: Thank you. The next question is from the line of Madhav from Fidelity Investments. Please go ahead.

Madhav: In the opening remarks you did mention that there has been some impact of payer mix on the performance. Could you just help us understand that for Lucknow and for Patna, how does the payer mix look like in quarter one, if you would share some broad understanding, like how much of scheme patients now, cash and insurance?

Pankaj Sahni: Yes. I mentioned that, more specifically with reference to our developing hospitals. So, let me talk about Patna, because Patna does have a PPP business. So, if you look at our obligations in Patna, we have 25% of the beds have been reserved for patients at the state government PPP rates. And up until, it was maybe August or September of last year, we didn't actually see any patient movement on the PPP side, because we hadn't got all the systems ready, and the government wasn't able to actually get that entire system moving. So, while the hospital has been operational for, almost two years by then or a year and a half, we saw zero PPP patients from this entire project.

Towards the end of August - September, this number of patients started to move given all the systems were in place. And if I recall in our last earnings call, it was somewhere around maybe 500 patients or so had been discharged towards the full financial year of FY25. Now those patients continue to scale up as we move into this quarter. So, when you are looking at the Q1 of last year, where the numbers were probably almost zero to Q1 of this year, you do see the impact of the PPP business in Patna. Now, the realizations from an ARPP basis of some of these patients has been very good, but we do also find that sometimes these patients tend to stay a bit longer in the hospital. So, the length of stay of some of the PPP patients is typically for the same specialty, a little bit higher than the length of stay for the cash patients. And so that then ends up having a negative impact, impact on the ARPOB. But if you really want to look at this, maybe

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ARPP would be a slightly better way to think about it, but the payer mix of PPP will of course

have a dilutive impact on ARPOB , especially in Patna.

We have not seen too much of that with respect to Lucknow . In Lucknow, we have not seen the ARPOB being impacted that much. In fact, if you look at our ARPOB in Lucknow, they have actually increased from Q1 FY24 to Q1 FY25 .

Madhav: And on the PPP business, what would the patient volume mix be for quarter one, broad number is fine like, it was almost zero last year, Q 1 FY24, in Q1 FY25 how does that look like?

Pankaj Sahni: Yes, Patna PPP business is around 5% to 7%.

Madhav: This in terms of volume, right , patient volume mix?

Pankaj Sahni: Yes.

Madhav: Perfect. And you said to the ARPP here is quite good. Generally, my understanding has been

that, the tariff is a lot for cash patient vs the scheme patient, is it better than cash, like what 's the dynamic that is playing out at this facility ?

Pankaj Sahni: So, just coming back to a little bit this model in Patna, the way it works is that the government has various medical colleges and various state authorities that will actually send referral patients to us for treatment under this scheme. So, many of the patients obviously are those patients who could not get treated in maybe a routine manner in some of the government or other facilities. So, probably these are sicker, maybe more complicated patients who require that kind of high-end treatment. So, this is my baseline assumption , that why the patients are having a pretty good ARPP because they are quite sick , so they are coming for complex procedures. So, we are not getting PPP patients for snake bites or for routine kind of medical management, it's far more patients who are coming for complex procedures in cardiac or in cancer, and those would therefore have a slightly higher realization.

Madhav: Got it. And just on the Lucknow facility as well, cash and TPA like you said was very high until last year because we didn't mean to take scheme patients, but in Q1 FY25 , if you could give some sense in terms of how that has moved in terms of the payer mix for Lucknow?

Pankaj Sahni: I don't think the payer mix has changed very much in Lucknow at all, because we really only started our Ayushman business towards this quarter. So, a very insignificant change in payer mix in Lucknow currently. But, as we said, as that scales up you will see some of that business coming in. Also, we have not even obtained the empanelment for CGHS and railways, etc. It is only Ayushman that we are currently impaneled with. So, I would say that right now, no change really in terms of payer mix from what you have seen in the past, but we do anticipate that as we move forward that will come on board.

Madhav: And this last question was on the developing hospital, we did about 29 % to 32% margin in FY23-24 some of it, my guess is that was helped by the very good payer mix that we had until Global Health Limited – Medanta

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now. So, as we move ahead, although our topline growth will remain quite healthy, should we expect that with the change in payer mix margins normalize a little bit lower ? Don't want a specific numbers but just directionally was trying to understand this.

Pankaj Sahni: Well, like I said, I can't predict what the EBITDA margins will be as we move forward. What I can do is , articulate for you some of the levers that we operate on. So, the first thing actually to think about, specifically with respect to our developing hospitals is that we operate, as I said on a fixed cost model with respect to our doctors . So, our doctors are fully employed , they are largely on a fixed salary model. We have seen a dip in some of the volumes in the last few months, and as we do see some of those recoveries coming in, as we do see some of the improvement coming in, obviously the increased volumes and occupancies has a disproportionate impact on the marginal profit because of the operating leverage concept. So, as volumes grow, assuming the cost base doesn't increase too much, you should expect to see improvements in the EBITDA margins as you move forward.

When it comes to the case mix, again it is very relevant to understand what is the exact type of mix. So, if we see a scenario, let's say similar to Patna, where patients who are coming in for in these slightly lower paying categories are actually highly complex patients . If you see that the cost to serve them may not be that high. So, just to give you an example, the cost to serve patients in certain specialties may not be actually higher than the cash patients, vis-à-vis the margins that you get but in some cases, it is

. So, it really depends on the specialty mix. It really depends on what is the complexity of the patients that come in .

Overall, our belief is that because the marginal cost to serve every incremental patient for us is significantly lower. We generally believe that increased volume, even at lower realizations is margin accretive, and we have seen that play out again and again, year-over-year in our Gurugram facility, where we have been taking these scheme patients for many years. So, that is the reason we have always stated that there is no harm in shying away from this

business. We do expect that with volume increases, you should see positive momentum on the margins .

Madhav: And one on the Mumbai expansion, which you all have spoken about. So, in the press release you have mentioned Rs. 1,200 crores investment for 500 beds so it's about, let's say Rs. 2.4 crores of CAPEX cost per bed. So, is the view that Mumbai is probably be a high paying market, the location is quite attractive. So, ARPOB here could be very good guys only, then probably justifies the Rs. 2.4 crore bed cost, is that the right way to think ?

Pankaj Sahni: So, broadly your logic is right. Let me just make couple of clarificatory points, which are important for everybody to be aware of. The first point is that, we have not yet got a complete exact amount of project budget that we will have, because there is some amount of understanding of exactly how much amount of money needs to be paid for acquiring the additional FSI to construct. So, Bombay allows you to buy additional FSI. These are things which are being still finalized and still worked out .

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The second thing is that the exact designing of the hospital is not been completed in terms of how many specific beds, so we had mentioned +500. So, whether it's going to be 500 or 550 or more, I am not sure that also depends on how we think about all the other supporting services, which are very important for us, in terms of operating rooms, etc. So, as this gets finalized, we will actually have a much more definitive idea on the exact project. But you can say that this is a reasonable number for us to estimate as we move forward that's why we put this number out there.

The second aspect which you said is very correct. In general, Bombay is more expensive to build. Unfortunately, we have been seeing that over last many years, maybe last five - six years, including pre COVID to post COVID, the cost of construction per bed has increased. It used to be below one crore per bed, even at the kind of quality and infrastructure that we put up. Now that number is much closer to Rs. 1.2 crores to Rs. 1.4 crores per bed, at the kind of investments and the kind of infrastructure that we do, and that is all cost, excluding land. Now, when you come to Mumbai, the numbers that you articulated include the cost of land, which is not normally how we look at it. So, if you take out the cost of land, that number would be a little bit lower.

But yes, Mumbai does have a higher cost to construct. And you are very right, typically the ARPOB and the realizations in Mumbai are higher. Our estimations, as we think about building out these in Mumbai will try to be as much in-line with our stated philosophy. So, we typically look at being competitive in the market when it comes to the tariffs and the pricing, as I mentioned earlier, we try to be as conservative as we can be relative to the kind of quality and infrastructure and medical services that we provide.

We feel that in light of all of those things, the investment is very much justified. And obviously you are aware getting a clean plot of land coming from a kind of a government option itself is a very big challenge in a city like Mumbai, and we have typically also mentioned in the past that we are very, very high on ensuring

that whatever we do with respect to the investments in real

estate. Or the structure is extremely clean from a governance point of view, and therefore we are very happy with this kind of a project, because state government auction so worked out well for us.

Moderator: Thank you. The next question is from the line of Alankar Garude from Kotak Institutional Equities. Please go ahead.

Alankar Garude: Sir in the fourth quarter, the issues highlighted by you in Lucknow were around seasonality, extreme winter, high, dengue base, etc. Can you help us understand what were the exact challenges this time around? You spoke about competitive intensity and some other challenges, possible to get into some details please?

Pankaj Sahni: So, Alankar there is no one specific thing that I can say that this is the reason, it's possibly a combination of factors that have been playing out. I do believe that some of the factors which you mentioned remain. The competitive intensity has also kind of played out not just with the branded institutions, but a lot more of the local institutions actually getting quite active, a lot more intensity you see from some of the local as well as some of the other institutions with

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respect to their sales and marketing, with respect to their branding, with respect to their various approaches, with respect to how they feel, they should be attracting patients.

Some of the things that Medanta is not willing to do in terms of compromising on our approaches and our philosophy, and it has also been something which we look at to say that, we do believe that there were certain areas where we were struggling with respect to being able to provide the

supply. So, in certain cases, just to give you a very simple example, say in our nephrology department, we had a constraint on availability of clinicians. We had a constraint on availability of dialysis beds. We have hired in during the quarter a senior director in our nephrology department, and we see immediately that there is an increase in the patient volumes for that service. So, it's a combination of making sure that we bring in the right talent. It is a combination of making sure that we retain as much as possible the talent, given the increased competitive intensity, but also some reactions and some learning's for us that maybe we needed to get a little bit closer to the community, both inside Lucknow as well as outside Lucknow. So, some of the efforts which we were doing in Gurugram in terms of reaching out to the residents associations, various associations that activity has actually started now in Lucknow, and we are seeing a very positive response for that.

Also it was important for us to reiterate our commitment to the city and to the people, and also focus in on ensuring that we are continuing to add in talent. So, maybe you can say we got a little bit complacent with such high occupancies and volumes, some of the departments which we hadn't completed out. We hadn't really put as much effort in hiring doctors for those departments, now we are putting in a lot of focus on bringing those on board as well. Beefing up the talent pools in the existing department. So, it's been a good learning for us last couple of months, and that's allowed us to refocus and recommit all efforts to making sure that we are able to deliver for Lucknow what we promised.

Alankar Garude: That is helpful Pankaj. A quick follow up, have we seen any senior management churn in Lucknow, over the last few months, senior doctor churn I mean?

Pankaj Sahni: So, last couple of months we have not seen anything. We have had one or two doctors who have left us post June 30th, quarter end so as we speak today. But no real significant, I am trying to think now exactly maybe one or two doctors who have left us, some going into newer facilities,

some going into existing facilities and also, like I said it's not only the branded players, some of the local players also getting more active. We have also hired in doctors from these facilities, as well as some of the doctors from the government facilities, both in the quarter which you see in the investor presentation, but also subsequent to quarter end. So, these are all obviously public announcements since the doctors are all on board. So, to give an example our senior radiation oncology director has joined us post quarter end. He's come in from SGPGI, like I said, we have a senior nephrologist come in during the quarter. So not a very high churn, only two names that I can think of post quarter.

Alankar Garude: Okay, understood. The second question is, around Mumbai. Can you take us through the thought process behind entering Mumbai. The reason for asking that is, as one of the previous

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participants asked, in terms of CAPEX per bed, it's clearly higher in Mumbai. There are only a few corporate hospitals in the city. So, how should we look at the economics in the city and if you can help us understand the pros and cons of the market, that would be great.

Pankaj Sahni: If you think about, the Mumbai market first of all, it's a little bit misnomer to think of Mumbai as one market. I actually think that there are three or four markets in Mumbai. So, if you think of let's say, from Bandra towards South Mumbai, maybe you could group that into one market, although some people may say it's more than one. If you look at, let's say North of Mumbai, maybe going up past Andheri, and maybe all the way to Goregaon you may think of that as one market. And then, people do also consider the Western side moving into Navi Mumbai as a different market and then you can of course, subdivide that in many ways. So, there is no one market of Mumbai and if you really ask me, Mumbai remains an attractive opportunity of course subject to availability of land and the ability to build out these hospitals in each of these micro markets.

Now, obviously a city like Mumbai, one of the greatest challenges is getting access to land. And in our particular case, as I mentioned in the past also, the kind of infrastructure and space required to deliver the kind of care that Medanta is famous for requires a certain minimum amount of land. So, not much fun to put up 100 bed hospital with limited ability to do the kind of care that we deliver in a city like Mumbai. And then lastly, and very importantly as I also mentioned, there is a lot of challenges with getting access to land so Medanta has typically stayed away from questionable transactions where, not everything is 100% above board in terms of the accessibility to land and the kind of deals or the constructs that go into acquiring land or entering into contracts with the appropriate parties.

So, we have been highly conscious of the right ethics and governance. And since this was a plot of land that was available from the government authorities in a very clean auction, we thought it's an excellent opportunity. The land is exclusively for putting up a hospital. It met our requirements in terms of the location, which we believe in an excellent location, it will cover a very large part of the hardcore Mumbai area, but also some of the drainage from some of the

suburbs of Mumbai into that area.

It gives us an opportunity to put up a fairly big facility. If you take out the cost of land, if you are able to actually squeeze in a few more beds than the 500 which we have quoted you may actually find the cost per bed numbers significantly lower, more or less coming into lines with what we see in other parts of the country. That, being said, Mumbai has always been a market where you assume that it's not going to be Rs. 1.4 crores - Rs. 1.5 crores per bed it's going to be

a little bit higher, maybe closer to Rs. 2 crores. And again, these are without cost of land, but given that we got a clean plot of land, we thought it was a great opportunity so inclusive of cost of land, yes it comes to Rs. 2 crores a bed, our modeling actually shows that with the realizations and levels that are available in Mumbai, this still actually seems fairly attractive at approximately Rs. 2 crores a bed, 500 or 500 plus beds, if you can get a few more, say, 600 beds there. The economics seem to be very, very strong and more than that, Alankar, it's not a question of Global Health Limited – Medanta

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economics purely, it's a question of making sure that we are able to bring in our kind of care to the city.

Mumbai has historically been a city which has survived on a lot of older institutions, initially charitable trusts, etc, and people have tried to refurbish and rework some of those. There's been limited, brand new kind of investment in that city in the healthcare domain. And we feel that this is part of our duty as we have said always we want to deliver healthcare to the underserved healthcare markets, Mumbai in many ways, even though it's a high income and high per capita market it's quite underserved from a quality of healthcare point of view. And so it fits with our stated goal Mumbai of course is an attractive city in any case. So, absolutely in line with our approach we have been looking for locations in Mumbai for some time, and given that we found a good deal here, with respect to this government auction, we thought perfect opportunity and we were lucky to get it.

Alankar Garude: That's helpful Pankaj, and one final question if I may, any delays you are facing when it comes to Delhi, because construction activity is yet to start, what are the timelines we are looking at now for the Delhi hospital?

Pankaj Sahni: Yes, you are right Alankar, there has been a little bit of a delay in Delhi. We had mentioned in the past that we were waiting to get all the necessary regulatory approvals lined up to be able to move that construction in full force. That's taken a little bit longer than we would have liked. Also, some of those approvals are linked to not in our particular land, but in other associated cases which are going on the court. So, we have been waiting for some of those things to get cleared. What we have actually decided is to go ahead with some parts so the land is in kind of in two parts. So, we are actually going to start the construction very soon in one of the parts of the land, so that we should be able to see some of the construction activity that you articulated hopefully in this quarter itself. But ideally, one would argue that this could have been done, or we would have liked it to be done maybe six months in advance, so yes the answer is, it is a little bit delayed.

Alankar Garude: And similarly, on Indore the second hospital?

Pankaj Sahni: Indore, story is little different with respect to the construction activity. Indore as we had mentioned, got stuck into a little bit of litigation one or two prior owners back. So what we had done is, actually we had held off on signing the definitive agreement with our partner, although the partner remains committed. In our last update, I don't remember whether I said it in the last quarter, there was a judgment which was favorable for the landlord, where the High Court of Madhya Pradesh had dismissed the appeal which was being raised by one of the parties about the land, and the landlord that we intend to work with had won that case. I believe now it is at a revision level that appeal. So, we are just waiting for the legal part to be fully cleared, just to be doubly sure on the investments and the legalities although the partner is putting much of the Capex. So, we both remain committed to doing that, but I can't yet give you an answer on when

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the construction will start. It is very possible it may take another couple of quarters for that to close out before we move forward.

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Moderator: Thank you. As there are no further questions, I would now like to hand the conference over to the management for closing comments.

Pankaj Sahni: Thank you everyone for your questions and for joining today. As we move forward, our focus remains, as I said earlier on scaling our operations, enhancing our services and ensuring that we

continue to deliver the highest standards of care that we have been come to be known for. We look forward to interacting with all of you as we move forward. If there are any questions in the meanwhile, please feel free to reach out to our Investor Relations team, and we look forward to speaking with all of you soon and seeing many of you in the coming months. Thank you very much.

Notes:

1. This transcript has been edited for readability and does not purport to be a verbatim record of the proceedings
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Bandra (E), Mumbai - 400 051

Symbol: MEDANTA

Sub: Disclosure under Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 - Earnings Conference Call Transcript

Dear Sir(s),

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the transcript of Earnings Conference Call held on Thursday, November 14, 2024 , for the quarter and half year ended September 30, 2024 results.

The Transcript is also available at our website: <https://www.medanta.org/investor-relation>

This is for your information and record.

For Global Health Limited

Rahul Ranjan

Company Secretary & Compliance Officer

M. No. A17035

Encl: a/a

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"Global Health Limited – Medanta
Q2 FY25 Earnings Conference Call "
November 14 , 2024

MANAGEMENT : DR. NARESH TREHAN – CHAIRMAN AND MANAGING
DIRECTOR – MEDANTA – GLOBAL HEALTH LIMITED

MR. PANKAJ SAHNI – GROUP CHIEF EXECUTIVE
OFFICER AND DIRECTOR – MEDANTA – GLOBAL
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MR. YOGESH KUMAR GUPTA –CHIEF FINANCIAL
OFFICER – MEDANTA – GLOBAL HEALTH LIMITED

MR. RAVI GOTHWAL – HEAD INVESTOR RELATIONS –
MEDANTA – GLOBAL HEALTH LIMITED

MODERATOR : MR. AMEY CHALKE – JM FINANCIAL

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Moderator: Ladies and gentlemen, good day and welcome to the Global Health Limited, also known as Medanta Q2 FY25 conference call hosted by JM Financial. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference, please signal an operator by pressing '*' and then '0' on your touch -tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Amey Chalke from JM Financial. Thank you and over to you, sir.

Amey Chalke: Thank you. Good evening and a very warm welcome to all the participants on the Global Health Limited Q2 FY25 earnings call hosted by JM Financial. Joining us today from the management team, we have Dr. Naresh Trehan, Chairman and Managing Director, Mr. Pankaj Sahni, Group CEO and Director, Mr. Yogesh Kumar Gupta, CFO, Mr. Ravi Gothwal, Head of Investor Relations. I will now hand over the call to Dr. Trehan for his opening remarks. Thank you and over to you, Dr. Trehan.

Dr. Naresh Trehan: Thank you. Good evening to every one of you who is on the call and thank you for joining us today for Medanta's Q2 and H1 FY25 earnings conference call. The press release and earnings presentations have already been uploaded on our website and the stock exchanges and I trust you have had an opportunity to review them.

Medanta's continued dedication to raise healthcare standards has resulted in another quarter of strong performance. As we expand our reach, we see steady progress across our facilities, aligning with our commitment to delivering accessible and highest quality of healthcare.

The quarterly update that I would like to share with you that, I'm pleased to announce that last week we signed an Operation and Management Agreement to operate and manage a state-of-the-art 750 bed super specialty hospital in Pitampura, New Delhi.

This is a significant milestone in Medanta's growth journey, allowing us to extend our presence into the high growth densely populated micro markets of Northwest and West Delhi. This hospital to be jointly developed with the society over the next 4 years and will offer comprehensive world-class healthcare services, including multi-specialty robotic surgeries, regenerative medicine and a full spectrum of transplant services.

Our commitment to quaternary care remains steadfast as we focused on delivering cutting-edge medical services to transform healthcare delivery.

Notably, Medanta Gurugram reached a significant milestone by completing 3,000 kidney transplants. Additionally, our doctors achieved a breakthrough in treating Idiopathic Chylopericardium, a rare condition using minimally invasive approaches. Medanta Gurugram also performed its first CAR T-cell therapy, an advanced cancer treatment that harnesses the power of the patient's own immune system to fight cancer.

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We also introduced the Da Vinci Xi surgical robot at Medanta Lucknow, which will support precision driven, minimally invasive surgeries, further enhancing patient outcomes. We continue to strengthen our clinical capabilities and on-boarded over 25 doctors in Q2 FY25.

As we look ahead, we remain optimistic about our growth trajectory. Our focus will be on expanding our services offering, enhancing patient care and leveraging technology to deliver world-class healthcare services. Now I will hand over the call to Mr. Pankaj Sahni, our group CEO, who will walk you through the financial performances of the quarter. Thank you.

Pankaj Sahni: Thank you, Dr. Trehan. Good evening and welcome everyone to our Q2 and H1 FY25 earnings call. I am pleased to report a strong performance this quarter, achieving our highest

ever

quarterly revenue and EBITDA. Growth momentum was robust across our network hospitals and Lucknow unit also showed signs of recovery.

During the quarter, the company delivered consolidated total income of INR 9,748 million, which is a growth of 12.7% year on year and 10.4% on a sequential basis. The growth was driven by a combination of factors including higher patient volumes, increase in bed occupied days and improvement in realization.

EBITDA was INR 2,465 million, up by 5.5% year on year and a robust 18.4% growth on a quarter-on-quarter basis. EBITDA margins were at 25.3% in Q2 FY25, an improvement of 170 basis points on a sequential basis.

Profit after tax was INR 1,308 million, growth of 4.5% year on year and 23.1% on a quarter-on-quarter basis. PAT margins were 13.4% compared to 14.5% in Q2 FY24.

In the first half of FY2025, INR 370 crores of capex were incurred, including INR 131 crores related to the acquisition of land in Mumbai. Our balance sheet remains strong with a net cash surplus of INR 7,186 million at the end of September 2024.

Now moving on to some operational performance highlights.

During the quarter, we have successfully expanded our capacity by 118 beds, comprising 52 beds at Lucknow and 66 beds at Patna. Average occupied bed days increased 9.1% year on year with a strong occupancy of 64.3% on increased bed capacity. Inpatient volumes during the quarter increased by 8.8% year on year to over 45,000 and outpatient volumes increased by 6.6% year on year to 7.75 lakhs. This consistent volume driven growth reflects the sustained trust and

confidence our patients place in Medanta's dedication to delivering exceptional patient -centred care.

Average revenue per occupied bed or ARPOB for the quarter was INR 62,140 compared to INR 61,003 in the same quarter last year, a marginal increase of 1.9% , largely driven by changes in case mix and some amount of tariff increases in our mature d facilities.

Coming to the matured versus developing hospital performance update . Our matured hospitals comprising Gurugram, Indore, Ranchi continue to deliver strong growth across key performance

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indicators. Matured hospital revenue was INR 6,882 million, which is a growth of 13.5% year on year and 7.8% on a quarter -on-quarter basis.

EBITDA was INR 1,739 million, a growth of 12.5% year on year and 9.4% on a quarter -on-quarter basis. EBITDA margins remain strong at 25.3% in Q2 FY25.

Average occupied bed days increased by 6.4% year on year, representing strong occupancy of 67%. ARPOB grew by 6.2% to INR 67,059 in Q2 FY25, driven by tariff increases in Gurugram and Indore and changes in our case mix.

Coming to the developing hospitals, which include Lucknow and Patna , delivered revenue of INR 2,754 million, w hich is a growth of 4.2% year on year and 16.2% on a quarter -on-quarter basis.

EBITDA was INR 823 million, down by 8.2% year on year and a strong growth of 41.7% on a quarter -on-quarter basis. EBITDA margins improved from 24.5% in Q1 FY25 to 29.9% in Q2 FY25. Average occupied bed days during the quarter increased by 14.5% year over year, representing an occupancy of 60% on increased bed capacity. ARPOB declined by 5.9% year on year due to a change in payor and case mix to INR 53,724 .

To add some further colour to these numbers that I've just mentioned, coming first to a Patna unit, it continues to perform well, very strong growth in patient volumes resulting in robust revenue and profitability growth and increasing its contributio n to the developing hospital classification and to the group in general.

Lucknow performance during the quarter showed signs of recovery. Sequential growth is seen in inpatient volumes, revenue and EBITDA as is highlighted in our numbers.

During the quarter, we have strengthened our clinical capabilities in Lucknow by on -boardin

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six senior directors across various specialties. As part of our commitment to innovation and technology, we introduced the Da Vinci Xi Surgical Robot at Medanta Lucknow, further elevating our surgical precision and complex care capabilities in Lucknow.

Aligning with our mission to provide inpatient care access across all types of patients, Medanta Lucknow has already enrolled in the Ayushman Bharat and Deen Dayal Health Schemes providing cardiac and radiation oncology services to a wider patient base.

Furthermore, as of October 2024, we have secured the empanelment of the Central Government Health Scheme (CGHS) expanding our reach within the community. We continue to make further investments in medical equipment and clinical talent additions at our Lucknow unit.

As far as our growth outlook comes for the future , in the first half of FY25, we have significantly expanded our capacity, adding approximately 280 beds across our network with 150 beds coming in Lucknow, 84 beds in Patna and 50 beds in our Gurugram facility. Our focus is on ramping up the occupancy across these new beds, which will include enhancing clinical teams and investing in advanced technology to ensure seamless and high-quality care.

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We are also planning an additional 50 beds in Patna, which we hope to come on board in the next coming months. Furthermore, we have capacity within our current infrastructure to add 200 more beds in Lucknow and approximately 150 beds in Patna, which we will bring online in the near future.

Construction of our Noida facility is progressing well and we anticipate opening in Q1 FY2026. This expansion wi ll be an important addition and is in the next step towards serving the Noida and East Delhi catchment of the Delhi NCR region.

I am also pleased to share that as of last week, Medanta has signed an Operations and Management Agreement to operate and manage a state -of-the-art 750 bed super specialty hospital in Pitampura, New Delhi. This facility developed in partnership with the Dr. Narayan

Dutt Shrimali Foundation International Charitable Trust Society is strategically positioned to address the healthcare needs of Northwest Delhi and its surrounding regions.

With this, I request the operator to open the line for questions. Thank you.

Moderator: Thank you very much. We will now begin the question -and-answer session. We have the first question from the line of Tushar Manudhane from Motilal Oswal Financial Services. Please go ahead.

Tushar Manudhane: Yes, thanks for the opportunity. So firstly, if you could highlight the surgical medical mix for developing hospitals, for the quarter.

Pankaj Sahni: Not sure that I have that number off the top of my head, Tushar. But if you look at our revenue mix in the quarter, and this is for the group I'm talking about not developing, approximately 21% is in Cardiac, 12% is GI and Gastroenterology and about 14% is Cancer. This is on our investor presentation in the revenue mix slide, on slide 11.

Tushar Manudhane: I didn't mean the case mix wise. I meant there has been a decline in ARPOB, but the revenue has grown, which implies that the volume has grown significantly for the developing hospitals.

So maybe there has been a higher proportion of OPD. Is that the way to understand? And hence the medical mix is higher for the quarter for developing hospitals?

Pankaj Sahni: No, I don't think there has been a significant change in our OP to IP mix. I think that the two points, as I mentioned very briefly, one is that in the developing hospitals, we include Lucknow and Patna. Obviously, Patna has grown very well and has continued to add a greater share of the total revenue for the developing hospitals. And as you're aware, Patna has a lower realization and therefore a lower ARPOB. So to some extent, the developing hospitals will have a dilutive

effect

effect on the ARPOB as the share of Patna increases in that portfolio pool.

Now, our Lucknow Hospital actually has completed five years in November. So that, I guess, in many ways is now moving from being in the developing stage to being in the mature space if you use the five-year timeline. But that is one of the reasons.

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The second reason is that we have definitely, as with every other institution, seen a lot of growth in our Cancer work. And as you're aware, the Cancer work is maybe at a slightly more medical orientation if you take Medical Oncology or Radiation Orientation.

Although, this may add actually to an increased ARPOB because the length of stay of some of these patients is lower. So I think that the major reason which you will find for the ARPOB share change is because of the increased mix of Patna in the developing hospital portfolio.

Tushar Manudhane: Got you, sir. And from a capex point of view, Indore project is also sort of stuck because of the litigation. Delhi project is also, if you want to clarify how temporary the assault is. And given that we have new Pitampura project and the Mumbai project, so if you could lay out the kind of capex outflow for say second half of FY 25, FY26 and FY27.

Pankaj Sahni: So let me just give you a quick overview on the projects and I'll ask Yogesh to walk us through on the capex. You are right, Tushar. There has been a delay in both Indore as well as our South Delhi project. And that is unfortunate even as far as we are concerned. Our Indore project, as you rightly pointed out, is stuck in litigation. We are, of course, exploring all other as well as current opportunities in Indore.

And our DLF project or a partnership with DLF in Delhi is very much on track. Unfortunately, given some of the pollution challenges in Delhi, we have been a little constrained on some of the construction activity. But that project will happen, although it is delayed as well, unfortunately, but that project is fully on track as far as our intention and our partnership continues. Let me hand over to Yogesh who can maybe share some of the capex numbers for the next few years.

Yogesh Kumar Gupta: So capex numbers, I think we have already shared this on the slide 23 of the investor deck. If you see that what we are maintaining here is that there will be about INR 470 crores of capex on the existing units. And then the INR 500 crores on the Noida which will be spent in the somewhere Q4 FY25 and Q1 of the next financial year when we order major medical equipment, etcetera.

South Delhi and Indore is something which are delayed a little right now. South Delhi may come up, but Indore is delayed for a longer period. Rest, Mumbai, Pitampura, Medical School, Guest House, all these projects will get spent maybe equally over the next three to four years. For hospitals, major chunk of medical equipment will get spent in the last part of the development of those hospitals.

So we haven't broken up right now into the financial year wise as you asked, but what we have given is the overall strength on these projects. And except for Indore and South Delhi where we don't know right now how much time it will take, but all other projects will get completed in the

next three to four years.

Tushar Manudhane: Thank you, sir. I have some more questions and I'll come back in the queue .

Moderator: Thank you. The next question is from the line of Damayanti Kerai from HSBC. Please go ahead.

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Damayanti Kerai: Good evening and thank you for the opportunity. My question is again on status of DLF and Indore. So Indore unit you menti oned is pushed slightly longer, but on DLF, can you clarify like by when we should be getting complete clarity on all the aspects, whether it's environmental o

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some other pending issues?

Pankaj Sahni: So DLF facility, if you recall, I may have mentioned in our last quarter earnings call that we were going to proceed with the barricading and soil testing of our site there in South Delhi. That work actually got stalled because of the pollution requirements in Delhi. And so that has create d a little bit of delay, but that work we will commence. There is no other major challenge there on one part of that.

On the other part of it, we are actually working out to see how we can look at addressing some of the concerns that exist with respect to the site and some of the challenges that may be on the site. But the work will progress and the project will progress.

I would say that if you look at it on an overall basis, I think it's probably been approximately one year that this project has been delayed because of the various compliances and approvals that we've been trying to get. But the project will continue and is on .

As far as the Indore project goes, I think there the situation is a little bit more ambiguous. And that is why we have also commenced alternative thoughts in Indore to see how we can look at expanding our capacity. But we continue to work with our partners there to resolve these legal issues and continue to work with them to try to get these cases e xpedited and as you may be aware, I've mentioned in the past, currently the landlord has won the case which people had put against saying that there was an inappropriate transfer, etcetera.

So we are just waiting for the appeals process to play itself out before we go ahead there. But a little bit less visibility, frankly, in Indore than in Delhi. And that's why we are looking at what all other options can be played out in Indore.

Damayanti Kerai: Just a clarity on DLF. So you have two blocks, right? An d you started work on one side and then you're waiting for some clearance on the other block. Is that the situation?

Pankaj Sahni: Correct. Right now we are waiting for the Delhi Government pollution card requirements to allow for construction and to begin the soil testing.

Damayanti Kerai: My second question is on your margin profile for the developed hospital clusters. So I guess it's more consistent at mid -20s. So do you see more scope to improve from these levels or this is more of a steady level which should continue ahead also?

Pankaj Sahni: So if you look at the matured one, you will see that the margins have moved around 25 %, 25.5 %, 25.3%. So yes, if you look at the last several quarters, it does appear that they are reasonably stable. I don't want to say though that there is no opportunity to become more efficient. I do believe that there is always an opportunity to become more efficient.

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But we've repeatedly maintained this stance that we would like to ensure that we're able to provide the patients with a very reasonable price of care and given the quality and the services that we offer. So we have not gone all out to jack up the tariffs and you would have seen that over not only last quarters, but last several years, we've been very conservative with tariff increases.

So of course, there are opportunities. The question is that how aggressive do you want to be just to extract every last basis point of margin? I think that we feel that a slow and steady approach is much better. But yes, there are some efficiencies that we believe that we can look into.

Although when you look at the industry trends in terms of material costs or manpower costs, they seem to be very much in line with some of the best-in-class peers.

Damayanti Kerai: And my last question is on your empanelment proceeding in Lucknow. So you mentioned you are done with Ayushman, Deen Dayal and maybe CGHS. So out of the total bed, how much is currently going for these patients?

Pankaj Sahni: So we don't

have any bed allocation for these patients. So it is not like we reserve X percent of beds for these patients, like some of the least requirements in certain hospitals in Delhi and other areas. There's no such requirement for any beds. This is really a function of how patients come and how we want to service them and how the various specialties apply.

I think the larger thought is that our intention has always been to service every kind of patient from every category. And we had maintained always that once we reach a certain bed threshold, which I think in March we had crossed the 700 number or thereabouts, that we would go forward with all these various empanelment to ensure that our reach is higher.

I think if you look at our peer mix across the group, as well as even if you look at it on developing versus mature hospitals, I think almost 80 % to 85% in Lucknow, probably even higher, business is coming from cash and TPA. So, like we've always said, 10 % - 15%, if you do business at scheme rates, nothing wrong.

Damayanti Kerai: So that's helpful. Thank you very much.

Moderator: Thank you. We have the next question from the line of Alankar Garude from Kotak Institutional Equities . Please go ahead.

Alankar Garude: Hi, good evening, everyone. So in the previous call, you had outlined a host of issues around Lucknow, including competition, approach of some of your competitors, availability of clinicians, community connect, et cetera. So while there would have been a seasonal benefit at Lucknow in Lucknow in this quarter, similar to the other facilities, can you comment on how has been our progress in addressing some of these challenges seen over the last few months? And what are our expectations going forward?

Pankaj Sahni: So, you're right in the last quarter, I had mentioned a couple of improvement opportunities that existed for us as a group , specifically in Lucknow. And if you go through the last couple of quarters, you will see that there's been some fairly aggressive clinical talent additions in

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Lucknow. So that was one intervention that we made. And by the way, we will continue to make that.

What we're finding actually is that as happened in every other city where Medanta has opened, the baseline of healthcare has improved because we have gone in, we have set up a new system. Doctors have come from all over. Other hospitals have imitated us and come following us into the various catchments that we've gone into.

And that actually is good because it elevates the quality of care in the community. It elevates the health ecosystem of that community and that correspondingly transfers to greater availability of clinical talent and so on. So we have done aggressive hiring. Six department heads have been hired in the last three months. And we continue to do aggressive hiring and you will see that playing out as we move forward, not only in quarters, but over years actually.

The second thing that we've done is that there's been a pretty significant amount of investment. And we had mentioned in the last call that we don't look at this quarter to quarter , we invest for the long run and we've invested both in terms of a Da Vinci Xi Robot recently, which is the most visible equipment, but other equipment as well, which may be less known. And we continue to do that into this quarter as well across the specialties. So that's the second thing, which is investment in medical equipment.

The third thing is that we've continued the bed build out. So you will see that every quarter, some beds are getting added and we will look at how that bed build out should continue, but also looking at adding in newer specialties that we may not have.

So to give you a very small example, we did not have an Ophthalmology service maybe six months ago at all. That has been added now. We have two s

enior doctors that we brought on

board. And so it's a very small example, but an example nonetheless. Similarly, there are other departments which we did not have or had at some scale and we have added those in. So the service offerings have increased. That's the third thing. And we hope to continue to increase those service offerings as we move forward.

The fourth thing, as I'd mentioned, is that obviously we have got a greater sense of touch base with the community. We've already signed up several residential communities where we now have our implanted clinics . I think soon we will be in double digits in that and the idea is to take that forward. We've connected with very close by and community -based nursing homes, doctors' hospitals, et cetera. So almost 100 touch points on that front. And so there's a lot of action which is happening at the community level itself. Not to mention the increases in our awareness, et cetera.

Also, we are now five years in Lucknow. This November, we would have completed five years in Lucknow. So you can say, success, commitment, the embrace that we have received from the Lucknow community over five years is something that we very much value. And we are not in

this for even five years. We are in this for the next 500 years. So the way we want to think about this is what do we need to do to continue to scale up? So I personally believe that there's a lot more to be done.

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We are by no means over in the work which we need to do in Lucknow. At the same time, we see a lot of positive momentum and a lot of excitement in the unit as well as in the city. But once again, if I take the example of Gurgaon, we've been running this facility for 15 years. Over the 15 years that we've been in Gurgaon, there have been multiple quarters or years where things go up and down. So I don't think that that worries us too much. We've got to keep our head down and focus on what we stand for, which is to continue to deliver very high-end care at the right ecosystem, right values, and hopefully at the right price, which our communities and our patients can afford.

Alankar Garude: That's very helpful, Pankaj. A couple of follow-ups there. One is, in terms of competitive intensity, what is your assessment now? Are we in a slightly better position today as far as the market is concerned versus six months back? Or it has only got worse? Any thoughts on that front? That's one.

And the second question is, from a margin perspective in Lucknow, of course, we have come down a lot from our peak levels. I think last time you had mentioned that the recovery will be gradual. So some of these initiatives you spoke about, should we expect the recovery to continue to be gradual as far as our margin trajectory in Lucknow is concerned?

Pankaj Sahni: I think Alankar, after having interacted with me for so long, you'll always know that my answer will always be that everything should be gradual. Nothing should be in radical terms. So I would not like to answer that question any other way, but to say that, look, everything that we do moves the organization forward in the right direction.

The financial results that come out at the end of it are an outcome of our efforts. We don't run the company to deliver only the financial results. So I would say, look, allow us the time to get all the things that we need to do to make Lucknow finally a thousand-bed mature facility. We are still not there. While it has been five years, it's still a growing facility.

We are still adding talent. We are still building beds. We are still adding equipment. We are investing in cath labs. We are investing in robots. We are investing in OT weld-outs. A lot of action is happening there. So I don't necessarily think that these things can be tracked. Now, let's say I start a new department. If that department

ent takes off very suddenly, it's very hard to predict.

If you bring in a new doctor, that doctor takes six, nine months to get up and running, that's absolutely fine.

So I don't think that it's very easy for us to lay that forward, and neither are we hiring the clinician, thinking that, who can give me a financial return in the next three months? We look at hiring exceptionally talented doctors that will be with us for as long as possible and deliver very high-quality care. So that's how we've looked at it. That being said, yes, we've had a fairly positive momentum shift, potentially at faster than what other people would have expected.

But it is not like we had a timeline in mind for this. We said, let's put our head down and get things fixed, especially where it was not done. And one of the most significant feedback, as you rightly mentioned, was this absence of a community connect. If you ask me, are we done with our community connect, I would say, no, not at all. We are just getting started.

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So I think that there's a lot more to do, and I would urge that people look at us as a more stable growth organization than radical organization on that front. The second thing that I would say is that when you look at the margin profile of Lucknow, as you are aware, we have a model which our doctors are all full-timers and are kind of on fixed models. So obviously, the volume growth has a very disproportionate impact on margin because our marginal cost to serve every incremental patient is potentially lower than others.

So you can see that very clearly in the numbers when you look at quarter to quarter. As the volumes increase, we do believe that the margin profile will improve. So our focus is to continue to do that, continue to drive the growth of the organization through volumes as opposed to price. We still haven't taken a single tariff increase in Lucknow in five years. And that should give an indication of the approach and philosophy of the organization.

Alankar Garude: Fair enough, Pankaj, that's helpful. The second question is, you briefly spoke about the ups and downs over the years at Gurgaon. Now, if you look at the next three to five years, there are roughly 3,000 beds coming up in the city by leading multi-specialty chains. So while we might have seen intermittent competition over the years, the intensity of competition coming up over the next, say, three to five years seems to be much higher than what we have ever seen in the past. So, I mean, firstly, how do you expect the supply-demand equation to evolve in the city? And secondly, how do we plan to ensure minimum doctor attrition in the market? Thank you.

Pankaj Sahni: Okay. So let me try to break that down into, I think, three different questions. The first one, let's talk about how do we think about the competition in the city or the Delhi NCR region? I think the first very important point is that we have to think of Delhi NCR as multiple different cities. So West Delhi behaves very differently from Noida, Noida behaves very differently from Gurgaon. And I would assume that even Gurgaon behaves maybe differently from, say, South Delhi or East Delhi.

So really, Delhi is such a huge population, or the Delhi region is such a huge population, is really multiple catchment areas within itself. Now, obviously, Medanta as an institution has always been lucky that we have been able to capture a patient base that extends far beyond Delhi NCR. I think I may have mentioned it before, almost more than 50% of the patients from Delhi in our Gurgaon facility come from outside the Gurgaon region. So we do get a lot of patients who come from out of town because of the nature of the institution that we are, which is the destination care that we've talked about. To the extent that continues, I don't see that, or I don't see any reasons

on why that would drastically change because we will continue to be providing care at that standard. And so therefore, I think my only caution would be, don't look at it so simplistically to say that it's one city. It's not.

The second thing is that when you look at the competitive intensity, see, it's not like there hasn't been competition before, there has. It's just that it has been less visible because the industry as a whole, less listed players, less awareness on what is happening with respect to the results. Maybe many were more private. It was more a private equity game. Today, it seems to be more of a capital game. There has also been a very clear shift of capital inflow into this industry, especially post COVID.

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And so frankly, a lot more excitement from the financial community. I don't think that the, at least in our case as operators, we have really changed our thought, but it's been a lot more excitement and interest from the financial community. And therefore, the flow of capital has meant that people are interested in building hospitals. Now, as I've repeatedly maintained, building hospitals is easy. It just takes money and land. Running hospitals is far harder. And you see that with the respect to the number of hospitals not doing so well or coming up for sale. And there are people trying to make a quick buck and flip hospitals over two, three years. That game is not for us. So there will be this game that will happen. Keep in mind, adding capacity takes time. It takes three to four years to build out a hospital. It takes another two, three years to get that hospital stable.

So it's not like all this capacity is coming tomorrow. Most of the capacity that has been announced, including ours, it has to come from the ground up. So you're talking three, four years before it's built. So if you look at the portfolio across healthcare chains, it's not like this capacity is coming on board in the next six months. It will take time. And even after it takes time, there will be a gestation period.

So I don't worry at all about the capacity because the amount of time it takes, there will be a huge demand. Today, if we complete all the capacity announced by every player, tomorrow morning, it's still less from a supply-demand point of view. So supply-demand doesn't worry me at all.

And the last point that I will say is that, look, the push towards quality healthcare is very real post-COVID. The paying power as the economy grows is improving. People feel the need for high-quality care. These are all very positive signs for the industry and for the economy and for the country. And again, I take you back 15 years ago when Medanta Gurgaon opened, people used to ask, what are you doing in Gurgaon? And today, people talk about Gurgaon as a healthcare hub. I mean, who do you think made it the healthcare hub? It is people like us, people like other peers who have come and built into these institutions.

So the creation of these healthcare hubs is a good thing. It will add clinical talent. It will add availability of high-quality care for the communities. And hopefully, it will become a destination not only for India, but for patients from around the world. So I'm not at all worried. I think whatever is happening in healthcare growth is a good thing. And I think that if we double or

triple it, it's still not enough for a country like us.

Alankar Garude: Thank you, Pankaj. I had a small follow-up, but yes, maybe I'll come back in the queue. Thank you.

Moderator: Thank you. The next question is from the line of Sumit Gupta from Centrum. Please go ahead.

Sumit Gupta: Thank you for the opportunity. So just want to ask on the matured hospital side, so like what is driving the overall ARPOB growth? So is it the mix or what kind of prices that you have taken?

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Pankaj Sahni: So I don't know if you had kind of followed some of our earlier calls. You know, for many years, right after COVID, we hadn't taken much of a tariff increase. In fact, zero tariff increase. Starting, I believe, January 2023, if my dates are correct, we started to take small amounts of tariff increase across different specialties and that continued almost for a year. And that also includes some of the renegotiations with the insurance companies.

So some of the growth which you're seeing in realization is because of the tariff increase that we took, over the course of let's say last one year, which is playing out both on the cash as well as on the insurance side. The other part of it is that, you know, there has been some amount of movement towards complex work. So we now actually have, as I mentioned in the comments, a second robot, Da Vinci Xi Robot in Gurgaon.

So we are running two robots here. In fact, we used to have three and we may even add more than that to really build out a complete robotic program. So the complexity of the case mix is also laid out as one of the reasons. And as you start to get more and more complex work happening, you see that the ARPOBs of the realizations will grow. So I would say that it's a combination of a tariff increase that we've taken in Gurgaon and Indore. We haven't yet taken it in Ranchi and some amounts of increased complexity of work, things like robotics, things like some of our other complex procedures, TAVI procedures, have grown very significantly in cardiology. CAR T-cell is another example of a complex procedure in medical oncology and hematology, et cetera. So I think it's a combination of both of those things is what I would say.

Sumit Gupta: And the second question, on the margin side of matured hospitals only. So over the next two to three years or four years down the line, how do you see the overall margin profile of matured hospitals increasing, considering you are deploying all the robotics and everything improving the ALOS? So just want to understand on that as well.

Pankaj Sahni: So I think we have to look at the overall margin profile of the matured hospitals in lights of three things. The first is that today we are operating these facilities at somewhere around a 67% - 68% occupancy level. So if you think about given our model, as I mentioned earlier, of fixed cost, every percentage point or two of increased occupancy, obviously adds disproportionately to the bottom line because of the marginal profitability model.

The second thing, as I mentioned, is that if I look at it objectively, there are probably some inefficiencies in the system, whether it is material cost, whether it is manpower cost, et cetera, that can be pulled out of the system. And we should work on doing that. We've already started some activities on optimizing things around supply chain, et cetera.

But one very important point as you build out your models, et cetera, to keep in mind is that our complete corporate costs and our shared services costs are fully loaded on our mature facilities, in fact, on our Gurgaon facilities. So to that extent, if you're comparing just hospital unit to hospital unit, these margins, which you see may be a little bit subdued because they include the corporate costs of the whole group, both on manpower side, as well as on other shared services side, like, call centers and IT and things like that. So these are the two, three things.

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The other thing that I would say is that, look, we do believe that we should continue to remain conservative in our tariff increases. Obviously, the easiest way to bump up the margin is to take more aggressive tariff increases. If you look at our list tariff, we'll still be pretty low compared to some of the other

players in the NCR region. But I would not assume that we will use tariff as an overly aggressive lever, just for the sake of boosting margins. I don't know if that answers your question.

Sumit Gupta: Understood sir, thank you.

Moderator: Thank you. The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services. Please go ahead.

Tushar Manudhane: Thanks for the opportunity again. So on this Pitampura project, it is O&M, but at the same time, it's a JV. So if you could just firstly help us understand in terms of the overall project capex, and out of which, let's say we are willing to spend INR 600 crores, what kind of regulatory approvals would be required for this project? If you could share some more details related to this project, thanks.

Pankaj Sahni: Sure. Just to clarify, Tushar, the project is not a JV. It is a pure O&M transaction. The building, the land, everything will continue to be in the hands of the society. We have a contract to operate and manage the hospital. In fact, there was a complete process that was run by the society. We have been able to win that contract to operate this. And like with any other O&M, there's kind of a rent and a revenue share that will be paid out to the society.

And we will have the complete autonomy for running the medical operations. But the society will continue to be the owner of the land, continue to be the owner of the building. And all the, you can say basic licenses which are pertaining to the site or the property, those will be kind of obtained by the society. And we will obviously take all the responsibility of the complete medical and operational aspects of the hospital.

The site, as you would have read in the release, is about seven acres. The society actually also has an Ayurvedic facility over there, which they have the desire to scale up. And so on the same site, they may be putting their services in, which we've always believed is very complimentary to us. But on the Allopathic medicine site, Medanta will be running it as a pure O&M with a rent or a revenue share kind of a model being paid to them.

Yogesh Gupta: And this Ayurvedic hospital will be outside of the Allopathic hospital of 750 beds.

Pankaj Sahni: Yes, so that will be outside of the scope of this contract. The society continues to run that on their own. As far as capex, I would assume that the partner would be putting in somewhere INR 200 crores or maybe a little bit more, INR 200 - 250 crores of capex because they'll be putting up the civil structure, etcetera. And we will be putting in the services and the medical equipment and of course, the finishing to operate it.

Tushar Manudhane: Understood, sir. Thank you so much. Thank you.

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Moderator: Thank you. The next question is from the line of Alankar Garude from Kotak Institutional Equities. Please go ahead.

Alankar Garude: One question on Gurgaon again. Now, for any upcoming corporate hospital in Gurgaon, Medanta will be a key hunting ground for clinical talent. Now, we had seen about seven, eight years back impact of senior doctor attrition in this facility. So the question is, how do we plan on minimizing the impact this time around?

Pankaj Sahni: I think Alankar, sorry, you may have asked that question earlier. I forgot to answer it. My apologies. So you're right. We did see three senior department heads leave in 2016 and all the three senior guys came back within two years. Something must have attracted them to come back to us. That being said, I think there are three, four things.

One is that times are a little different now with the breadth of clinical talent that is available both within what Medanta has been able to develop, as well as what is available in the market or in the city. And interestingly, more talent continues to

come into the Delhi NCR region. So I think

that while talent is always in short supply and always the most critical, it's very different world from what it was, say, eight, ten years ago.

The second point is, then I did mention this, I think in our last call. I do foresee in the industry across the board, not only in Delhi, this kind of war for talent, so to speak, across not only clinical, but maybe even non-clinical areas. And I do believe that this is something that has the potential to create this potential short-term orientation for increasing the cost on the clinical talent or this idea that people will look to us to coach.

But I would only revert back to our last, maybe 12 months or so, forget now what's the exact timeline is. In a hospital that's 13 - 14 years old, we have still hired in very senior talent. If I recall, that was almost somewhere around 80 senior doctors that had been hired in last year in a 13 - 14-year-old facility. So we have never stopped looking at high-quality talent. This is something that given our size and our scale and frankly, the growth that's coming up with our opportunities to move our talent across various parts of NCR and various parts of the network, we will continue to hire aggressively.

But yes, we will, I'm sure, be a hunting ground. I guess in many ways, we should look at it that we should be flattered, that people feel that Medanta has the best clinicians around and they want to come and get them for us. But equally, we get a lot of positive impact from our clinicians

saying that this is the most doctor -friendly institution in the healthcare space. So by and large, I think most of the doctors, I would hope, are happy to come and work with us. And only last thing I would add is that, yes, you have to compensate your doctors well, you have to respect them well and you have to give them the necessary platform. I don't think anybody has a better platform than Medanta across any of its units. And this is evident in the excitement that doctors are showing in joining us in Noida. So the hospital is not even open, but we're already starting to get lots and lots of inquiries, including from senior clinicians from across the private and government space.

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So I think that there will be challenges in getting good talent across the industry given the number of hospitals coming up. But we feel we are probably at a position where we are one of the preferred employers of clinical talent.

Alankar Garude : I understood. That's helpful, Pankaj. The other question is, you spoke about 85% odd mix in Lucknow from cash and insurance. Possible to comment on same mix in Patna? How has been the PPP offtake till now?

Pankaj Sahni: Lucknow actually may very well be more than 85%. I was actually referring to the group as a whole. Lucknow, I would say, has very little scheme business. Because I think the numbers which you are seeing in September don't even represent CGHS. We had taken Ayushman only in, I think, April or May. So very negligible scheme business, I would say, in Lucknow. I would assume it would be far higher than 85%, but we can double check on that and get back to you. I think that in Patna, we started out somewhere around 10% - 11%, if I'm not mistaken, of the PPP business, which is the only kind of scheme business we have. I think now it's probably heading up a little bit more, maybe closer to 15% - 20%. But I think about 80% of our business is still cash and insurance in Patna.

Alankar Garude : And this is as a percentage of volumes, right?

Pankaj Sahni: No, I'm talking about revenue. I would assume that volumes would actually be more or less the same. Actually, the realizations of our PPP business in Patna are pretty high. So it's 19% is the PPP volumes and 11% on revenue.

Alankar Garude : Fair enough, understood. That's ver

y helpful. Thank you and all the best.

Moderator: Thank you. The next question is from the line of Amey Chalke from JM Financial. Please go ahead.

Amey Chalke: Thank you for the opportunity. I have first question on the Medicity where we are building Guest Houses and we are spending around INR 250 crores for the same. The way I see it, it will help us obviously to generate revenue from the Guest House as well as it will help us to improve our patient mix towards international patients or the outside patients.

Is that understanding, correct? And also in relation to that, if we can also provide international patient mix for the Medicity and number of guest houses we plan to or the rooms which we plan to build through this INR 250 crores spend.

Pankaj Sahni: Yes, so you may be aware that the Medicity campus is almost a 43-acre campus across our Gurgaon area with about 25 acres is the area for the hospital. And in the area which is the 25 acres, we do intend to put up a medical school also. So the actual area which is utilized for the real hospital is only 12 and a half acres currently. That being said, let me first touch on the guest house.

The guest house land, which I believe is about six acres of land, we intend to actually use less than half of it. And we intend to put up a service apartment or a guest house. And as you very

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rightly pointed out, it is not only for international patients, it is also for patients who come from out of town. Or to give you a very simple example, if you look at patients who come for chemotherapy, they may not even need to stay in the hospital. So they are staying in all kinds of unauthorized and unofficial guest houses here and there, which have no real standards or no real basis even on things like infection control, etcetera.

So our ability to provide our patients and their families with a safe and reasonable accommodation was the driving force behind moving this and that was part of the original plan of Medicity. That's why the name is about medical city, right. And what we've done is that we've decided to go ahead and do this ourselves. And actually work out possibly different types of

rooms. Some may have even extended stay facilities.

So in a scenario where you are running short of beds and patients could be discharged one or two days early, or they require some kind of home care or extended stay support that also can be provided. So I think there are many, many options that are available to us. How we actually utilize this facility. And what you see increasing are the trends around extended care, elderly care, assisted living. So there are lots of options that play out once this facility is up. And then of course, even some of the supportive facilities and nice big hall for CMEs, etcetera. There are many things that add to the entire campus and fit in fairly seamlessly. So we feel pretty good about putting this up. 250 keys is what we're planning right now. And we'll see how it goes. Like I said, we are building on less than half of the actual land. So if it is a success, maybe we need to scale it up.

Amey Chalke: Sure. What will be the rough timeline for this?

Pankaj Sahni: Well, normally it takes about three to four years for anything to get up and running. Obviously a little less complex than building a hospital because you don't have office rooms and all those fancy things inside. But I think three to four is a good timeline for you to think about.

Amey Chalke: Sure. And the second thing which you also mentioned is about the medical college. Is it any plan in place in terms of number of seats, anything to spend? Because that has not been disclosed on the present.

Pankaj Sahni: So medical college is something we're very excited about actually. The reason it has not been disclosed is because the drawings and the

finalization of that is not exactly confirmed yet. We've actually put in applications to the state government for moving this process forward. And as you may be aware, subsequent to that, NMC will require to get your approval from. The way it works is that in your first year you can only get application, you can only get approval for 100 seats. Now that can scale up.

There are some changes that are coming in the various guidelines for medical schools after the NMC has taken over from the erstwhile MCI. So earlier you could go up to 200 seats or so. I think now that number is 150, but you have to start with 100. That's the rule that we have. So if you think about it from the point of view of five -year undergraduate program, that would be 500

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students, plus if you think about a two -year postgraduate program after that, and then a super specialized program. So I think these are things that are still being worked out.

What we believe is that the medical school building will probably be somewhere around 4 lakh or 5 lakh square feet of build -out area out of the total one million square feet of FSI, which is left for us on the 25 acres of the hospital medical school area. So about less than half of that we are utilizing. And I think our estimate is that 4 lakhs building like that should probably also cost somewhere around INR 250 crores, maybe thereabouts INR 300 crores, let's say, if you budget.

Amey Chalke: The third question I have is related to the ARPOB and the casework mix movement in the second half. You expect it to be better for developing hospital or what could be the trend in the second half for the developing hospital?

Pankaj Sahni: Sorry, just to clarify, not only for you, but for everybody on the call, the Investor Presentation does actually have the capex laid out of INR 300 crores for the medical school on slide 23. Could you repeat your question in terms of ARPOB for?

Amey Chalke: ARPOB s and case mix for developing hospital, you expect them to be better in the second half of FY25?

Pankaj Sahni: I mean, I don't know that I would classify something as better or worse. The way I think about this is that the case mix is a function of three things, right? First is that depending on what you're defining, first is the actual specialties. So if you look at our last six months in Lucknow, there are certain specialties that didn't really even exist. One example is plastic surgery. This specialty didn't exist really. It's come in, I think in the last three to four months, grown very quickly. And so to the extent that new specialties get added, I do see that the case mix may change. I don't consider one specialty or another to be better or worse, because I think it really depends on what metric you're looking at. If you look at ARPOB, you may say somewhat secondary care specialties with shorter length of stay may be better. If you're looking at how complex is the work, you may say that transplant is better, but may have a longer length of stay. So I'm not sure what your definition is.

The second thing is that depending on some of the clinical talent additions that we have, because in addition to new specialties, we are actually hiring second, third teams in existing specialties. And that will change as those specialties come on board. So obviously if I hire a second doctor in cardiology or cardiac surgery or neurology, that grows as the doctor comes on board. So that will change over time.

And the third thing, of course, is the complexity of the cases. So with a robot coming into Lucknow, you now have robotic cases, which wasn't there for the last three, four years. So that changes, although it's the same specialty, the nature of treatment within the specialty changes. I think that the way in which I would answer this for us as Medanta and for Lucknow in general, is

that when we think about taking a hospital from 400 beds to 750 beds, and then from 750 beds to about 950 beds, you are continually adding in what is the service needs and what are the patient needs of that area.

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Now, we did not anticipate, for example, two years ago or one and a half years ago, this much of a demand for our mother and child services. But it has been far greater than what we anticipated. And therefore, we may give them additional space. In a similar scenario, we may see things as we build out from 700 to 900, that may change, it may move on. So we are not going to be adding in specialties or changing case mix purely to drive one financial metric like ARPOB or revenue or anything like that. It would be totally dependent on the need of the community, and what is the clinical talent that we are able to put into place to deliver and service that need. The financial outcome will be what it is.

Amey Chalke: Sure, sure. Thank you so much.

Moderator: The next question is from the line of Sumit Gupta from Centrum. Please go ahead.

Sumit Gupta: Hi, thanks for the opportunity again. So I was talking about the international patient revenue mix. So basically, pre-COVID it was at around 11% to 12%. Now that we have, like post-COVID, it is stabilized around 6%. So going forward, how do we see the international patient contributing to the overall revenue over the next three to four years, considering the majority of the incremental bed capacity is also going to come in Delhi NCR only?

Pankaj Sahni: So, Sumit, I think first of all, let me just clarify some of the numbers. Pre-COVID was really only Gurgaon and maybe to some extent Indore and Ranchi. So international business is only really in Gurgaon. As Lucknow and Patna contribute to the total pool, the share of international business will naturally reduce. And that's why you're seeing it as 6% because of the fact that Lucknow and Patna don't have any international business.

Definitely as NCR facilities come on board, there should be an increase in the share of international business that comes on board, because obviously Delhi will get a much larger share of international patients than Lucknow or Patna. And there is now an airport coming up in Noida, which I'm told will get activated within the year. Then there is Delhi facility that we have, which will have international patients and of course, let's not forget our Mumbai facility, which will also have access to international patients.

So I think as the units and the cities where those units are, that profile changes, our international patient profile will grow. That being said, international business is also dependent on a lot of external geopolitical factors. Afghanistan was a big source of international patients. For India, that has become basically zero. These days, we see challenges in Bangladesh, which is another big source of international business. So I think that international business will be there. But given what happens across various countries that goes up and down as well. So obviously, the core will be the baseline domestic business.

Sumit Gupta: So just one follow up. So basically, considering the geopolitical situation in Bangladesh, so how much of this Bangladesh contributed to your international revenue? Second is, if there is any negative, then have you shifted your focus towards other countries also?

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Pankaj Sahni: So the way in which it works, Sumit, is that we regularly look at all the various countries that we can attract, right? And we try to look at certain newer, challenging markets. So for example, Medanta gets a lot of patients from say Rwanda, right? Now, this is not a very well-known country that sends too many international patients to India. But we i

ndex quite highly on that.

Similarly, we used to be one of the only few hospitals that was getting patients from the CIS countries.

As those markets opened up, many more people have followed us into those countries. And this will kind of continue. So I think that there's a lot of factors that play. Sometimes the government

of a particular country will be very keen to interact and engage with the government of India through ministerial functions, and that opens up a pipeline. I think just Bangladesh's familiarity with India and the way in which patients and the seamless movement of the populations was obviously one of the reasons why it was one of the leading and the maximum number of medical visas in India were given to Bangladesh.

But again, it's such a negligible part of our total business. I don't think it moves the needle in any which way. And I'm sure Bangladesh will come back. These are temporary blips. But our focus is not that we are not looking at Africa, Middle East, CIS, we continue to look at all these regions. And I'm sure Bangladesh will stabilize and come back into the fold because these places don't have the kind of care and clinical care that we can offer.

Sumit Gupta: Thank you, sir.

Moderator: Thank you very much. I will now hand the conference over to the management for closing comments.

Pankaj Sahni: Thank you, everyone. I know it's late on a Thursday evening, but thank you for your questions and for joining us today. As we conclude this call, I'd just like to reiterate our commitment to delivering excellence in healthcare, advancing our mission to provide world class medical services, and also making sure that we are doing this with a slightly longer term, high quality, high ethics ecosystem and values that we have always talked about.

As we look ahead, we are very excited about the various opportunities that we've outlined, whether it is the new facility and partnership in Pitampura or Bombay, or some of the other cities that we have outlined in the various interactions that we've had.

We remain focused on delivering what we have always said, strengthening our core services in our existing facilities, expanding our reach and patient care into newer areas where we believe we can make a difference and driving sustainable ethical growth. Thank you for joining us once again. And please feel free to reach out to our Investor Relations team, in case any questions have remained unanswered. Thank you.

Notes:

1. This transcript has been edited for readability and does not purport to be a verbatim record of the proceedings
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Call: Feb 2025

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Dear Sir (s),

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the transcript of Earnings Conference Call held on Wednesday , February 05 , 2025 , for the quarter and nine months ended December 31 , 2024 results .
The Transcript is also available at our website: <https://www.medanta.org/investor-relation>

This is for your information and record.

For Global Health Limited

Rahul Ranjan
Company Secretary & Compliance Officer
M. No. A17035

Encl: a/a

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"Global Health Limited - Medanta
Q3 & FY25 Earnings Conference "
February 05, 2025

MANAGEMENT : DR. NARESH TREHAN – CHAIRMAN AND MANAGING
DIRECTOR – MEDANTA – GLOBAL HEALTH LIMITED

MR. PANKAJ SAHNI – GROUP CHIEF EXECUTIVE
OFFICER AND DIRECTOR – MEDANTA – GLOBAL
HEALTH LIMITED

MR. YOGESH KUMAR GUPTA – CHIEF FINANCIAL
OFFICER – MEDANTA – GLOBAL HEALTH LIMITED

MR. RAVI GOTHWAL – HEAD INVESTOR RELATIONS –

MEDANTA – GLOBAL HEALTH LIMITED

MODERATOR : MR. SUMIT GUPTA - CENTRUM BROKING

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Moderator : Ladies and gentlemen, good day and welcome to Global Health Limited, also known as Medanta Q3 & FY25 Conference Call hosted by Centrum Broking.

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” then “0” on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Sumit Gupta from Centrum Broking. Thank you, and over to you, sir.

Sumit Gupta: Thank you. Good afternoon and a very warm welcome to all the participants on the Global Health Limited Q3 FY25 Earnings Call hosted by Centrum Broking.

Joining us today from the management team, we have Dr. Naresh Trehan – Chairman and Managing Director ; Mr. Pankaj Sahni – Group CEO and Director, Mr. Yogesh Kumar Gupta – CFO and Mr. Ravi Goyal – Head of Investor Relations .

I will now hand over the call to Dr. Trehan for his “Opening Remarks ”. Thank you and over to you, doctor.

Dr. Naresh Trehan: Welcome to the Q3 FY25 Earnings Conference Call. I hope all of you had the chance to review the results and presentations that were released yesterday.

I will begin by highlighting some of the key clinical achievements of the quarter, following which I will hand over the call to our group CEO – Mr. Pankaj Sahni, who will provide an overview of our “Financial and Operational Performance ” for Q3 and the nine months ended FY25.

Medanta continues to advance its mission of delivering world -class healthcare , with Q3 FY25 marking another quarter of steady progress and growth . Our focus on clinical excellence , operational efficiency and patient -centric care remains at the core of our success .

In Lucknow , we commissioned the Da Vinci Xi robot last quarter and our clinical teams have set a new benchmark for performing 50 robotic surgeries within the first 90 days, showcasing both the rapid adoption of advanced technology and expertise of our surgeons. At Medanta Gurugram, our multi -specialty team successfully performed neuro -intervention on a newborn with Choroidal Vein of Galen Malformation, an extremely rare condition requiring a highly specialized procedure , available at only a few centers globally.

Additionally, our vascular team at the Gurugram effectively managed a complex case of peripheral arterial disease using the Stealth Orbital which is another 360 Atherectomy system, establishing a new standard of care with no metal behind approach. That emphasizing long -term vascular health and minimizing the need for future intervention.

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We are proud to share that our four Medanta doctors have been recognized as among the world's top 2% scientists in the prestigious Stanford University Global ranking. A testament to their dedication to advancing medical sciences and delivering exceptional patient care. These recognitions reflect Medanta's continued commitment to setting benchmarks in clinical excellence and expanding access to advanced healthcare.

Furthermore, our board has approved a long -term lease agreement for a newly built 110 bedded hospital in Ranchi, strengthening Medanta's presence in the region and enhancing access to high and tertiary care. Our Noida hospital construction shall be operational within the next 3 to 6 months and then we have 1,500 beds of greenfield capacity coming up at Delhi -Mumbai market in the next three years.

With this, I will now hand over the call to our group CEO – Mr. Pankaj Sahni, to share the financials and operational highlights for the Quarter and overall , 9M FY25 Results . Thank you for your indulgence.

And Pankaj, can I hand it over to you now,

please?

Pankaj Sahni: Thank you, Dr. Trehan . Good afternoon, everyone, and welcome to our Q3 and 9M FY25 Earnings Call.

Let me begin with the financial performance for Q3 FY25 :

During the quarter, Medanta delivered a total income of Rs. 9,595 million compared to Rs. 8,545 million same quarter last year, registering a growth of 12%. EBITDA for the quarter was

Rs. 2,538 million, an increase of 8% year -on-year with an improved EBITDA margin of 27.5%. Profit after tax for the period was Rs.1 ,429 million, marking a strong year -on-year growth of 16%. PAT margins for the quarter improved to 14.9% compared to 14.5% in the same quarter last year.

Overall, our growth during the quarter was primarily driven by a combination of factors, including consistent volumes growth and strong contribution from developing hospitals. Our inpatient volumes during the quarter increased by 13% year -on-year to 44,856, and the outpatient volumes increased by 9% year -on-year. During the quarter, we have added 34 beds and 219 beds were added in the 9M FY25. Our Average Occupied Bed Days for the quarter increased by 10.5% year -on-year, with strong occupancy of approximately 64% on increased bed capacity. Average Revenue Per Occupied Bed for the quarter was Rs. 61,307 compared to Rs. 60,571 in the same quarter last year, a marginal increase of 1.3%, largely due to a change in case mix. During the quarter, revenue from international patients was Rs. 541 million compared to Rs. 473 million in the same quarter last year, an increase of 14%.

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Coming to the Matured and Developing hospital performance update :

During the quarter, revenue from our matured hospitals was Rs. 6,466 million compared to Rs. 5,875 million in the same quarter last year , registering a year-on-year growth of 10%. The EBITDA of mature hospitals stood at Rs. 1,617 million, reflecting a growth of 9% year -on-year with a margin of 25%. Average Occupied Bed Days increased by 6.8%, representing an occupancy of 63%. ARPOB grew by 3.3% to Rs. 67,303 in Q3 FY25 due to a change in case mix.

Developing hospitals which includes Lucknow and Patna continue to show year -on-year growth and sequential growth momentum. The revenue from operations during the quarter was Rs. 3 ,004 million compared to Rs. 2 ,742 million in the same quarter last year, registering a growth of 9.5%. EBITDA from these hospitals during the quarter was Rs. 1 ,015 million compared to Rs. 968 million in Q3 FY24. Average Occupied Bed Days during the quarter increased by 16 .8% year -over-year, representing an occupancy of 64% on increased bed capacity. ARPOB for the quarter was Rs. 52,502.

I would like to share some highlights on the Lucknow unit.

We are pleased with the performance of our Lucknow unit over the last few months, marked by a notable increase in patient volumes and revenue growth. Our ongoing efforts to expand market outreach and strengthen engagement with the Lucknow community continue to yield positive results.

In line with our stated objectives of serving all sections of the community in Eastern UP, we obtained empanelment with Ayushman , CGHS, Railways, as well as various corporate and PSU's . We are proud to serve the patients covered by these schemes and are encouraged by the positive response from the community.

Medanta Lucknow continues to be a preferred choice for the highest quality clinical talent. We are continuing our recruitment efforts to enhance our medical teams, to fill in any specialty gaps that remain, as well as deepen our expertise within existing departments. We continue to invest in infrastructure and advanced technologies to support sustained growth. We remain watchful on our growth in Lucknow and continue to obtain feedback from the community and remain committed to optimizing our operations to manage seasonal variations.

Now, with

respect to Medanta Patna , Medanta Patna has consistently delivered strong performance across key metrics. During the quarter, we expanded our capacity by adding 28 ICU beds, bringing the total bed additions to 112 in the nine -month period ended December 2024. In parallel, we strengthened our clinical capabilities by onboarding over 20 new doctors during this fiscal year. This combination of experienced clinical teams and enhanced infrastructure coupled with our unwavering commitment to patient -centric care, has driven higher inpatient volumes, increased occupancy, and an improved case mix, particularly in
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complex specialties. These factors have collectively supported sustained growth and reinforced Medanta Patna's position as a leading destination for complex medical care in this region. With respect to various projects , in the last nine months, Medanta has added over 200 beds across matured and developing hospitals. This includes approximately 50 beds in Gurgaon, 112 beds in Patna, and 58 beds in Lucknow. The construction of our 550 -bed hospital in Noida is in

full swing, and we are expecting to commence business operations within the next six months. I'm also pleased to report, our board has approved a long-term lease for a newly built 110-bed hospital, strategically located just 1.2 kilometers from our existing facility in Ranchi. This expansion reinforces Medanta's commitment to delivering world-class healthcare in Jharkhand and the Ranchi region. Spanning 1.33 acres with a built-up area of 1,08,000 sq. ft., this state-of-the-art facility is equipped with advanced operation theaters, high-end medical technology.

Designed to enable the highest level of tertiary medical care, this new hospital will bridge critical gaps in the region's healthcare services. It will expand our medical capabilities, which have been constrained by the existing infrastructure in our existing Ranchi hospital. As we operationalize the new hospital, we will optimize specialties and services across both sites and introduce key missing medical specialties and treatments to enhance patient care.

Overall, we have roughly 1,000 bed additions planned over the next two years, including Noida and the remaining expansion, which is happening in our existing hospitals in Lucknow, Patna and now Ranchi. We also have 3 major Greenfield projects underway, comprising approximately 1,600 beds. These include Mumbai Oshiwar 500+ beds, Pitampura 700+ beds and our South Delhi project in Greater Kailash, which is about 400 beds. These projects are in various stages of approval and construction, and will take 3 to 4 years to commence.

With this, I request the operator to open the line for questions. Thank you.

Moderator: Thank you very much, sir. We will now begin the question-and-answer session. Anyone who wishes to ask a question may press "*" and "1" on their touchtone telephone. If you wish to withdraw yourself from the question queue, you are requested to press "*" and "2". Ladies and gentlemen, please wait for a moment while the question queue assembles. We have our first question from the line of Amey Chalke from JM Financials. Please go ahead.

Amey Chalke: Thank you for taking my question and congrats to the management for the good set of numbers. I have first question on the performance which we have shown in developing hospital that the occupancy has gone up to 64% during this quarter. Should we consider this occupancy as a base case going ahead or is it sustainable? Or you expect this occupancy to normalize in coming quarters?

Pankaj Sahni: So, I think that two points, as we may have mentioned in the past, and I mentioned also in the notes, we do continue to add beds in both Lucknow as well as in Patna. In fact, in Patna, we do

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an

anticipate additional beds coming on board in the next couple of months as well. So, occupancy as a percentage may be a little inappropriate to just look at as a single metric just because of the fact that the denominator keeps changing with respect to the bed additions. However, if you look at the actual occupied bed days, you can see that the actual occupied bed days are also increasing. In fact, we have seen an increase of about 16.8% in the occupied bed days or the inpatient volumes, whichever way you want to look at it, more or less they transfer to the same thing. And that has actually gone up from about 48,026 in Q3 FY24 to about 56,089 in Q3 FY25 and even sequentially it is up. So, we do believe that the volumes and the occupied bed days will continue to increase. I guess the absolute percentage is a little bit a function of how the denominator changes. So, my only caution would be don't look at the percentage number in absolute terms. Keep in mind as the bed additions are there, the percentage numbers may vary a little bit. But yeah, we are very bullish on both the units and we actually see a fairly strong demand in terms of our Patna unit as well, which is just now coming into about three years of operations. In fact, we are, kind of really pushing to get additional beds there because the hospital is quite full on both the ICU as well as in the ward.

Amey Chalke: Just to summarize, it's fine to assume that the occupied bed will continue to increase in coming quarters.

Pankaj Sahni: I mean, I don't want to tell you what assumptions you should take, but of course we are very strong believers in the fact that that will increase.

Amey Chalke: The second question I have, we have on-boarded 49 doctors in 3Q, is it possible to give breakup of this doctor in what all facility units they have been added and is the cost fully baked in during this quarter?

Pankaj Sahni: I don't have that off the top of my head. What I can tell you is that the bulk of the doctors have come in in this particular quarter in Lucknow and in Patna, obviously, because those are still the more growing hospitals. However, if you look over a nine month period, you'll notice that even our Gurgaon facility, which is a 15 year old facility, has added in 50 beds. And I believe, I forget exactly which quarter it was now, but even in a couple of the prior quarters, we have also been hiring in talent.

I can also tell you that our plans for as we move forward, not only in the next quarter, but also in the next full fiscal year, will include hiring in all the units, including our Gurgaon unit. And

this is done for 3-4 reasons. The first of course is that in some cases, you have to fill in the gaps like adding in specialties which may be missing. In certain other cases, you are adding very niche or expert capabilities. So, for example, you may be having a particular department, but you may not have a robotic surgeon in that department. So, you may add a robotic surgery capabilities. And then of course you are also adding because we have huge capacity, so we are able to occupy this or there is good talent that is available in the market and we always like to keep our eye on talent. So, what I would say to you is that doctor recruitment and clinical talent additions will continue in all the hospitals. Obviously, the cost of that depends on the kind of talent you're bringing in, the specialty, the nature of the market, etc. But for the most part, if you look at our
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manpower cost to revenue ratios, you see that they are doing quite well in terms of the overall cost. I think we are across the industry, we perform quite well on the manpower cost to revenue ratios.

And when you look at Noida, of course, which will come on board in the coming months, there we will definitely see much higher cost of hiring because that will be a new uni

t so we are

bringing all the clinical talent and not only clinical, we will be bringing in all the talent. So, next maybe 6 to 9 months, you will definitely see an increase in hiring in the group and a lot of that will be because of Noida starting.

Amey Chalke: Thank you so much. I will join back.

Moderator: Thank you. We have our next question from the line of Prithvi Raj from Unifi Capital. Please go ahead.

Prithvi Raj: Sir, my first question is on the new hospitals, I mean business. We continue to see a sharp turnaround here. Was there any seasonality factor that pushed up the numbers in this quarter, or is it all due to the various collective measures that you have been taking in the last few months, and it is all organic growth that we witnessed?

Pankaj Sahni: So, you know, the only thing that I would caution before just blanketly using this word seasonality is that it means different things in different cities and it means different things for different natures of hospitals. So, to give an example, we had during Q2, significant amount of patients in some of our units with respect to dengue and some of the seasonal diseases, but that did not necessarily happen in the other units. So, it varies across the board. So, each unit has to be looked at slightly differently and seasonality plays a different role in each unit depending on the nature of the case mix, etc. over there. So, that's just one word of caution in general.

The second thing is that you see the way in which we look at it is that there's a bunch of activities that we had mentioned now on the last couple of earnings calls that we had taken to arrest some of the concerns that had been highlighted specifically in Lucknow and those included as I mentioned outreach to the community, clinical talent, additional growth, investing in infrastructure and technology, etc. All those activities obviously we continue to do in fact, but all those activities actually we started to see taking shape towards the middle half of the calendar last year. And what you're seeing is just simply a reflection of that in the numbers that get reported out on a particular quarter. But it's not like all those activities started only on 1st October and ended on 31st of December. Obviously, they continue over the period of time. So, I don't think that there is only a seasonality impact, we have definitely taken very active measures across the Lucknow unit. At the same time, there is of course seasonality in all the various quarters depending on what you look at. That's with respect to Lucknow.

Now when it comes with respect to Patna. Patna, yes, there are, again, some seasonality factors in the Q2 with dengue and some of those types of things, but I think in Patna, the real thing is capacity enhancement. So, as and when we have been able to add in beds, we have seen that the
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volumes have grown and some of that is because we are actually quite fully choked up in Patna. So, a lot of the growth has come from clinical talent, from infrastructure and capacity enhancements.

We are about 470 beds there now, we are adding in another 30-50 beds over the course of the next few months, and then we actually will have the second tower also getting built out. So, that should add in another 180-200 beds hopefully. So, a lot of the growth I would say in Patna is very typical of newer hospital growth, as and when we are providing the capacity and bringing in the talent and technology, we are seeing growth. So, that's a little difference between both the units is how I would look.

Prithvi Raj: That's quite helpful. And my second question, could you give us a number on scheme patients for the Lucknow facility and how will this number evolve going forward?

Pankaj Sahni: So, I don't have off the top of my head the exact number for the scheme patients for Lucknow.

What I can tell you is that our overall sales mix has a pretty low percentage, I think just about around 10% of our patients are towards the scheme business. And by far the largest base of scheme is in Gurgaon because we have been doing it for many years. So, I would assume that Lucknow would be even ~4% of the scheme business. I'm told by Yogesh and the team that it's probably around 4%-5%.

Prithvi Raj: And can we expect the event is to move towards 10% kind of number in the next one year?

Pankaj Sahni: I'm sorry, so just one more point before I answer that question, because I think it's a little important, especially in context of what we are seeing in Patna. There is an implicit assumption that the scheme business may not necessarily be a high-realization business. And what we have seen in Patna with our PPP business is that there is actually a pretty healthy realization of that business. I think this idea that all scheme business is low realization business is not necessarily true. I think that it again is different for different types of hospitals. Obviously, we do very high end complex care and it is also different in each specialty.

So, realizations for example for scheme business in certain departments like say Cancer or Cardiac may actually be pretty healthy whereas in some of the more secondary care oriented specialties like ENT or Plastic Surgery may not be that good. So, it really depends on the sales mix of this scheme business with respect to the specialty. And that's an important point.

The only thing that we have seen on this business is the length of stay tends to be a little bit longer and that may affect ARPOB as opposed to ARP P because of length of stay. And the fact that to the extent that the scheme business is a traded business that may be reflected in the debtors on the balance sheet as opposed to the realizations in the P&L. So, I think these are the two, three things that we said. In fact, last couple of months we worked quite actively to reduce the length of stay of our scheme business, especially in Patna, and we have had some good success in that. But other than that, just one word of caution so that you don't assume everything is lower.

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Tariffs are lower, but realization is not always the case. Sorry, you had another question I interrupted you.

Prithvi Raj: Yes, understood. So, my final question is on the international piece. Medanta didn't focus actively on international patients in the last couple of years. And it's almost at 6% of your volumes. So, do we see any further improvement there or the focus continues to be on the domestic patients?

Pankaj Sahni: I don't think it's necessarily correct that we don't focus on international business. We do very much focus on international business. I think that the reality is that given the physical locations of our hospitals, international business is really restricted only to our Gurgaon facility. We don't really get international business in Lucknow, Patna, etc. We do expect that when Noida comes on board, that will also become a potential facility for international business, and we will, of course, be focusing on that. If you look at the international business are specifically only for Gurgaon, I think it would be significantly higher than 6% of the topline, maybe closer to 10% which seems to be more or less in line with the industry. So, I don't think that's not that we don't focus on it. I think that it's a question of the nature of where our facilities are and where international patients come. 6% is what you're looking at is basis, the overall group revenue, not the individual unit revenue. So, that's one.

The second thing is that I did mention in the call, we have actually seen growth of international business about 14% when you look at year-on-year over the quarter. So, we do see growth

in

international business. One of the challenges I think on international businesses, not just for us, but for the industry, is that Bangladesh was always one of the leading countries sending patients for international medical care to India. That of course, given the challenges that we see and what's happening in Bangladesh has taken a little bit of a hit over the course of the last several months, maybe half a year or nine months. And to the extent that we do assume that that will eventually stabilize, we should see that also coming back. The good part for Medanta specifically is that we are seeing a lot of flow coming in now from countries that weren't always necessarily that popular for sending patients to India, countries from Africa, countries from CIS countries. So, historically the biggest drainage has been from countries like Bangladesh, Afghanistan and Iraq. Obviously, Afghanistan is closed, Bangladesh is under a little bit of turmoil right now. So, we are seeing this pick up in other countries, which is good news not only for us but for the country and for the industry as a whole.

Prithvi Raj: Thank you, that's all from my side and congrats for the phenomenal turnaround of Lucknow business.

Pankaj Sahni: Thank you.

Moderator: Thank you. We have our next question from the line of Tushar Manudhane from Moti Lal Oswal Financial Services. Please go ahead.
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Tushar Manudhane : Yes, thanks for the opportunity and congrats for the good set of numbers. So, while there has been interestingly good pick up in terms of say occupancy or even in terms of revenue and profitability for developing hospitals. The ARPOB is sort of witnessing bit of down trend probably due to high patient flow from Patna. So, where do you think this ARPOB will settle maybe in FY26 as the proportion of Patna continues to increase further? That's my first question.

Pankaj Sahni : So if you look at ARPOB for the developing hospitals, you are right, it is, I would say a little bit down from same quarter last year about 2% down hovering in the 53,000 - 52,000 range. This is to some extent linked to the PPP business in Patna and may also be a little bit linked to the scheme business which, if you look last year we didn't really have Ayushman, CGHS etc. One of the bigger things is really less around ARPOB and more around length of stay. So, if you look at some of the challenges which we have with the length of stay for some of the scheme patients, it tends to be significantly higher than cash patients. And that actually, because of the basic math, ends up being a little dilutive towards ARPOB. Although we may not have seen actually a reduction in average realization per patient So, our average realization per patient actually continues to grow. And that is not so affected with the scheme business. It's really around managing the ALOS which then drives up or down the ARPOB. So, that's one important point to note.

That being said, we see that the PPP business still is not at the 25%, which we had said, would be kind of the as per our agreement in Patna, it's still significantly lower than that. I think it's somewhere around 15% to 17% if I'm not mistaken. But that may grow depending on whether the government sense. And we may also see some increases in the scheme business in Lucknow as we continue to add in the scheme business there, which we had always said that we would start this financial year. We are not, frankly speaking, overly concerned about the ARPOB as long as the realizations are healthy from an ARPOB point of view. So, as we continue to work on the ALOS reduction, ARPOBs will grow.

And then the other thing that I just wanted to mention is that we have been now operational in Lucknow for five years. We have not taken a single tariff increase in Lucknow. This January, we would have been operational

in Patna for three years. We have not taken a single tariff increase in Patna. So, a lot of our insurance contracts, a lot of our cash patients, a lot of our other corporate contracts are still running on five year old tariffs in case of Lucknow and three year old tariffs in case of Patna. So, we do have these opportunities available with us, but like we have always maintained, we try to drive as much growth as possible through volume. But we will of course keep in mind how the market dynamics play out to look at what we should be doing on tariffs and realizations.

Tushar Manudhane: Got you, sir. So, secondly on Noida, it seems there's a bit of delay in terms of commencement of operation. Any particular reason you would like to call out?

Pankaj Sahni: No, I don't think there's any delay. If you look at the hospital, we started construction in September 2022. So, we hope to be live by the summer of this calendar year 2025. So, if you really take it, it's less than 3 years, which is pretty fast time for construction. We were hopeful

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to get it operational by March. Looks like that may be a little bit more towards the June, July time frame. But I don't think there's a very significant delay. Broadly we are on track. Obviously we all want it done faster so that we can ensure that we are able to serve the patients as soon as possible. Very strong demand for the patient community there as well as very positive feedback from the clinician community to join us there. So, we are quite excited about Noida, come along quite well. Of course, the sooner it opens the better it is for everybody.

Tushar Manudhane: Got it. And thirdly, just on Ranchi, where we already have our existing hospital, if you just share in terms of the industry perspective at Ranchi, which sort of gave you this or compelled you to get into tie up for another hospital at Ranchi?

Pankaj Sahni : Sure. So, we have a hospital in Ranchi, it's approximately 200 bed. The infrastructure of this hospital is very old. And in fact, what we realized was that over the last couple of months, almost maybe a year, we found that there has been a very strong demand for certain services in Ranchi. We have struggled a little bit with the existing infrastructure in terms of scaling up, especially scaling up some of the kind of work which we do, like we wanted to add another 20 - 30 ICU beds, we wanted to add the capabilities of more advanced procedural work. Currently in Ranchi, we don't do any cancer work. So, we don't have space actually to put up a medical oncology

unit. We don't really have surgical oncology. Our dialysis is running full in all the shifts. So, we were actually quite constrained in the physical infrastructure of the facility. And also given the age of the actual infrastructure, quite a bit of renovation needs to be done. Now in a running hospital, which is quite busy, doing some of these changes is hard. We were fortunate to be able to find an existing hospital, fully -baked, I think we had some pictures in the investor deck as well, state-of-the-art operation theaters, which can do all the complex work we do, state-of-the-art Cath lab, MRI, etc. So, all of this is ready-made for us, so we thought that it's a great opportunity to kind of create a little bit more of a campus connection and scale up from 200 beds to maybe, hopefully, 300 -350, even if possible. And what this allows us to do is gives us some time to actually take shutdown of couple of floors, renovate the old building, and then revitalize that as well. I think the biggest thing is that we will be able to start Oncology, which we have not been able to offer to the people of Ranchi so far. And given that it was so close to the existing hospital, I don't want to use the word hub and spoke because that also may be too far. I think it's really two sites, same camp

us kind of feeling. And

we have had a very phenomenal response in the last couple of years post COVID in Ranchi. And these are the kind of places which need the kind of quality of care that we can give. So, I think it fits very nicely with the idea of expanding in beyond 200 to make it around the 300 -350 bed campus across two locations that are frankly speaking very close. So, we will be able to get all the operating entities. It will be run by the same team, etc. So, it made complete sense for us to do, and we were lucky to get a very nice ready-made facility, which we hope to operationalize very soon.

Tushar Manudhane: Got you, sir. Just one last on this, so how much investment you would be requiring on this and the kind of sort of rent or revenue share you would have with the existing partner?

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Pankaj Sahni: So, it's a straight rental agreement, there is no revenue share. Obviously, we are lucky that we believe that we have got it at fairly good commercial terms. It's a 15-year lease extendable for two more additional periods of 15 years, so that makes it a 45-year lease. What we have decided to do, just because we believe that we may be better suited to manage it, we have decided to buy over the existing equipment from the landlord that had already been put in there so that we can kind of integrate that with our own balance sheet and our own management system. So, we will be spending somewhere around maybe Rs. 20 crores to Rs. 30 crores to buy over all this equipment. And then we have kept a buffer for maybe another Rs. 20 crores of additional equipment investment etc. that we may want to do as and when we get move forward. So, I would say overall, the complete investment should be under Rs. 50 crores.

Tushar Manudhane: Thanks, sir. Thanks a lot for this.

Moderator: Thank you. We have our next question from the line of Nancy Yadav from Allegro Capital Advisors. Please go ahead.

Nancy Yadav: Hi, thank you so much for the opportunity and congratulations to the management for a great set of numbers. Most of my questions are cleared. I just wanted to get two numbers. Sir, could you firstly tell me the net debt or cash position of the company currently?

Yogesh Kumar Gupta: Overall cash position of the company is Rs. 1,085 crores. And the gross debt at Rs. 321.

Nancy Yadav: Okay sure. And also sir, could you help me out with the IndAS 116 adjustment number?

Yogesh Kumar Gupta: I think it's not very different from the last year. I don't have the number right now ready with me. You can get that from the last year financials.

Nancy Yadav: Okay, sure. Thank you, sir.

Moderator: Thank you. We have our next question from the line of Madhav from Fidelity. Please go ahead.

Madhav: Yes, good evening. A couple of questions. The first one was last year in quarter three, Lucknow had obviously seen very strong seasonality related to Dengue cases, if I remember correctly.

Could you give some qualitative sense in terms of whether that intensity of those cases this year was similar, higher or lower? Just to get a sense in terms of how that played out?

Pankaj Sahni: Yes, so as we mentioned many times, obviously disease doesn't follow the neat quarterly reporting that our financial community forces us to get into. So, actually, Dengue typically starts depending on the, or it's not only Dengue, right? Dengue, Chicken guinea, sometimes it's some of the other seasonal diseases with respect to the summer like Gastroenterology, etc. These typically start sometime in the summer depending on as and when how the rain's hit and what the environment is and they continue on. So, that could be sometimes July, sometimes August, sometimes September. And that continues on depending on how that season goes away and we get the winter season coming, whether it is October -November and frankly speaking, it's

also

affected basis when Diwali and some of the other festivals are.

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I think in last year, the impact of dengue in Lucknow was significantly higher than what we saw this last nine months. But the impact is typically not Q3. It is spread out over Q2 and Q3. So, I think that a little bit lower impact this time around compared to last year. But I just want to caution you. You should not only look at Q2 and Q3. But maybe if you combine Q2 - Q3 that would be more appropriate to look at.

Madhav: For developing hospitals in terms of seasonality, last year Q4 occupied bed days had come a bit versus Q3. So, giving you the seasonality plays out differently for our different facilities. Is usually Q4 for developing the two key facilities? Is it usually a softer seasonality versus Q3? Or how should we look at it?

Pankaj Sahni: We mentioned this, I think, last year. And if you look at Lucknow, of course, Patna was a little different story last year because it was really growing from 200 beds to 400 beds. So, I don't think Patna is in a stable enough state to make a fair assessment. But Lucknow, a little bit more stable, although we did grow from about 430 beds to about 700 beds in Lucknow as well. Last couple of years, we have seen Q4 being a little bit softer than Q3 in Lucknow. In fact, in the case of January - February - March 2024, significantly softer, but I don't think all of that was seasonality. Some of that we clearly now know was some of the challenges that we had in the unit. But even if I go back to January - February - March 2023, we did see that as a little bit softer than the previous two quarters.

So, whether this is a clear trend in Lucknow that Q4 is softer or not, I think still we need to wait it out and see because it's been a couple of years where we have been adding in beds and then I think this will get a greater sense. What I can tell you is that it little bit depends also on how the winter season plays out. Is it very cold? One thing that my assumption is, and again it's still an assumption which we are still testing, is that I think that the quarter four plays out a little differently in places like maybe Delhi, Bombay, Bangalore than it does in places like Lucknow, Patna, and some of the other tier-2 cities. But I think this we will figure out over time, and we will also see the performance of various players who have now ventured into some of these places, maybe following us into these kind of cities. And we will see whether that makes sense. I know many people don't report out unit by unit numbers. But my hypothesis is that the metros may perform a little bit differently with the sales mix as compared to some of the other areas. And depending on the demographics, the disease patterns in those cities, you may find some changes. Today, you do not really see too much seasonal diseases in places like Delhi, Bombay, Bangalore. It's a lot more the non-communicable diseases. So, I think it depends on the city to city and how the medical disease pattern changes over time as those cities develop.

Madhav: So, if I can ask one last question, the Noida facility, which starts in six months, would you be able to hazard a guess in terms of what could be the EBITDA break-even timeline for the facility, given it's a greenfield one? I know it's early days, but any sort of broad sense in terms of how year one, year two could play out, that would be interesting? Thank you.

Pankaj Sahni: You may have noticed on some of my previous calls or statements, I tried to make it a point never to hazard a guess, and especially never to hazard a guess on what the EBITDA will be. I

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think our focus has always been, let's try to deliver the best quality of care that we can and let the numbers speak for themselves from that point

of view. So, I don't have any guidance for you,

I'm sorry, on what the EBITDA would be in Noida. What I can only tell you is that we do intend to look at operationalizing and running our Noida facility with the same dedication and focus towards delivering exceptionally high quality and high-end tertiary care.

Now this means two, three things. The first thing that it means is that obviously we will be looking at very highly capable and highly talented clinicians, bringing in a workforce that allows us to do that. Obviously that workforce has a cost. Like with any new facility, there is a certain time that it takes to get ramped up. If you look at our history in Patna and Lucknow and even frankly, if you go back many years in Gurgaon, we have been very, very fortunate that we have been able to obtain break even or even profitability on an EBITDA basis in a very short period of time. But I don't think that that is a reasonable thing for any management to plan for EBITDA break even in six months or one year or even 18 months. So, our models continue to assume that it takes a couple of years to breakeven in any facility. And if we can do better than that, then that's great. But I don't want to take a chance to guess that yes, we will be able to break even in X months or Y months.

Madhav: Thank you.

Moderator: Thank you. A reminder to all participants, you may press "*" and "1" to ask a question. The next question is from the line of Bansil Desai from J.P. Morgan. Please go ahead.

Bansi Desai: Thanks for taking my question. My first question is on Gurgaon facility. So, we have seen a decent improvement in occupancy here year-over-year for last couple of years. We have added 50 beds here. How should we think about optimal occupancy for this particular unit? I understand our infrastructure layout for Gurgaon Hospital and for that matter, all our hospitals is quite different compared to peers. So, how should we think about optimal occupancy levels? Will it be difficult for us to reach 70 %-75% occupancy levels or do you think that's achievable with our specialties or exposure to specialties that we have?

Pankaj Sahni: Let me answer that question in two, three ways. First, if you look at, and I will step back from the occupancy percentage number just for a minute. If you look at our overall inpatient volumes over the course of the last four years or so, you see that we have been able to get a CAGR of double digit growth, I think somewhere around 10% plus volume growth just on inpatient volumes in our Gurgaon facility over the last four years. Now, I also may have mentioned in some of the earlier calls that we didn't even take tariff increases. So, a lot of the revenue growth which you've seen up until maybe the last couple of quarters was fully driven by volume and then some amount of sales mix and realization improvements. And it's only really last year that we took some tariff increases. So, you're seeing some amount of the revenue growth driven by tariff, but still a lot of it coming from volume growth. So, we do intend to continue to drive the growth as much as possible through volume additions.

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Now, the reason that I say that occupancy is not necessarily a standalone metric is, I forget now whether it was Q2 or whether it was Q1, in the last nine months we have operationalized a 60 - bed Chemotherapy ward in our Gurgaon facility. And that actually took our existing census beds, which were night stay beds, and we shut them down and renovated them and we converted them to daycare. So, what happens with daycare is that there's no patient at night, so the occupancy statistics look different because you don't consider them at midnight occupancy. So, we won't actually be overly worried about what the percentage number is as long as the volumes continue to grow.

In fact, there's a lot

of demand to reduce length of stay to move more and more work to daycare, to look at the network of clinics. As you're aware, we operationalize the clinic, another clinic in our Gurgaon area. We may be looking at more of that. So, we will look at various means to use the infrastructure and talent we have to deliver care, not only occupancy percentage. We will continue to ensure that we are doing the cohorting and the quality and the specialty management that we would like to try to do without compromising on that to the extent that we can grow the occupancy, grate. But frankly for us, it's much more around volumes, absolute volumes than percentage.

Bansi Desai: Got it. So, the reason I asked is that in the last 2-3 years, if I see, our average length of stay has remained pretty much the same. So, what probably has allowed us, so to that extent, the inpatient volume growth is probably reflected in the occupancy increase, except for the fact that you mentioned that, you know, some of the chemotherapy beds etc. are not included in our occupancy numbers?

Pankaj Sahni: No, so unfortunately not that straightforward at the macro level. You have to dig down by specialties. So, I will give you an example. If you look at our sales mix across specialties, you'll find that Oncology business is growing. A lot of Oncology business tends to be potentially shorter space than say some of the other specialties. So, you will find some changes across specialties in how that plays out. So, daycare having some amounts of preferences. At the same time, you may find that a lot of the work which we are doing, say in Cardiac or in some of the other specialties, are actually quite complex and the length of stay there is sometimes even increasing. So, when you get into this, you have to actually look at length of stay, specialty by specialty.

Now is there an opportunity for Medanta to increase a couple of basis points of occupancy percentage? Of course there is. Do I say that the hospital is full? No, it's not. But I think that we still keep this kind of cohorting and the kind of way in which we fill up the beds as largely sacrosanct as long as you're not compromising on that. Yes, I think that there's opportunities to increase the occupancy percentage. But again, we do that through volume driving, right? If a patient can go home and not stay at night, that's a good thing. So, we don't optimize for occupancy percentage and keep the patient in the hospital just because that looks good on numbers. We'd much rather drive volumes. And if the occupancy grows, it grows. But higher volumes with lower length of stay is always better.

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Bansi Desai: Got it, this is helpful. And just extending previous participant question on seasonality, especially in Lucknow, you mentioned probably trends are like, you'll monitor those trends. Just, if you could comment on how January has played out for you, is it any different from past years in your opinion?

Pankaj Sahni: So, you are asking me things that will get me into trouble with the regulator if I give you data beyond 31st of December. What I can tell you is that there is definitely a very visible and very strong growth and positive momentum that we have seen in Lucknow and continue to see in Lucknow, vis-à-vis where we were, let's say maybe 9 to 12 months ago. And I think that a lot of that is, a lot of the efforts that have been put in by the team. And we see that in the response from the community, in the clinical work which we are doing, in the kind of complex work which we are doing, like I think Dr. Trehan also mentioned, amongst really, really fast growth in our robotic program in Lucknow. So, all indications are in the positive way. But this is a business which you can't be looking at it only quarter-to-quarter. And you have to keep as we learned, right? So

, you have to be connected to the community, and you have to keep continuing to serve the community. So, when I say that we keep a close trend and watch on all of these things, I think that we should not assume that work is over by any means, lots more to be done. And it's also important to understand we are currently a 750 bed hospital in Lucknow. It is still not complete. Our aspiration is to go to about 950 – 1,000 beds. We still look at adding in additional clinical talent, still some investment to be done in new medical equipment and medical technology, some of that already underway, you will hear about it in coming months. And we will continue to scale up our beds. We will continue to add in a complete new floor of outpatient clinics as well. So, Lucknow Hospital is not complete. It's very big, but it's still not complete. So, lots to be done there.

Moderator: Thank you. We have our next question from the line of Aman Goyal from Axis Securities. Please go ahead.

Aman Goyal: My question is related to margin difference between mature hospitals and developing hospitals. So, as we look at in last quarter developing hospital reported margin somewhere around 30% , now 33% , where our mature hospital reporting margin at 25%. So, what is the key difference between 700 - 800 basis points between these two hospitals over the longer term, like the next two years, where we could see the developing hospital could report a margin on average basis?

Pankaj Sahni: So, as I mentioned earlier, I try to avoid giving predictions on what the margin would be. I think that's never a healthy thing to do. But what I can tell you is that our complete corporate costs, our complete overhead costs are currently booked in our Gurgaon facility. So, what you see reported out may not be reflective of the true operating margins of an individual unit. Our developed hospitals comprise Indore, Ranchi, and Gurgaon . And our developing hospitals is Patna and Lucknow. So, there is some amount of averaging out across the ecosystem. But I think one major point which you look at is that what you see as the matured hospital margin is sometimes wrongly assumed to be the Gurgaon unit margin. That's not necessarily the case

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because of the fact that certain costs are booked here that may not be actually completely Gurgaon-centric.

Aman Goyal: Sir, another question is, Max has also entered in Noida with Jaypee acquisition. Could you see any competitive landscape on that part?

Pankaj Sahni: See, I think that if you reflect back on what is happening in Indian healthcare since the COVID pandemic went away, is that the broad perception of actual competitive intensity depending on which way you look at it, is increasing across the board because there has been far more focus on healthcare, there has been far more investment in healthcare and there has been a lot of excitement with the Private Equity and the capital markets around healthcare . So a lot more capital flows into this industry. So, definitely we do see increased amount of infrastructure additions, we see increased amount of hospitals opening up. And our belief as Medanta is that this is always a good thing, we still have a very long way to go with respect to the absolute amount of beds that we have to serve our population and to serve the population in need and that's able to afford.

If you look back now several years, there was nothing in Gurgaon when Medanta opened. And now people reference Medanta as a medical hub. So, what ends up happening, and the same story is in Lucknow , we started in Lucknow 2 years ago, people asked us that have you gone crazy? Why are you in Lucknow? And everybody seems to be chasing us into that market. We do believe that we are trendsetters in this place , in this way. We go where the need is the most. If people follow us into that

market, it's very good. It elevates the quality of care for the community. It elevates the accessibility of talent. It creates a draw for both patients as well as

doctors in terms of creating these so-called medical hubs.

So, our belief is actually slightly contrary to what you're saying. The more people that come to a particular market and create an ecosystem of healthcare, the better it ends up being for everybody, for the patients, for the doctors, for the accessibility of talent in terms of nurses, doctors, as well as frankly speaking, for the hospitals that kind of do well. Now, along with this one word of caution, is that in the last 15 years in Gurgaon we have seen many hospitals open and close, and some have been sold more than once over. So, it's not that easy to run a hospital, it's easy to build a hospital, just requires so much capital. But running a hospital is a hard business, and running it with the right values and the right qualities is even harder. So, we welcome the increase in beds across the system. I don't see any concern whatsoever. And if that adds to the quality of care of the community and expands the base, it's good for everyone.

Aman Goyal: Thank you, sir.

Moderator: Thank you. We have our next question from the line of Alankar Garude from Kotak Institutional Equities. Please go ahead.

Alankar Garude: Hi, thank you for the opportunity. So, first question, you had earlier spoken about starting construction in one half of the land parcel at the South Delhi Hospital due to certain legal issues.

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Now with the soil testing started, should we assume that all the legal issues as well as the pollution-related stoppages have been resolved?

Pankaj Sahni: So, on the parcel which we talked about, yes, no legal issues as far as building out. On the pollution, it keeps starting and stopping. So, we have started and stopped at least six, seven times already. I'm given to understand right now there is no pollution related stoppages, but it depends on how the government changes that. I am hopeful now that here the worst of the air pollution is behind us in the Delhi area. So, we should be able to continue on that. So, I think if the question is that, is the construction moving ahead? Yes, it is. Are there pollution or other related stoppages that may come? That is hard for me to tell. But from our side, the intention is very clear that we will build this out and that is actually what is happening.

Alankar Garude: Understood. And similarly, Pankaj, if you can talk about the timelines for the Indore hospital as well?

Pankaj Sahni: Indore, as I mentioned in the, I think, I don't know which one it was the last or even before that, we have had some challenges with respect to the legal issues there. That is on hold, it is status quo because the matter is subjudice. I mentioned in the last call that we are exploring other options, those are actually already underway as well. So, I don't actually have complete clarity on what will happen in Indore. It is definitely a delayed project which is not great for anyone involved but we also don't want to do anything which is not appropriate.

So, Indore we are exploring all options. We haven't yet pulled the plug on the deal that we have because we feel might as well keep it open if it's possible to get done. We remain committed to Indore and seeing what we can do there, but yes, this particular site, unfortunate that it's gone into a little bit of a delay and potentially, I don't know whether it will come out of this delay or not, or we will have to look at an alternative. But that's what we are looking at.

Alankar Garude: Understood. So, basically Pankaj, the broader question is, how should we look at the timelines for these 1,950 beds planned to be added post FY27? So, should we assume Mumbai and Pitampur would be more towards the

end of FY28 and possibly South Delhi being after FY28?

Very broad timelines?

Pankaj Sahni: So, the three hospitals, which is your Delhi, your South Delhi, Pitampura and Mumbai, are all broadly on the same time frame in terms of the construction activity, the drawings, the digging of the sites, etc. So, they all more or less will move simultaneously from our point of view. Now obviously, construction has its own challenges. So, maybe one of them gets built faster than the other. But I would say that broadly speaking, see, it takes around three years or so. I mean, we hope to get Noida done before that. But it normally takes about 3 to 4 years to build up any hospital. Now, which one will come six months before the other, that I'm not in a position to tell you today. But I would assume safely that these hospitals are about three years or so up.

Alankar Garude: Got it. That's it from my side. Thank you.

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Moderator: Thank you. We have our next question from the line of Anshul Agarwal from Emkay Global.

Please go ahead.

Anshul Agarwal: Thank you for the opportunity. So, my question is on Lucknow facility, given the good ramp up that we are seeing here. Would we be sort of opening new beds here in the next six months itself?

Would we be reaching peak capacity in the next six months in Lucknow?

Pankaj Sahni: I don't think we will be reaching peak capacity in the next six months. Peak capacity will be somewhere around 950 to 1,000 beds. We are currently at about 750 beds. We do hope to add in beds over the next six months that you're talking about. Whether it will be exactly six months, whether it will be nine months, whether it will be 50 beds or 100 beds, that I'm not sure. But the two considerations that go into this. One of the considerations as we start to get towards the closure of the absolute capacity is whether or not we want to keep any beds in the back pocket for potential future options for new specialties. And the example that I will give you, if you're not familiar with it is our last floor in Gurgaon got completed in April of last year, I believe, which is almost 14 years after the hospital started. Now, if we did not have that floor, we would not have been able to start mother and child services in Gurgaon.

So, sometimes you do keep one or two things in your back pocket, this is the specialty and how things are planning out. But I think that the broader point around the fact that will you continue to build out and finish up the hospital and build out the beds, the answer is yes. We will keep as much flexibility as possible to allow us to plan for future specialties and new kinds of services that we may like to have. Like we don't have a dedicated pediatrics program in Lucknow, right? Now that is something that we may decide to add in today. If that comes in, then it may change the basic construct and layout. But other than that, I don't think there's any desire to delay any build up.

Anshul Agarwal: Great, that answers. The second question I had was for Yogesh sir, probably if you could just help me out with the CAPEX guidance for the next couple of years.

Yogesh Kumar Gupta: If you see the slide 25 of our investor presentation, we have already given that guidance. So, total CAPEX for the next 3 to 4 years is somewhere around Rs. 3,000 crores. And the breakup by project has been given there.

Anshul Agarwal: That's it from my end. Thank you so much.

Moderator: We have our next question from the line of Anubhav Sahu from Macro Research. Please go ahead.

Anubhav Sahu : Coming again on the Lucknow thing, I think it's a pretty good quarter for us. And I can see you have indicated for 200 beds addition for in a 6 to 9 months, with the flexibility you just talked about. At this point, from a strategy perspective, are you looking for something else other than the unit which

you have in Lucknow, whether for this city or maybe in the nearby city like Kanpur?

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Pankaj Sahni: Let me just clarify the question. Is your question, are you looking at a second site in Lucknow or looking at Kanpur? Is that the question?

Anubhav Sahu : Exactly.

Pankaj Sahni: So, I don't know whether we would be looking at a second full hospital site right now in Lucknow. We will definitely be looking at how to increase our coverage in Lucknow, whether that is through hospital or whether that is through alternative means. We have in the past talked about the fact that there are many ways to kind of cover the community, not all of them translate to building a hospital, especially not possibly a thousand bed hospital. But we will look at increasing the coverage of care that we can provide in the Lucknow city area specifically.

As far as Kanpur goes, or for that matter, even some of the other bigger or well-known cities in the eastern part of UP, which could include Varanasi, Allahabad, Gorakhpur, etc. I've mentioned before, these are definitely on the radar for us. All cities, including Kanpur, Varanasi, Ayodhya, Allahabad, Gorakhpur, all these cities are of interest to us, given just such a huge demand of health care and an absence of high quality care in these areas.

At this point in time, there is nothing definitive that I can tell you that, yes, we will be opening in this particular city. But all the cities are in play and we have a very strong commitment to the state of UP and in general, to that part of the country. And we will be looking at opportunities to ensure that we are able to deliver our services in all of these various places.

Moderator: Thank you, sir. That was the last question for today, and I now hand the conference over to the management for closing comments.

Pankaj Sahni: So, thank you all for taking the time and for joining us today for our Q3 FY2025 earnings and results call. Thank you for your insightful questions and for joining us today. If you have any other questions or if you have any other specific information and questions remain unanswered, please feel free to reach out to our investor relations team. And we look forward to speaking with you and seeing you all soon. Thank you very much.

Notes:

1. This transcript has been edited for readability and does not purport to be a verbatim record of the proceedings
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Call: May 2025

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Dept. of Corporate Services

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Mumbai - 400 001

Scrip Code: 543654 The Manager

Listing Department

National Stock Exchange of India Limited

Exchange Plaza, C -1, Block G,

Bandra Kurla Complex,

Bandra (E), Mumbai - 400 051

Symbol: MEDANTA

Sub: Disclosure under Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 - Earnings Conference Call Transcript

Dear Sir(s),

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the transcript of Earnings Conference Call held on Friday, May 16, 2025, for the fourth quarter and financial year ended March 31, 2025 results.

The Transcript is also available at our website: <https://www.medanta.org/investor-relation>

This is for your information and record.

For Global Health Limited

Rahul Ranjan

Company Secretary & Compliance Officer

M. No. A17035

Encl: a/a

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"Global Health Limited – Medanta
Q4 FY '25 Earnings Conference Call "
May 16, 2025

MANAGEMENT : DR. NARESH TREHAN – CHAIRMAN AND MANAGING
DIRECTOR – MEDANTA – GLOBAL HEALTH LIMITED

MR. PANKAJ SAHNI – GROUP CHIEF EXECUTIVE
OFFICER AND DIRECTOR– MEDANTA – GLOBAL
HEALTH LIMITED

MR. YOGESH KUMAR GUPTA – CHIEF FINANCIAL
OFFICER – MEDANTA – GLOBAL HEALTH LIMITED

MR. RAVI GOTHWAL – HEAD INVESTOR RELATIONS –
MEDANTA – GLOBAL HEALTH LIMITED

MODERATOR : MR. TUSHAR MANUDHANE – MOTILAL OSWAL
FINANCIAL SERVICES LIMITED

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Moderator: Ladies and gentlemen, good day, and welcome to Global Health Medanta Q4 FY25 Earnings Conference Call hosted by Motilal Oswal Financial Services Limited. As a reminder, all participant lines will be in the listen -only mode, and there will be an opportunity for you to ask

questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” then “0” on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Tushar Manudhane from Motilal Oswal Financial Services Limited. Thank you, and over to you, sir.

Tushar Manudhane: Thank you, Yusuf. Good evening, and a very warm welcome to all the participants on Global Health Limited, 4Q FY25 Earnings Call, hosted by Motilal Oswal Financial Services.

Joining with us today from the management side, we have Dr. Naresh Trehan, Chairman and Managing Director; Mr. Pankaj Sahni, Group CEO and Director; Mr. Yogesh Kumar Gupta, CFO; and Mr. Ravi Gothwal, Head of Investor Relations.

Over to Naresh sir for the opening remarks.

Naresh Trehan: Good afternoon, everyone. Thank you for joining us for today's Medanta Q4 and Full Year FY25 earnings conference call. Our results were published yesterday, and we have already uploaded the press release and earnings presentation on the website and the stock exchanges.

I'm pleased to share with you that our flagship hospital in Gurugram, widely known as “Medanta - The Medicity” celebrated its 15th anniversary. And over the years, we have consistently set benchmark in clinical excellence and higher standard of patient care.

I'm also proud to share that all our accomplishments have been achieved because Medanta Gurugram has once again been ranked India's best private hospital by Newsweek's for the 6th year consecutive. And we are proud to know that we are the only one in the first 150 hospitals of the world. This, of course, is a reaffirmation of our quality and the Medanta quality of care and systems.

Let me now highlight the clinical achievement of this year :

In addition to the many new technologies that we introduced for the benefit of the patients, we have accomplished the first set of CAR -T cell therapies, starting with Lucknow making a significant milestone in the advanced cancer care.

Following this, Medanta Gurugram also conducted CAR -T cell therapy, an innovative treatment, which leverages the patient's own immune system to combat cancer.

Medanta Lucknow has emerged as a sector of excellence for robotic surgeries in Uttar Pradesh, having successfully performed 100 robotic procedures within the first six months of operations.

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Medanta Lucknow crossed the milestones of 250 + kidney transplants since inception and established itself as a leading center for renal transplant in the region.

Medanta Gurugram secured a patent for a brachytherapy bracket therapy device, known as MAOLO developed for the advanced treatment of cervical cancer. This has been appreciated very widely and we are now in the process of getting a manufacturing individual who can actually bring into market.

Doctors at Medanta Gurugram employed minimally invasive and novel techniques to treat a rare cardiac conditions, idiopathic chylopericardium and many other surgical procedures, which were done for the first time.

These clinical milestones are not isolated achievements, they reflect Medanta's deep institutional expertise, cross-disciplinary collaboration of highly skilled doctors to deliver superior patient outcomes.

During the year, we have onboarded 100 + doctors across Medanta network of hospitals. We have also continued to invest in latest technology to enable our doctors to deliver high-end medicine with precision.

As we look ahead, we plan to add approximately 1,000 beds over the next 2 years, of this, our Noida facility with a capacity of 550 beds is set to become operational in Q2 FY26.

Further in line with our commitment to expanding access to high-quality tertiary and coronary care, we have announced this year 3 large-format greenfield hospitals in high-growth regions of Mumbai, Pitampura and most recently, Guwahati.

With that, I would like to now hand over the call to Mr. Pankaj Sahni, group CEO, to share further details and the financial update for the year. Over to you, Pankaj. Thank you.

Pankaj Sahni: Thank you, Dr. Trehan. Good afternoon, and welcome, everyone, to our Q4 and FY25 earnings call. Let me take you through the key highlights of our annual performance for the year. After that, I will hand you over to our Group CFO, Mr. Yogesh, who will discuss the quarterly performance for Q4 FY25.

During the year, Medanta delivered a total consolidated income of INR 37,714 million, registering a growth of 13% year-on-year. EBITDA was INR 9,562 million, a growth of 9% year-on-year with a strong EBITDA margin of 25.4%. Profit after tax for the period was

INR 4,813 million after adjusting it for onetime exceptional expenses of INR 499 million related to the merger of Medanta Lucknow, i.e. MHPL with Global Health Limited. Adjusted profit after tax stood at INR 5,186 million, a growth of 9% with a margin of 13.8%.

I'm also pleased to share that the Board has recommended the first-ever dividend of 25% amounting to INR 0.5 per share.

Now moving on to the operational highlights for the year :

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During the year, 219 new beds were added across the Medanta network, representing a 7.8% growth in bed capacity. This includes the addition of our 49 bedded dedicated floor for mother and child at Medanta Gurugram, 112 beds were added in Patna and 58 beds were added in Lucknow.

The average occupied bed days for the year increased by 10% year-on-year with an occupancy of 62% on increased bed capacity. ARPOBs were INR 62,722, a marginal increase of 1.3%. Increasing PPP and scheme business has been marginally ARPOB-dilutive, but it has been offset by growing contribution from cancer and changing case mix. Inpatient count increased by 12% year-on-year and outpatient count increased by 10% year-on-year.

Coming to the mature and developing hospital performance update :

Our matured hospitals revenue from them was INR 26,119 million, reflecting a growth of 11% on a year-on-year basis, driven by a combination of increased patient volumes and improved realizations. EBITDA was INR 6,481 million, up by 12% year-on-year with an EBITDA margin of 25%. Occupancy was at 64%.

Our developing hospitals, which includes Lucknow and Patna, registered a year-on-year revenue growth of 10%, amounting to INR 10,940 million. EBITDA was INR 3,290 million, a growth of 2% year-on-year with strong EBITDA margins at 30%. The occupied bed days at our developing units increased by 19% year-on-year with an occupancy rate of 59% on an increased bed capacity. ARPOB declined by 4% year-on-year to INR 54,303, largely due to increased share of PPP patient and scheme patients in Patna and Lucknow.

We continue to see strong financial performance in Partner despite the lower realizations with extremely high revenue and EBITDA growth numbers, and very robust EBITDA margins.

As highlighted in the past, our Lucknow operations have stabilized, and we remain committed to further strengthening our clinical capabilities in Lucknow. In line with this, we have added 7 new directors and over 20 senior clinicians across various specialties. We have also commissioned our Da Vinci Xi surgical robot and a new cath lab, significantly enhancing our surgical and interventional capabilities.

Additionally, we enrolled the Ayushman Bharat

and CGHS and other schemes, enabling us to

serve a broader patient population. In other key updates, our revenue from international patients increased by 8% year-on-year to INR 2,086 million, contributing approximately 6% of our total revenue. Our OPD pharmacy business continues to register strong growth with revenue increasing by 26% to INR 1,400 million in FY25.

During the year, we have launched Medanta clinics at Golf Course Road in Gurgaon and at Ranchi in the city center, bringing high-quality outpatient services closer to the patients making health care more accessible to them.

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Capex of INR 6,449 million was incurred during the year, of which INR 1,367 million was towards the Mumbai hospital, while the remaining was spent towards expanding bed capacity, upgrading technology and our Noida construction.

Overall, we ended the year with a strong net cash surplus of INR 8,123 million positioning us well to drive future expansion plans and growth.

Moving on to our various projects :

In Ranchi, we have signed an O&M agreement this year to operate a 110-bed hospital with advanced OT and critical care facilities, and we expect to operationalize this in the first quarter of FY26. This is in addition to our existing 200-bed hospital in Ranchi. In Noida, the development of our 550-bed greenfield hospital is in full swing and is expected to be operational in Q2 FY26. Mumbai, during the year, we acquired 9,288 square meters of prime land in Oshiwara to develop a 500+ bed super specialty hospital there, and that activity is also underway.

In Pitampura, Delhi, we have signed an operations and management agreement to jointly build and operate a 750-bed super specialty hospital at Pitampura, both Mumbai and Pitampura hospitals are in various stages of drawing preparation and regulatory approvals.

Most recently, our Board has approved a new project to acquire land and build a 400-bed super specialty destination care hospital in Guwahati, Assam, with the aim to serve the entire Northeastern region of India. Overall, we have built a strong pipeline to add approximately 1,000 beds in the next 2 years and another 2,000 beds across various greenfield projects over the course of the next 3 to 4 years.

With this, I hand over the call to Yogesh to share with you the performance for the quarter.

Yogesh Gupta: Thank you, Pankaj. Now I would like to discuss consolidated financial and operational performance for the quarter Q4 FY 25.

During the quarter, Medanta delivered total income of INR 9,542 million compared to INR 8,361 million same quarter last year, registering a strong year-over-year growth of 14%. EBITDA for the quarter was INR 2,476 million, an increase of 20% year-on-year, with an improved EBITDA margin of 26.0% compared to 24.7% in the same quarter last year. Profit after the tax for the period was INR 1,014 million with a PAT margin of 10.6%.

Overall, our growth during the quarter was primarily driven by the consistent volume growth over the network.

Operational performance for Q4 FY25 :

Our inpatient volumes during the quarter increased by 15% year-over-year to 42,901 and outpatient volumes increased while 13% year-over-year. Average occupied bed days for the quarter increased by 12% year-on-year with the occupancy of approximately 61% on increased bed capacity. Average revenue per bed for the quarter was INR 63,629, a marginal increase of

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1%. During the quarter, revenue from international patients increased by 17% year-over-year to INR 557 million.

Now coming to the developing hospitals : During the quarter, developing hospital clusters continue to grow strong, registering the total in terms of INR 2,814 million, a growth of 24% year-over-year. EBITDA for the quarter was INR 850 million, increase of

36% year-on-year

with an improved EBITDA margin of 30%. Average occupied bed days increased by 36% year-on-year, representing improved occupancy of 60% on increased bed capacity. ARPOB during the quarter was INR 53,818, compared to INR 57,696 same quarter last year, de-growth primarily due to uptick in the new patients at Patna and Lucknow.

Now coming to the mature hospitals : total income for the quarter was INR 6,541 million, up by 8% year-over-year. EBITDA was INR 1,685 million compared to INR 1,493 million same quarter last year, up by 13% year-over-year. Average occupied bed days increased by 2%, representing an occupancy of 62%. ARPOB, grew by 6% to INR 69,592 in Q4 FY25 due to change in the case mix.

Now coming to our capex plan, as Pankaj highlighted, Medanta is on our track to double its bed capacity by adding approximately 3,000 beds across multiple projects over the next 3 to 4 years. We have earmarked the total capital outlay of around INR 4,000 crores for this expansion, which includes both the capacity addition and ancillary support services, along with the maintenance capex of about INR 450 crores. The majority of this investment is expected to be back-ended, and the projects will be funded through a combination of internal approvals and the debt financing.

I would like to draw your attention to the Statutory standalone financial, file with the stock exchanges, pursuant to the merger with MHPL, which was holding the Lucknow unit entity, with the holding company, GHL, the standalone financial results now include the performance of Lucknow units. Accordingly, the figures of comparative periods have been restated to include Lucknow unit as well.

The key benefits of the merger are export benefits under EPCG scheme will now be available to Lucknow against the Foreign earnings of the GHL. Free cash flow generated by Lucknow unit can be utilized by GHL without any additional tax obligations, enabling optimized fund deployment for both organic and inorganic growth, maximizing shareholder revenue and streamlining of legal regulatory and recordkeeping processes reducing complexity and compliance costs.

With this, I request operator to open the line for the questions. Thank you very much.

Moderator: First question is from the line of Vinayak Mohta from Axia Asset.

Vinayak Mohta: Sir, could you please repeat the capex numbers? I just missed out on the total capex for the beds. Did you say INR 4,000 crores?

Yogesh Gupta: Yes, I said INR 4,000 crores.

Vinayak Mohta: Okay. Perfect. And the second question I had, could you give a sense of how many transplants have been done in this quarter and in FY 25, and how have those grown on a year-on-year basis or maybe on a trajectory-wise, how have transplants grown for us?

Pankaj Sahni: So I don't have the number upfront with me. I just wanted to, however, quickly clarify 2 - 3 points because there are many different types of transplants. We do kidney transplants, liver transplants, bone marrow transplants, heart transplants. I don't have that number. I'm sorry, with me off the top of my head, but our Investor Relations team will be able to get that back to you. Only thing that I mentioned in our script is that in Lucknow, we have crossed 250 transplants in the kidney area. And I think that we do a significant number of transplants across liver, kidney, BMT of course in our Gurgaon facility. And then we do liver and kidney and BMT in Lucknow as well. And Patna, we have not yet started liver transplants, but we're doing all the others. Vinayak Mohta: Understood. And sir, last question would be, how are you seeing the competitive intensity across your major markets? I mean all the larger players, I think except for Patna, at least in Lucknow as well as in the NCR region, have been expanding and doing their own work as well.

I. So just a sense on how are you seeing that competitive intensity play out across the board.

Pankaj Sahni: So I think there are 2 ways to look at this. One way that you see is a lot of what you may hear because of the various listed players that you mentioned, who are expanding their build-outs across various cities of India, not only in the North, but also in the South. So we do see an increase in the amount of beds announced, the amount of hospitals announced. Although, I also want to caution a lot of the announcements are just that announcements, the hospitals are not yet being built that will, of course, take a certain amount of time to build the hospitals. So we do see some amount of that.

The second thing that may not be so visible to our capital markets investor community is that we do also see a scale-up of local hospitals. So you see renovations, you see increasing amounts of innovation coming in and new talent coming into the smaller local hospitals that may not be listed, but may be relevant in one city or the other of the country. Now our point of view actually is that all of these things are very, very positive for the industry as well as, of course, for the patient community.

So the reason I say that it's very positive is that, if you look today at the Gurgaon market, many people refer to Gurgaon as a medical hub and what that results in is two things. One, of course, we get patients coming in from all parts of the country or at least the northern part of the country, but we also get clinical talent that is gravitating towards the city and that enables people to hire better and so on and so forth.

So this idea that Gurgaon today is a medical hub is not something that was always there. In fact, before Medanta Gurgaon opened, there was hardly 1 or 2 smaller hospitals in Gurgaon. In fact, we were very much part of building it into a medical hub and that actually creates a phenomenon where, to put it, frankly, it's a rising tide will lift all boats type of phenomenon. And we believe that, that is likely to happen in several markets.

So we have already seen when we initially went into Lucknow, there's a lot of skepticism about why are you going to tier-2. We have seen our competitors following us into those markets. And we do believe that the Lucknow-Kanpur region will also become a medical hub in the years to come.

Similarly, we believe that there will be additional development of the Delhi region, all aspects of it. Frankly, both the central Delhi region as well as places like Noida into other medical hubs. And of course, with the huge and expanding population of Delhi, we personally feel that there's still a lot of capacity addition that is required to service that need over there.

And then lastly, if you look at some of the movements that you've seen over the course of the last year or two in the western part of the country, which again was not very active with the established players, you see now a lot more activity happening in Maharashtra, Mumbai, Nagpur, then Pune, etc. So these in our personal opinion are actually all very positive for the industry and very positive for the patient community. So we are not overly worried. We feel that in the long run, this will be very beneficial.

Moderator: Next question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: My first question is on Lucknow and Patna hospitals. So, I understand most of the patients are cash-paying patients like self-pay. And then you mentioned you have now empanelled Ayushman and some other schemes. So just want to understand, should we assume these 2 hospitals will mostly remain cash heavy and the contribution coming from the insurance channel might not be very significant in near to medium term. How should we look at the payer mix there?

Pankaj Sahni: So I think just to cla

rify, Damayanti, I'll let Yogesh also chime in on the numbers. But just to clarify, I think when we use this word cash, we typically actually mean cash plus TPA. I think the more appropriate word would be to use our listed tariffs rather than the actual mechanism of cash. And I think the intention is not to define cash versus credit, and since cash was insurance, but a lot more to say that these are the published rates of the hospital, which are a little bit of course, the scheme rates as you're aware, are lower than the published rates.

So if you look at our payer mix across the group, we still have about 82% of our payer mix coming from the cash TPA market. And if you look at our Lucknow and Patna markets, I think that may be even closer to 90%. Of course, we do have some PPP business in our Patna market. And as we had mentioned, I think in maybe a year ago plus earnings call, we did always intend to take some of the scheme business in Lucknow to serve those patients. But Yogesh can correct me on the numbers. I think state upwards of 80% - 85% is cash and TPA.

Damayanti Kerai: Okay. Yes, I think I was considering only cash, but thanks for clarifying it's cash plus TPA.

Okay, that's the first question. Second is your ARPOB, as we saw in last few quarters, single-digit growth. So just want to understand whether you have any plans to take any tariff hike or like in the past, you will just continue to focus on volumes and business mix.

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Pankaj Sahni: No. I think we will be taking some tariff hikes. We have, in fact, taken some tariff hikes already, which, in Gurgaon at a nominal rate. And as you are aware, some of our discussions and negotiations with the insurance companies, they happen typically once in a 2-year time frame. So those negotiations and discussions will also happen.

As you rightly pointed out, we have not yet taken a tariff hike in Lucknow and Patna. That may also be something that we may do for the course of this financial year. However, as I've always mentioned, our tariff hikes are typically nominal, we don't make very big jumps in tariff hikes, referring to maybe stick to numbers which are near about maybe the routine inflation. And the intention is not to overly burden the patient community with increased tariffs, but just to cover for the inflationary and input cost aspects.

It will be selective unit by unit. And even within the unit, it may be selective for certain departments because we do take the clinicians into confidence before doing any of this.

Damayanti Kerai: Okay. So for ARPOB, the growth trajectory, which we should look for next few years should be in single digit, right, maybe low to mid-single-digit range?

Pankaj Sahni: I mean I think that would be fair. I would be a little hesitant to say that it could be in double digits, barring some very strange kind of input cost changes, which we saw right after COVID. Typically, if you go back to earlier times pre-COVID, we did see it in the single-digit maybe mid-level single-digit time, which is broadly in sync with how it moves globally, a little bit more than inflation is typically health care and inflation.

Damayanti Kerai: Got it. And my last question is on your Pitampura unit, although like it's a bit away. But nonetheless, in your presentation, you mentioned you'll be incurring around INR 600 crores of capex. And you are doing this hospital jointly with the society. So, this INR 600 crores is your portion or that's the total capex, which you have planned?

Yogesh Gupta: This is our portion because structure, etc. will be built by the society. So we are not counting that capex out.

Damayanti Kerai: Okay. And this would be obviously much back loaded, right? Nothing maybe in, say, next one or two years, it's a bit late in terms of timing.

Naresh Trehan: Nominal amount, because design invest

ment is our, so we incurred some expense on the design side, but mostly, this expense will be after they build the facility.

Pankaj Sahni: And just to clarify, even our greenfield projects, which we will completely are typically on a cash flow point of view, typically also back-end loaded because a lot of the expenses come in the interiors and medical equipment. The greenfield projects upfront have typically the civil cost, but I would say, even if you are taking 100%, I would say almost 60% or so is backloaded in the routine cash flow of a greenfield project.

Damayanti Kerai: Okay. Pankaj, can you also clarify in a typical greenfield, the majority of cost is towards the medical equipment and servicing, etc and the civil constitute relatively smaller parts, the way you said maybe it's 40 -50, or how should we look at that?

Pankaj Sahni: No, I was referring to the cash flow, not the breakup between civil. So, any project has basically, I would say, 4 major buckets. One, of course, I'm not including the land cost, but let's if you take out the land cost, there would be the civil and the construction -related activities, there will be all the services, which is your technical, mechanical, air conditioning HVAC services. That's the second component. The third component would be your interiors and the fourth component would be your medical equipment. This broadly is how you think about the project cost. I'm not including any of the preoperative expenses and the consultancy expenses, etc, but those are also there.

So most of the costs beyond civil are back -end loaded because it does take about a year, 18 months to get the civil structure up and then probably another 18 months or so, maybe a little bit more to get the interiors and then the medical equipment is the last of course to come in.

So the cash flow is a bit backloaded, but I would say the breakup between these four components, maybe a little bit more on the interiors and services part because obviously, it depends on how many operating rooms, etc. Civil is probably the lowest component of it now. But I would say probably between 20% to 30% is each component, maybe with civil being a little bit lower.

Moderator: Next question is from the line of Jainil Kothari from Vasuki India Capital.

Jainil Kothari: My question actually revolves around a gross block per bed or maybe capex per bed. Our gross block has always been around INR 9.5 milli on to INR 10 million per bed, but our country is actually witnessing a good acceptance of pre -engineered buildings, especially in a very capex - heavy business, which has to lower down the capex spend and which eventually helps to have a superior return profile.

So hospitals being a very capex -heavy business, along with a good gestation period for breakeven. What's your point of view regarding the acceptance of pre-engineered buildings in this sector?

Pankaj Sahni: So we have been looking at this very closely. I think that the jury is still out on this. And I can give you two very specific challenges or concerns. One, of course, as you understand, hospitals have significantly higher requirements for various types of services because of our operating rooms, our ICUs, etc, and the way in which Medanta builds these facilities in line with the best international standards, we may have even higher than many others in the industry.

So we do need to be cautious about these pre-engineered types of buildings. We've also looked at steel structures versus cement structures. But the second major concern in all of this construction is around safety. And some of that safety is related to fire safety and the ability to bear the fire ratings as well as the structural safety of the building.

So unfortunately, we have not yet seen

very mass adoption of this in the hospital sector, probably because of these two reasons, and that is the reason why till now we have not gone with any of these structures, but new technologies, new materials, new techniques are being done. And we are evaluating this and also evaluating whether kind of a hybrid structure can be put in place. So we are looking at this hybrid structure for, say, for example, our Mumbai project to see if that is possible, and that may help with more than anything, maybe speed to construction, but it's still to be determined.

Moderator: Next question is from the line of Anshul Agrawal from Emkay.

Anshul Agrawal: Sir, a few questions or clarifications on upcoming projects. Could you help us with the time line for the DLF project and the Pitampura project?

Pankaj Sahni: So let me go one by one. Our DLF project is currently in the phase of where we have started the barricading activity and the soil testing activity, but that construction activity will still probably take about 3 years, I would say, to go to complete finishing.

We've started in our Pitampura facility doing the drawings, and we are ready to do those submissions. So as soon as we get the approvals, we'll be in a position to start that activity as well. And that also is likely to take somewhere around 3 years. So both the projects about 3 years out.

Anshul Agrawal: Got it. And just a follow -up on these two projects, sir. Will be sharing rent or will be sharing

part of revenues in both these projects?

Pankaj Sahni: So I mean, it's more or less the same thing. Part of revenue is, in a way, a rental income only for the party that owns the land. So, depending on the commercial arrangements with the party, it is typically rent or revenue share or some combination of both with a minimum guarantee rent.

Anshul Agrawal: Okay. We have the same kind of conditions in both the projects, similar or same.

Pankaj Sahni: Yes. I mean there are slight differences in both of the projects, but really, that is a question of what is the commercial arrangement that you come to terms with respect to the landlord or the party that is in construction, in the hospital partnership.

It depends on how much money the other party may be putting in and building. So obviously, if somebody gives you just the land, it's a different rate, if somebody gives you land in the cold shell, it's different. If you get land plus warm shell, it's different. So that's how it typically works in the industry.

Anshul Agrawal: Sure. Second question I had was on the Noida project. We would have already hired certain doctors, clinicians for this project. Are those costs sitting in our Q1, have we incurred any of these costs in the current quarter, Q4, I mean?

Yogesh Gupta: We have not hired any doctors as of now because doctors will get hired near to the opening of the facility. So we are talking to various people.

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Pankaj Sahni: Offers are out in the market, but the cost is not yet in the P&L, except for the people that are already sitting in our other units who may move to Noida.

Moderator: Next question is from the line of Sumit Gupta from Centrum.

Sumit Gupta: So sir, strong momentum was visible in developing units in this quarter. So do you see the momentum continuing for the, let's say, forcible future, for the next at least 1 to 2 years.

Pankaj Sahni: So I mean, I always caution on these calls and in my meetings that we don't run this business quarterly, we don't really look at how things move quarterly, those just happen to be the outcomes basis, how you put the calendar reporting together. What I would say is that we feel still fairly confident about the future growth both in the short term as well as in the longer term on all the developing hospitals.

Now just to kind of keep in mind that as we move back now into what we have

ad originally at the

IPO time classified as developing and non-developing I think we have taken about a 6-year time frame. So theoretically or technically, I guess Lucknow will now start to move into the mature hospitals depending on how you define that. But that's really just how you report out in the investor presentation.

But I think if you forget about this nomenclature of classification of developing versus non-developing and you talk specifically about both the units as well as the both the cities. I think we have a good amount of growth to come in both Lucknow and Patna because we still have bed additions to do there, first of all. Secondly, we are still doing very aggressive hiring in both the units. And thirdly, if you look at the potential of these markets, both in terms of the city of Lucknow and the city of Patna as well as the surrounding areas, there's quite a lot of scope for all of this to grow. And I did mention as well in response to another question that we do believe, at least the Lucknow-Kanpur region will develop as a more robust medical hub. And we've seen that already with Gurgaon, we've seen that with other cities like Delhi and even places like Bangalore, Hyderabad, they end up delivering a good amount of growth for all the hospitals in the ecosystem once they become these more established medical hubs.

So I think Lucknow and Patna very, very early days for those markets on a health care industry point of view, and they will continue to develop and continue to grow.

Sumit Gupta: And sir, second question is on the Noida facility, which is about to come. So how do you see this facility ramping in the first year? And do you see competition from Max or Jaypee will add to the competitive intensity and some other hospitals are also there like Jaypee. So how do you see the competitive intensity in the Noida region particularly?

Pankaj Sahni: So first of all, just to clarify, Max, Jaypee is the same now because Max has taken over Jaypee. If you look at the Noida region, right, the population explosion just in Noida itself, forget about the surrounding cities, all the way up to Agra, Bareilly, Meerut, Saharanpur, etc, is just very, very, very high, very, very dense. And frankly speaking, historically, Noida has had a very short supply of hospital beds, leave alone quality hospital beds. So we believe that there is a very big opportunity in Noida. We believe that even after the kind of our hospitals open and some of the

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other expansion that has been discussed with some of the other hospitals, there's still quite a lot of capacity required in that territory. That's our belief. So we don't see any big issue with respect to oversupply, if that is the first question.

As far as the ramp-up goes, look, again, hospitals typically do take a couple of years to ramp up. Medanta has been very fortunate that we've been able to deliver EBITDA breakeven in our recent facilities of both Patna and Lucknow within the first year. I would say that is good and fortunate. We shouldn't always plan for that. There is a certain buildup time.

And a lot of it depends on how the clinicians come on board, how the community reacts to our services. And we are feeling fairly confident about this. But any normal hospital, especially hospital of this size does have a certain ramp-up period. And we are planning for that but of course, we are hopeful for as fast as possible.

Moderator: Next question is from the line of Tushar Manudhane from Motilal Oswal Financial Limited.

Tushar Manudhane: So with respect to the addition of operating beds, maybe specifically for FY26-27, like with Noida getting added. Is there a scope to add more number of beds at Lucknow, at Patna maybe specifically for FY26-27, if you can just bridge the bed additions for next 12 to 24 months given the other projects are

under construction phase?

Pankaj Sahni: Yes. So if you look, Tushar, at our investor presentation, I think it's Slide #32, if I'm not mistaken. You will see that there is bed addition plan not only in Noida, but also in Lucknow and Patna over the course of the next, let's say 12 to 18 months.

So we do expect to add approximately 1,000 beds across all of these 3 units in the next few months and years. And that doesn't include or maybe that's part of that also, we'll have our new 110-bed facility, which we have just taken on board, I think, in January of this year, if I'm not mistaken in Ranchi, which is augmenting our existing 200-bed facility because we had some capacity constraints there and wanted to upgrade the technology and infrastructure.

So the Ranchi facility, we hope to come on board this quarter itself in Q1 FY26 and then all the 3 other facilities, Noida, Lucknow and Patna, should have bed additions as well.

Tushar Manudhane: Got it, sir. If you could elaborate on this Guwahati at your new addition in terms of location in first place for us and subsequently, what sort of investment specifically over next 1 year or 2 year and then subsequently in terms of construction? While you have already highlighted the overall capex, but if you could just break that into, say, construction cost and land purchase cost?

Pankaj Sahni: Yes. So we have been looking actually for land in Guwahati or in the Northeast region for some time. And I believe when we announced this as part of the governments Invest in Assam Summit or Initiatives, we have been able to get land from the government and to build out a 400-plus bed super specialty. We currently have been allocated 3 acres of land in Guwahati.

I don't know how familiar you are with Guwahati, but this is on one of the main highways, which actually connects Guwahati to Meghalaya and Shillong as well. So it has a good drainage, not

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only from the Guwahati region, from the upper Assam region as well as from parts of Meghalaya. So very well situated from that point of view.

The project cost is about INR 30 crores to INR 35 crores is the cost of the land, and the total cost is approximately about INR 500 crores. And like with most greenfields that should take us about 3 years or so to get going. So we have already triggered some of the planning as far as the clinical planning and the drawing activities for that. But of course, that will take some time. We just got the land, I think, in a month or so.

Yogesh Gupta: We got the agreement from the government and now we have to get the registry done, we'll take the position after that.

Pankaj Sahni: Yes.

Tushar Manudhane: Understood. Just on this INR 500 crores for 400 bed given the locations is Guwahati, still INR 1 crore per bed sort of an investment, typically tier 2 city correct me if I'm wrong, this is not a Tier 2 city, but this typically takes lower bed cost, so any particular reason why the spending is much higher.

Pankaj Sahni: Tushar, we at least in Medanta ecosystem, if you have seen our kind of hospitals that we have built in Lucknow and Patna both from a civil, interiors and infrastructure point of view as well as in terms of the medical equipment that we put in there, whether it's the latest CT, MRI or in fact, the latest and most advanced LINAC machines, we have not compromised on any of the quality in these cities.

So the basics of construction remain the same. Our belief is that we should be able to deliver this kind of standard of care and build out these kind of facilities. And in fact, one of the reasons

why the government was very keen that Medanta come into that region as opposed to maybe many of the other options was exactly because they understood that when we go in, we bring a standard that is second to none, and we don't necessarily compromise on the quality of the facility

that we put up just because it may be a city that is a non-metro city.

So having this in mind, yes, there is a difference between the land cost in Guwahati, and say Delhi or Mumbai, and we've been fortunate to get land at very attractive rates, thanks to the scheme of the government. But the basic cost is more or less the same. I think today, if you build a hospital with any kind of standards like what we put up I think INR 1 crore or INR 1.2 crore, sometimes even going a little bit higher, is a typical cost, especially if you're going for bigger scale, you could possibly do it at less for a smaller 200-bed facility but this will be 400 beds and maybe even higher than that. So I don't think it's very different from building anywhere frankly. Tushar Manudhane: Got it, sir. And just lastly, while you've outlined the overall INR 4,000 crore capex for the next 5 years, but if you could really highlight the capex for FY26 and FY27?

Pankaj Sahni: Yes, I'll let you Yogesh take.

Yogesh Gupta: Tushar, we haven't done a breakup year wise at present.

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Moderator: Next question is from the line of Dhawal Khut from Jefferies.

Dhawal Khut: So I'm not sure if you covered it earlier, but I wanted to know the outlook for Lucknow hospital. Do you expect significant volume growth to continue? When are you adding additional beds into the unit? And did the quarter have any positive or negative impact because of Mahakumbh? And have you seen those impact reversing in the first quarter? How are the trends in April, so that's one? Second, needed data point, what's the census bed count in our developing hospital units.

Pankaj Sahni: Okay. So a couple of questions. Let me see if I remember all of those. I think the first question was around Lucknow, right? What was the growth on that?

Dhawal Khut: Yes, the outlook, and when do you see adding more capacities within the unit? Do you see profitability being steady or improving and growth being higher than what we have seen or steady or any kind of adverse impact?

Pankaj Sahni: Okay. So if you recall our various quarterly presentations over the course of the last few quarters, you will, of course, remember that we had a little bit of a lower growth rates in Q4 and Q1 specifically as Q4 of last year and Q1 of this year and some drag of that into Q2 to some extent of this year. And we had mentioned significant steps being taken to bring that unit back to the kind of volumes that we had seen earlier.

So happy to report since Q3 - Q4, we see fairly strong performance in Lucknow, very good EBITDA margin profile, very good growth volumes, good amount of performance across all of that unit. So very happy with how that has played out over the last couple of quarters. Obviously, as you look at all the 4 quarters together, there's some impact of that on a year versus year basis. But we do see that growth having come back already, and we do see good opportunities as we move forward.

We will be adding beds in Lucknow. We have a certain amount of beds to add in on various floors in our existing facility, which is already partially complete. We could be adding currently maybe mother and child additional number of beds, maybe adding some more beds for some of our transplant and complex specialties. I mentioned, I think Dr. Trehan may have also mentioned, we've added in pretty advanced equipment there in terms of robots. I think we've already done, within a period of about 6 months, we've done over 100 robotic cases, which is the highest number in the Lucknow area, I think, in a short period of time. And we continue to invest in clinical talent as well as technology there.

So for us, we are not stopping anything in Lucknow, going full speed ahead with all of the activities there. And we do believe that there is good potential left in the city. We'll be adding

an additional specialties also in the coming months and quarters. So there are certain specialties,

we still don't have them. And we have plans to acquire clinical talent to be able to deliver those specialties as well. So that's the story with Lucknow.

What was your second question, sorry, if you can remind me again?

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Dhawal Khut: Yes. So do you think your operations were impacted because of Mahakumbh during the quarter? Was there any delay in elective surgeries that you felt, that kind of impacted occupancy? And maybe that could have started to come in from April onwards? Or overall, you were neutral or you saw some benefit because of a lot of population coming into the UP region and Lucknow region also.

So was there any kind of impact because of the entire population moving into this UP region out there?

Pankaj Sahni: So I mean, I don't have any specific data that tells me people who are coming from Mahakumbh suddenly decided to also use it as an opportunity for any kind of health care experience. So I don't think it was positive in that way in terms of adding any great population inflow into the health care facilities.

There is, of course, definitely some impact on these kind of festivals in terms of people having elective procedures. We see this pretty much every season where during, of course dengue and some of the seasonal diseases side, we see that during, say, the Diwali, Dussehra, Chhath Puja kind of time frame where the festivals are on. We do see some kind of dip in the elective procedures.

And so to that extent, I guess, Mahakumbh also has more of a negative impact than a positive impact because of the population coming into the UP region, I think. But I don't think that I can tell you very specifically exactly what was the number of people who did not come or came because of Mahakumbh, I don't think we track that. But, yes, some movement across different weeks travel constraints, etc, it does impact patient movement to some extent.

Dhawal Khut: Got it. And lastly, just -- yes, data point, what on the developing hospital side.

Pankaj Sahni: So if you look at our census beds on the developing hospital side, we have about 600 or so beds in Lucknow and about 320 beds in Patna. So around 930 is the census beds for the developing.

Moderator: Next question is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: Sir, first question, matured hospital margins have come down on a year-on-year basis by 120 basis points. Can you please call out the reasons for the same?

Pankaj Sahni: Yes. So on a year-on-year basis, full year EBITDA margins in matured hospitals have increased from 24.5% to 24.8%, so nominal increase. But I think if you look at it at a macro level, it's more or less stable. Of course, there is some fluctuation with respect to how we see the various specialties.

So as we find certain specialties, which are maybe more heavy on material costs, they have some kind of an impact. Also to the extent that we did some hiring over the course of the last year or so, there is some amount of clinician cost and manpower cost that comes in. But broadly speaking, I would say that I would consider these to be more or less stable in nature, not much

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difference between a 24.5% and 24.8%. But yes, maybe some amount of material cost increase a little bit between the years.

Alankar Garude: And sir, in the presentation, Slide 26, it shows the margins have come down from 25.2% to 24%, that's why I've asked that question.

Pankaj Sahni: Just looking at 2 quarters in isolation doesn't necessarily, I think, give the whole picture because obviously certain specialties have some impact in certain months. It's not very, very crystal clear to look at Q4 versus Q4. I would say that a

more realistic picture would be to look at full year versus full year.

Alankar Garude: Understood. Sir, just maybe 1 clarification there. No acquisition merger-related expenses have been accounted in that margin number, right?

Pankaj Sahni: No, not in the EBITDA, that's come below the EBITDA.

Yogesh Gupta: So that's below the EBITDA, as an exceptional cost.

Alankar Garude: Got it. And the other question was on Indore. I see the second hospital has not been included in our capex plan. So is it still possible to go ahead with that, or as a matter of precaution, we decided to kind of not include that in our current plan?

Pankaj Sahni: Yes. Alankar, we had mentioned a couple of times that this hospital has gone from a nonbinding agreement some time ago. And I said that it was more appropriate to say that we just put it on the back burner from a disclosure point of view.

We haven't terminated our conversations with that party, and we still maintain that there is a door open there for us to do it. I did also mention in some of our earnings calls, maybe one or two quarters ago that we are looking at alternatives as well. But I thought it was more appropriate for us to pull it out of the current disclosure to not mislead that this is something which is imminent because it will require kind of a revisit.

Yogesh Gupta: And also since we don't have a very clear time line on the legal case, so we don't really know

that whether it will happen in the next 4 years or not. So that's how we have taken it out and put it into the notes.

Moderator: Ladies and gentlemen, we will take this as the last question for the day. I would now like to hand the conference over to the management for the closing comments.

Pankaj Sahni: Thank you, everybody, for your questions and for joining us today. Just to wrap up, as we step into our next phase of growth, we, of course, remain committed to our original philosophy of delivering the highest quality of care at the highest standard to the highest values, we believe that through this approach, we'll continue to expand our footprint in a responsible manner, invest in our people in a responsible manner and also continue to bring the highest quality and higher standard of infrastructure as close to the communities as possible.

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Please do feel free to reach out to our Investor Relations team in case there are any questions that remain unanswered. Otherwise, all the very best and thank you for joining. See you soon.

Notes:

1. Please refer to the revised Investor Presentation for the fourth quarter and financial year ended March 31, 2025 Result uploaded on the website and stock exchanges on May 16, 2025. Also refer to slide no 13, 19, 22 and 26 of the presentation for the changes and also refer to the note "Matured and Developing hospitals numbers have been restated due to the merger of MHPL (Lucknow Unit) with GHL (holding company). The intercompany transactions between two companies were eliminated as per the requirement of accounting standards. The above script has been edited to reflect the updated numbers for Matured and Developing hospitals.
2. This transcript has been edited for readability and does not purport to be a verbatim record of the proceedings
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Call: Aug 2025

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Dept. of Corporate Services
BSE Limited,
P J Towers, Dalal Street,
Mumbai - 400 001

Scrip Code: 543654 The Manager
Listing Department
National Stock Exchange of India Limited
Exchange Plaza, C -1, Block G,
Bandra Kurla Complex,
Bandra (E), Mumbai - 400 051

Symbol: MEDANTA
Sub: Disclosure under Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 - Earnings Conference Call Transcript

Dear Sir(s),

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the transcript of Earnings Conference Call held on Friday, August 08, 2025, for the first quarter ended June 30 , 2025 results.

The Transcript is also available at our website: <https://www.medanta.org/investor-relation>

This is for your information and record.

For Global Health Limited

Rahul Ranjan
Company Secretary & Compliance Officer
M. No. A17035

Encl: a/a

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"Global Health Limited - Medanta
Q1 FY 2026 Earnings Conference Call "
August 08, 2025

MANAGEMENT : DR. NARESH TREHAN – CHAIRMAN AND MANAGING
DIRECTOR – GLOBAL HEALTH LIMITED – MEDANTA

MR. PANKAJ SAHNI – GROUP CHIEF EXECUTIVE
OFFICER AND DIRECTOR – GLOBAL HEALTH LIMITED
– MEDANTA

MR. YOGESH KUMAR GUPTA – CHIEF FINANCIAL
OFFICER – GLOBAL HEALTH LIMITED – MEDANTA

MR. RAVI GOTHWAL – HEAD INVESTOR RELATIONS –
GLOBAL HEALTH LIMITED – MEDANTA

MODERATOR : MR. AMEY CHALKE – JM FINANCIAL

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Moderator: Ladies and gentlemen, good day and welcome to Global Health Limited Q1 FY '26 Earnings Conference Call hosted by JM Financial Institutional Securities Limited. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Amey Chalke from JM Financial. Thank you and over to you, sir.

Amey Chalke: Thank you, Avirath. Good afternoon and a warm welcome to all the participants on Global Health Limited 1Q FY '26 earnings call hosted by JM Financial. Joining with us today from the management side, we have Dr. Naresh Trehan, Chairman and Managing Director; Mr. Pankaj Sahni, Group CEO and Director; Mr. Yogesh Kumar Gupta, Chief Financial Officer; and Mr. Ravi Gothwal, Head of Investor Relations. I will now hand over the call to Dr. Trehan for his opening remarks. Thank you and over to you, doctor.

Dr. Naresh Trehan: Thank you. Good afternoon to everybody. Thanks for joining us today for Medanta's Q1 FY 26 earnings conference call. I hope all of you have had the chance to review the results and presentations that were released yesterday.

At Medanta, we continue to build on our vision of delivering world-class, patient-centric and compassionate care. Clinical excellence remains the cornerstone of our identity and during the quarter, we remain focused on strengthening that foundation through consistent and disciplined execution. During the quarter, we have had on-boarded over 150 doctors, including 30 senior clinicians across specialties.

This strengthens our medical department and the depths with which we are able to deliver complex and multidisciplinary care. I am also pleased to share that we have expanded our presence in Ranchi by operationalizing a new 110-bedded hospital in July 2025 under a long-term lease agreement with the owners. This additional unit enhances our ability to serve the underserved region.

Furthermore, we look forward to commissioning of our 550-bed Medanta Noida facility, which is poised to commence operation in the coming weeks. This landmark addition to our network will significantly enhance our capacity, strengthen our presence in the National Capital Region and further our mission of delivering world-class healthcare to a broader community. As a result of our focus on delivering the highest standard of quality care, we have been able to deliver strong financials and operational performances during this quarter.

With that, I will now hand over the call to Mr. Pankaj Sahni, our Group CEO, who will walk you through the quarterly performance.

Pankaj Sahni: Thank you, Pankaj. Thank you, Dr. Trehan. Good afternoon and thank you for joining us today. I am happy to share that Medanta had a strong start to the fiscal year. In Q1 FY 26, we have delivered our highest ever quarterly total income and EBITDA while maintaining healthy

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margins and driving operational efficiencies. Our performance reflects the strength of our integrated care model, disciplined execution and the growing trust of our patients.

Let me begin with the key financial performance highlights for Q1 FY 26.

During the quarter, Medanta delivered total income of INR10,513 million compared to INR8,830 million same quarter last year, registering a strong growth of 19% year-on-year.

EBITDA for the quarter was INR2,553 million, an increase of 23% year-on-year, with an improved EBITDA margin of 24.3% year-on-year. Please note that EBITDA is before non-cash ESOP expenses of INR79 million.

Pro

fit after tax for the period was INR1,590 million, year-on-year growth of 50%. PAT margins for the quarter improved to 15.1% compared to 12% in the same quarter last year. In Q1 FY 2026, profit after tax is higher due to a non-recurring exceptional income of INR196 million, arising due to reversal of earlier accrued interest liability on EPCG imports in the Lucknow unit, following the transfer of the EPCG licenses to GHL pursuant to its merger that was announced last quarter.

Our overall performance was driven by sustained growth in patient volumes across units and improved realisations, especially in Gurugram. Our inpatient volumes during the quarter increased by 14% and the outpatient volumes increased by 13% year-on-year. Our average occupied bed days for the quarter increased by 13% year-on-year, with occupancy of approximately 63% on increased bed capacity.

Average revenue per occupied bed for the quarter was INR66,584 compared to INR64,035 in the same quarter last year, an increase of 4% year-on-year. This increase was largely driven by an increase in realisations in the Gurugram unit and change in specialty mix. During the quarter, revenue from international patients was INR636 million, an increase of 34% year-on-year.

Coming to the matured hospital performance, during the quarter, total income from our matured hospitals was INR7,006 million compared to INR6,328 million in Q1 FY 2025, registering a year-on-year growth of 11%. The EBITDA of mature hospitals stood at INR1,640 million, reflecting a growth of 7% year-on-year, with a margin of 23.4%.

Inpatient volume growth was 6% year-on-year. ALOS declined by approximately 4% year-on-year. These two combined factors resulted in an average occupied bed days increasing by 1%, representing an occupancy of 62%. ARPOB grew by 9% to INR73,256 in Q1 FY 2026, primarily driven by increase in realisation in Gurugram and change in specialty mix.

Now, when it comes to our developing hospitals, developing hospitals comprises Lucknow and Patna. To note, Noida Hospital, which is yet to commence business operations, has now been included in the developing hospital categorisation.

Noida expenses of approximately INR30 million, of which 50% are attributable towards employee cost and the remaining towards other expenses, have been included in Q1 FY 2026

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numbers. Our Lucknow and Patna hospitals continue to show strong momentum as they move towards operational maturity.

Total income during the quarter was INR3,219 million, compared to INR2,369 million in the same quarter last year, registering strong growth of 36% year-on-year.

EBITDA was INR942 million, compared to INR589 million in the same quarter of last year, registering a robust growth of 60%, with margins remaining strong at 29.3%. Average occupied bed days increased by 39% year-over-year, representing an occupancy of 64% on increased bed capacity. Lucknow ARPOB's declined by 11% year-on-year due to an increase in ALOS by 17% year-on-year, driven by an increase in share of scheme patients.

Patna hospitals saw an improvement in ARPOB's of 8% year-on-year, driven by a 7% reduction in ALOS as part of our ongoing operational efficiency. The combination of these factors resulted in developing hospital ARPOB declining by 3% to INR56,704.

During the quarter, 20 beds were added on the 9th floor of Tower A at Medanta Patna Hospital, marking the completion of Tower A.

Moving on to our projects update :

We are excited about the upcoming launch of our 550 -bed Medanta Noida facility, which is expected to be operational in the coming weeks. We have already onboarded over 230 employees, all of whom are undergoing comprehensive training programs.

In July 2025, we successfully operationalised our 110 -bed hospital in Ranchi, further deepening our presence in the eastern region

of India and enabling access to high -quality healthcare in an underserved market .

Interior fit -out and commissioning for Tower B in Patna and two floors in Lucknow is in progress, which will add additional capacity in these locations.

Our broader expansion pipeline now includes 2,000 beds over the course of three to four years. These projects are in various stages from design to execution and are aligned with our strategy to scale in high -demand markets. Over the near term, we aim to add 1,000 beds supported by the development of advanced medical technology and expansion of clinical teams. This forms the foundation for sustained, long -term growth across our network.

With this, I request the operator to open the line for questions. Thank you.

Moderator: Thank you. Thank you very much. The first question is from the line of Vishnu, an individual investor. Please go ahead.

Vishnu: Sir, do you have any expansion plan of 500 -550 beds in Noida Hospital ?

Pankaj Sahni: As of now we have only plan of 550 beds in Medanta Noida. As we have told earlier will start from 300 beds, as and when demand increase will go upto 550.

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Vishnu: Thank you .

Moderator: Thank you. The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services. Please go ahead.

Tushar Manudhane: Okay, great. Sir, just with respect to the additional operational cost of Noida and Ranchi, if you could call out how much to factor for a full year.

Pankaj Sahni: Operational cost for Noida?

Tushar Manudhane: And Ranchi, the new hospital.

Pankaj Sahni: So, Noida operational cost right now only includes about INR3 crores of cost in Q1 as EBITDA expenses, basically. There's no revenue there. As we move forward, Tushar, it really depends on how we scale up the unit.

Ranchi, on the other hand, will be mostly supported by the existing unit as far as the administrative and overhead costs go. So most of the costs in the new unit at Ranchi will only really be for the clinical nursing teams. But exact numbers, I can't tell you because I don't know what it will lay out in the future.

But as we scale up, as the doctors come on board, there will of course be some amount of cost which we may incur as and when the doctors and the nurses and everybody comes on board. But exact numbers right now, I can't project out into the future. In any case, we don't really give the future projections.

So, I'm not sure. But I don't see any real difference why there would be a very significant change in the cost or performance profile of the Noida unit versus any other unit, except of course for the initial startup kind of costs.

Tushar Manudhane: Got it, sir. And there's 150 plus doctors onboarded. So, broad breakdown, I presume this is largely for Ranchi and Noida. Or correct me if I'm wrong. And just sort of help us understand where all these...

Pankaj Sahni: Yes, so I would say that the majority of the 150 doctors is actually for our existing units. That includes almost all the units, Gurgaon, Lucknow, Patna, even some additions in Indore and Ranchi. The addition for the new unit in Ranchi will be very negligible. And in Noida, it will be hardly about 15 to 20 doctors. So most of the 150 doctors that you see are actually all for the existing units.

Tushar Manudhane: Got it. So, we should be expecting a reasonably more number of doctors getting onboarded in the coming quarters for Noida and a new unit of Ranchi, right?

Pankaj Sahni: Yes. So, if you recall, Tushar, we have been consistently mentioning over the last several quarters that we actually still have quite a bit of growth to add into the Lucknow and Patna unit, both in terms of the bed build-out as well as in terms of the clinical hiring over there. So, just to give you an example, we are currently building out in Patna, our mother a

nd child floor in Tower

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B. And there will, of course, be a complete hiring of the department there. So, it is not like the hiring is only for Noida. It is very much for the other units.

We have also hired in units in Gurgaon. So, to give you an example, we have a complete new department – team for ophthalmology, which we have hired into our Gurgaon unit. We've hired oncologists into our Gurgaon unit. We've hired nephrologists. We've hired cardiac surgeons. So the hiring which you see is very much part of our agreed and announced strategy of, I believe, we called it on focusing on the core. And we will continue to do that in the coming quarters as well. And of course, there will be a complete hiring for Noida.

Tushar Manudhane: All right. Sir, just lastly, on Matured Hospital, there's been a reasonably good increase in ARPOB made the year-over-year, quarter-over-quarter. But at least sequentially, and despite occupancy remaining stable, we've seen a reduction in the margins. So any particular reason to call out here?

Pankaj Sahni: So, I think that the margin reduction is largely the impact of the salary increments which come in typically in quarter one. So, when you look at it on a sequential quarter basis, or even if you look compared to last year, large percentage of this is employee costs, which then gets, you know, kind of across the year gets managed out. And we saw that last year as well. Your second question was ARPOB, if I'm not mistaken, right?

Tushar Manudhane: Yes.

Pankaj Sahni: Yes. So ARPOB has seen actually a good amount of growth. What we've seen is that across many of the specialties in Gurgaon, we've seen some amount of realization growth. And then of course, as with the country and the industry as a whole, a lot of work moving towards oncology

in terms of the sales mix. So, that has a natural growth towards ARPOB. Also, certain efforts have been made on ALOS management, which we continue to do.

So, if you see our average length of stay, it's quite low for a complexity of this size at about three days. So, really have those impacts of low ALOS actually also impact the ARPOB growth.

Tushar Manudhane: Just on your comment of ARPOB realization across therapy, so this is like more of a price hike, which we have taken in Gurgaon, is that the same assumption?

Pankaj Sahni: No, actually, when we look at it internally, yes, we have taken some amount of price hike. And, some of it also plays out with respect to, at least maybe with respect to quarter last year, because last year, we took some insurance renegotiations. So, that may not be reflected in Q1 -25.

I think that impact probably came in Q2, Q3 and onwards. So, that may be a little bit of it. But I would say that, Yogesh, correct me if I'm wrong, that would be less than 2% of the impact would be tariff. Most of it is just complexity and realization. Daycare procedures are increasing. In many cases, sales mix across because you're looking at it consolidated.

Tushar Manudhane: Reduction in ALOS also -- higher as well.

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Pankaj Sahni: Yeah. But realization increases largely linked to, within the specialty also, we've seen realizations increase. And some of that is just linked to, more complex work happening. So, I think we had mentioned this last time also, if you do say robotic versus non-robotic, there's a change in the ARPP. In some cases, in neuro and cardiac, we've seen some changes. So, those are the kinds of things.

Tushar Manudhane: All right. Thanks. Thanks.

Moderator: Thank you. The next question is from the line of Bino Pathiparampil from Elara Capital. Please go ahead.

Bino Pathiparampil: Hi. Good evening. Congratulations on a great set of numbers. A couple of quick clarifications. One, I see that the depr

eciation and amortization cost has come down this quarter compared to last few quarters. Why is that?

Yogesh Kumar Gupta: There are certain assets which have completed their life, like computers, etc. When they complete their life, their current depreciation becomes zero. And plus building life, which was taken as 30 years, has been revised now in the industry standard to 60 years, but that impact is very minimal.

Bino Pathiparampil: Okay. So, for existing assets, this will be the new level at which there will be D &A. And then as you add assets, it will pick up. Okay. And second, any update on the indoor O&M that you are planning?

Pankaj Sahni: On which one, sorry?

Bino Pathiparampil: Indoor, there was a planned hospital, right? Any update on that?

Pankaj Sahni: Yes. We had announced, I think, maybe one or two quarters ago that that had gone into the back burner and that we were not very confident that that would come back. So, as on date, no update on that.

Yogesh Kumar Gupta : So, that was basically a legal case going on between the previous owners and that was not moving. So, we have said that we may not we are not very sure of the progress on that. So, we will be putting it into kind of till we get the clarity, we are not very sure about the project.

Bino Pathiparampil: Got it. Thank you. Thank you.

Moderator: Thank you. Before we take the next question, we would like to remind participants that you may press star and one to ask a question. The next question is from the line of Damayanti Kerai from HSBC. Please go ahead.

Damayanti Kerai: Hi, thank you for the opportunity. My question is again on Noida Hospital. So, you mentioned you have hired around 15 to 20 doctors for the unit and initially you will start with 300 beds. So, just want to understand, say, how do you open a bed? So, up to 300 in initial two to three months, how many will be actually operational and then at what occupancy level you generally open up further beds, if you can walk us through this?

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Pankaj Sahni: Yeah. So, just to clarify, we have released we have currently, as on today, the number which I mentioned, 15 to 20 doctors are currently on rolls and working for the company. Some of them actually sitting in our clinic in defense colony as well. Of course, offers are out for a far greater number of that upwards of 100. So, they will be coming on board in the next days and weeks

actually.

Now, to go to your question as to how do you start? So, for operationalizing any hospital, you need basically three, four of the key areas to be operational. Of course, we need the ward beds, we need the ICU beds, but more importantly than that, we need the operation theatres and the cath labs and the basic support systems beyond beds to be operational. So, we are currently planning to operationalize with approximately five OTs in the next few days and then two cath labs.

And then as and when we get almost all our ICUs are actually complete. And then the ward floors, you know, we keep adding in. So, we had mentioned that we would start out with 300 beds.

I don't think we will wait. Again, in the , I think , the previous earnings call, I had mentioned that since it's a single -tower property, we would not wait for the buildout for the remaining floors. So, we will continue to build that out over the next months.

There will be no pause in the construction buildout. So, we will continue to move to 500 beds.

And in this particular case, unlike Lucknow and Patna, we don't need to actually put in fresh services or build out a second tower. It's a single tower building. So, we'll just continue the buildout over the next few months and hopefully complete it as soon as possible. And then it will really depend on how the occupancy grows as and when we'll continue to fill out the beds.

Damayanti Kerai: Sure. S

o, day one itself, 300 beds across these, you know, different categories will be available.

And as and when you scale up, may be you'll -- your operations will pick up accordingly?

Pankaj Sahni: Yes.

Damayanti Kerai: Okay. That's helpful. And very broadly, right now, 15 , 20 doctors that you mentioned, how many, like, further offers have been rolled out? Like, what are the plan on the clinical team size for this unit?

Pankaj Sahni: So, we have more than 100 offers out in the market across various departments and across various levels. I can't tell you exactly how many doctors will finally be working in the hospital.

But just to give you some context, if you look at Medanta , Gurugram , which is a 1 ,500-bed hospital, we have almost 1 ,000 doctors who work here.

And out of those 1 ,000 doctors, I would say that probably about 500 to 600, maybe a bit more are what we would consider as the senior category doctors, what we call Associate Consultants ' and above. So, you can apply similar ratios for the 500 beds in Noida.

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Damayanti Kerai: Sure. And on the non -clinical side, your presentation mentioned already you have admin and nurses, etcetera, who have been onboarded. And then, are the costs for those non -clinical staff reflecting in 1Q or some like only some parties right now visible there?

Pankaj Sahni: So some parties whoever, obviously, whoever's on rolls before June 30th, their cost is reflected. The total costs in queue one for Noida is about INR3 crores only. And then, as and when people come on board, of course, their cost is getting added. So, you will see, obviously, a scale -up in costs in Q2.

Damayanti Kerai: Sure. And if you can explain, sorry, I missed out, in the developing hospital segment, why did we see a decline in ARPOB ?

Pankaj Sahni: So, the developing hospitals are largely Lucknow and Patna.

Damayanti Kerai: Yeah.

Pankaj Sahni: If you look at our two facilities, we found that our Patna facility, which I'll take on first, that actually saw an increase in ARPOB of about 8%. And that is largely driven by a reduction in ALOS of about 7%. In Lucknow, we saw a decline in ARPOB by about 11%. And much of that is because of the increase in the scheme business, which has a longer length of stay typically. So, you did find that as the length of stay increases, the ARPOB reduces and, of course, vice versa.

Damayanti Kerai: Okay. So, from the current quarter level of around INR56,000 ; INR56,000; INR57,000, how should we look at the ARPOB trend in this developing hospital vertical?

Pankaj Sahni: So, you know, the way in which we think about ARPOBs, and we mentioned this consistently, is that if you look at this on a quarterly basis, it has got a lot of variation in some quarters, you have different diseases and others you have different diseases. So, we don't really study it on a quarterly level. But if you look at this, over the years, we've consistently maintained that typically we think about a ARPOB growth in normal times, which is a little bit more than inflation.

Of course, this is dependent on certain things like ALOS reduction, etcetera . So, if you notice our ALOS in our facility in Patna, we found quite a significant amount of difference in the ALOS for our PPP patients. So, as we started working on operational efficiency of bringing down the ,

ALOS of the PPP patients, it reduced.

Damayanti Kerai: Okay, that's clear. And my last question is, you have taken Tariff hike in Gurugram unit, that's what I understood. What about ...

Pankaj Sahni: That was last year.

Damayanti Kerai: That was last year?

Pankaj Sahni: It was in forecast but that was last year.

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Damayanti Kerai: Okay, so that impact is getting reflected in 1Q number, right? You haven't taken in 1Q?

Pankaj Sahni: In the first quarter, we haven't taken any impact, maybe there is some carry forward from the last year, especially when you compare it to Q1 of 2025. And then there may be, you know, some small increases. But even if you consider the tariff increases for I think the whole of last year, it's fairly negligible on the overall realization in this.

Damayanti Kerai: Okay. Anyway, like it's a smaller part of your ARPOB will increase, but any plans for any Tariff hike during the current fiscal in any hospital?

Pankaj Sahni: Yes, maybe. We will possibly look at some amount of tariff hike in Gurgaon as we come into the two years from our last tariff hike, especially with respect to the insurance companies, etcetera. Those contracts will need to get renewed.

We've also never taken a tariff increase in Lucknow or Patna. And we may consider a tariff increase there. But these are also maybe not, it is not like it may be blanket, it may be in certain specialties, certain areas. So typically, Medanta has been extremely conservative vis-à-vis the rest of the industry on tariff increases. And we broadly would like to maintain that posture as much as possible.

Damayanti Kerai: Okay. Thank you. These are very helpful.

Pankaj Sahni: So, if the realizations are increasing without tariff increases, then no real need to take tariff increases.

Damayanti Kerai: Okay. Thanks for your response. I'll get back in the queue.

Moderator: Thank you. The next question is from the line of Harith Ahamed from Avendus Spark. Please go ahead.

Harith Ahamed: Good evening. Thanks for the opportunity. My first question is on the new Ranchi Hospital. Is this hospital going to be operated independently from the existing hospital? And if not, then with 110 beds, how should we think about our ability to offer all the specialties in this unit? Wouldn't we be constrained by the number of beds? Or are we looking to expand this hospital further so that we can have more specialties here?

Pankaj Sahni: Yeah. So, I think in one of our earlier investor presentations, we had discussed that this hospital is very near to our existing hospital and we do intend to operate them both as one kind of unified campus. So we will not be looking at them as completely independent. We will operate them as a unified campus. We will also look at, which are the specialties, which will move to the new hospital, which one will remain in the older hospital.

But more importantly, there are certain specialties that we were not able to add in our Ranchi facility because of space constraints, as well as because of some of the infrastructure challenges, because the infrastructure is actually quite an old building. So this new hospital allows us to do two things. One, it allows us to commence the renovations of the old hospital and put some amount of maintenance into that facility, which was challenging while it was running.

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And the second thing that it allows us to do is expand new specialties and hire more clinicians and add -in certain departments that were missing, as well as increase the capacity for very high demand departments. So, to give you a very simple example, we are completely full and overflowing with a waiting list for our dialysis work in Ranchi. So, adding capacity allows us to shift some care to the new facility and then make more space for dialysis. So, examples like that across the existing specialties will also be factor.

Harith Ahamed: Okay. That's helpful. My second question is on ARPOB, both in the Mature Hospitals cluster, as well as the Developing Hospitals segment. In Mature Hospitals, not sure if you've addressed this before, but the 9% growth that we saw this quarter is above what we've seen in recent quarters, where we saw mid-single-digit ARPOB growth in this segment. So, what's

driving the

higher ARPOB growth and how should we think about ARPOB growth here going forward?

Pankaj Sahni: So, I did mention earlier, but you know what much of the growth from the Mature Hospitals has been driven by increased realizations in Gurgaon, as well as some amount of ALOS reduction and operational efficiencies there. As we think about ARPOB growth going forward, again, I wouldn't like to give you a guess on what would happen in this quarter or the next quarter. But when we look at this on an annualized basis, we do see typically anywhere in the range of at least in the Medanta ecosystem, maybe 3% to 7% is our normal.

There are times when you find that the increases are a little bit higher, either because there's been operational efficiencies, or in the case of what we do see this quarter as well, there has been an increase in the complexity of work. So as I mentioned earlier, that sometimes you go from say non-robotic surgery to robotic surgery, so that would increase your realization. You may go into in the cardiology arena into some of the more complex procedures like TAVI, etc.

Those are expensive procedures that increase your realization. Similarly, in neurosurgery, you may have complicated procedures, so that increases your realization. And also as we move into a more precision oncology and immunotherapy type of care in cancer, those drugs tend to be a little bit more expensive, so that also increases your ARPOBs because you have a higher price on maybe a smaller day's day procedure.

Harith Ahamed: Okay. And similarly on the developing hospital side, we've had a few quarters of our public line, and we've discussed some of the factors for this, like the allocation of beds for PPP patients, etcetera. Where exactly are we on this? Have we reached the 25% level? And when can we expect ARPOB growth returning in this segment?

Pankaj Sahni: So, just to clarify on the PPP first, our PPP business in Patna is somewhere in the range of about 15% to 17% of our total volumes. So, to answer your question, have we reached the maximum of 25%? The answer is no.

We continue to work very closely with the government to increase that business, actually, because we do find that the realizations are good, as well as, as we mentioned in the past, the payment terms are very good. And of course, it is our obligation as well to work with the government to do that. That work is on quite aggressively.

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The good part of what we've been able to do in Patna over the last several months is actually reduce the length of stay of the PPP patients to bring them more in line to the cash and PPA patients for the same specialty. So, we do find that there has been an operational efficiency which has been brought in in Patna and that actually has resulted in a pretty good ALOS reduction of 7% and that gets translated to an ARPOB increase of about 8%. So, that's what's happened in Patna.

I would not say that it's stable because for two reasons. First of all, we have about 100 beds coming on board over the next couple of quarters in Patna, as well as we have new specialties coming on board, as I mentioned, say like mother and child. We also would be very happy and keen to have the PPP business get to that 25% number.

And so, I do see that Patna will continue to have growth and therefore some amount of changes across various metrics that we study. As far as Lucknow goes, Lucknow has in Q1 of last year had almost no business on various schemes because if you remember when we had our last year earnings call in either Q4 or Q1, we talked about the fact that we had only really started and signed up for schemes like Ayushman, CGHS, etcetera in FY '25.

So, Q1 would have very, very little of that. And therefore, the length of stay is significantly optimized because it's purely cash insurance patients.

As you get into scheme patients, they tend to have a little bit longer length of stay and that results in an ALOS reduction – ALOS increase – sorry, and therefore an ARPOB reduction.

We will work on optimizing that as we move forward. But again, Lucknow also has beds to get added. So, I would say still various amounts of growth and therefore some amount of changes that may come into the metrics that you look at in Lucknow as well.

Harith Ahamed: Thanks. Thanks for this. That's very helpful.

Pankaj Sahni: Sure.

Moderator: Thank you. The next question is from the line of Anshul Agrawal from Emkay Global. Please go ahead.

Anshul Agrawal: Hi. Thank you for the opportunity. Hope I'm audible?

Pankaj Sahni: Yes.

Anshul Agrawal: Great. So, my first question is on the occupancy in developing hospitals. As per your assessment, is there any element of seasonality here? I believe the occupancy has risen quite sharply and it has even exceeded the same of mature hospitals. So, is there any element of seasonality or any

onset of monsoons, earlier monsoons, anything that you could share on this?

Pankaj Sahni: So, I don't think, I mean, I don't have exactly the impact of things like dengue, etc. off the top of my head, but I don't think that really has impacted in Q1. Typically, we see that kind of impact more in Q2 and Q3.

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There is definitely a seasonality impact with certain specialties like -- I forget now whether it was FY 2025 or 2024. I think it was FY 2024, we had a very significant growth in volumes because of dengue in our Lucknow unit. We do get seasonality impact, but that's typically Q2, Q3, so not so much in Q1. So, I would assume not much impact of seasonality. We do have the growth seen coming in in terms of our occupancy in our Lucknow unit. And so, when you look at our Lucknow unit, that has seen a significant growth in volumes and therefore a significant growth in occupancy, not just on the scheme business, but on all aspects and across the board as well. So, if you look at the absolute volume growth in the Lucknow unit also, that's increased pretty significantly. And that's why you're probably seeing an increase in the occupancy numbers.

Anshul Agrawal: Great. Would you be able to share the scheme patient mix, revenue mix from schemes in Lucknow?

Pankaj Sahni: So, we don't break up the payer category by each area. But if I look at it, it would be, you know, reasonably negligible. I mean, I would say that across all hospitals, across Lucknow, all the scheme business would probably be somewhere in the 10% or under range.

When you look at our business as a consolidated, I think also it ranges in this area. With the PPP business of Patna, I think adding into that a little bit. So, if you look at our investor presentation, the PPP business is actually categorized in the others category. So, that is not shown in the -- just trying to quickly check which slide it is. But it is not shown in the revenue mix of CGHS, ECHS. I think it's on slide number 11 of the presentation.

Pankaj Sahni: You can see it shows about 11% business as a whole of CGHS. And then there's about 3% in others. That includes the PPP business of Patna, which is at CGHS tariffs, although realizations are still high.

Anshul Agrawal: Got it, sir. Is it 11 plus 3 for the entire portfolio?

Pankaj Sahni: So, not all the 3% is PPP business. Some of that includes business from some of our government contracts. But yes, I think at an overall level, you can say 11 plus 3 would be at a non-hospital tariff rate.

Anshul Agrawal: Got it. Clear. The second question that I had was, what could be peak occupancy levels generally for our hospitals? I see that in our mature portfolio, Gurgaon Hospital has hit about 70%-71% in the past. Is that like the peak occupancy levels? Because I believe midnight occ

upancies would

be higher or we can go higher than that also at current capacity.

Pankaj Sahni: No. So, just to clarify, midnight occupancy is typically lower than peak occupancy during the course of the day because patients get admitted and discharged. And so, therefore, midnight occupancy is always typically lower than what the peak occupancy is during the course of the day, not the other way around.

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Now, if you look at the ALOS which we have, for the group, it's about three days, 3.03 days if I'm not mistaken. And if you just increase that ALOS by half a day or one day, you would find that the occupancy would go upwards of 70% to 75%. And the reason I mentioned this is that occupancy on its own is not necessarily a good metric because it means patients are staying in the hospital for a longer period of time.

And that doesn't necessarily make it a good thing. So, if you look at our ALOS, it is amongst the best in the industry and something which we've always maintained. We work on very aggressively because the objective is to get you home, not to make you stay at night.

And so, therefore, occupancy on a standalone basis would not be the right metric in my opinion. And the right way to look at this is to see are we seeing inpatient growth, volume growth, which we have seen consistently across the group and across both mature as well as developing hospitals. But to answer your question, what is the peak occupancy?

I think depending on the kind of complex work which you do, that would range, you know, probably anywhere from 70% to 75% I think would be what a big system like Medanta, which

is doing very complex care, would look at. I mean, there are systems around the world that operate on higher occupancy. But some of them are far more institutionalized and organized in various ways.

But the system starts to get choked once you cross that at a midnight occupancy level. But again, occupancy on a standalone basis is not necessarily a good thing. The right way to think about it is more around throughput than who stays at night.

Anshul Agrawal: Noted. Very good. Thank you so much for your answers.

Moderator: Thank you. The next question is from the line of Amey Chalke from JM Financial Institutional Securities Limited. Please go ahead.

Amey Chalke: Yes. So, first question I have on international patient revenue. This quarter, we have seen a good jump of 34% year-on-year on revenue side. I believe the contribution largely is coming from Gurgaon, right? So, how much will be the contribution now for the Gurgaon Hospital for the international patient? And is it the reason why the ARPOs are also going up?

Pankaj Sahni: So, yes, you're right. International business is almost exclusively Gurgaon. I would not say largely, it's almost exclusively. If you recall over the last few quarters, there has been some amount of reduction and impact of the international business. And that is driven by three factors. One factor would be the Afghanistan change in leadership there.

Second is some of the challenges which we've seen in Iraq. And the third one is the challenges which we've seen in Bangladesh. So, if you look at all of these three things, they have impacted international business over the last several quarters.

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Now, a lot of that is one, of course, stabilizing with the exception of Afghanistan, but Iraq is definitely stabilizing. Bangladesh, still some stabilization to continue. But what we've seen is that actually there's been a growth in international business from other areas.

So, Africa is one notable area where we've seen good amount of growth in the Medanta system and I'm sure across the industry. And the second is the CIS countries. So, I guess as we look at some of the challenges, because B

angladesh was definitely one of the largest countries sending medical tourism patients to India, that has been offset to some extent by patients from Africa, CIS and other parts of the world.

We do expect this to continue. And we hope also that Bangladesh will continue to stabilize and then restart, the original stream of business from there. The second important point as far as the Medanta ecosystem is concerned is that we do expect that Noida will have an international patient base, because being part of the NCR region, that is a market which is more likely to attract international patients than say a Lucknow or a Patna. So, that's the second important point. Then to answer your question that approximately I would say 11% to 12% of our business in Gurgaon comes from international. And while we've seen about 1 percentage or 1.5 percentage uptick in that vis-a-vis the same quarter last year, I don't think that that alone has contributed to the bulk of the realization increase or the growth.

I think our business is very much driven fundamentally on domestic patients with international really more of the icing on the cake. So, much of the growth is really coming from, as I said earlier, the realization increases on domestic patients.

Amey Chalke: Sure. The second question I have on the expansion beyond Noida, is it possible to give any updated timelines for South Delhi, Mumbai and Pitampura?

Pankaj Sahni: Well, I don't have really any more timelines as far as dates or anything like that. As you see in the investor presentation, we mentioned various stages of work. So, for example, in our Mumbai project, we have received the additional FSI approval that we were seeking. And that actually allows us to build the kind of beds and get the kind of scale that we are looking for. We have got the building plans being under preparation.

In our Pitampura project, we have submitted the building plans for approval and we are under process for all of that. As far as timelines, I would say that at the overall level, just like with our Noida project, things typically take in the range of three to four years to get built out. So, I don't see any very significant shift in the absolute timelines.

Amey Chalke: Any one of these units or assets you expect to see in FY 2029 or should we assume the FY 2030 as a time to commission these units?

Pankaj Sahni: Well I would say that that is a timeline that I don't want to predict out into the future. You know that you can't get a building up in a day. So, it takes a certain amount of time. I don't know whether we should be more confident or less confident of build out in places like Mumbai and

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Guwahati than we have been in UP and Bihar. But we are very happy to get it in FY 2029 if we could.

Amey Chalke: Sure. That comes to the third question basically. So, we have a good amount of cash on books. The capex requirement looks like till we looks very comfortable to generate a good amount of FCF over the next two years. So, what would be the priority going ahead of the users for this capital? So, would you be like willing to do acquisition to fill up any gap in terms of that addition if these three hospitals going to come, let's say, beyond FY 2029 or you would still go with the greenfield approach or not?

Pankaj Sahni: Sure. So, we are very much open to acquisitions, and I will just qualify that to confirm that acquisitions of every kind of variety, the pure typical M&A which we are maybe more used to in the financial world but also acquisitions in the nature of O&M contracts, asset-light models, etcetera, that we've seen gain popularity in this industry. So, absolutely looking in to the appropriate acquisitions.

I think the key word here is appropriate. So, it has to be -- as I said, it has to be the right -- as I said in the past, it has

to be the right kind of quality of an asset that fits well with the Medanta ecosystem. It has to be in a geographical area or a market which we believe is appropriate for us where Medanta can come in, make a difference in the quality of care we provide.

And then of course, the valuations and the other financial parameters have to be logical and make sense for us to do. So, if all these three parameters are checked, we will absolutely be looking into the acquisition, O&M and other kind of associated models.

The second use for the cash, which we have on hand, would be to continue to double down on our investments in the latest technology. And also, let's not forget, we do still have a good amount of cash to be spent as we think about the build-out of the existing four hospitals, that's two hospitals in Delhi, Mumbai, as well as then Guwahati. Although that cash requirement is typically back-loaded when we build out hospital, typically the bulk of the spend happens in the last year of the build-up.

So, you're right, we have a very, very strong balance sheet, very happy with the current cash position that we have, but lots of sticks in the fire, so no shortage of growth opportunities in our industry and especially with Medanta. So, we have a lot of exciting growth opportunities in our system to come.

Amey Chalke: Sir, thank you. Thank you so much. I will join line.

Pankaj Sahni: Thank you.

Moderator: Thank you. As there are no further questions from the participants, I now hand the conference over to the management for closing comments.

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Pankaj Sahni: Thank you everyone for your questions and for joining us today. As we scale, we continue to focus on what is always defined Medanta, clinical excellence, operational discipline and a commitment to delivering outcomes that matter to our patients.

We're happy that you joined us. Please feel free to reach out to our Investor Relations team in case you have any other questions that remain unanswered. Look forward to meeting you and seeing you also. Thank you.

Moderator: Thank you. On behalf of JM Financial Institutional Securities Limited, that concludes this conference. Thank you for joining us and you may now disconnect your line.

Notes:

1. This transcript has been edited for readability and does not purport to be a verbatim record of the proceedings
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Call: Nov 2025

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Mumbai - 400 001

Scrip Code: 543654 The Manager
Listing Department
National Stock Exchange of India Limited
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Symbol: MEDANTA
Sub: Disclosure under Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 - Earnings Conference Call Transcript

Dear Sir(s),

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the transcript of Earnings Conference Call held on Monday, November 10, 2025, for the second quarter ended September 30, 2025 results .

The Transcript is also available at our website: <https://www.medanta.org/investor-relation>

This is for your information and record.

For Global Health Limited

Rahul Ranjan
Company Secretary & Compliance Officer
M. No. A17035

Encl: a/a

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"Global Health Limited – Medanta
Q2 FY 26 Earnings Conference Call "
November 10, 2025

MANAGEMENT : DR. NARESH TREHAN – CHAIRMAN AND MANAGING
DIRECTOR – GLOBAL HEALTH LIMITED – MEDANTA

MR. PANKAJ SAHNI – GROUP CHIEF EXECUTIVE
OFFICER AND DIRECTOR – MEDANTA – GLOBAL
HEALTH LIMITED

MR. YOGESH KUMAR GUPTA – CHIEF FINANCIAL
OFFICER – MEDANTA – GLOBAL HEALTH LIMITED

MR. RAVI GOTHWAL – MEDANTA – HEAD INVESTOR
RELATIONS – GLOBAL HEALTH LIMITED

MODERATOR : MR. RAHUL JEEWANI – IIFL CAPITAL

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Moderator: Ladies and gentlemen, good day, and welcome to Global Health Medanta Hospitals Q2 FY 26 Earnings Conference Call hosted by IIFL Capital. As a reminder, all participant lines will be in the listen only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” then “0” on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Rahul Jeewani from IIFL Capital. Thank you, and over to you, Mr. Jeewani.

Rahul Jeewani: Hi. Good afternoon, everyone. This is Rahul from IIFL Capital. On behalf of IIFL, I welcome you all to the second quarter earnings conference call of Medanta. From Medanta, we have with us today Dr. Naresh Trehan, Chairman and Managing Director; Mr. Pankaj Sahni, Group Chief Executive Officer and Director; Mr. Yogesh Kumar Gupta, Chief Financial Officer; and Mr. Ravi Gothwal, Head, Investor Relations. Over to you, sir, for your opening comments.

Dr. Naresh Trehan: Good afternoon, and thank you for joining us today for Medanta's Q2 FY26 Earnings Conference Call. I hope all have had the chance to review the results and presentations that were released Friday.

Over the years, Medanta has remained committed to one core belief that world -class medicine should be accessible and delivered with compassion. Every quarter, as we reflect on Medanta's progress, I am reminded that what we are building is not merely a network of hospitals but a movement in health care, a movement grounded in clinical excellence, compassion and trust. This quarter marked another proud milestone as we commenced operations at our Noida Hospital, Medanta's sixth facility. This 550 -bed state -of-the-art patient -centric hospital embodies our vision of expanding access to ethical, high -quality health care. The hospital began operations in September and is currently functional with 226 beds, including over 80 ICU beds and 5 operating theaters , as well as 3 cath labs, all equipped with advanced medical technology. Our teams are fully committed to ensuring a smooth ramp -up and to extend Medanta's care to more lives in the region.

We also announced plans to establish a 400 + bed super specialty hospital in Guwahati to serve the entire Northeast region. In October, Medanta completed the acquisition and possession of a 3.5-acre land parcel in Guwahati and the Bhoomi Pujan ceremony was held on 31st of October 2025, paving the way for construction to begin.

Turning to our Mumbai project. We are pleased to share that we have received additional FSI approval, enabling us to expand our plans from a 500 to 750-bed super specialty hospital. This is a significant development and we are focused on expediting the construction process to create one of the most advanced health care facilities in Western India.

With that, I now hand over the call to Mr. Pankaj Sahni, our Group CEO, who will walk you through the financial performance. Thank you very much.

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Pankaj Sahni: Thank you, Dr. Trehan. Good afternoon, everyone, and thank you for joining us today. I would like to begin with a key milestone of this quarter, the commencement of operations at Medanta Noida, our sixth hospital, which became operational in September 2025. This is a significant achievement for all of us at Medanta. The construction of this hospital began in September 2022 , and within just 3 years, we have successfully brought this world -class facility to life, a testament to our strong commitment and execution capabilities. Currently, 226 beds are operational, including 81 ICU beds, 5 operating theaters and 3 cath labs. The hospital is equipped with cutting -edge medical technologies

such as the Da Vinci Xi surgical robot; next -generation O -arm; Artis Icono AI-driven BiPlane CATH Lab; and state -of-the-art diagnostic systems, including a 3 Tesla MRI, dual source CT, PET scan and gamma camera. These capabilities ensure that our clinicians have access to the most advanced tools for diagnostics and treatment, and this aligns with our vision of delivering world -class care with precision and compassion.

Medanta Noida offers comprehensive tertiary and coronary care across more than 20 super specialties including Cardiology, Oncology, Neurosciences, Gastroenterology and Orthopedics. We have already onboarded over 150 experienced doctors including more than 30 senior director level clinicians who bring deep expertise across key specialties.

Our emergency and critical care services are fully operational, ensuring round-the-clock readiness for complex medical needs. The early response from the community has been extremely encouraging, reaffirming the trust that Medanta name commands. We are confident that Medanta Noida will evolve into a strong pillar of our network and emerge as a significant contributor as it scales up over the coming quarters.

In the first month of operations, Medanta Noida generated INR 39 million and reported an EBITDA loss of INR 197 million, which is in line with expectations during the initial ramp-up phase. We are very excited about Medanta Noida and what it will offer to Noida, the Greater Noida and wider region, given the strong clinical talent that we have onboarded, supported by the highest quality and ethical medicine that Medanta is known for.

Now moving to the consolidated performance update for Q2 FY 26.

During the quarter, consolidated total income was INR 11,189 million compared to INR 9,748 million same quarter last year, registering a strong growth of 15%. EBITDA ex-Noida grew 14% year-on-year with INR 2,804 million with a margin of 25.2% compared to 25.3% in Q2 FY 25. EBITDA, including Noida, was INR 2,607 million, up 6% year-on-year with margins at 23.3%. Profit after tax for the period was INR 1,584 million, a strong year-on-year growth of 21%. PAT margins for the quarter improved to 14.2% compared to 13.4% in the same quarter last year. Overall, our operating cash flows for H1 FY 26 stood at INR 3,295 million, and capex was approximately INR 4,252 million, primarily towards our Noida project and upgradation of medical equipment across the network. Our balance sheet remains extremely strong with a net cash surplus of INR 7,082 million at the end of September 2025.

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Now moving to the operational performance highlights for the quarter. Inpatient volumes during the quarter increased by 13% and outpatient volumes increased by 15% year-on-year. Our average occupied bed days for the quarter increased by 8% year-on-year with strong occupancy of approximately 64% on increased bed capacity.

Average revenue per occupied bed for the quarter was INR 65,570 compared to INR 62,141 in the same quarter last year, an increase of 6% year-on-year. During the quarter, international patients revenue increased by 49% year-on-year to INR 762 million in Q2 FY 26, driven by increased international patient volume.

Coming to the matured and developing hospital performance update :

During the quarter, revenue from our matured hospitals, which includes Medanta Gurugram, Indore and Ranchi, was INR 7,200 million compared to INR 6,833 million in the same quarter last year, registering a year-on-year growth of 5%.

Now please note that the OPD pharmacy business of Gurugram unit which is now operated by a 100% subsidiary of the company named GHLP & Diagnostics Private Limited. As a result, the OPD pharmacy revenue which is not part of the matured unit reported numbers in Q2 FY 26 in the presentation. And therefore, excluding the

impact of the Gurugram OPD

pharmacy business, the matured revenue has actually grown by 7.9% year-on-year.

Growth was visible across four specialties whereas decline was seen in internal medicine, primarily due to the lower incidence of vector-borne diseases during the quarter compared to the same quarter last year. EBITDA of matured hospitals stood at INR 1,696 million compared to INR 1,689 million in the same quarter last year with a margin of 23.5%.

Adjusting for the impact of OPD pharmacy business switching, EBITDA growth in the matured hospitals was 2.2% year-on-year. The EBITDA in Q2 FY 26 includes the impact of additional manpower cost largely in Gurugram and Indore as well as the new unit in Ranchi. This increase includes the impact of increments as well as new hiring in the matured units.

On the volume side, inpatient admissions grew by 5% year-on-year, and the occupancy remained healthy at 61% on an expanded base. ARPOB for mature hospitals stood at INR 73,447 in Q2 FY 26 compared to INR 66,989 in the same quarter last year, registering a strong growth of 10%, primarily driven by a favorable change in specialty mix.

Moving to the developing hospital portfolio, which includes facilities less than 6 years old, which is Medanta Lucknow, Patna and now Noida. During the quarter, 37 new beds were added in Patna, taking the total bed addition to 57 beds in the H1 FY26 in Patna. No bed additions in Lucknow, while 226 beds were added in Noida with our operationalization of the unit there.

Both Lucknow and Patna continued to deliver strong double-digit growth. Revenue from developing hospitals, excluding Noida grew 28% year-on-year to INR 3,531 million, including the initial contribution from Noida, total revenue stood at INR 3,570 million, reflecting an overall growth of 30% year-on-year. Overall, EBITDA from developing hospitals,

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excluding Noida was INR 1,112 million, registering a strong year-on-year growth of 34% with strong EBITDA margins of 31.5%.

EBITDA, including Noida, was INR 915 million with a margin of 25.6%. Inpatient volumes were up 26% year-on-year, reflecting broad-based growth across Lucknow and Patna, along with an early contribution from Noida, which became operational only in September 2025. Occupancy stood at 70% with an ARPOB of INR 54,854 across the developing hospitals. Including Noida, occupancy was 67% with an ARPOB of INR 54,980.

Now coming to our project updates :

In Guwahati, we completed the acquisition of a 3.5-acre parcel of land for ~ INR 600 million. The site located along National Highway 27 offers excellent accessibility across the entire Northeast region. Allotment and possession are complete, and I'm pleased to report that subsequent to quarter ended on 31st October 2025, we concluded the Bhoomi Pujan activities and construction of the Guwahati facility can now commence.

In Mumbai, Oshiwara we received additional FSI approval, enabling us to expand our bed capacity from the earlier reported number of 500 to 750 beds. The Board has approved a new project cost of INR 15,300 million to cater to the increased bed count of 750 beds. This cost includes the cost of land, additional FSI purchased as well as the construction and medical equipment and also includes INR 850 million for staff residential apartments.

Overall, Q2 FY 26 was a quarter of steady growth with consistent performance and disciplined execution as well as the launch of the exciting new opportunities that Medanta Noida holds. Our expansion projects continue to progress well across various stages of execution with a strong pipeline, disciplined capital allocation and a healthy balance sheet. We remain confident of sustaining growth momentum and delivering on our strategic objectives. With this, I request the operator to open the line for questions. Thank you

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Moderator: The first question comes from the line of Tushar Manudhane with Motilal Oswal Financial Services. Please go ahead.

Tushar Manudhane: So just on the mature hospital, while even after adjusting for the pharmacy and other income, the EBITDA growth still seems to be just 2% - 2.5% with the addition of manpower cost. So are we more or less done with the increase in the manpower cost? Or is there something more to come? And subsequently, when do we see the EBITDA growth in line with or better than the revenue growth?

Pankaj Sahni: So thanks, Tushar. So there are two parts in the developing hospitals, although the nomenclature may be a little misleading because it does also include the 110 beds which are new in our Ranchi facility, which was the expansion of that facility. So while it is part of the mature hospital group because Ranchi is part of the mature group that includes about 110 beds of new facilities. So there is some amount of expenses, which go into the launching of that new facility, which we hope will stabilize as we get that facility up and running, and we are actually hiring additional manpower in Ranchi not only for the new facility, but also to add in some of the specialties

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which were missing in the old facility, which is one of the reasons for taking this. So we do expect to see growth in Ranchi as the new unit grows as well as the old unit gets additional specialties. That's point number one.

Point number two is that last year in Indore, we had an unseasonably high amount of vector-borne diseases. So when you look at quarter-to-quarter, there's a little bit of impact of a very high base from Indore last year. And Gurugram also, we had some amount of vector-borne diseases, which I think across the industry is a bit lower.

However, we did hire and we did also do some manpower increases in Gurugram. And we do expect that to play out over the coming quarters as well as going forward. And as we look at scaling up and beefing up our core, which is part of our original strategy to strengthen the core,

we will look at continuing to invest into our mature units. So we do expect to see the results of that as we go forward. And we do expect to see in the coming quarters that you will see the EBITDA growth a little bit more in tune. But if you look at it even from a year-on-year basis, if you take the complete 6 month to 6 month basis, you will find that there is a significantly better performance in the mature facilities over the complete 6 months.

So not getting too much caught up into what happens in one quarter versus the other. If you look at it more of a 12-month run rate annualized basis, you will see fairly healthy growth in the mature facilities as well. So I don't know if that answers your question, Tushar.

Tushar Manudhane: That helps. And secondly, on the developing hospital side, given that there has been, I guess, decent traction as far as institutional patients are concerned. But when do we see the ARPOB further scaling up with probably the change in case mix?

Pankaj Sahni: So if you have to look at, because the ARPOB, is blended for Lucknow and Patna, which is quite different in both the cities and both the geographies. If you look at the ARPOB growth, we've had fairly strong ARPOB growth in our Patna facility, a lot of that is also the addition of more and more complex work.

So just to give an example, we added a robot in Patna, I think when you look at it over the last year or so. And we see now robotic work picking up. We see some of our cancer work picking up there. So a lot of the cancer work, as you're aware, is on lower length of stay. So therefore, you're seeing ARPOB growth.

In Lucknow, we are actually seeing also because we didn't have some of the specialties, say, a year or so ago, which were maybe lower ARPOB in nature

like pediatrics, like some of the other

work. So that is getting added in. But it's not a question of any kind of growth or de-growth. It's really just a question of some of the ARPOBs in Lucknow stabilizing towards what we expect to see as a more rational ARPOB from what it was in very early days. Very healthy ARPOBs there, above INR 65,000. Patna also almost INR 48,000 – INR 50,000 ARPOB. So we see very strong ARPOBs in all these areas. But we also look at a lot of the volume-driven growth in both of these units as you're aware.

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And if you look at the volume-driven growth in both of these units, we've seen very good growth in Lucknow, almost 30 + % growth in inpatient volumes, similar kinds of growth numbers coming in, in Patna. So I think ARPOBs, as I've always maintained, is really just an outcome of some of the mathematics that's worked out.

But if you see realization strong in both units, continuing to grow. If we see volumes in both units, also strong, continuing to grow. I think some amount of length of stay optimization still can be done in both the units. But like I said, I would prefer to look into how the realizations are going rather than just the ARPOBs on a stand-alone basis.

Tushar Manudhane: One Clarification, sir. In Lucknow, certain specialties, because of those specialties, there has been sort of a check as far as ARPOB growth is concerned, correct?

Pankaj Sahni: So when you look, Tushar, at the ARPOB growth in Lucknow, one, of course, is that like I said last year, some of the vector-borne diseases, patients who end up staying in the ICU for a longer periods of time last quarter versus this quarter. But if you look at it across a slightly longer period of time, you will see that growth is there in Lucknow ARPOBs as well.

If you look at how we think about the growth in Lucknow, it's adding in some of the lower ARPOB specialties that is correct. Pediatrics has just started. We will continue to scale that department up. And if you remember several calls back, we talked about adding in some of the longer tail specialties in Lucknow. So those have come in. And I would say that compared to what you may have seen, let's say, about maybe 12 to 18 months ago of ARPOB in Lucknow, these are more stabilized set of ARPOB in Lucknow in this kind of INR 65,000 range.

Tushar Manudhane: Got it, sir. And lastly, as far as Guwahati is concerned, is there any sort of guidelines to have the way we have in case of say Patna in terms of certain beds allocated for institutional patients? Or this is more like entirely we can use it for private patients?

Pankaj Sahni: No. We can use, there are no guidelines or restrictions in any of the beds in Guwahati. This is part of the Advantage Assam 2.0 investment project that the government of Assam has done. So there are no restrictions whatsoever on any of the beds in Guwahati. However, Medanta will very much look at providing care to all the spectrum. It will not be that we will not be looking into various different types of payer classes as we have done in every one of our units, irrespective of the requirements of the lease. But no PPP, this is a pure kind of land which we have procured from the government as part of this investment initiative.

Moderator: The next question comes from the line of Damayanti Kerai with HSBC.

Damayanti Kerai: My question is on your Noida unit. So you mentioned you have already onboarded 150 doctors,

including senior directors, et c. So are you broadly set in terms of having the required medical team, which you intend to put up there? And does 2Q FY26 operational cost reflect the Noida unit cost largely? And how do you see that panning out in say, 3Q and 4Q FY26 ?

Pankaj Sahni: Okay. So first of all, to your question on have we hired most of the clinical talent? I would say that we have hired

a large part of the clinical talent but not all the clinical talent. So to give you

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a simple example, our operations team is not complete , our paediatrics team is not complete. That being said, I think because Noida, we will get one complete tower in one shot. I think in Noida compared to some of the other units, especially most recently Patna, where the hiring was a little bit more staggered. In Noida, I think a lot of the doctors have come in sooner within the first month itself in many . In Patna, you saw them coming in over a period of 6 to 12 months. In Noida, I would say a majority of the senior clinicians have come in on board in the first month itself. And that is part of the reason why you're looking at some of that cost. Now obviously, that is a huge opportunity for us, and we're very excited about that.

And to be very frank, it's also an indication that there is a very huge excitement of senior and established clinical faculty to join us. So everybody is raring to go. We are hoping that the official inauguration will happen in the coming weeks. It's been just about a month or so that the hospital has been opened.

We are still actually building out the rest of the beds. So 220 beds or so are there as of September. We have added in some more beds. So as on date, we are sitting on approximately 300 beds, but we intend to continue this build-out without stopping over the next few months. So I do believe that some amount of costs will continue to come in as we'll hire in more doctors, more nurses, maybe not too many senior directors because that we have mostly filled out. But you will, of course, need manpower to fill out all the beds.

So we're very excited actually about Noida as an opportunity and very excited with the response that we are getting. And hopefully, we will have a grand inauguration soon.

As far as the costs for Q3 and Q4 coming in, I would say that we do have costs which will get added in there. We will also, of course, have revenues that will get built up into that. I don't know exactly how that will play out. But our perspective is that what we've been able to do is bring in the firepower that now enables us to get running. So as soon as we get all , we only have 5 operation rooms active right now, another 9 to 10 are coming on board in the next few weeks. So as we get all this ready to go, we are quite excited about getting this unit moving in. And so far, the response has been very good.

Damayanti Kerai: Okay. So in view of great start for Noida unit and then largely, your team is now in place. Do you think Noida can achieve cost breakeven within 12 months of operations, or it can be sooner?

Pankaj Sahni: So as we said in many of our last earnings calls, Medanta has a good track record of achieving operational breakeven in what may be considered extremely short periods of time. But that's not how we think about it. That may be an outcome, we may have been fortunate. We actually look at investing in the right resources and right infrastructure and right talent to build out for the long run. Whether that will break even in 6 months, 9 months, 12 months, I would not like to project out into the future. All I can say is that we remain very excited about it. I see no reason to worry about any of how this will play out. But I don't want to guess on which month it will break even. I think let time tell that.

Damayanti Kerai: Okay. And my last question is in NCR market. Obviously, for Noida, you said doctors are really excited, and that's why you saw most of hiring happened in first month itself, most of joining,

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etc. But when we look at the overall NCR market, what kind of competition you are seeing on the talent hiring part, especially from some of your larger peers? Have you seen

anything like

attrition more than usual in your Gurugram unit or something?

Pankaj Sahni: No, we have not seen any attrition in our Gurugram unit in the last many quarters, I think at least 4 to 6 quarters at any of the senior positions of the heads of department or heads of department minus 1. Of course, at the junior level, there remains a certain amount of churn. But we have seen almost no significant attrition in the last, I would say, I think, at least 4 quarters, if not 6

quarters.

That being said, I maintained in past calls that the competition and the ability to attract and retain high-quality and high ethical, more importantly, clinical talent, especially in the kind of full-time model that we have, will remain a challenge, and it is not only in NCR, I think this challenge will remain in most of the major markets. And it will remain so not only for the listed players because there are also hospitals, which are single-unit hospitals, unlisted hospitals. All of them are growing. All of them are investing in scaling up the bed count from charitable hospitals to unlisted private hospitals to the listed players, and indeed, even the government hospitals. So I do feel that access to clinical talent will remain one of the key elements and key things to focus on for all the players in the industry. Medanta has been very fortunate that we are considered to be a very attractive place for doctors to work. And we've seen that with the Noida hiring. And so far, no attrition.

In fact, no attrition even in Lucknow or Patna that I can actually think of at these levels. The only thing that's happened is that one of our department heads from Patna has moved to Noida. So we've done some juggling around the units, but we haven't seen any major changes in the headcount.

Moderator: Next question comes from the line of Dhananjay Bagrodia with Alchemy.

Dhananjay Bagrodia: Sir, this is more a broad-based question. How are we seeing ARPOB for the industry? Are we seeing saturation levels? Do we see that continuing going ahead? Any thoughts on that?

Pankaj Sahni: So I think there's a lot of nuances to this question. If you look at, first of all, ARPOBs are very different. First of all, it's important to understand ARPOB is a derived metric, it's a function of realization and length of stay. If you look at Medanta's length of stay, it's probably amongst the lowest in the industry for institutions of this caliber and complexity of work that we do, around 3 - 3.06 days.

However, if you look at ARPOBs as is commonly studied, it's very much a function of the cities where the hospitals are. So as hospitals expand and you would see that across various listed players, as hospitals expand into markets which may not be in the metro cities or may not be a Delhi, Mumbai kind of market, you will see different markets. Obviously, a Patna ARPOB or a Ranchi ARPOB will be different from a Delhi ARPOB or a Mumbai ARPOB.

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As in our case, we move into, say, a Mumbai market, you may see ARPOB increase because Mumbai ARPOBs are typically higher even than Delhi. Delhi ARPOB tend to be a little bit higher than, say, Gurugram or Noida. So it's very much a function of where your hospitals are. And I think that is how we see it across the group.

So what you see as reported is a blended ARPOB and therefore, it depends on where you're operating. South maybe has a different kind of an ARPOB. So I think you have to study it slightly more granularly than just looking at it on an overall basis.

That being said, I think the overall growth in ARPOB, which till a few years ago was -- a lot of it was linked to post-COVID very aggressive realizations. I think you see is tapering out. You see ARPOB growth across the players now maybe middle to high single digits, in some cases, maybe early double digits and a lot of

of that is actually increasing complexity of work. Some of that is increasing complexity of work in terms of, say, robotic surgery versus non-robotic surgery. Some of it is in terms of increasing pharmaceutical costs like things like immunotherapy and other expensive treatments in say cancer.

There has recently been a revision of the central government rates also which will play out as we move forward. I don't know if that would, hopefully that would have an increase as well because those rate revisions are happening after a long period of time. So I think you have to study it more granularly. I don't think that there's one answer to that.

Dhananjay Bagrodia: Sure. And sir, a follow-up. You had mentioned in your answer right now that something like robotics has an impact on ARPOB. I just want to understand now with even international players talking about how robotics have started coming in operational surgeries. Wouldn't that, over the longer term, actually reduce prices because the robot can now be used for 24 hours cost like that? Or is that naive thinking, just to understand?

Pankaj Sahni: No. I mean there is a certain amount of consumable cost for operating a robot in every case. The instrumentation, the items which get used in the surgery actually have a cost. So it is not that because you're using a robot for more period of time, it's cheaper. And it's not like robot is doing the surgery without the surgeon. There's still all the other costs associated with the surgery. So I don't think necessarily that's true. But robot is only one part of it, right? There is also a cost of the pharmaceuticals. There is also cost of advanced procedures like CAR-T cell therapy, there is also cost of increasing antibiotic resistance and therefore need to put in higher levels of

antibiotics. So there's all kinds of things that are going on. It's not a kind of a one size fit all kind of a simplistic answer that I can give you.

Dhananjay Bagrodia: Sure. Sir, just last question. So going ahead, let's assume if ARPOB for most or maybe if you not for you but for the industry, if ARPOB is as we said, growing in the single-digit level. So then now most of the growth will just be coming from bed growth. Is that the right way to look at it now going ahead?

Pankaj Sahni: No, that is not true. You can generate volume growth without bed growth as we have done very successfully in many of our units. Bed growth alone doesn't lead to volume growth. Volume growth means different kinds of procedures. You can actually generate more efficient

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operations. If you study Medanta as a group, our ALOS has been falling consistently over the last several years. So you can get more efficient about getting patients home sooner, which is always a good thing.

So there are many different ways to generate growth. Of course, basics of volume and realization is one part, but you can generate volume growth without bed growth as well. It's not necessary that you need new beds. And by the way, you can also generate bed growth without new hospitals by optimizing how you are actually putting up beds, how you actually have them utilized. So lots of things can be done.

Moderator: Next question comes from the line of Rahul Jeewani with IIFL Capital.

Rahul Jeewani: Sir, you indicated that the Lucknow hospital saw 30% IP volume growth during the quarter. So can you comment in terms of what has led to such strong volume growth for the Lucknow Hospital, given that this year, we haven't added any incremental beds at Lucknow apart from, let's say, 50 - 60 beds, which we had added last year. And if you can also comment in terms of how has the occupancies been at the Lucknow hospital?

Pankaj Sahni: Yes. So I think in the quarter, if I look at the occupancy in Lucknow, it's been upwards of 65%, I think, around 67% or so. And this growth, see, one is that we study only one quarter and another

quarter. However, I think that you have to take a little bit broader view. So we have seen very strong growth in Lucknow. And that growth has come in from many of the actions that we've been taking over the last maybe 12 months or so. You just are looking at a snapshot period in time. If I look at this growth even in the previous quarter, it was in a similar kind of percentage and if I look at it over 6 months or if I look at a 12-month running period, it's also been fairly robust.

So I don't think that this is anything very unique that has something special has happened only in Q2. It is a combination of the clinical talent that we added in over the last 12 months or so. It is a combination of our increasing beds. We started out with about 400 - 500 beds. We scaled that up to about 750 beds. It also includes the addition of some clinical talent there. We've added in some Paediatrics talent, we've added in talent in our Oncology function, we've added talent in some of our Surgical and Neuro functions. Even subsequent to quarter end, we have added clinical talent in our Neurosurgery department. And we will continue to add in clinical talent in the coming months. So we added in a very senior Cardiologist, I think, about 6 months or so ago. So a lot of this is talent that we've been building up in the unit that you see playing out over the various months.

One snapshot, I don't think is only that something extra special happened in Q2. It is the ongoing investment in that unit, which will continue. We still have more beds to go in that unit. And we'll be adding in maybe 1 or 2 more specialties as well. We'll be building out a comprehensive rehabilitation center there as well.

Rahul Jeewani: Sure, sir. And in terms of competitive intensity in the Lucknow market, so let's say, the kind of competitive intensity, which you were facing from some of your peers 12 to 18 months back. Have we seen some letup there, which is also helping us to drive this strong volume growth?

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Pankaj Sahni: So, I mean I don't know if let-up is the right word or even what happened 12 to 18 months ago was driven purely by competitive intensity. I think that we have definitely not seen any significant impact in terms of any clinical attrition, no significant dip in patient volumes or any kind of feedback from the patient community that they are preferring to go to another hospital versus Medanta. And I think that maybe last 3 - 4 quarters or more, we have seen everything

firing on all cylinders. So I haven't seen any change actually in anything that's been happening there in the last several months. So no real impact of this. I'm sure that competitive intensity exists. I'm sure it will remain. But we haven't seen any difference anyway.

Rahul Jeewani: Sure, sir. My second question is with respect to the Noida Hospital. So by when do you expect the insurance empanelments to get completed for the Noida hospital?

Pankaj Sahni: So we are working very closely with all the insurance providers. I'm hoping that they will be able to move a little bit faster than they have so far. We have good, strong relationships with all of these people. And we hope within days and weeks, we'll be able to get this closed. But we are waiting for them to get a little bit faster to their responsiveness. Let's hope that plays out quickly.

Rahul Jeewani: Sure. So by end of the year, do you think that most of the empanelments for the Noida Hospital should be largely done?

Pankaj Sahni: I hope so. I hope it's done sooner than that.

Rahul Jeewani: Okay, sure, sir. And sir, last question before I join back the queue. You talked about the benefit on the CGHS business. Now if I see from your presentation, our CGHS revenue contribution is around 11%, 12% of overall revenue. So what

kind of a price increase let's say, on a blended basis, the price increase which you are expecting on the CGHS portfolio to play out from FY 27? And what could be the quantum of margin benefit as well to us?

Pankaj Sahni: So it's a very nuanced answer to that question. So I don't have a number to give you. I'm not even sure whether we have done the complete calculation on that yet. What we have seen is that the CGHS authorities have seen fit to increase the rates after several years. Keep in mind that this currently applies only to CGHS, not ECHS, and that hopefully will come out if they choose to follow suit as well. And some of the other state governments and some of the other public sector undertakings, etc, which follow a similar guidance may follow soon.

We've seen some positive trajectory over there, especially in some of the specialties and departments, which were very, very low and almost unsustainable. So departments like ENT have seen high growth. In some of the bigger departments like Cardiology or Cancer, we've seen maybe 5% to 10% growth in different procedures. But I think each procedure has a different growth rate and so therefore, it requires quite a bit of getting nuanced understanding into that. As with any price increase since we assume that there would not be a proportionate cost increase, that should all then flow through to the bottom line, but that remains to be seen. I think the biggest challenge, frankly, with this business remains the payments and the collections. So while we have seen some increases, it is not necessarily that suddenly, this has become on par with the cash or insurance business because collections also remain a problem.

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That being said, given our model, our large-scale model, our fixed cost of doctors, etc, we actually feel that we should continue to serve this section of the public, and we intend to do so and across all our hospitals. And as we build out the new ones, we will do them, do some amount of work there as well.

Rahul Jeewani: So you said, obviously, 11% - 12% is including both CGHS and ECHS. So what is the contribution just from the CGHS business?

Pankaj Sahni: I don't know, Yogesh, if we know that off the top of our head, but I would say more than 50% would be, 50-50, maybe.

Moderator: Next question comes from the line of Harith Ahmed with Avendus.

Harith Ahmed: So your comment about moving the operations of the Gurugram Hospital pharmacy unit to a new subsidiary. So just trying to understand the thought process here and if you would be moving more pharmacy operations to this subsidiary? And then also the lab operations, we plan to consolidate under this entity?

Pankaj Sahni: I think maybe it's actually the reverse, maybe you're not aware of the history. This entity already has the complete retail lab operations. This entity also already has the retail pharmacy and outpatient pharmacy for all other hospitals. In fact, only the Gurugram pharmacy was left to move in.

I don't even remember now in which quarter, but in several subsequent periods, we have moved all the outpatient pharmacies and all hospitals to this entity as well as some of our out-of-hospital retail pharmacies and all retail labs are running in this entity. So only Gurugram was actually left over to transition. And as soon as we got the relevant licenses for that, that was undertaken. And so you are seeing actually the last transition impact. Now all outpatient pharmacies are in this subsidiary.

Yogesh Gupta: And you should also note that it's only the retail business or OPD business, which has gone to their pharmacy. And another question which you asked us about the lab business being consolidated with this. All our retail labs are done by our subsidiary since beginning. All hospital

labs are run by hospitals, and we don't have any such pla

ns to move hospital lab business to this entity. So this will remain as two separate businesses only.

Harith Ahmed: So the IP pharmacy would remain under GHL?

Yogesh Gupta: That's what we said it's only OPD pharmacy, basically retail business, which is done by our 100% subsidiary company, and Gurugram was the last leg of that transition. All other transition has happened in the previous years. Retail labs was always part of this retail company only, which will continue to do. Hospital labs are part of the hospital business and this company doesn't do any part of the hospital business.

Harith Ahmed: Okay. Got it. And if you can give an update on the additional 110 beds that we have commissioned in Ranchi, that would be helpful.

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Pankaj Sahni: Sorry, what kind of update were you looking for?

Harith Ahmed: How ramp -up has progressed and how we are looking at it?

Pankaj Sahni: Yes. So just to give you a sense, in Ranchi, our volumes have grown on an annualized basis, somewhere around 15% year -on-year. The new facility is still getting its various empanelments done. So we expect that as that gets done, we expect to see some amount of growth there. Once we get the empanelments done, we will optimize the specialty mix across both the physical sites, although we will continue to run these as one campus.

I think the thing that has not yet happened, which we are looking into and do intend to do is to add in the Oncology services into our Ranchi campus, which were thus far missing. So I think that is still pending. And as soon as we get the specialty aligned across the two facilities, we will be adding in the Oncology services.

Harith Ahmed: Okay. And last one, on the Ongoing projects at South Delhi and Pitampura, if you can give an update on the progress. And also, any comment on the timelines that we should expect for the four new projects? I understand these are beyond FY 27, but how should we think about the commissioning timelines FY 28 or FY 29, some color would be useful.

Pankaj Sahni: Yes. So if you look at , let me first go to our Delhi projects. So in our South Delhi and Pitampura projects, we have commenced the digging activities in our South Delhi project. Pitampura project, we have submitted our plans for approval and as soon as those get approved, we will be commencing all our various construction activities. Both those projects are moving well.

Mumbai also moving well. I mentioned in earlier comments that we have been able to obtain additional beds. So that's excellent news , we now can put up a 750 bed hospital in Oshiwara as opposed to a 500 -bed hospital. And we also will be looking to get now the approval for starting the construction activities there, but we just recently paid off and acquired the additional FSI there. Guwahati, like I said, we've just done the Bhoomi Pujan on 31st October. We have possession of the land. We have paid out for that land and now drawings, et cetera, will be built for these.

All the hospitals are typically about 3 years away, I would say, Noida we built in 3 years from starting construction until now. So , but any hospital typically takes 3 to 4 years. So I don't have an exact date what will come in FY28 - FY29 like that, but it'll take about 3 to 4 years to build out all of them.

Moderator: Next question comes from the line of Amit Thawani with Clear Blue Capital.

Amit Thawani: Can you tell me what the impact of the CGHS revision is on our revenue?

Pankaj Sahni: So I'm not sure if you heard my answer to the earlier question, but I did answer that earlier when there was another speaker who had asked. We don't know this right now because it's quite a nuanced analysis and quite a nuanced expectation of how this will play out. So as of date, I don't have a number to give you, as I mentioned to the earlier speaker.

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Amit Thawani: But any ballpark would also be helpful.

Pankaj Sahni: I am sure. However, we don't like to speculate, so I'd rather prefer to give you facts rather than ballpark speculations. So I don't have that number.

Amit Thawani: Okay. My second question is on the insurance revisions. Typically, we are seeing some kind of a tussle between hospitals and insurers on the revisions. So what is the current scenario? Are we

done with our revisions for the next 2 years, maybe?

Pankaj Sahni: Yes and no, again. Requires a little bit more nuanced answer. See, we have different units and we have different insurance companies, and each of them have different contracts with different durations. So for example, we may have done with one particular insurance company a renewal last year, which is valid for 2 years. So therefore, we are not due for that.

Some are coming up in the next several months for renewal. Some may already have passed. So there's no one answer for that. What I would say is that currently, the with the exception of Noida and our new facility in Ranchi, all Medanta units are empanelled panel with all insurance companies for cashless treatment.

Amit Thawani: Okay. Yes. Actually, that was the question I was asking. My last question is you alluded to some kind of payment issues when it comes to CGHS. Can you just elaborate on that? And I mean, what kind of debtor days are we seeing today in CGHS and in non -CGHS government business?

Yogesh Gupta: See, overall basis, our debtor days remains about 80 to 90 days of our credit business. But in CGHS and ECHS in the government business, average credit realization happens somewhere between 7 - 8 months. Sometimes it goes to 9 months plus period also depending upon how they are playing out. It's again, like CGHS has moved on to a new portal now. That new portal will take some time to stabilize. We expect that with the new CGHS thing coming up, new prices, some delay will happen, some kind of a working capital investment will be required. So again, question -- answer to your question is that it takes 6 months, 7 months, sometimes 9 months to realize from CGHS or ECHS.

Amit Thawani: But is that also applicable to ESHS and the PSU business?

Yogesh Gupta: No, PSU business are much faster. They don't follow that longer period, but specifically CGHS, ECHS takes longer.

Moderator: Next question comes from the line of Tushar Manudhane with Motilal Oswal Financial Services.

Tushar Manudhane: Sir, just maybe if I have missed the Noida operational cost, while currently it is EBITDA loss of INR 20 crores. What kind of numbers to think about, let's say, over the next 1 year in terms of cost or EBITDA loss?

Pankaj Sahni: Tushar, you are asking me to gaze into the future, which I'm always tentative and hesitant to do. So I don't have a number to give you. You have followed us for some time and seen that we've had a reasonably good record of being able to deliver consistent and strong growth in reasonably short periods of time.

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I do not want to comment, as I mentioned to another speaker that we would break even in this month or that month. But we are very excited about this opportunity. And with the kind of talent and responsiveness, which we are getting, I think that as this unit opens up fully, as we get our inauguration done, we do hope to see a good response.

I don't know what that means for next month or the month after that. But I think in the next, let's say, 12 to 18 months, we should see a good kind of performance coming out of here from our quality of work, which we're doing in terms of complex work as well as on the financial side.

We have everything there is state of the art. It is not like it is anything less than the kind of clinical talent, the kind of equipment.

Most hospitals, day 1, you don't open with a Da Vinci robot, right? We've already got all of this stuff there. The Radiation Oncology machine in Lucknow, we brought in, I think, after 3 to 4 years of operation. We've already got that on site. It's getting installed now. So I think we are fairly gung-ho on this opportunity and fairly gung-ho in terms of putting in whatever it takes to deliver a complete solution. Now if that breaks even in 1 quarter or 2 quarters or 3 quarters, I don't know. But we are not holding back in terms of investing into the unit.

Moderator: Next question comes from the line of Sanidhya with Unicorn Assets.

Sanidhya: So my question is more on the EBITDA margin side. When we see the difference between the mature versus our developing hospitals, what we see a trend has been that as soon as we launch a new facility, likely a large facility like we are launching Noida right now and expanding our existing Ranchi and other hospitals?

So what we see is there is a certain kind of say, depressedness in the EBITDA margins that we see for shorter periods. And eventually, our EBITDA margins more aligned with the long-term trajectory. So do you think that's what the company is going through right now this quarter, maybe even next quarter as well?

Pankaj Sahni: I mean I'm not sure what EBITDA margins you have been analyzing over the various units. What I can tell you is that if you go back in history, in the first year of operations, Lucknow delivered somewhere around 15% EBITDA, if I remember correctly. Patna delivered somewhere around an 8%, I think, if I'm not mistaken.

So different units play out differently. I think Gurugram took about 18 months way back when to breakeven. So different units behave slightly differently. It's difficult to tell exactly how Noida will behave. But obviously, any unit starts up, there is some incremental initial costs that come in.

I think also, if you really look at the way we categorize our developing hospitals, Lucknow has just completed 6 years. So we'll probably, as per this definition, maybe from maybe next financial year onwards, kind of move into what may be considered to be the mature hospital phase. But I think these are just nomenclatures that are there in investor presentation and talks. I think if you really look at each unit slightly more individually, you will find that all of them continue to perform extremely well, especially in context to their individual location and

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situation. And it's not like we say that a hospital like Gurugram, which is 15 years old, is finished. We've been delivering strong growth even in a 15-year-old hospital. So I think each unit plays out slightly differently. And we will see this in Noida, sorry, as we scale up. And we will understand how similar or different Noida is to any of the other units. But our internal management approach is to consider each unit differently and not worry about whether it's like Patna or like Lucknow or like Gurugram, but to run that unit for what it is. And we feel quite excited about Noida. So we are very happy that it's up and running. I think that was the first step. And now we have to get it open properly, get everything finished, get the inauguration done and be ready to serve all the people that are hopefully excited to come to Medanta facilities if they need any kind of medical care.

Sanidhya: Yes, I understand. And just on a follow-up for the same question. So I heard you saying that when we look at the Lucknow vs Noida, we are on the way all through going into, in terms of technology and all the machines that we need from start in the Noida facility. So my question is more on the approach the management is taking when it comes to Noida versus how we have been doing in the past when it comes to, say, Lucknow or Ranchi or Patna. So how dif

ferent is

our approach when it comes to Noida this time?

Pankaj Sahni: I think the main thing is that Noida is very much part of the Delhi NCR market. Obviously, we are a fairly well-known and important player in the NCR market with our presence in Gurugram for so long. Noida, we have kind of falls in that same territory and therefore, we have a lot of patients on this area, and we have a lot of brand recall in this area. There's also maybe a little bit more familiarity, easier potential availability of clinical talent in the NCR market.

Patna and Lucknow were new territories, right? Most of the private health care industry had never even been there. If you look at when we walked into Lucknow, there was a lot of question marks about how successful we would be and many of the other members of the industry saying that they would not be interested in that market.

Now we proved many doubters wrong, and we see a lot of people following us and copying us into these territories. Same was the story in Patna. There were a lot of question marks about whether Patna can be not just clinically successful, but financially successful. We have proven that as well. And very likely, we will see people follow us there. I would not be surprised if people follow us into other markets which we've gone into.

We've kind of established ourselves as a trend setter on this front. We've always said that Medanta will go where the people need us, not necessarily just where the perceived high per capita income is, but we will go where the people need us. And we have made all those units financially well and clinically successful.

And Noida, given that it's part of the NCR market, we were a little bit more confident to go in with more talent and more investment sooner rather than actually going into a newer city like Lucknow or Patna where we had to learn also about that center, learn about those cities. And the doctors had to also learn about us. I think in Noida, we are maybe a little bit more familiar to the community.

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Moderator: Next question comes from the line of Rahul Jeewani with IIFL Capital.

Rahul Jeewani: So we did quantify the EBITDA loss from the Noida Hospital. Can you similarly call out the number for the new Ranchi Hospital where we commissioned 110 beds. So what was the EBITDA loss from the new Ranchi hospital in 2Q FY26?

Pankaj Sahni: We can't because that's part of the Ranchi unit. I don't know Yogesh if you have more color.
Yogesh Gupta: Yes, we are running both the units as one single campus. So there are a lot of shared costs and shared management. So we really don't track what is the loss of the new hospital that will be unfair. These are not working as two separate units, they are working as a single unit. Same doctor is working here and there also and same management is managing that unit also. That's why we do not call it out separately.

Rahul Jeewani: Okay. Point noted. And sir, second question is with respect to price increases across your network. So let's say, I think we were supposed to take price increases across Lucknow and Patna. So have we already taken that at the beginning of the year? Or when do we plan to take those price increases?

Pankaj Sahni: No, we have not taken Patna increases I think, correct me if I'm wrong, Yogesh, but I don't think we have taken a Patna price increase since starting of Patna.

Yogesh Gupta: Patna remains with same tariff.

Pankaj Sahni: Same tariff from what we started. Never taken a tariff increase in Patna. It's now our fourth year of operations, I believe. Lucknow, also we haven't taken any tariff increase. We have taken some nominal. So, nothing this quarter that to call out. I think maybe last quarter or even I think at the start of the calendar year, we had mentioned that we would be taking nominal increases in Lucknow

and Gurugram. And those, I think, have mostly already been taken before the quarter.

I don't think there was any major increase this quarter, if I'm not mistaken.

Yogesh Gupta: No, no. Lucknow increased somewhere in this quarter.

Pankaj Sahni: Maybe some in this quarter, but it would be hardly a few percentage points increases.

Rahul Jeewani: Sure, sir. And does the PPP agreement in Patna stop us from taking price increases or because you said you haven't taken price increase in that market for 4 years now?

Pankaj Sahni: No. There are no constraints on taking any price increases for the non-PPP part of the business, which is currently about 75% - 80%, but 25% is reserved for PPP. So no constraints on the remaining 75%. Frankly speaking, we have not taken price increases because the unit is doing so well. And we've always been an organization that wanted to be conservative on how much we charge to patients.

With the kind of financial performance we see in Patna, we don't really see any reason to charge more to our patients just because we can. So we would like to be as reasonable and as

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conservative on pricing as possible. We continue to open more beds there. We continue to see growth across the board there.

So what is the need to just blanketly take a price increase because you have to. So that's our philosophy and our approach and I think well stated for many years. But as and when we feel that there are some inflationary impacts to manage in Patna, we will look at that as well.

Moderator: Ladies and gentlemen, as there are no further questions, we have reached the end of question-and-answer session. I would now like to hand the conference over to the management for closing comments.

Pankaj Sahni: Thank you, everyone, for your questions and for joining us today. Please feel free to reach out to our Investor Relations team in case you have any questions that remain unanswered, and we look forward to speaking with you and meeting many of you soon. Thank you.

Notes:

1. This transcript has been edited for readability and does not purport to be a verbatim record of the proceedings
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Call: Feb 2026

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Symbol: MEDANTA
Sub: Disclosure under Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 - Earnings Conference Call Transcript

Dear Sir(s),

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the transcript of Earnings Conference Call held on Thursday, February 05, 2026, for the Third Quarter and Nine-months ended December 31, 2025 financial results.

The Transcript is also available at our website: <https://www.medanta.org/investor-relation>

This is for your information and record.

For Global Health Limited

Rahul Ranjan
Company Secretary & Compliance Officer
M. No. A17035

Encl: a/a

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Q3 FY26 Earnings Conference Call"
February 05 , 202 6

MANAGEMENT : DR. NARESH TREHAN – CHAIRMAN & MANAGING
DIRECTOR , GLOBAL HEALTH LIMITED – MEDANTA

MR. PANKAJ SAHNI – GROUP CHIEF EXECUTIVE OFFICER
& DIRECTOR , GLOBAL HEALTH LIMITED – MEDANTA

MR. YOGESH KUMAR GUPTA – CHIEF FINANCIAL
OFFICER , GLOBAL HEALTH LIMITED – MEDANTA

MR. RAVI GOTHWAL – HEAD INVESTOR RELATIONS
GLOBAL HEALTH LIMITED – MEDANTA

MODERATOR : MR. AMEY CHALKE – JM FINANCIAL INSTITUTIONAL
SECURITIES LIMITED .

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Moderator: Ladies and Gentlemen, Good Day and Welcome to Global Health Limited Q3 FY26 Earnings Conference Call hosted by JM Financial Institutional Securities Limited.

As a reminder, all participants' lines will be in listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing “*” then “0” on your touchtone phone.

I now hand over the conference to Mr. Amey Chalke from JM Financial Institutional Securities Limited. Thank you. And over to you, sir.

Amey Chalke: Thank you, Sir. Good afternoon and warm welcome to all the participants on Global Health Limited Q3 FY26 Earnings Call hosted by JM Financial.

Today on this call, we have with us from the Management, Dr. Naresh Trehan – Chairman and Managing Director, Mr. Pankaj Sahni – Group CEO & Director, Mr. Yogesh Kumar Gupta – Chief Financial Officer, and Mr. Ravi Gothwal – Head of Investor Relations.

I will now hand over the call to Dr. Trehan for his Opening Remarks. Thank you and over to you, doctor.

Dr. Naresh Trehan: Thank you. Good afternoon to everyone and welcome to Medanta's Q3 FY26 Earnings Conference Call. I hope all of you have had the opportunity to review the Results and Presentations that were released yesterday.

At Medanta, our primary focus continues to be on delivering high-quality, patient-centric care across our network, supported by strong clinical governance, robust processes, and continuous investment in medical expertise and infrastructure.

During the quarter, the company remained firmly committed to strengthening its clinical program, expanding across tertiary and quaternary care, and ensuring consistent clinical outcomes across its hospital network.

As doctors, we remain deeply concerned about the rising cancer burden in India, where early detection continues to be the most effective intervention. In this context, we launched a large-scale cancer awareness program during the last six months, focusing on Breast and Prostate Cancers. The initiatives were aimed at dispelling prevalent myths, promoting self-examination, and encouraging timely medical intervention.

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To meet the growing demand for advanced oncology care, our Gurugram facility commissioned its 5th Radiation Oncology Machine, significantly enhancing treatment capacity and adding some of the latest technology to this field.

I would like to acknowledge the dedication of our doctors, clinical teams, and support staff, whose collective expertise and commitment remain central to maintaining the highest standards of care while delivering personalized, patient-centric outcomes across our hospitals.

Our recently commissioned Noida Hospital completed its first full quarter of operations during this period. The hospital made encouraging progress across clinical onboarding, outreach initiatives, and infrastructure activation, thereby laying a strong foundation for the ramp-up of business operations and the delivery of high-quality healthcare services, not only to Noida city, but also across western Uttar Pradesh.

I am also pleased to share that Medanta, Lucknow has received the Prestigious Joint Commission International (JCI) quality accreditation in January 2026. Medanta, Lucknow is the first hospital in the region to be accredited by JCI. This certification reaffirms our commitment to deliver the highest global standards of clinical quality and patient safety in the region where we operate.

With that, I will now hand over the call to Mr. Pankaj Sahni, our Group CEO, who will take you through the financials and operational performance in detail. Thank you.

Pankaj Sahni: Thank you, Dr. Trehan. Good afternoon, everyone, and thank you for joining us today for our

Q3 FY26 Earnings Call.

Let me begin with the financial performance for Q3 FY26:

Total income for the quarter stood at Rs.11,428 million, delivering a robust 19% year-on-year growth, driven by sustained momentum across all our hospitals and continued improvements in operating metrics.

EBITDA, excluding Noida, grew by 11% year-on-year to Rs.2,814 million, with healthy margins of 25.4%, underscoring the resilience and operating leverage of our balanced portfolio. EBITDA, including Noida, stood at Rs.2,494 million, with margins of 21.8%, reflecting the expected impact of early-stage operating losses from our newest hospital.

Medanta, Noida, which commenced operations in September 2025, completed its first full quarter of operations during Q3. The hospital generated Rs.343 million in revenue and reported an EBITDA loss of Rs.320 million in line with the investment required to operationalize a facility of this scale and high standard.

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At Noida, we accelerated the ramp-up with the addition of 102 beds, taking the total bed count to 328 beds, with 9 new operation theatres added this quarter, taking the total count to 14 operation theatres. This is also supported by significant strengthening of our clinical teams. We remain confident of steady improvement in utilization and financial performance over the coming quarters. Consolidated profit after tax stood at Rs.950 million, impacted by higher depreciation and finance costs related to Noida, initial operating losses and a one-time statutory impact of Rs.366 million, arising from the implementation of new labor codes classified as an exceptional item. Adjusting for labor codes impact, profit after tax was Rs.1,224 million.

We also saw encouraging momentum in international business; international patient revenue grew by 30% year-on-year to Rs.703 million, driven by higher volumes, while our OPD pharmacy business grew 30% to Rs.465 million.

Operationally, the quarter demonstrated strong traction across the network. We added 144 beds, including 42 beds in Patna and 102 beds in Noida. Inpatient volumes grew by 14%, while outpatient volumes increased by 20% year-on-year. Our average length of stay reduced to 3.02 days, a 7% year-on-year improvement, while occupied bed days increased by 7%, translating into an occupancy of approximately 59% on expanded bed days. ARPOB increased by 10% to Rs.67,361, supported by improvements in ALOS and case mix.

Moving on to matured and developing performance update:

Our mature hospital consisting of Medanta, Gurugram, Indore and Ranchi delivered steady performance with revenue of Rs.7,020 million, up 9% year-on-year and EBITDA of Rs.1,675 million, up 7%, with margins of 23.9%. Matured margins during the period were impacted due to increase in employee costs and other operating expenses, including repairs and maintenance.

Our developing hospitals excluding Noida continue to outperform, recording 22% revenue growth to Rs.3,651 million and EBITDA of Rs.1,156 million, growth of 13%, with strong margins of 31.7% in Q3 FY26.

If we look at nine-months performance, both Lucknow and Patna facility continues to deliver double-digit growth in revenue and EBITDA, with Lucknow margins registering over 150 basis points improvements year-on-year, while Patna margins remain stable.

Developing hospitals including Noida stood at Rs.3,994 million, up 33% year-on-year, while EBITDA stood at Rs.836 million with margins of 20.9%, reflecting the expected drag from early-stage operations in Noida.

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Inpatient volumes across developing hospitals increased by 27% year-on-year, with overall occupancy at 62% and ARPOB rising 8% to Rs.56,853.

Project Update. During the first nine months of FY26, bed capacity increased by 18% year-on-

-year

with addition of 537 beds, comprising 99 beds at Patna, 110 beds at Ranchi and 328 beds at Medanta Noida.

We continue to have meaningful headroom for growth within our existing network, with the potential

to add 496 beds through Brownfield expansions, including 193 beds at Lucknow, 81 beds at Patna and 222 beds at Noida. These additions are expected to drive near to medium-term growth with minimal incremental capex.

On the expansion front, our pipeline continues to progress well. In Guwahati, barricading is complete and drawings have been submitted for approvals. In Mumbai, land acquisition is complete, additional FSI approvals have been received and drawings submitted for approval. In South Delhi, construction activities are underway following the completion of site surveys and soil testing. Pitampura, New Delhi project is going through the regulatory approvals phase.

Overall, nine months of FY26 reflect disciplined execution across the organization. Our core hospitals remain stable, developing hospitals continue to scale efficiently and Medanta, Noida is progressing in line with the expectations during its ramp-up phase. We remain confident that these investments will create sustainable value for all stakeholders.

With this, I would now request the operator to open the line for questions. Thank you.

Moderator: Thank you very much. The first question is from the line of Tushar from Motilal Oswal Financial Services. Please go ahead.

Tushar: Thanks for the opportunity. Sir, firstly the clarification, like in your opening remarks you highlighted about Lucknow and Patna margins. If you could just spell out again?

Pankaj Sahni: So, if you look at our developing hospitals performance, if you look at the presentation on the Investor Presentation, on slide #16, you will see that we have seen an income growth of 28% from the nine months' period, this is excluding Noida, where the revenue has grown from Rs.813 crores to Rs.1,040 crores. And if you look at the margins, that has also increased. In fact, the margins have increased from 30.0% to 31.1%, excluding Noida. And then if you factor in Noida, that is because of Noida loss, so that is coming to 25.0%. So, I hope that is the question if you are asking, Tushar.

Tushar: Sir, on Lucknow particularly, how has been the margin scale up?

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Pankaj Sahni: On Lucknow, we have done very well. Over the course of the last nine months or so, as I mentioned in the opening remarks, we have seen a margin expansion of over 150-basis points when you look at it on a nine-month to nine-month basis.

Tushar: Got it, sir. And secondly, now with respect to Noida, we have almost on-boarded 220 doctors. Are we more or less now done with in terms of on-boarding doctors or what is the sort of target as far as doctors are concerned, and how it has been like maybe the initial fillers with respect to Noida facility?

Pankaj Sahni: Yes, so as you are aware, we do not stop hiring doctors even after several years. But, of course, the hiring to be able to start the services is more or less underway. There are some departments which are still not active in Noida. So, some of the departments which are not active in Noida is that December 31st, would be the Pediatrics Department, the Obstetrics Department, Liver Transplant Department. Some of the departments is in some of the more niche specialties like Vascular Surgery. So, we are still continuing to look at these departments and hiring. But, I would say that the majority of the hiring has been undertaken, including adding in some of the talent to support the key teams. So, we now expect that most of that hiring is finished.

But, like I said, if we do find that we need to bring in more talent, we will not stop that hiring that will continue. And wherever the departments are not there, as I mentioned,

that hiring will take place.

I do not know whether it will happen immediately in the next few months or whether that will happen subsequent to March 31st, but that will be as per our availability of beds and our availability of talent and our plans as we move forward.

Tushar: Got it. And just lastly now that it is like three-four months into operation, so broadly, what kind of ARPOB is like as a starting point so to say, of course, this will further improve with the change in case mix, payer mix, but currently, if you could share what kind of ARPOB we are having at Noida Hospital?

Pankaj Sahni: ARPOBs in Noida Hospital are coming currently much higher than Gurgaon. But, I would not necessarily use this as a proxy for extending out into many months into the future, because we have just started, we have now signed up about four or five major insurance contracts, other insurance contracts are coming onboard. We have not yet taken any of the panels in Noida that may come on as we move forward in line with our broader philosophy of serving every strata of society and every category of patients. Right now, the ARPOBs are similar to Delhi ARPOBs, but I just caution that we should not assume that will maintain as we move forward. Once we have a more balanced, revenue mix, then you will see a slightly more balanced ARPOB as we move forward.

Tushar: Got it, sir. Thank you. That is it from my side.

Moderator: The next question is from the line of Devarsh Shah from Sundhi Securities. Please go ahead.

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Devarsh Shah: Yes. Hello, sir. Thank you for the opportunity. I wanted to understand, like, could you typically state the employee -to-bed ratio for the hospital, because as per my understanding, the employee -to-bed ratio varies for the different specialty, so on the overall basis, what would it be for your hospital?

Pankaj Sahni: I am not very sure, Devarsh, where you are getting these baselines from. We do not normally look at employee -to-bed ratios in any one area, because it varies very drastically depending on the nature of the specialty. So, neither have we ever published it, nor can I tell you that this is a standard number for a hospital. Just to give you a sense, a Liver Transplant department may have fewer beds, but very high number of clinical talent, whereas an Internal Medicine department may have a high number of beds and a very few number of doctors. So, it is not a very meaningful metric for us to look at in terms of any of the operational parameters. So, unfortunately, we do not have anything which we report out, which I can share with you. I am not sure if you have something deeper in mind in the question I can try to assist, but this metric, as such, we do not report.

Devarsh Shah: Okay, sir. And the second question was, like, could you tell the total census beds for each unit?

Pankaj Sahni: Noida census beds right now is 187. Our total is about 2,665 as at December, and maybe I can get this sent across to you from our investor relations team so we do not waste everybody else's time on the call. Each of the units will get the beds. Gurgaon is about 1,220 and Lucknow currently is operating at around 604. Patna the big facility is 386 is the census beds. But we will get this sent to you.

Moderator: The next question is from the line of Banshi Desai from J.P. Morgan. Please go ahead.

Banshi Desai: Thanks for taking my question. So, firstly, on Noida, how should we think about the losses from here on? I am assuming these would be the peak kind of losses that we report in the quarter given we have added doctors and a large part of commissioning expenses would be sitting in this quarter, and therefore, as the unit ramps up, should we see losses coming down? And also, given the fact that we are expected to add another 200+ beds here over the

next one year, how should we see the loss trajectory?

Pankaj Sahni: Yes. So, I mean, I hope so that we have seen the peak of the losses. You are right. If you look at our scale-up of clinicians, we added in a whole bunch of clinicians from September to November. And even within this quarter, if I was to really see, the performance really was more in terms of volumes growth, more started towards the second half of this quarter. So, you do not even see complete three months of run rate. So, we are seeing now, especially December and as we move into January, a better run rate trajectory on the revenue. So, of course, that will help with the loss containment. I do not know exactly when it will be fully concluded.

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One thing I just want to clarify for one of the earlier questions I had mentioned on the specialties, one thing which I forgot to mention is our Radiation Oncology and our Nuclear Medicine department, which is onboard in terms of the talent and the machines, that has not started yet. So, that is another thing I forgot to mention that the department which is currently missing.

But you are right. As we scale up, we should see now the business coming in, our insurance contracts coming onboard, our corporate empanelments coming onboard. And really now the question is, getting down and actually ensuring that we are able to service the people of Noida and Western UP. So, much of the initial work is done, although some of the project work is still undergoing. But the hospital is fully functional now, subject to the departments that I mentioned.

Banshi Desai: And when we think of adding 200 beds here, I would assume those would largely get absorbed with these 300 beds ramping up by that time?

Pankaj Sahni: Yes. So the additional beds would largely get absorbed with the teams which we have, plus some element of the teams which we are adding. So, I can tell you, for example, that we are already underway of creating our dedicated Transplant unit. So, although it was not completed and commissioned by December 31st, that is one area which we will be having, so, some of these additional beds will come into that. So, with the exception of really Transplant, which has dedicated infrastructure and maybe Obstetrics or Pediatrics, the rest of the infrastructure will more or less just scale up as and when the need is there and will be part of the existing clinical team. So, we do not see that there is any real need to hire new departments for filling those beds.

Banshi Desai: All right, that is clear. And my second question is on the matured units. We have seen significant decline in ALOS there; 2.9 days. Is it a quarter phenomenon or do we expect structurally ALOS coming off there? So, that is one. And the second is, you mentioned on margins. You had higher employee costs and certain maintenance-related expenses. So, with those behind, should we expect the overall unit to see improvement in margins from the next quarters as you see your revenues growing?

Pankaj Sahni: Yes, so let me take that in two parts. First, when you look at the ALOS, I think just picking up one

or two quarters may not be always appropriate, because there are some effects of how the seasons play out, how the Diwali holidays play out, how the certain seasonal diseases like Dengue, etc., play out, sometimes it is more in September, sometimes it is more in October. So I would suggest we look more at the annual or a nine-month basis at a minimum rather than quarter-on-quarter, just to get the numbers correct.

That being said, the ALOS reductions are also driven a lot by the increasing work for Cancer. And given the growth which you see on the sales mix, and I think it is a phenomenon across the country where Cancer treatment and Cancer detection is increasing. So, a lot of the Cancer work is in the Global Health

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nature of radiation oncology or medical oncology, which has a daycare component to it. So, there is an increase in daycare. There is also some amount of difference this year versus last year with respect to patients who are more in the Dengue and some of the other respiratory diseases, which tend to have a slightly longer length of stay vis-à-vis some of the shorter procedural departments. Other than that, I do not see any very significant structural change. I mean, we already have a very aggressive length of stay.

That being said, we continue to work, especially in some of the units like Patna and maybe even in Gurgaon, to actually ensure that we are optimizing our length of stay, and we will continue to try to see if we can convert a lot of the work from night stay to daycare. So, we want to work on how we also do things like improving discharge processes, streamlining the discharges for patients who are credit-patients. So, I mean, there is still some operational efficiencies I think we can bring in, but I think that three days length of stay for an ecosystem as complex as ours is very, very robust.

Bansi Desai: Understood. And just on margin, are we done with our investments into maybe adding employees and maintenance expenses or we expect that to recur?

Pankaj Sahni: Yes, obviously, the employee costs are recurring costs, they incur every month. But I think if you go back to last three, four quarters, I had been mentioning that there is some amount of war for talent that is coming, is likely to come. We are also aggressively hiring across the unit. So, there will be some amount of optimization and balancing out of this as we stabilize our hiring, as well as how other hospitals look at hiring in the market. We will also look at operational efficiencies where we can bring about as much as possible, both on the manpower cost as well as on some of the other expenses.

Some of these are cyclical in nature. So, for example, we do notice in our P&L that repairs and maintenance costs is whenever we have annual maintenance contracts coming up, which is out of warranty, new equipment comes out of warranty after maybe three, four years of procurement. Those come in and then they balance out over time. So depending on which horizon you are looking at, they more or less balance out. I do not see any very significant structural issues on the margins. I do believe there are efficiencies, especially in some of the older units that we can eke out, and we will work towards that. You will see some of those efficiencies are already playing out on some cost items; so, material costs are currently in the process of being optimized as we move, and we have seen some of that captured in the numbers so far, we hope that will continue. So, there will be some up and down across cost items, but overall, I think that we will be looking at trying to see whether we can get a couple of hundred basis points of efficiency out of the system as well.

Bansi Desai: All right. I have more questions. I will join back to the queue.

Moderator: The next question is from the line of Manish Poddar from Invesco AMC. Please go ahead.

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Manish Poddar: Just one question. So you said, I hope so for the peak losses to be behind for Noida. How are you thinking actually of the ramp up the other way around, let us say the initial expectation versus what you would have seen in the last few months, how do you think about getting to utilization, let us say 40%, 50%, 60%, and how is the journey faster than earlier expectation, just any color on that would be helpful?

Pankaj Sahni: So I think, Manish, the journey is very much as per our expectations. I think that as we look at getting, everything settled, not only operationally, but also some of the project teams moving

out of the

system. We started out this hospital while it is currently being constructed as well. So, some of these things will stabilize, like I mentioned, just as an example, our Radiation Oncology doctor is onboard, but the machine is not yet live because we are waiting for the approvals from AERB.

So, there will be some teething issues around this next couple of weeks, maybe months. But I think

that, for the most part, we are now ready to fire on full cylinders. What that time frame exactly will be? I am not sure, but I do not think that we have seen anything happen in Noida, which is totally counter to what our expectations were either on the hugely negative side or on the hugely positive side, more or less moving in line with what we expected.

Moderator: The next question is from the line of Damayanti Keraï from HSBC. Please go ahead.

Damayanti Keraï: Hi. Good afternoon and thank you for the opportunity. My first question is again on Noida, where you mentioned it is ramping up as per your expectation. So, just to clarify, the bed addition plan which you have, that will only be done once you have achieved cost breakeven at the existing bed set, right?

Pankaj Sahni: No, I do not think that is the right way to think about it. So, again, without getting into too much specifics, our Noida facility is a single tower facility as opposed to Gurgaon, Lucknow and Patna, where we had two towers, where we were able to build one tower, then wait and fill it, then operationalize the second tower. Because of the structure of this building, we intend to complete the project work as and when the project teams are able to do it. So, we will not actually demobilize and start again once occupancy increases, we will continue to build throughout. It just ends up being more efficient that way. And while it may be beds coming in a few months before we need it, we feel it is the most efficient way. So, we would not stop the build out is the answer to the question.

Damayanti Keraï: Okay. What will happen simultaneously, right, the ramp up the adjacent beds?

Pankaj Sahni: Correct. We will ramp up the adjacent beds. We will build out, now you may not hire every department, you may not hire the nurses for running one ward if the ward does not have patients, but the infrastructure and capex and the fit outs and all, we will conclude, and then we will see as and when we move out. We may also have some things in the back pocket for maybe one floor or so,

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depending on whether we need more ICUs or more OTs. You may need to optimize there. We are now seeing this after about four years performance in Patna. We are actually realizing that we may have a demand for additional procedure rooms as opposed to beds. So, we may actually redeploy some of the spaces which are allocated for beds to procedure rooms. So, today, I believe Patna is about 560 plus beds. So from the remaining 80 – 100 beds, I may choose to redeploy some beds to procedure areas, or we are trying to find if we can optimize there. So, barring these small adjustments, we will continue to build out Noida.

Damayanti Keraï: Got it. My second question is on the update which we heard on the C GHS rate revision some time back. So, anything you are picking up on that part in terms of your operations?

Pankaj Sahni: So C GHS rate revision has of course been beneficial for the industry. It has been a long demand after many, many years. There has been positive movement across the group on that in terms of tariff increases. We have captured some of that in the nine month numbers that you see. I think, high single digit in terms of gross is the impact on that. And, hopefully some of that will fall to the bottom line also. As we roll out, because these increases, I think, in middle or late Q2, so it is not even a full 12 months of impact yet. So, I

f I remember correctly, it was maybe sometime around October. And

then, also what happens is some of the other institutional companies that follow C GHS rates, they then follow up with their own orders, like the railways will follow up, saying, we also will activate these rates and so on and so forth. And also, we are working with our partners in Bihar that those rates are where we have PPP, those rates also get adjusted. So, the full year impact of this, I think, is not yet played out, but so far it has been positive.

Damayanti Keraï: And did you mention some high single digit impact?

Pankaj Sahni: That is right. I think it is somewhere in the range of Rs.7 to 10 crores of impact in nine months.

Damayanti Keraï: And my last question is on your plan for price increase, which is obviously not your focus, but as you focus more on the volume growth, but in terms of taking price hike for some of your units where you have not taken price hike for long, are those done?

Pankaj Sahni: So, if I remember correctly, from our last call in September till December, I think no real price increases have been taken in Patna. We have taken price hike in Gurgaon and Lucknow. But I think more important than that is that some of the insurance contracts, which were not renewed for almost two-to-three years, they are now getting renewed across the group. So, there will be some potential benefit for those price renegotiations as we move forward. So, I think, probably when we come to maybe June or September of next year, you will see more of a realistic picture of a full year of pricing impact across GGHS, our increases and insurance negotiations.

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Damayanti Keraï: Okay. Just a clarification on these rates, which you renegotiated with your insurance partners, what

kind of hike you might have got, just some indication, say compared to the last contract, when you signed the new contract, what is the difference?

Pankaj Sahni: So let me put it this way. When we sign up the contracts with the insurance companies, they were on a tariff list, which I think was somewhere around two years or three years ago, I think 2023 – 2024 tariffs. So, now when the insurance companies negotiate, they negotiated with our 2025 tariffs, which was what we had done last year. So, depending on how those tariffs increase, we kind of renegotiate basis that. So, it would be different in different units depending on when the insurance company signed up. But typically insurance companies will sign up for a two-year time frame and now it has already been a little over two years. So, you can say about two years plus of increases. And then insurance companies ask for, different levels of support in terms of discounting, etc. But on the whole, I think it will be a positive benefit for us from a tariff point of view.

Moderator: The next question is from the line of Ashutosh Nemani from JM Financial Office. Please go ahead.

Ashutosh Nemani: Thanks for the opportunity, sir. My question is on the earlier answer you told that war of talent coming up in the next three to four years. So, I wanted to understand more about it, like particularly what locations do we see this doctors supply and nurse supply more severe as compared to other on a national level, if you could just highlight on that?

Pankaj Sahni: I mean, I am not fully an expert to tell you at the national level in which city there will be more or less war for talent. I think this is a phenomenon the industry is facing, to the best of my knowledge across the board. I can tell you in the regions where we operate, which is more on the northern side. There are two ways in which we look at it, one way is where there are existing facilities, where there are lots of doctors and there is lots of demand for talent. There, it is a little bit driven by, some forms of competitive inte

nsity. Doctors are trying to get pushed by the other hospitals. Obviously, we would be also looking at hiring doctors from other institutions, some amount of retention also. We have been very lucky, we have been able to retain most of our senior doctors, if not all, even after 15-years in Gurgaon.

Then when you come to units which are in cities where there may be a slightly shorter tail of talent in terms of super specialists, we have been able to get in doctors from other cities, sometimes you have to pay for getting doctors to move into certain locations or you are actually looking at building up a talent base over there vis-à-vis taking time to build that specialty. So, just to give you an example, if I bring in a very talented Robotic Surgeon from one part of the country, they may not have a patient base, say, in Bihar or in UP. They build up that. So, that takes a little time to build up. So these are the kinds of things that we have seen from my conversations with colleagues in the industry. I think this is a phenomenon everybody is facing. Obviously, there is a lot of talk about hospitals, everybody getting very excited. One thing, though, we have seen is that I think clinical talent understands the Global Health Limited

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value of strong brands and strong institutions. Medanta has been very lucky to be a kind of what we believe is a preferred employer for clinicians. So, we have been lucky on this front. But I do believe that given the talent is scarce, this is something that the industry and the country will face for some years to come. I do not think this is something that is going to get solved in the next quarter.

Ashutosh Nemani: Understand. My second question is, in the next two to three years, what should be the ARP OB growth that you could assume for Medanta?

Pankaj Sahni: Again, in the next two, three years, I do not know how things will play out. But, we have always maintained that ARP OB's growth is 5% - 7% a year as opposed to what we have seen maybe in the last three, four years where we have seen much higher growth. Again, ARP OB is a little bit dependent on length of stay and case mix. But, we do not predict anything more than 5%, if we are very enthusiastic, maybe a 7% growth in ARP OB on an annual basis. In fact, Medanta typically projects much lower, 3% - 5% is what we try to think about. But again, that is more for us on tariff or realization side.

ARPOB a gain, if you optimize length of stay, then ARP OB will grow. But I think that base of 10% -15% ARP OB growth or realization growth, which we were seeing maybe a year and a half to two, that has already stabilized. You can already see it in the results of all companies. Last, maybe, a year to 18 months, you will see a significant slowing down of the ARP OB growth that was there two, three years ago. I think 5% - 7% is a reasonable number to project out if you are thinking about building a model. I would not go more aggressive than that, definitely, not double digits, I would keep it to single digits.

Moderator: The next question is from the line of Rahul Jeewani from IIFL Securities Limited. Please go ahead.

Rahul Jeewani: Thanks, sir, for taking my question. Sir, on the Noida hospital for, let us say, these 300 odd beds which we have operationalized, what kind of an exit occupancy we would have seen either for the December month or the January month? And if you can also talk about the occupancy which you need to achieve to, let us say, hit EBITDA break-even on these 300 beds?

Pankaj Sahni: So, we do not normally look at occupancy from a breakeven point of view because of the fact that

we believe much more in the volumes and the procedures rather than the midnight occupancy. So, at least, Medanta does not necessarily look at that. That being said, I think normally what we find is in a big syst

em somewhere in the range of 40 % to 50 % or maybe 40 % plus, occupancy typically breaks even on a long run basis for the kind of fixed cost model that Medanta employs. It may be very different in other industries.

Our goal is very simple in Noida. Our goal is that we have put everything that is required to deliver the highest standard of care ; that is investment in infrastructure, the technology, the clinical talent, Global Health Limited

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the processes, the IT systems, etc. , Our goal is to drive this occupancy to drive these volumes and this business through and through. Once we do that, whatever occupancy falls out of that, it falls out of that. Our focus is that we must be ensuring that we invest to deliver the highest standard of care , we have done that. And bas is the trajectory we see in the last few months, including post -December, we are pretty happy with how things are moving and pretty confident of the success of this institution. So, whether this occupancy will breakeven at 30 % or 40 % or 50 %, I do not know. It is also not a percentage number which we are chasing. Our goal is to ensure that we are able to drive as much patient volumes into the system. And we are pretty confident, given our track record, these things will then work out well.

Rahul Jeewani: Sure, sir. And for, let us say, these another 200 beds which you would commission at Noida, how many incremental doctor hirings would we need to do, given that, obviously, for the initial phase of 300 beds, we have already on -boarded 200 odd doctors?

Pankaj Sahni: So, not many. As I mentioned, I think in response to an earlier question, we have some departments which we may add. So, they, of course, are not linked to beds , and they w ould have additions of Obstetrics which is an example of that, which is a totally new department. The rest of the departments, all the seniors or most of the senior talent is already there. So, when you add in beds, it is really more of the junior staff who need to do more of the maintenance management and taking care -of-the-care, nurses, of course, will get added and other staff. But the senior doctors, I do not think, are necessarily linked to the bed addition.

Rahul Jeewani: Sure, sir. So, the incremental OPEX for these ano ther 200 beds, obviously, would then be lower ?

Pankaj Sahni: Yes, correct.

Rahul Jeewani: Sure, sir. In terms of the mature d hospitals, where we have seen some sort of a margin pressure this year, while you alluded to higher employee and maintenance costs, is the Ranchi O&M hospital also led to some sort of a margin drag for the mature units?

Pankaj Sahni: I mean, defi nitely, the new Ranchi Hospital also has a little bit of impact. I am not sure it will play out in the numbers which you are seeing because it is very small ; it is only about 100 beds on a 2,000-3000 -bed overall base. So, I do not think it has an impact as to what you see. Of course, it is a drag, but I do not think it is significant on material, on the financial, on the absolute rupee amount. Just to let you know, obviously, Ranchi and lower margins are of course lower than Gurgao n. But other than that, I do not think it is a very significant drag.

Moderator: The next question is from the line of Nitin Agarwal from DAM Capital. Please go ahead.

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Nitin Agarwal: Thanks for taking my question. Over the last few months and quarters, we have had a multiple sort of media items, news flow around increased difference between hospitals and insurers. I mean, two questions here. One, is there a change in relationship between insurers and the corporate hospitals , in the sense as insurers become larger in clou t? And two, what kind of implications does it have for rates and business as we go along?

Pankaj Sahni: Yes, so I think maybe I will gi

ve three, four specific points on this. I think, first of all, as with many things, the media likes to sensationalize things much more than they are in reality. So I would not always necessarily go with either the substance or the tone of what we hear in the media. That is point 1. Point 2 is just again at a higher level. See, every player in this industry, whether it is a provider like us, whether it is an insurance company, or whether it is even the government authorities, including at every le vel of government, they all understand that our collective responsibility is first and foremost towards the patient. Neither can we survive without the insurance companies , neither can the insurance companies survive without the providers. And by the way, the same logic applies to the pharmaceutical and medical device companies, which are also part of this ecosystem and also part of some of these conversations that you are alluding to , right? So I think we all get it. And

everybody is reasonably mature in that.

That being said, there is always a commercial negotiation and a commercial discussion that happens between any two parties engaged in a commercial conversation. Now, we have so far been very, very clear on our approach that we are open and ready to talk to anybody. And we have. We have been successful in those activities. And all conversations in good faith between two parties in a professional and in a non-collusive manner are something that we stand for. And we have been successful in moving that forward. Of course, if I would like to pay Rs.10 and you would like to pay Rs.5 or I would like to charge Rs.10, you would like to charge Rs.5, of course, we will negotiate on that price. Nothing wrong with that. And we have been successful in closing out many of these contracts and many of these negotiations. I think that we could have been faster and more efficient. I think that some of the players in the industry who are maybe trying to project that we will only talk as one body or only it is only the hospitals that are making money and the insurance companies are not making any money, I think these are facts which are being a little bit manipulated one way or the other, I think the truth is obviously somewhere in the middle, as with most cases. We do realize that the insurance companies have challenges, but many of those challenges are also linked to their operating model and some of the issues in the industry. Similarly, I think they understand that while you look at EBITDA many times, but the real money at the bottom line coming on the profit after tax, which is the money that has to go into capex refresh and investing in all of these areas, is required for hospitals to be able to grow, and, frankly speaking, people like us to invest in places like Patna, Bihar, Ranchi and Northeast. And the government also understands that it is very important that they encourage private investment in all these areas because that is how the care will get delivered.

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So, I, frankly speaking, see that there is no long-term relationship impact between the two parties. We have been very comfortable interacting with everybody. But, of course, we will negotiate on price. I do not see that stopping this year, next year or anytime in the future. As any entity feels they have more power, they would like to be stronger in a negotiation position. It is exactly the same thing that I would do with my suppliers. So, it is normal. I think some of it is blown out of proportion and some of it as few stakeholders in the ecosystem on both sides are probably mischaracterizing, misusing one party or another party on their side of the industry for their own personal benefits, which is also not correct.

Nitin Agarwal: Very helpful. And secondly, if I missed it, did we guide to a time period when we expect Noida to break

even on EBITDA?

Pankaj Sahni: No, we do not guide on future earnings as a policy. All I did allude to was the fact that we are pretty hopeful that we are now in a position to see strong positive momentum into the system. I do not know when that breakeven will happen. But, we are not worried that it is all slipping south or something like that. It is very much as per our expectations on how the ramp up of these units will do.

Nitin Agarwal: But, is it fair to say that this quarter EBITDA loss is kind of close to the peak EBITDA loss will do on the asset?

Pankaj Sahni: And that is the question which I said, I hope so. I do not want to commit to you one thing or the other. But I leave that to you guys to figure out. But yes, we are feeling confident that things are moving in the right direction.

Moderator: The next question is from the line of Naman Bagrecha from IIFL Capital Service Limited. Please go ahead.

Naman Bagrecha: Thank you for the opportunity. Sorry to press on the mature hospital margins again. So, if you look earlier, we used to make almost around 24.5% - 25% kind of margins, but is it because of now the incremental cost or war on talent, once should look at, let us say, lower margins structurally, let us say, 100 bps lower versus what was the strategy earlier for mature hospitals?

Pankaj Sahni: No, I would not necessarily agree with that, right? I mean, if you look at the margin profile, which we have in the nine-month period then the mature facilities, it is just under 24%. We have maintained for a long time and I have maintained for a long time that anything in the range of 22% to 25% is very strong performance for a system like this, especially such a big system like the ones which we have. I think that, 100, 90, 80 basis points swing here and there, especially in one period over the other is fairly insignificant. I mean, our industry, you have to look over three to four years, not nine months or even one year. And even within the nine months, the swing to me does not seem very significant.

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So, I do not think there is a structural difference. There was a structural reset post-COVID, which we

have talked a lot about on various calls. I think we are now in a fairly balanced way around that. I also had always maintained that I do not see this margin profile jumping up every year 30% or 300 basis points to head towards the 30% margin. I had always said that those were expectations that were being incorrectly set in the market. This number is a fairly balanced range, 22%, 23%, 24%, 25% this number is a very balanced range. You do a little bit, better, it could go up, but I do not think there is any big structural issue. Between one period or the other, you try to eke out as much efficiencies as you can. We will see this stabilize in these ranges only. I do not think they are going to change significantly.

The only thing in Medant's case in this, how we report out is, which is a little different, just so that you are aware, is all our corporate costs are loaded in our Gurgaon unit. So, what you see as the margin of the individual units is a little bit deflated because of the corporate costs. Probably, if you took out those corporate costs and you allocated them across, you would see a slightly more balanced margin profile between all of the bigger units at least in our group.

Naman Bagrecha: Fair. Thanks. And just one book-keeping question. What would be our net cash position as of December?

Yogesh Kumar Gupta: We have a loan of about Rs.600 crores and a cash position of over Rs.1200 crores, so, net cash is about Rs.600 crores.

Moderator: The next question is from the line of Abin Benny from JM Financial Institutional Securities. Please go ahead.

Abin Benny: Hi, sir. Thank you for the opportunity. While you did allude to the kind

of ARPO B growth that we

can be expecting, but my question was regarding the newer clusters. While we add more complex specialties, we add on to the newer ones, what kind of dynamics do you see regarding the kind of ARPO B that can change in these specifically, if you can give some color on that dynamics?

Pankaj Sahni: So, I mean, I do not have a number for you, but let me just give you a little bit of perspective, right? So, one of the things the industry has been commenting on for the last couple of years or maybe, six quarters, is this entire shift to, say, robotics. Now, robotics means a higher realization, but also potentially a lower length of stay because a patient gets to go home faster. So, obviously, that has a disproportionate increase on ARPO B because your ALOS will fall and your realization will increase both. So, it is one example of something that may boost up when you look at it at a ARPO B level rather than a ARPP-level.

The second thing, as I said earlier, was that when you get into increasing work in Cancer, especially if it is Cancer work in the daycare nature like Radiation Oncology, where you may not admit the

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patient or Medical Oncology where the patient just stays for one day for treatment, you will find a disproportionate growth to ARPO B, because the occupancy is not that long as opposed to, say, somebody coming in for Cardiac surgery. So, I think that these things are actually in favor of ARPO B growth because ARPO B is a derived metric.

I think if you look at it and step back towards realization, which is what the patient will actually be, I think, as I mentioned earlier, you will see a more balanced realization growth, there will be some growth towards realization because of complex work, some amount of increase in complex work across our newer units in Patna, even in Noida as that gets ramped up to some of the services, including Transplant and things like that. We are also starting to see some of this complex work coming into places like Indore with robotic surgery happening there now reasonably well. So, you will see some of this and then some of the changes of the CGHS rates, as I mentioned earlier, which happened in October, and the tariff increases will play out over, maybe the course of the next six to 12 months. So, you will see some positive trajectory towards this, but I think, again, it will be measured. I would not assume you would see 10%. I would say single digit is a fair number to be looking at.

Abin Benny: Got it. Thank you. And, sir, if possible, can you give us the kind of expectations on CAPEX outlay for next year, given that majority of the expansion will be there?

Pankaj Sahni: Yes, I think there was a slide, correct me if I am wrong, Yogesh, on the CAPEX plan in the presentation, slide #23 lays out the CAPEX. We have been somewhere in the Rs.1,000 crores range, I think, on next year CAPEX. So, this is Rs.3,000 crores over the next five years. Next year, maybe a bit less, actually, now that a lot of the cost goes to Noida. So, last year we had, I think, announced about a Rs.1,000 crores CAPEX in the year, out of which almost Rs.600 – Rs.700 crores was Noida. And I do not see that this FY27 CAPEX will be much lower than Rs.1,000 crores, maybe some maintenance CAPEX in the older units, some CAPEX on the margin in Noida. There will be not much CAPEX on the other units, because they are still in the digging and construction phase. So, a lot of the CAPEX in the new projects is back-ended towards maybe two, three years from now. So, next year CAPEX is probably, below Rs.500 crores would be our estimate.

Moderator: The next question is from the line of Chirag Gupta from Allegro Capital. Please go ahead.

Chirag Gupta: Thanks for taking my questions. I just wanted to understand why the margins in your developing facilities are higher t

han your mature facilities?

Pankaj Sahni: So, as I mentioned I think to one of the earlier speakers , one reason, of course, is that our corporate costs are fully loaded in Gurgaon, which is part of the mature facility. The second , if you look at our historical performance, is that our Lucknow unit and our Patna units have been doing an exceptionally
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good performance right through -and-through, barring one or two quarters in Lucknow. So, what we have actually seen is a good amount of growth, a good amount of operating leverage come in. So, just to contextualize for you, Lucknow is about a six -year-old asset , Patna is about four years old now, and we have continually been adding beds to these facilities. So, what you are seeing in these facilities in addition to the strong growth in these regions, you are also seeing improvements in newer specialties getting added ; just to give you an example, we have added three or four robots, including orthopedic robots in Lucknow. So, we are getting more complex work. We are adding more complex work into our facility in Patna , and you are also seeing the benefits of operating leverage kick in, depending on which year you are looking at , after a couple of years, Patna would have kicked in , Lucknow would have kicked in , plus, we would have added, I think, last year or 18 -months ago, we added almost 200 additional beds, taking Lucknow from about 400, 500 to about 700 census beds. So, those beds operating leverage also kicks in after some months. So, I think these are the three or four reasons why there is a difference between the developing and the mature. And then the last reason, although it may not be very significant in absolute terms, is that our mature d units also include Indore and Ranchi, which are significantly smaller scale and have a significantly lower margin profile.

Moderator: Thank you. Ladies and gentlemen, due to time constraints, that was last question for today. I now hand over the call to management for closing comments. Over to you , sir.

Pankaj Sahni: Thank you, everyone, for your questions and for joining us today. Please feel free to reach out to our investor relations team in case you have any questions that remain unanswered . Thank you once again , and we look forward to speaking with you all soon.

Moderator: Thank you. On behalf of JM Financial Institutional Securities Limited, that concludes this conference. Thank you for joining us and you may now disconnect your lines.

Notes:

1. This transcript has been edited for readability and does not purport to be a verbatim record of the proceedings
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Dear Sir (s),

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the transcript of Earnings Conference Call held on Friday , May 15 , 2026 , for the Fourth Quarter and Financial Year ended March 31, 2026 results .

The Transcript is also available at our website: <https://www.medanta.org/investor-relation>

This is for your information and record.

For Global Health Limited

Rahul Ranjan
Company Secretary & Compliance Officer
M. No. A17035

Encl: a/a

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Q4 FY '26 Earnings Conference Call "
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Moderator: Ladies and gentlemen, good day, and welcome to the Global Health Limited, also known as Medanta, Q4 FY26 Conference Call hosted by ICICI Securities. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” and then “0” on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Abdulkader Puranwala from ICICI Securities. Thank you, and over to you, sir.

Abdulkader Puranwala: Good afternoon, everyone, and a warm welcome to all participants on the Global Health Limited Q4 FY26 Earnings Call hosted by ICICI Securities. Joining us today from the management, we have Dr. Naresh Trehan, Chairman and Managing Director; Mr. Pankaj Sahni, Group CEO and Director; Mr. Yogesh Kumar Gupta, CFO; and Mr. Ravi Gothwal, Head, Investor Relations. I will now hand over the call to Dr. Trehan for his opening remarks. Thank you, and over to you, doctor.

Naresh Trehan: Good afternoon, everyone, and welcome to Medanta's Q4 and Full Year FY26 Earnings Conference Call. I would like to start with reiterating the fact that at Medanta, our vision has always been to build a world-class institution that seamlessly integrates clinical excellence, research, education and advanced technology with compassionate patient-centric care, benchmarking ourselves against the finest global healthcare institutions.

I am particularly pleased to share with you the key milestones during this year. Our 550-bed Noida facility commenced operations during the year. We also acquired land to set up a 400+ bedded multi-specialty hospital in Guwahati, which will bring the highest quality of care to the Northeast region of our country. We also partnered to build a 400+ bed hospital in holy city of Varanasi.

During the year, we strengthened our clinical leadership, enhanced institutional capabilities and further deepened the trust that patients place in us across our network. Our team successfully managed several highly complex and high acuity cases across specialties, reflecting the depth of our clinical expertise. At the same time, we continue to strengthen our capabilities in robotic-assisted and minimally invasive procedures, underscoring our focus on improving clinical outcomes and enabling faster patient recovery.

Medanta Gurugram, our flagship hospital, was recognized by Newsweek as the best hospital in India. This marks the seventh consecutive year of the hospital being recognized by Newsweek, and we remain the only private Indian hospital to be featured among the top 150 hospitals of the world.

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Our Lucknow hospital received prestigious JCI accreditation in January 2026, becoming the first hospital in Eastern Uttar Pradesh to achieve this distinction and further reinforcing our commitment to global standards of clinical quality. I'm also pleased to share that Medanta Noida 550-bed multi-specialty hospital formally inaugurated in November 2025, secured NABH accreditation within the first 6 months of operations. This, of course, reflects the strength of our institution system and clinical protocols from inception. We have on-boarded a highly skilled team of clinicians and deployed advanced medical technologies enabling us to deliver world-class care to patients in Noida and Western Uttar Pradesh.

On the expansion front, we announced 400-bed multi-specialty hospital project in the heart of Varanasi. This project will further strengthen our presence in Uttar Pradesh and serve as a key tertiary care hub for Eastern Uttar Pradesh addressing a significant unmet need in the most populous state of India.

Subsequently to the year-end, in May 2026, we expanded our presence in Indore through

acquisition of a 80 -bed hospital. This will allow us to add oncology capabilities as well as complement our existing multi -specialty facilities.

As we continue to grow, our focus remains firmly on building integrated healthcare institutions, driven by clinical excellence, patient trust, ethical practices, research and long -term sustainability. Every step in our growth journey is guided by a clear purpose to make the highest quality of medical care accessible to a larger population and especially established where it is needed most.

With that, I will now hand over the call to Mr. Pankaj Sahni, our Group CEO, who will take you through the financials and operational performance in greater details. Thank you very much.

Over to you, Pankaj, please.

Pankaj Sahni: Thank you, Dr. Trehan. Good afternoon, everyone, and thank you for joining us today for our Q4 and FY26 earnings call.

FY 2026 was another important year for Medanta as we continued to deliver strong operational and financial performance across the network. This performance was driven by sustained growth in patient volumes, improving realizations and consistent execution across our hospitals, while we simultaneously invested in expanding our capacity and strengthening our clinical capabilities.

Importantly, the year reflects the strength of our operating model where mature hospitals continue to deliver steady performance, and our developing hospitals are scaling up with strong momentum and the launch of our sixth flagship hospital in Noida, creating a solid foundation to drive medium -term growth.

Let me begin with the financial performance highlights for the full year FY2026. Total income grew by 20% year -on-year to INR 45,089 million. Total income, excluding Noida, grew by 17%. The EBITDA excluding Noida, grew by 19% year -on-year to INR 11,343 million.

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EBITDA margins improved to 25.7% compared to 25.4% in the same period last year. EBITDA including Noida, stood at INR 10,560 million with margins of 24.2%, reflecting the expected impact of early stage operating losses from our newest hospital.

Noida remains a key strategic asset for us, and we are encouraged by the pace of ramp -up since its formal inauguration in November 2025. We have operationalized 382 beds, 98 ICU beds and 14 operating theaters. The hospital has onboarded over 200 doctors and 75 + senior clinicians. Clinical programs have scaled rapidly with over 1, 600 surgeries, including robotic procedures, over 5,000 Cath lab procedures, the initiation of advanced therapies, including CAR T -cells and bone marrow transplant and kidney transplants. We have also achieved NABH accreditation within 6 months, reflecting the strength of our clinical and operational processes. We have made strong progress on various empanelments, including insurance, corporate and PSUs. And we have also received CGHS empanelment in the month of May 2026, in line with our commitment to deliver the highest quality of care to all communities.

From a financial standpoint, Noida has delivered revenue of INR 906 million and an EBITDA loss of INR 783 million during FY2026. Importantly, Q3 FY2026 was the peak quarterly loss for Noida at INR 320 million. Q4 loss reduced to INR 236 million even as bed capacity expanded from 328 to 382. Consolidated profit after tax increased by 15.1% year -on-year to INR 5,541 million.

On the international business side, revenue grew by 33% year -on-year to INR 2,780 million. International mix has expanded from 6% to 7% of consolidated revenue. Despite some short -term challenges given the situation in the Middle East, we see continued runway as Noida's international funnel activates and new countries in Africa, Southeast Asia and the CIS region scale up.

In line with the company's strong financial performance, the Board has recommended a final dividend of INR 0.50 per share, representing 25% of the face value of INR 2 per share. Now for some operational highlights during the year. The total bed capacity increased by 20.5% year-on-year with the addition of 623 beds during FY2026 comprising 382 beds at Noida, 131 beds at Patna and the commissioning of the newly built 110 -bed hospital in Ranchi. Alongside capacity expansion, we continue to strengthen our clinical depth across the network and onboarded more than 550 doctors during FY2026, including over 200 senior clinicians across various specialties further enhancing our institutional capabilities and patient care delivery standards.

Volume growth remained robust. Inpatient count increased by 16% and outpatient count increased by 19% year -on-year. ARPOB grew by 6.1% year -on-year to INR 66,550, driven by improvements in ALOS and change in case mix.

Our average length of stay improved to 3.04 days compared to 3.17 days same period last year, a 4% improvement year -on-year. While occupied bed days increased by 11%, translating into

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an occupancy of approximately 62% on an expanded bed capacity. Our occupancy, excluding our new Noida facility, was 64%.

Moving on to the mature and developing performance update. Let me spend a moment on the mature hospital portfolio. Our mature hospital portfolio comprising Medanta Gurugram, Indore and Ranchi, delivered revenue of INR 28,482 million, up 9% year -on-year and EBITDA of INR 6,946 million, up by 7% with a full year margin of 24.4%.

Matured margins were nominally down by 40 basis points during the year. This is largely because of an increase in employee costs, which were offset by improvement in material costs. It is important to note that all corporate manpower costs are loaded onto the Gurgaon unit, so to that extent, there is some moderation required when we evaluate the Gurgaon unit stand -alone performance.

Moving to the developing portfolio, our developing hospitals, excluding Noida, continue to outperform recording 29% year -on-year revenue growth to INR 14,130 million and EBITDA of INR 4,470 million, a growth of 35% with margins improving to 31.5% compared to 30.1 % in the same period last year.

Revenue from developing hospitals, including Noida, stood at INR 15,036 million, up 37% year -on-year, while EBITDA stood at INR 3,663 million with margins of 24.4% reflecting the expected drag from early -stage operations in Noida. Inpatient volumes across developing hospitals increased by 32% year -on-year with overall occupancy at 60% and ARPOB rising 4% to INR56,500.

Moving to the projects update. In line with our long -term growth strategy, we continue to make steady progress across our expansion and capability building initiatives during financial year 2026. In March of 2026, the Board approved a 400 -bed hospital project in the heart of Varanasi under a build -to-suit lease arrangement with a partner. This is a build -to-suit structure where our capex would be towards MEP, interior fit -out and medical equipment.

This project further strengthens Medanta's presence in Uttar Pradesh building on the successful establishment of its 1,000 -bed hospital in Lucknow and the recently launched 550 -bed hospital in Noida. The company also completed land acquisitions for its proposed 400 -bed hospital in Guwahati and has secured various needed regulatory approvals.

During the year, Medanta operationalized a newly constructed 110 -bed hospital in Ranchi, complementing its existing 200 -bed facility and enhancing capacity in the region. Additionally, Subsequent to year -end, Medanta announced the addition of an approximately 80 -bed hospital Indore under a business transfer agreement expected to be operationalized in Q2 FY2027.

Strateg

ically located under 500 meters from the existing Indore facility, this unit will address the gap in oncology and will enable us to deliver a comprehensive and integrated cancer care program.

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Our South Delhi project is progressing well. The Diaphragm wall on site 1 has been completed and the civil tender is expected to be awarded by the end of next month. Our Mumbai and Pitampura projects in Delhi are at various stages of regulatory approvals.

Looking ahead, Medanta will further ramp up the recently added bed capacity. In the short -term, the company expects to add approximately 500 beds across its existing hospitals with a minimum capex investment. In addition, Medanta will add approximately 2,700 beds through 5 greenfield projects over the next 3 to 4 years.

Across the entire expansion pipeline, total project capex over the next 5 years is approximately INR 45,000 million. We exited FY2026 with a net cash position of INR 5,906 million and generated operating cash flow of INR 7,144 million during the year growing at a 4 -year CAGR of 21%.

The capital plan is fully supported by a combination of internal accruals and project -specific debt that we may take. Our balance sheet strength gives us the flexibility to execute this growth pipeline with appropriate discipline.

With that, I will now hand over the call to Yogesh, our Group CFO, for the quarterly

performance update.

Yogesh Kumar Gupta: Thank you, Pankaj. I will now take you through the financial performance highlights for the Q4 FY26.

Q4 FY26 was a strong quarter for Medanta with the company reporting its highest ever quarterly revenue and EBITDA. EBITDA highlighted today is excluding ESOP expenses, which are non-cash in nature. Total income for the quarter stood at INR 11,958 million, reflecting a growth of 25% year-on-year, driven by a strong patient volume, healthy occupancy and improving realization across the network.

EBITDA excluding Noida grew by 27% year-on-year to INR 3,142 million with improved margins of 27.5%. EBITDA including Noida stood at INR 2,906 million, reflecting a growth of 17% year-on-year with a margin of 24.3%. Profit after tax for the quarter stood at INR 1,417 million, registering a strong growth of 40% year-on-year with a PAT margin improving to 11.8% from 10.6% in Q4 FY25.

Operationally the quarter continued to demonstrate healthy momentum across the network. Occupied bed days during the quarter increased by 18% year-on-year representing occupancy of approximately 61% on expanded bed capacity and around 64% excluding Noida Hospital. ARPOB increased by 5% year-on-year to INR 66,687 supported by favorable case mix and improvement in ALOS. Inpatient volumes grew by 23%, while outpatient volumes increased by 27% year-on-year.

International patient revenue during the quarter increased by 22% year-on-year to INR 679 million. Our OPD Pharmacy business continued to witness strong traction with revenue growth

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of 46.2% year-on-year to INR 496 million during the quarter. Overall, quarter demonstrated a strong traction across the network.

Now coming to the mature and developing hospital performance update, our mature hospitals comprising Gurugram, Indore, Ranchi delivered steady performance during the quarter with revenue of INR 7,257 million. Growth of 11% year-over-year and EBITDA of INR 1,935 million registering a growth of 15%. EBITDA margins improved to 26.7% compared to 25.7% in the same quarter of last year.

ARPOB stood at INR 74,941 registering a healthy year-on-year increase of 7%, supported by improved case mix and ALOS reduction and sustained traction across these specialties. Average length of stay improved to 3.00 days compared to 3.17 days in the same quarter last year reflecting continued focus on clinical

efficiency and patient throughput. IPD volumes grew by 10% year-on-year while occupancy remained healthy at 61%, reflecting consistent demand across our matured hospital network.

Our developing hospitals, excluding Noida, continue to deliver strong performance, delivering the improvement in efficiencies, especially specialty expansion and scale up in operations. Revenue from developing hospital, excluding Noida stood at INR 3,728 million, reflecting strong growth of 33% year-on-year. While EBITDA stood at INR 1,207 million with registering a strong growth of 42% year-on-year with healthy margin of 32.4%. Including Noida developing hospital revenue stood at INR 4,253 million, reflecting strong growth in patient volumes and specialty programs.

Noida Hospital reported a revenue of INR 525 million during Q4 FY26, while EBITDA loss reduced to INR 236 million compared to INR 320 million in Q3 FY26, reflecting a steady improvement in operating performance as the hospital continues to ramp up.

Developing hospitals ARPOB stood at INR 57,349, registering a healthy year-on-year growth of 7%, while overall occupancy remains strong at 61%. Average length of stay improved to 3.13 days as compared to 3.22 days in the same quarter last year, reflecting continued focus on clinical efficiency and patient throughput. IPD volumes grew by 45% year-on-year, driven by a strong ramp-up of Noida facility, along with sustained growth across existing and developing hospitals. The company continues to maintain a strong balance sheet with a net cash position of INR 5,906 million as of March 31, 2026, providing flexibility to support our long-term growth initiatives and expansion pipeline.

With this, I request operator to open the line for the questions.

Moderator: Thank you very much. We will now begin with the question and answer session. The first question comes from the line of Amey Chalke from JM Financial.

Amey Chalke: Congrats to the management on good numbers. So first question I have on Noida unit. The unit is performing well. Is it possible assuming that 300 beds were operational from the day 1 of the

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quarter, is it possible to give what was the occupancy for the quarter for this unit? And do you expect this unit to break even by quarter 2 of next year?

Pankaj Sahni: So we do expect this unit to break -even during the course of this year. But I would say that probably we would look at the second half of next year rather than to look at giving you a number on Q2. But during the course of the second half of this financial year, we do expect the unit to break -even.

Our occupancy in our Noida hospital is currently running somewhere around 30%. And obviously, you'll appreciate that because we are still ramping up and even some of the impanelments are still coming on as recent as these last few weeks, that is actually changing quite dramatically month -on-month.

Amey Chalke: Sure. So if the breakeven is in second half, so should we expect some losses even on small amount next year for the Noida?

Pankaj Sahni: So on a cumulative basis, Amey, I guess it really depends how quickly we are able to get the occupancy to somewhere, I guess, in the 40% to 45% range, we normally find that the hospitals do break -even. So it really depends on the extent, right? This last q uarter, as I mentioned in the notes, we have just about INR 24 crores of losses in Q4. So if that continues to reduce or the revenues and EBITDAs continue to grow in this way, then we should be able to make up whatever losses we have in the first part of t he year. But I don't want to give you a definitive commitment because it really depends on how much accumulated losses we are carrying as we move towards the profitable quarters .

Amey Chalke: Sure. And the second question I have on the other developing uni ts, Lucknow and Patna, we

don't have breakup, but it looks like for these units typically 4Q is slightly softer quarter compared to 3Q, but these 2 units have also delivered good numbers. So we have bed expansion here in these 2 units next year.

So if you can provide occupancy for these 2 units as well? And is there a scope to add these beds in the first half of the next year or it will also be a back -ended considering the occupancy, I believe, should be in mid -60% for both these units?

Yogesh Kumar Gupta: Occupancy for these units during the quarter was 68 %

Pankaj Sahni: So let me first touch on Patna as far as the bed expansion is concerned. Occupancy in Patna actually a little bit higher than what as Yogesh said, closer to the 70%. And we are actually significantly constrained in terms of our need to add specifically ICU and critical care bed s in Patna. So we have actually already given orders for a bed addition there. And we do hope to add that within the course of the first half of the year. But proje ct construction work is on, so we will see when that comes up.

We also have project construction work happening in our Lucknow facility, and we will probably add in some of the beds in this half of the year only. Specifically, some of the super specialty

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areas like Neonatology , ICUs, et c., are being added into Lucknow. So we will likely see some amount of bed addition in both the units in the first half of the year as itself.

Now I also wanted to clarify that more important than the bed addition, both the units are adding operating rooms. In fact, we have about 4 or 5 operation theaters being added in our Lucknow unit and 4 or 5 operation theaters being added in our Patna unit.

So beyond beds, the actual constraint and the actual growth we will see coming in from our procedural capacity expansion beyond just beds. So that's an important point to note because as you are aware, we have a very high procedural orientation in our group. And this is a long pending demand from the local unit leadership. So we are fulfilling that.

In fact, just to touch on the operation theater expansion, even in our Gurgaon facility, we are adding in operation theaters. 2 operation theaters will be activated in Q1 itself and another 2 operation theaters, we hope to get activated either towards the late r part of Q1 or at least by Q2. And we will also be adding additional cath lab capabilities in our Gurgaon facility. So I just want to stress on the point that it's not only beds that drives the enhanced work, a lot of our enhanced work will come from procedural capacity addition.

Amey Chalke: Sure. That's clear. And the last question, if I can squeeze in, on our addition or the greenfield units, which are coming up, 2 in Delhi and 1 in Mumbai, etc. It looks like looking at the progr ess on these projects, do you think that any of these units can come up in first half of FY29 or these would be beyond FY29, looking at the progress on these projects?

Pankaj Sahni: So we are sitting in the first half of FY27. Just to give you an idea, our Noida facility from construction start to construction end took approximately 3 years. And normally, hospitals of this scale take about that much time to get built out. So I think that we should assume that they will take about the same amount of time. Maybe if you are sitting in May of 2026, you could say May of 2029 hospitals could be up and running. But that may not be FY29 numbers, that may be more FY30 numbers.

Moderator: Your next question comes from the line of Banshi Desai from JPMorgan.

Banshi Desai : Congrats on a good set of numbers. So my first question is on Noida. Now that we are inching closer towards break-even in this fiscal. Historically, you've seen very strong ramp-up for our newer units, be it Lucknow, Patna. I know, Pankaj, you've always cautioned us that don't s

imply

extrapolate. But should we think about Noida's ramp-up differently more so because this is also a region of our core strength where we have a very strong ecosystem. So should we think any differently post the break-even?

Pankaj Sahni: Let me answer that in 2 parts. I think structurally, I don't see any very significant difference in the way in which we operationalize or operate these units. I also don't see any very significant difference in terms of the complexity of the procedures there or the demand for our services from these communities. I will, however, just mention, I'm not sure if it was in the commentary.

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There are some specialties that we still have not started in Noida. And one of those is our pediatric services, which was started towards the end of this financial year FY27. And our obstetrics practice has still not started and our liver transplant practice has still not started. So there are a few of these specialties which will come in. They will, of course, have their own impact on the financials. Some have a slightly higher margin profile, some have a higher realization profile, and they will, of course, translate to various sets of financial metrics as those scale up as well.

But other than these specialties which are not there, I see no structural difference in Noida as far as our operations or the excitement for this ramp-up. Now the only difference that I would say with respect to both Lucknow and Patna is, please keep in mind that those hospitals opened right in the middle of the COVID pandemic. So there were some very different kind of situations in both the units. Lucknow opened just before the pandemic started and Patna opened maybe towards the later part of the COVID pandemic. So there are some differences.

One thing which has been an advantage for us as far as the operations are concerned, is that we were able to bring in a lot of very senior clinicians in Noida on day of opening. And as you may have recalled from my commentary in the last quarter earnings call, we consider this to be an investment in our clinical talent. So while it was hitting the operating P&L, and therefore, you see some EBITDA loss because of the manpower cost, this is really the core investment beyond the capex that we do make. So we have very strong, exceptionally talented clinical teams in place, and you are seeing that with respect to the kind of work we're doing in robotics, in cancer, even the complex cardiac work, whether it's TAVIs and so on and so forth.

So we have the firepower to get moving quickly and we don't see any real reason why we can't. But also keep in mind, Lucknow and Patna were different times, different cities. So that's my only caution.

Banshi Desai: Also, Pankaj, were these more underserved markets in general? I mean, Noida doesn't appear to be underserved at least on face of it, but you are highlighting that at least structurally demand doesn't seem to be a problem. So should we think of that as a factor as well?

Pankaj Sahni: So I think for this, we have to get a little bit into each of the cities. I'll take a second to do that. Let's start with Lucknow. I think Lucknow had, always had a very strong and very robust government medical ecosystem. So it is not like there was not clinical talent or complex work happening in Lucknow. Probably in the private sector, it wasn't that common, but there was talent available there. So I don't think that you can call Lucknow as a completely absent of medical care. It is just that it was delivered in a different kind of an ecosystem.

Patna, I think, yes, definitely, Medanta Patna kind of a facility has probably not been seen in Bihar before. And the scale and the nature of the kind of infrastructure, equipment and clinical talent that we have brought to Patna and even Greater Bihar probably didn't exist

in that area.

So maybe there was an element of being underserved in Bihar much more than, say, in other

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areas. That being said, as you're aware, Medanta does attract patients from all over. So we have getting patients both in Lucknow as well as in Patna from the broader region beyond those cities. Now as far as Noida is concerned, you have to break down Noida into three catchments. One is the local Noida catchment. It has some hospitals, but not many and not of that same caliber. The other is the catchment, which flows from the Delhi NCR area. There are, of course, hospitals in that region, but we are seeing a good demand actually from even areas like South Delhi moving into our Noida facility. And the third area, which I do believe remains underserved is entire Western UP. So we do get a lot of patients from that area. And we are seeing a lot of excitement about patients coming in even as far as places like Bareilly.

So I think that Noida has some elements of patients coming from underserved areas, but also some amount of local capabilities and local patients are coming in and also some amount of movement from the Delhi area towards Noida for treatment.

Bansi Desai: Understood. And my second question is on our matured units. We've seen some margin compression this year. You highlighted higher employee costs. So I'm assuming given we've done these kind of investments this year, incrementally going forward, should we expect margins to improve? What would be the key drivers if we have to think about margin for matured units over the next 2 to 3 years?

Pankaj Sahni: I think the way in which we look at this, right, as you're aware, we don't kind of give margin guidance. But if you look at the margin profile over the course of the year, we have gone from 24.8% to 24.4%. So it's just about a 40 basis points swing, right? So it's not that this is a kind of very significant movement.

If you look at our fourth quarter margin profile for the matured hospitals, it's coming in about 100 basis points over last year same period at about 26.7%. So we do have a good feeling around some of the actions taken to optimize on material cost. We are taking some further actions on the optimization of our cost in our Gurgaon unit. But also, we do have, as I mentioned on the call, the entire corporate costs are loaded on to our matured hospital group, in fact, are loaded on to our Gurgaon facility. So there will be, as we move forward, various types of scale and benefits of that because, obviously, you won't hire more and more corporate resources as you scale the operations.

But we are feeling fairly confident about the margin profile remaining in this kind of 24% - 25% range. We don't see any real reason for any kind of dips. And to the extent that you get the benefits of some incremental growth or some tariff or realizations growth, some of it kicked in last year with respect to some of the tariff increases and the CGHS benefits, so partial year. This year, it should be a full year impact of that. So you may see some upward tricking on that. But I think that we believe, generally speaking, that the margin profile of the matured units is largely stable, and we are working to see if we can tweak out inefficiencies wherever we can.

Moderator: The next question comes from the line of Viraj Shah from PGIM.

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Viraj Shah: Congratulations on great set of numbers. So my first question is that, I see that your developing hospitals is sitting at 60% occupancy as of now. And you mentioned that your Patna unit is at 70% occupancy. Along with Noida ramp-up and the next leg of hospitals broadly coming in and around FY29, what are the growth levers on a general basis, which you are seeing for the next 2 or 3 years?

Pankaj Sahni: So let me first

kind of address the lead into the question on occupancy, right? And I also just wanted to call out, if you look at our ALOS performance over the course of the last 5 years, you will see that despite all the complex work you do, there has been a consistent reduction in ALOS from 3.76 days in FY22, which is just after COVID, all the way down to 3.04 days in FY26. So occupancy is a function of how many patients sleep in the hospital at midnight. It is not necessarily the only metric to show what is the volume work and volume and throughput that is happening in the hospital. A lot of work is moving towards day care. This is a trend in healthcare globally.

Our particular case, as I mentioned in one of the earlier questions, we have a lot of focus on procedural work. And so for us, it's almost more important to have operation theaters, cath labs, endoscopy rooms and ICUs than it is to have only ward beds. So if I look at the example I gave you in Patna, we are in a very urgent basis trying to add an ICU. But we do believe, at least I do believe that we still have operational efficiency to reduce length of stay in our ward beds there. So beds for me is not the only constraint for us to find the growth lever. The growth lever will

come from 3 or 4 broad aspects.

First of all, of course, in the areas where we don't have the complete set of suite of services, like in Indore, we mentioned we'll be adding cancer. In Noida, we'll be adding some specialties plus the ramp-up. So growth will come first and foremost from adding in services, adding in procedures, adding in specialties.

The second thing that will happen is that there will be bed additions during the course of the year in all of the existing facilities, which includes Noida as well as some of the others.

The third is that there will be procedural additions. So I already mentioned we'll be adding somewhere around 10 operation theaters in our ecosystem in the existing hospitals, which have all been operating for 4 - 5 years. That doesn't even include Noida or the new Indore facility. So we will be adding in this capacity.

In addition to this, there has been a lot of momentum on minimally invasive, robotics, complex care like TAVIs, etc. So if you look at, for example, the volume of TAVIs, which we do, it's probably the highest in the country. If you look at the kind of volume and the awareness of robotic surgery happening, not only for us, but across the industry, you'll find that, that is also growing. So more and more the new age therapies will come in. And given our complex work profile, we will see a lot of that coming into Medanta as well.

So these are 3 - 4 areas. Now of course, beyond this, there's the routine activities around sales and marketing. There's the routine activities around international growth in new markets. I do

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believe that at some point in time, you will see the challenges in the Middle East taper off, and we hope that will come back.

We do believe Bangladesh also as an international market will eventually come back. But we've seen a lot of traction in Medanta Group from newer markets like Africa, Southeast Asia and CIS. And in fact, we've seen a good amount of growth in our international business, about over 30% growth in international business this financial year. So we are feeling fairly confident about the complete ecosystem and the portfolio that we have. And we will also go out into the market and look for additional clinical talent because in some cases, we do have our doctors extremely busy, and we do need to add in clinical talent, etc. I hope that answers all the levers that I would think of instantaneously.

Viraj Shah: Yes. Can we also assume, so what I'm seeing is that your matured unit ARPOB growth has been much higher in comparison to the developing ones, which is broadly Lucknow a

nd Patna at this point. So can ARPOB growth be higher going ahead for these 2 units?

Pankaj Sahni: You mean Lucknow and Patna?

Viraj Shah: Yes.

Pankaj Sahni: So again, ARPOB is a function of length of stay and that as well. But let me not just talk about ARPOB, let me talk about realizations. So if you look at realizations as a whole, there are 2 important points. One is that Patna has been operational now for approximately 4.5 years. We have not taken a single rupee of tariff increase in Patna. So there may be some tariff adjustments that we may look at in Patna basis the market conditions there, maybe some in Lucknow as well. However, beyond the tariff, we will also see certain more complex specialties adding. So robotic work will scale up in both the units. We already have a good portfolio of robotic work in Lucknow. We are thinking of adding in a second robot there. We will have additional services like liver transplant coming into place in places like Patna, chest thoracic surgery, lung transplant coming into places like Patna, which don't exist there. So even our Mother and Child services have really started in Patna just very recently and because they tend to have a shorter length of stay. So from an ARPOB growth point of view, they tend to be accretive, even though realizations may not be as high as some of the cardiac or other types of work. So I think there's 3 - 4 levers will play in. You may see an opportunity for ARPOB growth in all of these areas.

Viraj Shah: Understood. Just one last question in this. With the strong net cash position, which you have and cash generation is pretty strong on an annual basis, any plans for over and above this bed expansion, any plans for any inorganic expansion which you have on your cards?

Pankaj Sahni: Yes. Obviously, you are aware, we cannot reveal any of the transactions until they are fully complete and announced to the exchanges. So we have done a very small transaction recently with the Indore acquisition. There are a couple of other transactions that we are looking at, both on the O&M side as well as on the M&A side.

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Like we've always maintained, we will continue our philosophy on M&A, not going for M&A just for the sake of it, but looking at the quality of the asset, looking at the values of the partner, in case it things like O&Ms .

Looking at the structure and the ethics of the transaction and then, of course, looking at whether it makes financial sense and whether it is needed in that particular territory. So we are actively exploring various types of transactions, both pure acquisition as well as partnerships or O&M types of models beyond the greenfield.

Moderator: Your next question comes from the line of Tushar Manudhane from Motilal Oswal Financial Services.

Tushar Manudhane: Congrats on a good set of numbers. Sir, firstly, on number of doctors being onboarded in FY26, almost about 550, while the new hospital required a good set of doctors to be there at Noida.

How to think about addition of doctors in FY27 - FY28?

Pankaj Sahni: So what's happening, Tushar, is that across the network, let me start with Gurgaon, which is our oldest facility. Across the network, we are seeing actually quite a bit of demand for our clinicians and also seeing a very strong focus on actually adding capacity to be able to service the appointments, service the waiting list for getting access to certain doctors and so on and so forth. And as you're aware, we have a kind of outreach or a scale of activity that is happening now beyond the hospital. So with 4 clinics operating in NCR, there is a demand for access to doctors in these clinics as well. So we do believe that there is a need to hire clinical talent in our ecosystem. And we are also looking selectively at some of the clinical talent coming in as a preparat

ion for the assets that come on Board 3 to 4 years from now.

So just to give you a very simple example, let's say, I hire a cardiologist or a cardiac surgeon, who in 3 or 4 years would like to move to Guwahati and kind of take a senior position there.

This is something that we are actively looking into. So we will be hiring in almost every single unit. Gurgaon is the oldest and has the most density of doctors, but we are looking to actively hire here.

Lucknow and Patna, we will first look to fill the gaps, as I mentioned. So examples were obstetrics was recently added in Patna; liver transplant, not yet there; chest surgery, not yet there in Patna. So wherever there are clinical gaps, we will add those in. This last year, we added in pediatrics, advanced pediatrics in Lucknow. We may scale some of those services up further. We will also look at adding in additional band width at a lateral level in Patna and Lucknow, both, because those also are now reaching situations where the doctor's capacity is becoming an issue.

So there will be a reasonable amount of addition in all of these places. Of course, Indore, Ranchi, both of them have new bed capacity. So they will have doctor addition to meet some of that bed capacity. And of course, full fledged of cancer services will come on Board in Indore. All of this is in addition to the Noida facility ramp -up that you mentioned.

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Tushar Manudhane: Understood. And coming to Gurgaon now that 2 new hospitals coming up in Gurgaon probably in September -October period, competitor's hospitals. So how do you sort of think about across demand or, let's say, the inflow of patients and the doctor attrition or retention of doctors, whichever way you may look at as far as Gurgaon hospital is concerned?

Pankaj Sahni: Yes. So we've been talking about this now, I think, for 2 - 3 quarters. And every quarter, we believe that the hospital is coming onboard in the next 6 months. So we are aware of this. Our clinicians obviously are also aware of this. I'm proud to tell you that as on date, at least from what I'm aware of, no senior clinician has been lost to any of these hospitals, which are in the process of coming on Board . So we have not seen any very major attrition at least as on today. We have, as you're aware, a very strong clinical value proposition, and we hope that our clinicians are happy with the kind of work environment and the value system that we deliver for them to operate here.

That being said, obviously, we have to be aware of it. Obviously, we have to keep in mind what would be the manpower cost implications of this, as I've mentioned in the last couple of calls that there is always a shortage of high -quality talent, and therefore, we will see some amount of a war for talent in the clinical side at least. And we are prepared for that. We believe we have a good proposition.

We are also, as I mentioned earlier, actively hiring. So it is not that it's only Medanta doctors who are good. There are good doctors elsewhere in the market as well, and we are looking to actively augment and add in that capacity. So that's on the hiring part.

As far as the demand part goes, look, Gurgaon over the last 15 years plus that Medanta has been operating, have seen every kind of environment. We have seen environments where there was no hospital to where Medanta was just starting out. We've seen Medanta scale to over 1,500 beds. We have seen hospitals open, close, get acquired twice over. So for us, this doesn't sound like a very worrying issue as far as the demand and the success of Medanta or our ability to compete in this market. I think there is a demand. Obviously, that's why hospitals are being added. More and more patients will come to Gurgaon for care. And also, don't forget, as clinicians get added into hospitals, it becomes more attractive of a medical

destination for people from outside the region.

So if you have a group of 10 doctors working in 1 hospital versus a group of 500 doctors working in 10 hospitals, you will actually see a disproportionate growth in patient volumes because all doctors attract patients to the territory, and we see that as a good thing, by the way. So we are not overly worried either on the demand side or on our ability to attract or retain clinical talent.

Yogesh Kumar Gupta: And also, you see that the new beds then get added like when we opened Noida Hospital, we added new beds. We have not seen anybody saying that the demand has gone down in their hospital because of new hospital has come up. Similar story stands for when Max Dwarka opened up, right? They ramped up their hospital, but nobody said that we got hurt because of a new hospital has come up. So demand is enough. I think demand is enough and there is actually more demand for the quality beds.

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Tushar Manudhane: Understood. And sir, just lastly, if you could talk about the capex to be spent on in FY27 and maybe project-wise, if you could share?

Yogesh Kumar Gupta: Project-wise, we'll not be able to share with you right now. But the overall capex for the year will be somewhere around INR 800 crores to INR 900 crores.

Tushar Manudhane: And maybe for FY28 broad numbers, given that as highlighted in the presentation, like certain projects are where the drawings submitting or architectural drawings being submitted, which is why trying to just understand FY27 - FY28 how the capex spend will look like?

Yogesh Kumar Gupta: Capex spend for the year FY27 will be somewhere around INR 800 crores to INR 900 crores. Next year, it will be in the range of INR 600 crores to INR 700 crores.

Tushar Manudhane: Understood. And just lastly, if I may, on Ranchi, where we added beds in July 2025.

Pankaj Sahni: Sorry, Tushar, just one quick question. I know because in some of the earlier calls also, I think this point has come up. Keep in mind also that sometimes the capex is different from the cash flow or that the commitment of what has been announced in terms of the spend gets different and the cash flow gets different.

So you're aware that when we build out the hospitals, the greenfield hospitals, the cash flows are actually back-ended as opposed to being front-ended. So I don't know how you're looking at it. Some of it would be in CWIP and some of it will be in actual capitalized. So there will be differences in this and differences in the cash flow.

Tushar Manudhane: Yes. I mean, I'm not connecting this with the number of beds getting added, just purely understanding the cash outflow from the capex point of view, not connecting this. Just on Ranchi, if you could share like post-bed additions in July 2025, how the ramp-up of that unit has happened?

Yogesh Kumar Gupta: See, Tushar, we look at Ranchi as a single unit only. We don't look at them separately because it's run like a campus.

Tushar Manudhane: No, I meant to say with addition of beds and as an overall entire unit with addition of beds, how the business scale-up has happened in Ranchi?

Pankaj Sahni: So what's happening in Ranchi, Tushar, is that as we've added in this facility, we have also tried to scale up our various impanelments and various insurances and CGHS, NABH, etc. So this has taken a little bit of time to get up and running. We have now our CGHS impanelments. We'll get our railways. We have actually got our NABH also in the new unit. So some of this is an addition of that.

In addition, as I mentioned, I think when we went ahead with this addition of this hospital campus or part of the campus, we also intend to shut down some of the beds on the older hospital for renovation. So we have seen a reasonable amount of growth coming in this facility, approximately 15% to

16% growth we have already seen. And as this hospital gets up and

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running, we should see that scale up. I don't know if you're aware, but historically, we haven't seen a kind of 16% - 17% growth. Historically, it has been much lower.

Some of that was constrained by bed availability and the quality of the infrastructure given the old asset. So we do expect to see this growth hit upwards of 20% as this unit scales 20% to 30% like we've seen in some of the other units. But as Yogesh mentioned, it runs as one campus. So what actually we will do is some specialties will move to the newer site and some will remain on the older site. So we will also try to add in a few more specialties like oncology. So I think as this gets added, you will see this growth scale up.

Tushar Manudhane: Got it, sir. So just connecting this to the matured hospital category where the measures taken at Ranchi is sort of helping to realize the revenue growth. Similarly, with Indore 80 beds getting added with sort of the ready asset available, the scale up in that and subsequently, the improvement in further optimization of case mix, payer mix at Gurgaon, the matured hospital is also sort of set for a decent revenue growth along with improvement in the profitability. Is my understanding right?

Pankaj Sahni: I mean we believe so, yes. But obviously, given the scale of Gurgaon, Ranchi and Indore are very, very small as far as the overall scale of matured. So a 30% growth in Ranchi doesn't move the needle much when you look at it on an overall scale. So you're right, it will add to it, both revenue growth as well as profitability.

But I think also we should look at the continued focus on driving growth and efficiency in our Gurgaon unit is also something that we are taking very seriously, as I mentioned. So, I already told you that we will be adding clinicians into Gurgaon and we'll be adding operation theaters into Gurgaon and cath labs into Gurgaon. So if I look at the next year, I would say at least 5 to 10 procedural rooms will get added, and that doesn't even include some of the daycare areas. So we are investing quite a bit in putting in additional capabilities into our Gurgaon facility, even though you may not see it translate into bed numbers.

Moderator: Your next question comes from the line of Anshul Agrawal from Emkay.

Anshul Agrawal: Just one quick question on the Noida unit. Are there any insurance empanelment pending? Or are we done with all the empanelment in the Noida unit?

Pankaj Sahni: So all major empanelment are done or in the process of getting done within the next week or so. So by 31st of March, all of them were not done. But by today, almost all of them are done. I wouldn't say everything is done. I know that there are 2 or 3 which are pending. But I think most of the major ones are either done as we speak or in the process of getting done within a matter of a week or so.

Anshul Agrawal: Great. So occupancies as well as the revenue throughput of this unit should further accelerate in Q1 on the back of these empanelment, both scheme patients as well as obviously TPA empanelment. That would be a fair assumption to make?

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Pankaj Sahni: Yes. So scheme has literally come in, I think, 2 or 3 - 4 days ago. Other than that, of course, it's been all cash and TPA. So as we get various empanelment as this scales up, you will see growth in both of these areas. And then the various schemes, we will take a call as we get them onboard, CGHS, railways to do, not to do, when to do, how to do.

We'll have to taper that also with respect to the capacity availability and the clinician and infrastructure availability. But yes, we do believe that this will continue to grow, not only Q1, but as we move towards the full year, you will see

, I think, growth in all of these areas.

Anshul Agrawal: Great. Just one more follow-up question on the Noida unit. Could you help me with the ARPOB range of this facility currently?

Pankaj Sahni: I can. It is, I think, today actually performing almost in the INR 70,000 range or more, I think, maybe closer to INR 80,000. However, I would extremely caution that you cannot take this as a baseline because obviously, this is a 100% cash kind of tariff till now on a very small base. So please don't project out that this is a standard.

I think fair to say that this will probably be more or less same as Gurgaon, maybe a little bit lower than some of the higher-priced facilities in Delhi area. But I would say, in our thinking, we think that ARPOB ranges of Noida and Gurgaon are more or less same.

Moderator: The next question comes from the line of Sanidhya Agarwal with Unicorn Assets.

Sanidhya Agarwal: Hi, Pankaj. Hi, team. Good set, I think after 7 - 8 quarters now the company is back to 25% growth levels, which is excellent. And there is no reason to not believe that next 5 - 6 quarters will continue to be ramping up at this pace.

My question is more from the perspective of next year, maybe you can say 2028 calendar year. So while we see ramp-up continuing and new additions in this year with our existing facilities and some expansions in Noida as well. How do you see beyond this? Because what we also have seen in the past is once we ramp up the new facilities, or there comes a phase of now where we are somewhere in 15%, 12%, 14% range because we are trying to get the best out of the mix that we operate in, adding super specialties, getting the right ARPOBs, driving occupancy optimizations. And of course, no problem in that. But on the other side, why don't we capture the larger share, whether it comes through brownfield or some smaller facilities that we can expand further or something like a sister market to NCR, say, something like Jaipur or Chandigarh or something like that. And honestly, quite phenomenal transformation in Noida unit. So that would be the first question?

Pankaj Sahni: Okay. So a lot of elements of strategy that you have or theoretical strategy that you have laid out there. Let me first give you a couple of data points. See, our Lucknow facility, which is classified as developing is 6.5 years old. Our Patna facility is 4.5 years old. In the larger context of most hospitals, they would have supposedly as per your theory plateaued by now. They are giving growth of 30% more or less year-on-year for the last couple of years on a volume basis as well as on a revenue basis.

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So I don't believe that Medanta has followed what you articulate as a normal approach. I don't know if it is being done in other hospitals. But our hospitals even after year 4 - 5 are showing reasonably phenomenal growth. And keep in mind that these are not 200 - 300-bed hospitals, these are 600 and 1000 -bed hospitals. So on a very, very high base, we are seeing a very phenomenal amount of growth even after the so-called early years. So we don't believe that Medanta is just following any kind of other approach that we may be looking at. And we've seen a very, very strong performance across all of these units, and we continue to see that. In addition to that, as I mentioned on the call, we have a fairly significant bed addition coming in all these units, including Noida. So almost 200-plus beds will be coming in this year with almost negligible capex. So we do continue to believe that there will be a very robust growth in the developing portfolio. At some point in time, I guess, Lucknow will technically get classified as a developed hospital after it crosses whatever reporting threshold we have and we may change how we report out that information. But forgetting about the reporting, if you

you just look at the performance of Lucknow, still a lot of juice left in that asset, right, as well as in the other assets. Now as far as will we expand to Jaipur or will we look at other opportunities? I mean, of course, we are looking at new strategies all the time. We've talked about these as and when it is appropriate. And we are constantly evaluating brownfield, greenfield, bolt-on strategies, in some of the earlier calls, I talked about hub-and-spoke strategies in various cities. You see that Varanasi is an outcome of what we had talked about a couple of years ago after Lucknow stabilized and we said that we have 1,000 beds in Lucknow. We will look at other cities in Eastern UP. And Varanasi is a reflection of the execution of those stated goals. So of course, you will continue to see this. And at the appropriate time, we will share what we can with the community to see that this is how we are moving forward. But all things that you described are very much part of our business.

Sanidhya Agarwal: Great. That answers. Just last one, if you could highlight what would be the ARPOB for the developing or particularly the Patna and the Lucknow unit. Like in the developing, there was some decline in growth last year given the payer mix and since we have not taken any hikes in terms of prices, should we expect something in near-term?

Pankaj Sahni: So as mentioned in the investor presentation, the ARPOB for the developing cluster is about INR 56,500 for this year, which is an increase of about 4% over last year. And we do believe that as the complexity scales up, you will continue to see ARPOB growth. I also mentioned in response to one of the earlier questions that we do believe that there's some ALOS opportunity improvements, especially in Patna. So you may see some ARPOB benefits because of that because of the way in which ARPOB is calculated.

So all of this, by the way, has nothing to do with the tariff increases that we may take in Lucknow and Patna. And at the appropriate time, we will take the tariff increases, and we will hopefully see some benefits of realizations for that. But you may have followed the commentary earlier, we are reasonably conservative in tariff increases just because we can take tariff increases doesn't mean we do. But we'll take them as per what the market conditions and what we feel is appropriate for our business.

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Moderator: Ladies and gentlemen, this was the last question for today. I now hand the conference over to the management for closing comments.

Pankaj Sahni: Thank you, everyone, for your questions and for joining us today. We are very happy and confident with the results for FY26 and the growth that we have coming forward and very excited for what healthcare opportunities India has to offer us as we move forward.

With this, I would like to end the call. If you have any questions or if there's anything that has remained unanswered, please feel free to reach out to our Investor Relations team. Thank you once again, and see you all soon.

Moderator: Thank you. On behalf of ICICI Securities, that concludes this conference. Thank you, everyone, for joining us, and you may now disconnect your lines.

Notes:

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